



Thank you very much for purchasing this California-specific Home Health Manual. The manual is divided into two binders:

Operational Policies and Procedures
and
Human Resource Administration

We know you will find it helpful as you prepare for ACHC accreditation, Medicare certification and continue to improve your operational practices in the state of California.

For home health organizations that are departments of a facility or a component of an integrated health care system (such as a hospital-based home health program), please be aware that a number of operational functions and/or processes need to be integrated in an organization-wide fashion under ACHC standards (e.g., emergency management and patient safety program). Policies in this Manual may need modification to reflect this.

Please Note: These manuals have been designed to meet ACHC and Medicare Conditions of Participation and California Title 22 Standards. Your organization must review ALL policies and customize any that do not meet your specific requirements.

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Microsoft® Office 2016 or higher (including Office 365)
Microsoft® Word 2016 or higher
2 GB RAM or higher
Windows 10 or higher operating system (recommended, but not required)
Currently supported Intel or AMD processor

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NOTICE:

We recommend that providers go to www.cms.gov and download the most recent Federal/Medicare Conditions of Participation

We recommend you print and insert these Conditions of Participation in the Attachment section at the end of this manual.

POLICY NUMBERING AND REFERENCES

Policies have been numbered to identify the manual name, section, policy number and page.

Example:
2–043.1

- ☐ The 2 represents Section Two
- ☐ The 043 represents policy 43 within the section
- ☐ .1 represents page 1 of that policy

LEGEND OF EVIDENCE FOR ACHC ACCREDITATION	
D	Documents
I	Interview
O	Observation
S	Survey
P	Policy

Policies in each manual have been “crosswalked” to the corresponding Accreditation Commission for Health Care (ACHC) standards and Medicare Conditions of Participation. The crosswalk also contains a legend identifying the evidence required by ACHC.

**CROSSWALK OF POLICIES AND PROCEDURES
WITH ACHC STANDARDS AND
MEDICARE CONDITIONS OF PARTICIPATION**

SECTION ONE

Organization and Administration

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
1. Mission Statement				
2. Governing Body	P, D, I	74717 74721 74742 SB 188	HH1-1A HH1-1B HH1-2A HH1-2A.03 HH1-5A.01	484.105(a) 484.105(h) 484.65(e)
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4. Referral Disclosure and Care Decisions	P		HH1-4A.01	
5. Public Disclosure Statement	P, D, I	74665	HH1-1A HH1-1B HH1-4A.01 HH2-1A.01	484.100(a)
6. Administrative Qualifications and Responsibilities	D, I	74718	HH1-5A HH1-5A.01	484.105(b)
7. Appointment of Executive Director/Administrator	D, I	74717	HH1-5A HH1-5A.01	484.105(b)
8. Designation of Individual in Absence of Executive Director/Administrator		74718		484.105(b)
9. Home Health Administrator	P, D, O	74613 74703 74718	HH1-5A HH1-6B	484.105(b)
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15. Use of Organizational Chart	P, D, I	74661	HH1-6A HH1-6B	484.105
16. Uniform Quality of Care	D, I		HH1-1A.01	
17. Experimental Research and Investigational Studies	P, D, I		HH1-1A.01 HH7-10A.01	
18. Organizational Planning	D, I	74667	HH1-5A	484.100(a)
19. Contingency Planning				
20. Contingency Plan If Organization Closes				484.110(c)
21. Branch Control	P, D, I	74609 74669 74675	HH1-1A HH1-6C HH1-12A.01	484.100(b) 484.105(d)
22. Contracted Service Providers	D, I	74719	HH1-10A	484.105(e)
23. Home Health Contracted Services	D	74652 74709 74719	HH1-10A	484.105(e)
24. Written Agreements for Contracted Services	D, I	74652 74719 74721	HH1-10A	484.105(e) 484.80(h)
25. Scope of Services	P, D, I	74693 74695 HSC 1727.1	HH2-1A.01 HH2-12A.01 HH5-11A HH5-11F HH5-2C.01 HH5-2C.02	484.75 484.105 484.105(f)
26. Listing of Services Provided	D, I	74693 HSC 1727.1	HH1-7A HH5-11A	484.75 484.105 484.105(f) 484.105(i)(2)
27. Scope of the Behavioral Health Program	D		HH5-2C.02	484.105
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SECTION Two

Program/Service Operations

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3. Admission Criteria and Process	P, D	74695 74697 74743	HH2-1A.01 HH2-2B HH5-2B HH5-2C HH5-13A.01 HH5-14B.01	484.60 484.55 484.105(i)(1)
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15. Minimum Necessary Uses of PHI	P, I, O	74719 74731 74743	HH2-2A HH2-5A	
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21. Patient Requests for Privacy Restrictions	P, I, O	74719 74731 74743	HH2-2A HH2-5A	
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29. Sanctions for Privacy and Security Violations	P	74731 74743 74749	HH2-5A	
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4. Rehabilitation Care Planning	P, D, I	74697 74711	HH5-2A.01 HH5-2C.01	484.105(g)
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35. Medication Labeling	P		HH5-2F.01 HH5-2F.02	
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46. Discharge Summary	P, D	74735	HH5-6A	484.50(d) 484.60(d) 484.110(a)
47. Contents of Clinical Record	P, D	74735 HSC 1727.5(b)	HH5-1A.01	484.75(b)(6) 484.110 484.110(a)
48. Clinical Record Review	D, I	74742	HH6-4A.07 HH6-7A.01	
49. Patient Education Process	P, D, I		HH5-12A.01	484.75(b)(4) 484.75(b)(5) 484.102
50. Safe/Effective Use of Medications	D, I		HH5-2F	
51. Drug – Food Interactions	D, I	74697	HH5-2F	
52. Pain Management Education	P, D, I		HH5-12A.01	
53. Rehabilitation Techniques	P, D, I		HH5-12A.01	
54. Appropriate Use of Restraints and Supplies	P, D, I		HH5-12A.01	
55. Safe/Effective Use of Equipment and Supplies	P, D, I		HH5-12A.01 HH7-9A.01	
56. Storage, Handling, and Access to Supplies and Gases	P, D, I		HH5-12A.01	
57. Identification, Handling, and Disposal of Hazardous Waste	P, D, I		HH5-12A.01	
58. Infection Control Precautions	P, D, I	74721 74727 74742	HH7-1A HH5-12A.01	484.70
59. Natural Disasters/Emergencies	P, D, I	74721	HH5-12A.01 HH7-3B	484.102
60. Basic Home Safety	P, D, I	74747	HH5-2C.01 HH5-12A.01 HH7-9A.01	
61. Patient Education Related to Discharge Planning	P, D, I	74713 74719	HH5-12A.01	
62. Educational Resources	D, I	74711 74713 74715 74718	HH5-12A.01	484.75(b)(4) 484.75(b)(5)
63. Community Resources	D, I	74713	HH5-12A.01	
64. Record Retention	P, D	74731 74735 74723	HH3-3A.01 HH4-1A.01 HH4-1A.02 HH5-1B	484.110(c) 484.102
65. Assembly of Clinical Records	P, D		HH5-1A.01	484.110(a)
66. Intake Process	P, D, O		HH5-13A.01	

Organization's Name

SECTION FOUR

Provision of Care and Record Management

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
67. Care/Service Coordination	D, I	74703 74719		484.60(d) 484.75
68. Abbreviations and Symbols				
69. Missed Visits				

SECTION FIVE

Quality Outcomes/Performance Improvement

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
1. Improving Organizational Performance	P, D, I	74703 74705 74707 74718 74721 74742	HH6-1A HH6-1B.01 HH6-1C HH6-1D.01 HH6-3A.01 HH6-4A.01 HH6-5A	484.65 484.70(b) 484.75(b)(8)
2. Patient Focused Performance Improvement		74742		484.65
3. Measuring Performance of the Environmental Safety Program	D	74742	HH6-4A.02	484.65
4. Annual Organization Evaluation	P, D, I	74742	HH6-4A.04 HH6-4A.05	484.65 484.105(i)
5. Incident Reporting	P, D, I	HSC 1736.4	HH2-4A HH6-6A HH7-7A.01	484.65
6. Serious Adverse Events	P, D, I		HH6-6A	484.65
7. Root Cause Analysis/Action Plan	D, I		HH6-6A	484.65
8. Aggregation of Data/Information	I		HH6-4A HH7-9A.01	484.65
9. External Databases	I		HH6-3A.01	
10. Patient and Family/Caregiver Experience of Care Survey	I	74742	HH6-4A.05	484.65

SECTION SIX

Risk Management: Infection and Safety Control

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
1. Management of Exposures in Personnel	P, D, I, O, S	74721 74723 74742	HH1-1A.01 HH7-1A HH7-7A.01	484.70 1910.134 1910.1030
2. Record Keeping	P, D, I, S	74721 74723 74725	HH1-1A.01 HH7-1A	484.70 1904.1(a) 1904.4 1904.5 1904.6 1904.7 1904.8 1904.9 1904.10 1904.11 1904.29 1904.32 1904.33 1904.35 1904.36 1904.39 1910.1030
3. Occupational Exposure Information and Training	P, D	74721	HH1-1A.01 HH7-1A	484.70 1910.1030
4. Standard Precautions	P, D, O	74721	HH1-1A.01 HH7-1A HH7-9A.01	484.70 1910.1030
5. Personal Protective Equipment	P, D, O	74693 74671 74721	HH1-1A.01 HH7-1A	484.70 USP 800.7 1910.133 1910.1030
6. Hand Hygiene	P, D, O	74721	HH1-1A.01 HH7-1A HH7-9A.01	484.70
7. Clean vs. Aseptic Technique	P, D, O		HH7-1A	
8. Infection Control/Expanded Precautions	P, D, I, O		HH7-1A	484.70
9. Contaminated Materials Disposition	P, D, I, O		HH7-1A	
10. Contaminated Waste Disposal	P, D, I, O		HH1-1A.01 HH7-1A HH7-2A.01 HH7-2B.01	
11. Hazardous Waste Handling	P, D, I		HH1-1A.01 HH7-1A HH7-2A.01 HH7-2B.01 HH7-6A.01	USP 800.11.4

SECTION SIX

Risk Management: Infection and Safety Control

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
12. Accidental Exposure to Blood	P, D, I, O		HH7-1A HH7-2A.01 HH7-2B.01 HH7-6A.01	
13. Bag Technique	P, D, I, O		HH7-1A HH7-2A.01 HH7-2B.01	
14. Evaluating and Maintaining Records of Infections Among Patients	P, D		HH7-1A HH7-1D.01 HH7-2A.01 HH7-2B.01	484.70
15. Evaluating and Maintaining Records of Infections Among Personnel	P, D		HH7-1A HH7-1D HH7-2A.01 HH7-2B.01	484.70
16. Reporting of Communicable Diseases	P, D	74725	HH1-1A.01 HH7-1A HH7-2A.01 HH7-2B.01	484.70
17. Communication of Hazards to Personnel	P, D, O		HH1-1A.01 HH7-1A HH7-2A.01 HH7-2B.01 HH7-6A.01 HH7-6B.01	484.70 USP 800.8
18. Environmental Safety Program	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-6A.01 HH7-6B.01 HH7-7A.01 HH7-9A.01	USP 800.5 USP 800.9 USP 800.13
19. Environmental Safety—Office	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-6A.01	USP 800.5 USP 800.9 USP 800.13
20. Fire Safety—Office	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-5A.01	
21. Utilities Management—Office	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-5A.01	
22. Equipment Management—Office	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-5A.01	
23. Environmental Safety—Patient	P, I, O		HH5-2C.01 HH7-2A.01 HH7-2B.01 HH7-3B HH7-5A.01	

Organization's Name

SECTION SIX

Risk Management: Infection and Safety Control

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
24. Fire Safety—Patient	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-3B HH7-5A.01	
25. Utilities Management—Patient	P, D, I, O		HH1-1A.01 HH7-1A HH7-1D	
26. Equipment Management—Patient	P, D, I, O	74693	HH1-1A.01 HH7-1A HH4-2C.01 HH7-7A.01	
27. Infection Control Plan	P, D, I	74742	HH1-1A.01 HH4-2C.01 HH4-2D.01 HH7-1A	484.70 1910.1030
28. Tuberculosis Exposure Control Plan	P, D, I, O	74723	HH7-2A.01 HH7-2B.01 HH7-9A.01	484.70
29. Bloodborne Pathogens and Hepatitis B Exposure Control Plan	P, D, I, S	74742	HH7-2A.01 HH7-2B.01	484.70 1910.1030
30. Safe and Appropriate Use of Medical Equipment and Supplies	P, D, I, O	74693	HH7-2A.01 HH7-2B.01 HH7-9A.01	
31. Storage of Medications and Nutritional Therapies	P, D, O		HH1-1A.01 HH7-2A.01 HH7-2B.01 HH7-7A.01 HH7-9A.01	
32. Medical Equipment Malfunction	P, D, O		HH7-2A.01 HH7-2B.01 HH7-5A.01	
33. Safe Medical Device Act	P, D		HH7-2A.01 HH7-2B.01 HH7-5A.01	
34. Organization Personnel Safety—Personal Safety	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-7A.01	
35. Organization Personnel Safety—Unsafe Home Visits	P, D, I, O		HH7-2A.01 HH7-2B.01 HH7-3A HH7-3B HH7-3C HH7-3D HH7-3E	
36. Vehicle Accident Reporting	D, I		HH7-2A.01 HH7-2B.01 HH7-7A.01	

Organization's Name

SECTION SIX

Risk Management: Infection and Safety Control

POLICY/PROCEDURE	EVIDENCE	CALIFORNIA TITLE 22 STANDARD	ACHC STANDARD	MEDICARE COP
37. Emergency Management Plan	P, D, I	74721	HH7-2A.01 HH7-2B.01 HH7-3A HH7-3B HH7-3C HH7-3D HH7-3E	484.102
38. Waived Testing	P, D		HH1-11A HH4-6C.01 HH7-8A.01	484.100(c)
39. Home Glucose Monitoring	P, D, O		HH1-11A HH4-6C.01 HH7-8A.01	484.100(c)

ATTACHMENTS

Attachment I:

[ACHC Crosswalk](#)

Attachment II:

[Glossary of Terms](#)

SECTION ONE

Organization and Administration

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SECTION ONE

Organization and Administration

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1-024.A

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1-025

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1-025.A

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1-026

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1-027

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1-028

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1-028.A

Addendum: Home Health Change of Care Notice (HHCCN)

1-028.B

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1-028.C

Addendum: FFS Expedited Review Detailed Notice

1-028.D

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1-028.E

Program Planning

1-029

Marketing Plan

1-030

PURPOSE

To define the organization's mission.

POLICY

The organization's programs and services will reflect its written mission as stated in this policy. Published statements will be available to personnel, patients, and the general public. These statements will be reviewed and revised as needed and approved by the Governing Body at least every 36 months. Organization's Name senior management personnel will continuously monitor employee performance to ensure it supports the organization's mission.

Philosophy

Organization's Name is driven by the philosophy of commitment to our patients, leadership and excellence as defined below. We recognize the unique physical, emotional and spiritual needs of each person. We strive to extend the highest level of courtesy and service to patients, families/caregivers, visitors and each other.

We deliver state-of-the-art home care services with identified centers of excellence. We engage in a wide range of continuing education, clinical education, and other programs for professionals and the public.

We strive to create an environment of teamwork and participation, where, through continuous performance improvement, people pursue excellence and take pride in their work, the organization and their personal development. We believe that the quality of our human resources—organization personnel, physicians, and volunteers—is the key to our continued success. We provide physicians an environment that fosters high quality diagnosis and treatment. We maintain financial viability through a cost-effective operation to meet the organization's long-term commitment to the community.

Mission

Organization's Name is guided by a tradition of personal, clinical and technological excellence. We believe that serving patients in the community setting is the most important activity of the organization.

We are dedicated to providing the highest quality of patient care with compassion and respect for each person.

PURPOSE

To outline the roles and responsibilities of the Governing Body.

POLICY

The Governing Body will assume full legal authority, responsibility, decision-making and accountability for the overall management and operations, the provision of services, fiscal operations, human resource management, review of the organization's budget and operational plans, and its quality assessment and performance improvement (QAPI) program. The Governing Body of Organization's Name will serve as the governing authority for the home care program, which will function according to Organization's Name's bylaws.

PROCEDURE

1. Governing Body members will be selected based upon their industry experience, expertise, and professional relationships. How these selection factors support and strengthen the mission of the organization will be determining factors for placement as a Governing Body member.
2. The Governing Body will establish operational and human resources policies that are consistent with the organization's mission and approve new or revised policies as needed.
3. The Governing Body will appoint a Governing Body committee to work with Senior Leadership and the Performance Improvement Committee to review and develop policies and procedures consistent with the organization's mission; annually review and revise policies and procedures; prepare an annual evaluation of Organization's Name in relation to its mission; and assist in identifying goals and measuring accomplishments of the organization's operations.
4. The Governing Body will establish an appropriate forum to address ethical issues. (See "[Ethical Issues](#)" Policy No. 2-038.)
5. The Governing Body will routinely review all fiscal affairs and hold senior management accountable for the financial solvency and adequacy of the financial resources of Organization's Name.
6. The Governing Body will approve the organization's budget and capital expenditures plan, as applicable.
7. The Governing Body will assist with development and approve strategic, marketing, operational and human resources plans.

8. The Governing Body will ensure that the agency does not refuse service to or employment to or in any way discriminate against any person based on sex, race, hairstyle, color, religion, ancestry, national origin, sexual orientation, disability, medical condition, marital status, or registered domestic partner status.
9. The Governing Body will appoint a qualified Executive Director/Administrator and establish procedures of systematic communication between the two. Performance of the Executive Director/Administrator will be monitored annually through a procedure established by the Governing Body. The process and documentation will reflect that:
 - A. Relevant findings of performance improvement activities are consistently provided to the Governing Body to allow for oversight of the quality assessment and performance improvement (QAPI) program. The governing body will ensure the following:
 1. That an ongoing program for quality improvement and patient safety is defined, implemented, and maintained;
 2. That the organizational-wide quality assessment and performance improvement efforts address priorities for improved quality of care and patient safety, and that all improvement actions are evaluated for effectiveness;
 3. That clear expectations for patient safety are established, implemented, and maintained; and
 4. That any findings of fraud or waste are appropriately addressed
 - B. Other information relevant to the quality of patient care (i.e., unusual occurrences in care delivered) is also consistently provided through a defined process
10. The Governing Body will authorize adequate resources and support to establish and maintain an organization-wide performance improvement program. Information related to the performance improvement program will be provided to the Governing Body.
11. The Governing Body will appoint a Corporate Compliance Officer who is responsible for the day-to-day oversight of the Corporate Compliance Program and coordination of all compliance initiatives. The Corporate Compliance Officer will report to the CEO/Administrator and the Governing Body.
12. The Governing Body will appoint a Corporate Compliance Committee (The Committee) who will be charged with the support and implementation of the Corporate Compliance Program in conjunction with the Corporate Compliance Officer. The Committee will consist of members from the Governing Body and senior-level managers. The Committee members will be trained and well-informed of the policies and elements of the Compliance Program. The Committee will meet at least quarterly.
13. The Governing Body will implement a written conflict of interest policy that includes guidelines for the annual written disclosure of any existing or potential conflict of interest. Disclosure statements will be retained by the organization. (See "[Conflict of Interest](#)" Policy No. 1-003.)

- A. In the event that a situation exists where a member of the Governing Body could use confidential or privileged organizational information for personal gain, he/she is obligated to report that potential to the Governing Body. The Governing Body will then render a decision of the member's eligibility to vote on any particular issue.
 - B. Disclosure of a potential conflict and the Governing Body's decision regarding the conflict will be noted in the minutes by the Secretary.
14. The Governing Body will evaluate the organization's performance on a regular basis.
15. The Governing Body will review legal and business documents including articles of incorporation, bylaws, and legal agreements at least every 36 months.
16. The Governing Body will share the organization's mission with the community while monitoring the organization's efforts to identify and address the community's needs and concerns.
17. All new Governing Body members will participate in and evaluate an orientation program that includes:
- A. Review of their responsibilities and accountabilities in improving organizational performance
 - B. Review of their responsibilities and accountabilities as defined by the organization bylaws/articles of incorporation
 - C. Review of their responsibilities and accountability as related to the corporate compliance plan
 - D. Review of the organizational structure, the values, mission and goals; overview of program services, philosophy of care; grievance policy and procedure for employees and patients and confidentiality
18. Arrange for outside, professional advice, as indicated.
19. Documentation of each member's orientation will be maintained by the organization. See ["Governing Body Orientation Checklist"](#) Addendum 1-002.B.
20. The Governing Body will comply with the organization's bylaws in relation to:
- A. Meeting the notice requirement for scheduled and special meeting
 - B. Frequency of meetings
 - C. Appointment of officers and terms of office
 - D. Attendance requirements at meetings and quorum determination
 - E. Executive sessions

- F. Committee structure and function
 - G. Conflict of interest disclosure requirement
 - H. Overall responsibilities
21. All actions taken by the Governing Body will be documented in meeting minutes, which will be retained by the organization for a minimum of five (5) years. Minutes will minimally include attendance records, summaries of agenda items and action taken, and motions documented as stated and acted upon. Minutes will be distributed per organization policy.

See "[Governing Body Members](#)" Addendum 1-002.A for a list of names and addresses of the Governing Body.

ADDENDUM 1-002.A

GOVERNING BODY MEMBERS

GOVERNING BODY MEMBERS

MEMBERS	PROFESSIONAL AFFILIATION	TERM EXPIRATION DATE
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
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Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	
Name: Telephone: Address:	Occupation: Title: Employer:	

ADDENDUM 1-002.B

GOVERNING BODY ORIENTATION CHECKLIST

Governing Body Orientation Checklist

Name

:

Mentor

Name:

Date

Completed: _____

Provide General Information	
History of the organization	
Organization/Governing Board (GB)/Staff Structure	
Copy of by-laws and constitution	
Vision and mission of the organization	
Strategic priorities, goals and objectives	
Summary of programs and services	
Grievance procedure for employees and patients	
Confidentiality (HIPAA) policies and procedures	
Roles and Responsibilities	
Reviewed GB member's role	
a. Corporate compliance activities	
b. Performance improvement activities (QAPI)	
c. Legal duties and obligations/Laws governing organization	
Reviewed Executive's/Officer's role	
Reviewed staff roles	
Reviewed committees and their roles	
Discussed expectations of new GB members	
Policies and Procedures	
Provided GB policies and procedures	
Discussed GB policies and procedures	
Discussed GB role in organization's policies and procedures review	
Financial Management	
Discussed budget process	
Provided current year's budget	
Provided a copy of the most recent audited financial statements	
Provided a copy of the most recent annual report	
Other Information	
Provided copies of minutes from previous GB meetings	
Provided marketing materials	
Provided copy of the most recent agency annual evaluation	
Gave tour of the offices	
GB member signed the required paperwork (i.e., Conflict of interest statement, Confidentiality agreement, etc.)	
Provided copy of the GB Manual	

PURPOSE

To ensure that all organization personnel, as well as members of the Governing Body and advisory boards, adhere to organizational guidelines for professional conduct.

POLICY

The affairs of the organization by all personnel (including Governing Body, and outside entities providing services to the organization and patients) will be conducted in accordance with the highest standards of integrity. There can be no deviation from complete honesty in business transactions. Use of organization funds or internal business information for improper purposes and dishonest practices is absolutely forbidden.

GUIDELINES

1. The organization will ensure that:
 - A. Ethical patient care issues will be referred to the ethics committee.
 - B. All marketing literature will be reviewed with the marketing/planning director or risk management personnel to ensure that services promoted are provided.
 - C. Billing practices will be monitored for accuracy to ensure that only services provided are billed.
 - D. The Governing Body and senior management personnel will be accountable to the conflict of interest policy, related to disclosure of any benefit.
 - E. If a patient requires transfer to another organization, he/she will be informed of any financial benefit to the organization, and provided the option to choose a provider of their choice.
2. Members of the Governing Body and executive personnel will:
 - A. Act in the course of their duties solely in the best interests of the organization without consideration to the interests of any other organization with which they are associated, and to refrain from taking part in any transaction where such person(s) do not believe in good faith that they can act with undivided loyalty to the organization.
 - B. Disclose any material, financial or other beneficial interest to any entity engaged in the delivery of goods or services to the organization or its members.

- C. Annually disclose any transactions with the organization that would result in any benefit to themselves, their immediate families, or any entity in which they hold a significant financial ownership or other interests, and refrain from participation in any action on such matters except upon approval of the Governing Body after full and frank disclosure.
 - D. Refrain from utilizing any inside information as to the business activities of the organization for the benefit of themselves, their immediate families or any entity with which they may be associated.
- 3. During the orientation process, all personnel will be required to sign an employment agreement that includes statements related to:
 - A. Confidentiality
 - B. Noncompetition
 - C. Return of records, papers, and equipment
 - D. Conflict of interest
 - 4. All personnel will agree to devote his/her best efforts to the company and not directly or indirectly be engaged in or connected with any other commercial pursuits whatsoever without written authorization of the organization.
 - 5. In the event that a situation arises whereby organization personnel could use confidential or privileged organization information for personal gain, they are obligated to report that potential to the Executive Director/Administrator.
 - 6. Disclosure of a potential conflict and the Executive Director's/Administrator's decision regarding the actions will be noted in a log file kept by the Executive Director/Administrator.
 - 7. No full-time salaried organization staff member will engage in private practice of a service similar to that provided by the organization within the geographic area serviced by the organization, without the written permission of the Executive Director/Administrator. Persons violating this policy will be subject to probation or termination.
 - 8. In the event that a situation arises whereby a member of the Governing Body or senior management personnel could use confidential or privileged organization information for personal gain, he/she is obligated to report in writing, that potential to the Governing Body.
 - 9. The Governing Body will render a decision of that member's eligibility to be part of voting, if applicable.
 - 10. Disclosure of a potential conflict and the Governing Body's decision regarding voting will be noted in the minutes of the meeting.

Attestation Statement: I have read the Conflict of Interest policy set forth above and agree to comply fully with its terms and conditions at all times during my service as an employee, contractor (outside entity), or Governing Body member. If at any time following the submission of this form, I become aware of any actual or potential conflicts of interests, or if the information provided below becomes inaccurate or incomplete, I will promptly notify the Executive Director/Administrator in writing.

Disclosure of Actual or Potential Conflicts of Interest:

Printed

Name:

Signature:

Date:

ADDENDUM 1-003.A

CONFLICT OF INTEREST STATEMENT

(Insert Organization Statement)

REFERRAL DISCLOSURE AND CARE DECISIONS**Policy No. 1-004.1****PURPOSE**

To ensure that all patients are informed about the relationship between the use of services and financial incentives between the organization and other service providers. To ensure that the integrity of clinical decision-making is not compromised by financial incentives offered to leaders, managers, clinical personnel, or physicians.

POLICY

When a patient is referred to another service organization, the patient will be informed of any financial benefit to Organization's Name. To promote efficient quality patient care, clinical care decisions will be based on identified patient health care needs.

[Cross-reference "[Intake Process](#)" Policy No. 4-066, "[Admission Criteria and Process](#)" Policy No. 2-003, "[Transfer/Referral Criteria and Process](#)" Policy No. 4-043, "[Initial and Comprehensive Assessment](#)" Policy No. 4-018, "[Ongoing Assessments](#)" Policy No. 4-019, "[Physician Participation in Plan of Care](#)" Policy No. 4-002, and "[Verification of Physician Orders](#)" Policy No. 4-003]

PROCEDURE

1. The Program Director will be responsible to inform the patient or family/caregiver of any affiliation or financial incentives between Organization's Name and other service providers.
2. The patient may choose referral of services to other organizations.
3. All referrals will be documented and include name, date, time, and reason for referral.
4. The referrals will be monitored, reviewed, and reported each month by the Program Director. Any areas of concern identified, will be reviewed by the Program Director and Executive Director/Administrator as part of the organization's performance improvement process.
5. All clinical decisions will be based on identified patient health care needs. Decisions will not be based on organizational compensation or financial risk shared with leaders, managers, clinical personnel, or physicians. All personnel are educated and understand this.
6. The organization will accept only those patients whose needs can be met by the services it provides and who meet admission criteria.
7. Initial and ongoing patient assessment data will identify patient health care needs.

8. In compliance with standard medical practice, all services will be delivered under physician's (or allowed practitioner's) orders and in compliance with state law and ethical policies.
9. Any areas of concern identified will be reviewed by the Program Director and Administrator as part of the organization's performance improvement process.
10. Information regarding financial incentives to leaders, managers, clinical personnel, or physicians will be available upon written request.

PUBLIC DISCLOSURE STATEMENT**Policy No. 1-005.1****PURPOSE**

To ensure compliance with federally mandated disclosure regulations.

POLICY

Organization's Name will comply with the reporting requirements of 42 CFR Part 420, subpart C (Program Integrity) of the Medicare Conditions of Participation as they apply to the organization.

PROCEDURE

1. Organization's Name will annually complete a written public disclosure statement including:
 - A. Names and addresses of individuals, corporations, or subcontractors having a combined direct or indirect ownership or 5% or more in the organization
 - B. Names and addresses of those persons directly related (spouse, sibling, parent, child) to individuals named in "A"
 - C. Names and addresses of individuals in "A" or "B" with an ownership or controlling interest in a Medicare or Medicaid facility
 - D. The name and address of each person who is an officer, a director, an agent, or a managing employee of the home health agency
 - E. The name and address of the person, corporation, association, or other company that is responsible for the management of the home health agency, and the name and address of the chief executive officer and the chairman of the board of directors of the corporation, association or other company responsible for the management of the home health agency.
 - F. If any persons described in (A, D, or E) has served as or currently serves as an administrator, general partner, trustee or trust applicant, sole proprietor or any applicant or licensee who is a sole proprietorship, executor, or corporate officer or director of , or has held a beneficial ownership interest of 5 percent or more in any other home health agency, health facility, clinic, hospice, Pediatric Day Health and Respite Care Facility, Adult Day Health Care Center, or any facility licensed by the Department of Social Services, the applicant shall disclose the relationship to the Department, including the name and current or last address of the facility and the date such relationship commenced and, if applicable, the date it was terminated.
 - G. Description of any criminal offense conviction involving titles V (Maternal and Child), XVIII (Medicare), XIX (Medicaid), or XX (Social Services) brought against any persons listed in "A", "B", or "C"

- H. Names and addresses of any individual currently employed in a managerial, accounting, auditing, or similar capacity who were employed by the organization's fiscal intermediary within the previous 12 months
- I. Changes in the Administrator, Program Director, or Medical Director during the previous 12 months
- J. The dates of any of the following:
 - 1. Actual or anticipated change in ownership or control in the previous or next 12 months (within thirty (30) days of change)
 - 2. Anticipated bankruptcy filings
 - 3. Operational changes by a management company
 - 4. Leasing agreements by another organization
 - 5. Address changes for the parent, or branches
- 2. The annual disclosure statement will be signed by the Executive Director/Administrator of Organization's Name.
- 3. Organization's Name will inform ACHC and other state/federal regulatory agencies, as appropriate, of negative outcomes from review/audits. The following outcomes must be reported to ACHC, but are not limited to:
 - A. License suspension(s)
 - B. Conditions/probation restrictions to license(s)
 - C. Non-compliance with Medical CoPs identified during survey by another regulatory body
 - D. Civil penalties of \$10,000 or more
 - E. Third Party Payer Recovery of insurance payments of \$10,000 or more
- 4. The organization will furnish updated information to federal/state regulatory agencies and ACHC at intervals between recertification or re-enrollment, within 30 days of a written request.
- 5. Additional information available for public review include:
 - A. Mission statement
 - B. Licensure and accreditation status, as applicable

ADMINISTRATIVE QUALIFICATIONS AND RESPONSIBILITIES**Policy No. 1-006.1****PURPOSE**

To outline the authority and responsibility for overall administration and management of Organization's Name.

POLICY

Qualifications for administrative positions will be clearly defined in Organization's Name position descriptions and will meet the needs of the organization in terms of position responsibility, scope of services, and complexity of the organization. Administrative and management personnel will have a combination of education and experience necessary to be successful in their assigned role.

Authority and responsibility for administrative positions will include supervision of contracted services and branches of the parent organization.

PROCEDURE

1. Organization's Name will maintain administrative and supervisory responsibility for all care provided through contracted individuals or vendors. This responsibility will be clearly stated in all relevant written agreements and contracts.
2. Organization's Name will obtain documented verification of all administrative personnel's qualifications including:
 - A. Resumes
 - B. Two (2) references reflecting performance and experiential qualification
3. Organization's Name position descriptions will clearly define specific responsibilities and include at least:
 - A. Organization and direction of the operations to assure availability and provision of care and services
 - B. Implementing Governing Body directives
 - C. Implementing the organization's policies and procedures
 - D. Responsible for ongoing oversight of the agency's quality management system
 - E. Compliance with applicable law and regulation
 - F. Maintenance of adequate staffing levels by recruiting, employing, and retaining qualified personnel

- G. Ensuring personnel access to continuing education such as inservices, continuing education for staff, etc.
- H. Evaluating the performance of reporting personnel according to organizational policy
- I. Providing leadership for performance improvement activities
- J. Managing operations in accordance with established fiscal parameters
- K. Program planning, development, implementation and oversight
- L. Representing the organization to other groups, organizations and the general public
- M. Ongoing review of marketing materials and public information documents to ensure accuracy
- N. Informing the Governing Body and personnel of current organizational, community and industry trends.

APPOINTMENT OF EXECUTIVE DIRECTOR/ADMINISTRATOR**Policy No. 1-007.1****PURPOSE**

To define the qualifications and selection process of the Executive Director/Administrator of the organization.

POLICY

The Governing Body will appoint the Executive Director/Administrator through a procedure established by the Governing Body. The Executive Director/Administrator will:

1. Be responsible for the day-to-day operations of the organization.
2. Have the necessary authority to implement his/her responsibilities for the operations, as conferred by the Governing Body.
3. Have at least two (2) years health related experience and the education, knowledge, and ability to fulfill his/her responsibilities. A master's degree is preferred.
4. Be knowledgeable of applicable law and regulation including Medicare Conditions of Participation as applicable, state regulations, licensure requirements, and any other applicable local/state/ or federal regulations.
5. Take reasonable steps to assure that:
 - A. The organization complies with applicable law and regulation.
 - B. Action is taken on reports and recommendations of any authorized planning or regulatory inspection organization.

PROCEDURE

1. The Governing Body will appoint the Executive Director/Administrator through the standard recruitment and selection process including, but not limited to:
 - A. Interviews with human resource personnel, various members of the Governing Body, senior management, etc.
 - B. Verification of educational preparation
 - C. Verification of references
 - D. Verification of experience and previous work history
2. Candidates will be narrowed to the two (2) or three (3) best candidates meeting the selection criteria and the job description qualifications. The President of the Board will make the final decision.

**DESIGNATION OF INDIVIDUAL IN ABSENCE OF
EXECUTIVE DIRECTOR/ADMINISTRATOR**

Policy No. 1-008.1

PURPOSE

To facilitate smooth and uninterrupted daily operations of Organization's Name during time off of administrative personnel.

POLICY

Organization's Name will have an Executive Director/Administrator or designee available at all times.

PROCEDURE

1. In the event that the Executive Director/Administrator is absent, the Clinical Director will assume the operational duties and responsibilities at Organization's Name. (See Clinical Director Job Description.)
2. In the event both are absent, an appropriately qualified nurse will assume the responsibilities.
3. These individuals have been identified as qualified to act on behalf of the Executive Director/Administrator in his/her absence.

Clinical Director: _____

Qualified Nurse: _____

PURPOSE

To define the overall responsibilities of the home health administrator position.

POLICY

The Executive Director/Administrator position will be responsible for the direction, coordination, and general supervision of all home health services.

The full extent of the Executive Director/Administrator's responsibilities will be defined in the applicable job description.

The Executive Director/Administrator will be qualified by education and experience including at least two (2) years of health-related experience and the knowledge and ability to effectively direct the program.

PROCEDURE

1. The Executive Director/Administrator's resume, diplomas/transcripts, and reference checks will be retained to validate the individual's educational qualifications.
2. During the interview process, the individual's knowledge regarding state regulations, Medicare Conditions of Participation, and other applicable regulations will be assessed. Only an individual with sufficient knowledge of these subjects will be considered for the position.
3. Responsibilities of the position will include but not be limited to:
 - A. Planning, organizing, directing and evaluating operations to ensure the provision of adequate and appropriate care and services
 - B. Ensuring organizational compliance with legal, regulatory, and accreditation requirements
 - C. Fiscal planning, budgeting, and management
 - D. Recruiting, employing, and retaining qualified personnel
 - E. Establishing and maintaining effective channels of communication, including integrations of technology, as applicable
 - F. Ensuring program personnel have current clinical information and current practices

- G. Directing and monitoring performance improvement activities
- H. Ensuring staff development including orientation, inservice education, continuing education and competency evaluation
- I. Assuring that all clinical services are furnished under the supervision of a physician or registered nurse
- J. Ensuring that appropriate service policies are developed and implemented to accomplish identified outcomes
- K. Assuring the development and qualifications for professional services and the assignment of personnel
- L. Assuring that the Clinical Manager and other appropriate staff supervision are available during all operating hours
- M. Ensuring the accuracy of public information materials and activities
- N. Appointing a similarly qualified alternate to be available at all times during operating hours in the absence of the Executive Director/Administrator
- O. Maintaining ongoing liaison with the Governing Body
- P. Implementing Governing Body directives
- Q. Informing the Governing Body and staff of current organizational, community and industry trends
- R. Ensuring standards of ethical business and clinical practice are maintained

PURPOSE

To ensure compliance with local, state, federal, and other regulatory bodies.

POLICY

The organization will maintain evidence of regulatory compliance, including but not limited to:

1. Current California license
2. Organization and ACHC policies and procedures
3. Medicare and Medicaid provider numbers and regulations
4. Business and occupational licenses for city, county as required
5. CLIA certification
6. Reports of reviewing bodies (ACHC, FDA, state licensure surveys, OSHA, DEA, Department of Transportation, etc.)
 - A. Negative outcomes affecting accreditation, licensure, or Medicare/Medicaid certification will be reported to ACHC within thirty (30) days of the occurrence.
 - B. Incidents that must be reported to ACHC are license suspension(s), license probation(s), non-compliance with Medicare/Medicaid regulations identified during survey, revocation of Medicare/Medicaid/Third-party provider number, Medicare condition-level deficiencies identified by another state or regulatory body and any open investigation by any regulatory or governmental authority.
7. D/B/A California registration, as applicable
8. Civil Rights Act of 1964, Equal Employment Opportunity Acts, Americans with Disabilities Act, Fair Labor Standards Act
9. HIPAA regulations
10. Omnibus Budget Reconciliation Act of 1987 and Balanced Budget Act of 1997
11. Section 1557 of the Patient Protection and Affordable Care Act
12. Public Health regulations relating to infectious disease

13. Professional standards and principles
 - A. Federal regulations
 - B. State practice acts
 - C. Commonly accepted health standards established by national organizations, boards, and councils
14. Medi-Cal Provider: Any person with a five percent (5%) or greater direct or indirect ownership interest in a home health provider, must submit fingerprints for a criminal background check in the manner determined by the Department.
15. If the home health agency is based outside of California, and provides services across state lines in California, it must have a parent office licensed in the state of California and meet all applicable requirements. If the home health agency is crossing state lines in order to serve rural, scarcely populated areas, it may meet the special conditions for a license on a case-by-case basis.
16. The licensee shall, within ten (10) days, notify the Department in writing of any of the following:
 - A. Change of name of home health agency
 - B. Change of location and/or address of home health agency
 - C. Change in licensing information required by Section 74661
 - D. Change of the mailing address of the licensee
 - E. Change of the principal officer (chairman, president, general manager) of the governing board. Such written notice shall include the name and principal business address of each new principal officer.
 - F. Change of administrator including the name and mailing address of the administrator, the date the administrator assumed office and a brief description of qualifications and background of the administrator.
 - G. Change of Director of Patient Care Services including the name and mailing address of the Director of Patient Care Services, the date the Director of Patient Care Services assumed office and a brief description of qualifications and background of the Director of Patient Care Services.
 - H. Addition or deletion of services.
17. A licensee may request in writing that a license be put in suspense. The Department may approve the request for a period not to exceed twelve (12) months.

18. Any licensee desiring to voluntarily surrender a license for cancellation shall notify the department in writing as soon as possible.
19. The license, or a true copy thereof, shall be conspicuously posted in a location accessible to public view in the main business area.
20. The following individuals shall submit to the State Department of Public Health an application and shall submit electronic fingerprint images to the Department of Justice for the furnishing of the person's criminal record to the state department:
 - A. The owner or owners of a private agency if the owners are individuals.
 - B. If the owner of a private agency is a corporation, partnership, or association, any person having a ten (10%) percent or greater interest in that corporation, partnership, or association.
 - C. The administrator of a home health agency.
21. The organization shall not allow newly hired administrators, program directors, and fiscal officers to have direct contact with patients prior to the completion of a criminal record clearance.
 - A. A criminal record clearance shall be complete when the Department has obtained the person's criminal offender's record information search response from the Department of Justice and has determined that the person is not disqualified from engaging in the activity for which clearance is required.
22. All required licensing information, including any changes, shall be provided to the Licensing and Certification Program. Changes shall be submitted within ten (10) business days along with any applicable fees.
23. If the criminal record review for owners, administrators, program directors and fiscal officers discloses a felony conviction or any crime that provides evidence of unfitness to provide home health services, the Licensing and Certification Program shall deny a license, or the individual shall be prohibited from providing services for the home health agency.
 - A. The individual may present evidence of rehabilitation and presently is of such good character to justify the issuance of the license or provision of home health services.
24. In addition to the background check, individuals shall provide the organization with information as to whether or not they have any prior criminal convictions, have any arrests within the past twelve (12) month period, or have any active arrests and shall certify that, to the best of their knowledge, the information provided is true.
25. All applicants for licensure shall provide information regarding any prior administrative action taken against them by any Federal, State or local government agency and shall certify that, to the best of their knowledge, the information provided is true.
26. Other state of California reporting requirements

ADDENDUM 1-010.A

ADDITIONAL STATE REQUIREMENTS

(Insert any state-specific regulatory requirements)

PROFESSIONAL ADVISORY COMMITTEE**Policy No. 1-011.1**

This is an OPTIONAL policy. A Professional Advisory Committee is no longer required by the Conditions of Participation but may be required by the state.

PURPOSE

To provide a process for the development and implementation of the Professional Advisory Committee.

POLICY

The Governing Body will appoint a multidisciplinary Professional Advisory Committee (PAC). The committee will consist of at least one (1) practicing physician, a nurse with community health or home care experience, representatives of other professional services reflecting at least the scope of organization services (such as physical, speech, or occupational therapy social work (masters prepared social worker) and discharge planning), and one (1) member that is neither an owner nor an employee of the organization. Members should be individuals who are aware of the needs of the community related to the population served.

This committee will meet at least annually, or more often as needed, and minutes of each meeting will be recorded. The committee is authorized by the Governing Body to advise the organization on professional issues, to evaluate program and services and to assist the organization to maintain relationships in the community. The committee will submit all policy revisions needed and annual agency evaluation to the Governing Body annually and as needed.

GUIDELINES

1. Responsibilities of the Professional Advisory Committee include:
 - A. The committee will establish and annually review policies and procedures governing the scope of services provided and document in the meeting minutes the oversight of the program, in the following areas:
 1. Admission and discharge criteria/policies
 2. Medical supervision and plans of care
 3. Emergency procedures
 4. Clinical protocols
 5. Clinical records
 6. Personnel qualifications and policies
 7. Infection control program

8. Clinical program policies
 9. Annual program evaluation
 10. Operating budget
 11. Quality assessment and performance improvement plans
- B. The committee will refer to state or national association for the most recent Medicare Conditions of Participation regulations and trends in home care.
- C. The committee will evaluate the organization's success in meeting community needs for home care services, provision of adequate and appropriate care to each patient, and progress towards financial stability.
- D. The Committee members will receive an orientation to their responsibilities and accountabilities. Orientation will include but not be limited to the following:
1. Organizational structure
 2. Employee and patient grievance policy and procedure
 3. Responsibilities for quality improvement program
 4. Organization's values, mission and/or goals
 5. Confidentiality practices and signing of a confidentiality agreement
 6. Overview of programs, services, and initiatives

A list of the Professional Advisory Members follows. (See "[Professional Advisory Committee Members](#)" Addendum 1-011.A.)

ADDENDUM 1-011.A

PROFESSIONAL ADVISORY COMMITTEE MEMBERS

PROFESSIONAL ADVISORY COMMITTEE MEMBERS

Organization's Name

Organization and Administration

[illegible]

PURPOSE

To identify a process for decision-making impacting organization operations.

POLICY

Organization's Name and its senior management team will maintain a written process for the participation in policy decisions affecting the organization.

GUIDELINES

1. Senior management personnel representing the clinical, financial, and operational components of the organization will meet monthly to discuss any of the following, as pertinent:
 - A. Performance improvement program/activities, including measures, outcomes, results, and quality improvement team activities
 - B. Human resource allocation/staffing, including recruitment, retention, and education
 - C. Management of information, including, as applicable, forms, and external data
 - D. Fiscal management, including monthly responsibility statements, and variances
 - E. Organization goals and objectives, including, but not limited to, community needs, strategic plans, and status of plans
 - F. Clinical operations (assessment, care and treatment, coordination of care, patient and family/caregiver education), including, but not limited to, services being provided, operational issues, new program development, utilization review issues, and patient and family/caregiver concerns
 - G. Environmental issues, including, occupational exposures, workers' compensation, safety issues, and hazards
 - H. Surveillance, prevention and control of infections, including, patient and organization personnel infections, and trends
 - I. Ethical/legal issues, including potential risk management issues, and ethical concerns
2. Senior management's involvement in decision-making regarding policy issues will be dependent on the scope and degree of the issue.

3. Senior management has the authority and responsibility to make decisions that affect patient care and services regarding any of the above as delegated by the Governing Body.
4. Issues which senior management must direct to the Governing Body include any issue, which cannot be resolved by senior management.
5. Minutes of senior management meetings will be recorded. The minutes will be kept in the office of the Executive Director/Administrator.
6. Content of discussion, results of discussions and actions taken, as a result of management meetings will be incorporated into the performance improvement program. This may include actions which address systems, knowledge and/or behavior/attitude issues.
7. Any member of senior management may participate in a performance improvement team/process. Senior management may undertake a process improvement project, which addresses senior management processes, as applicable.

DEVELOPMENT OF POLICIES AND PROCEDURES**Policy No. 1-013.1****PURPOSE**

To define how policies and procedures are developed, implemented, reviewed and revised.

POLICY

Policies and procedures will be developed as necessary to provide guidance to organization personnel in providing care and service to patients. Through the annual evaluation process, they will be reviewed and revised to address changing organization needs or requirements. (See "[Governing Body](#)" Policy No. 1-002, and "[Organizational Planning](#)" Policy No. 1-018.)

Administrative policies will address authority and responsibility in the areas of governance, planning, financial controls and personnel.

Operational policies will form the framework for planning, delivery and evaluations of care and services provided.

PROCEDURE

1. A policy and procedure may be developed by anyone in the organization, with the approval of his/her immediate supervisor. A draft will be given to senior management for review and possible revisions.
2. Senior leadership and the Governing Body will approve new or revised policies and procedures. The senior member of each group will document approval through signature.
3. The policies and procedures will be communicated through the management hierarchy, at department meetings, committee meetings, etc.
4. The policies and procedures will be implemented via education and training as well as during field supervision of clinical personnel.
5. Personnel will have access to policies and procedures at all times
6. Policies and procedures will be reviewed annually through the organization evaluation process in cooperation with the Governing Body and other oversight committees as applicable.
7. Professional experts, including physicians, surgeons and registered nurses will be involved in the development, review and revisions of policies and procedures, when applicable.
8. The annual review will be detailed in the minutes of the organization's annual evaluation.
9. All revisions will be assigned an effective date and if appropriate, a revised date and noted on the Administrative Policy Renewal/Revision Flow Sheet. (See "[Administrative Policy Renewal/Revision Flow Sheet](#)" Addendum 1-013.B.)

10. The following policies and procedures will form the minimum framework for Organization's Name policy manual.

Administrative:

- A. Conflict of interest disclosure
- B. Public disclosure of information
- C. Responsibilities of the ethical issues review group
- D. Complaint Process (internal and external)
- E. Exposure control plan
- F. Formal safety program
- G. Financial policies and procedures
- H. Research activities/investigational studies where applicable
- I. Corporate Compliance Plan
- J. Identity Theft Prevention Program

Operational:

- A. Nondiscrimination practice in regards to admission of patients
- B. Criteria for the acceptance or non-acceptance of patients
- C. Admission, continuation of service and discharge
- D. Standardized assessment process
- E. Referral to other providers of care
- F. Medical orders, verbal orders and physician oversight where applicable
- G. Emergency service
- H. After hours service
- I. Confidentiality of protected health information
- J. Emergency/disaster management
- K. Health, safety, and security of personnel during working hours
- L. Services/products provided directly and under contract

- M. Standards of practice for all disciplines, as applicable
- N. Standards of operation for all products, as applicable
- O. Infection control
- P. A TB exposure plan in compliance with the most current CDC recommendations
- Q. Medical Device Act
- R. Accepted medical term abbreviations
- S. Organizational performance improvement activities

Personnel:

- A. Conditions of employment
- B. Respective obligations between employer and employee
- C. Nondiscrimination information
- D. Grievance procedures
- E. Employee orientation
- F. Employee exit interviews
- G. Maintenance of health reports and protected employee information
- H. Employee record confidentiality and record retention
- I. Recruitment, retention, and performance evaluation of personnel

California:

1. The organization shall ensure that all policies and procedures regarding the purchase, storage, furnishing and transportation of legend devices are maintained, implemented, reviewed and revised as needed annually by a group of professional personnel, including a physician, surgeon, pharmacist and registered nurse.

Note: Legend devices means any device that bears the label "Caution: Federal Law restricts this device to sale by or on the order of a _____" or words of similar meaning.

ADDENDUM 1-013.A

REQUIRED POLICY CHECKLIST

REQUIRED POLICY CHECKLIST

REQUIRED POLICIES	PRESENT
<i>Administrative:</i>	
1. Conflict of interest disclosure	
2. Public disclosure of information	
3. Responsibilities of ethical issues review group	
4. Client Rights and Responsibilities	
5. Internal and external complaint management	
6. Exposure control plan	
7. Formal safety program	
8. Financial policies and procedures	
9. Research activities/investigational studies as applicable	
10. Corporate Compliance Plan	
11. Identity Theft Prevention Program	
<i>Operational:</i>	
1. Nondiscrimination statement addressing admission of clients	
2. Defined criteria for acceptance and non-acceptance of clients	
3. Admission, continuation of service and discharge	
4. Standardized assessment process	
5. Referral to other providers of care or services	
6. Medical orders, verbal orders and physician oversight as applicable	
7. Emergency service	
8. After hours service	
9. Confidentiality of protected health information	
10. Emergency/disaster management	
11. Health, safety and security of personnel during all hours of work	
12. Services/products provided directly and under contract	
13. Standards of practice for all disciplines	
14. Standards of operation for all products as applicable	
15. Infection control	
16. A TB Exposure plan in compliance with the most current CDC recommendations	
17. Medical Device Act	
18. Accepted medical term abbreviations	
19. Performance improvement activities	
<i>Personnel:</i>	
1. Conditions of employment	
2. Respective obligations between employer and employee	
3. Non-discrimination information	
4. Grievance procedures	
5. Employee orientation	
6. Employee exit interview	
7. Maintenance of health reports and protected health information	

Organization's Name

Organization and Administration

REQUIRED POLICIES	PRESENT
8. Employee record confidentiality and record retention	
9. Recruitment, retention and performance evaluation of personnel	

ADDENDUM 1-013.B

**ADMINISTRATIVE POLICY RENEWAL/REVISION
FLOW SHEET**

ADMINISTRATIVE POLICY RENEWAL/REVISION FLOW SHEET

[illegible]

CLINICAL POLICIES AND PROCEDURES**Policy No. 1-014.1****PURPOSE**

To ensure that patient care and services are guided by current and relevant clinical policies and procedures.

POLICY

The organization will maintain current, up-to-date policies and procedures manuals for clinical personnel to utilize in the provision of patient care and service.

Clinical policies and procedures will be revised according to state/federal guidelines and current clinical practice.

PROCEDURE

1. Clinical policies and procedures manuals may be developed internally utilizing relevant and current professional practice guidelines or may be purchased commercially. Purchased manuals will be individualized to the organization's practice.
2. Clinical policies and procedures will be reviewed and revised, as indicated, and approved and dated at least annually.
3. Approval of clinical policies and procedures may be completed by the Clinical Director, the Medical Director, or a clinician with recognized and documented expertise in a specific clinical arena. Final approval will be by the Governing Body.
4. Clinical policies and procedures, professional journals, and discipline-specific practice guidelines will be accessible to personnel at all times.
5. The following clinical policies and procedures will form the minimum framework for Organization's Name policy manual:
 - A. Plan of care development, implementation, coordination and evaluation
 - B. Review of the plan of care by the physician and home health staff
 - C. Verbal orders processing
 - D. Communication with the physician regarding changes in the patient's status or needed revision to the plan of care

- E. Sixty (60)-day summary reports
- F. Care coordination
- G. Supervision of paraprofessional services, LPN/LVNs and therapy assistants
- H. Medication management protocols, including at a minimum:
 - 1. Drug utilization review
 - 2. First dose in the home policy
- I. Emergency care protocols
- J. Advance Directives
- K. Reporting patient abuse, neglect, fraud or exploitation
- L. Patient safety
- M. Clinical laboratory test processes, as applicable
- N. Accepted medical term abbreviations
- O. Annual program evaluation
- P. Clinical record retention upon discontinuation of operations
- Q. Patient consent for release of information

USE OF ORGANIZATIONAL CHART

Policy No. 1-015.1

PURPOSE

To facilitate effective overall management and administration of the organization and establish communication channels for all organization personnel.

POLICY

There will be defined lines of authority, which clearly establishes responsibility and accountability for all organization personnel.

PROCEDURE

1. Organizational charts (see "[Organizational Charts](#)" Addendum 1-015.A) will be used to define relationships and lines of authority within the organization.
2. Organizational charts will be distributed as part of orientation to ensure personnel understand the organization's reporting structure.
3. The organizational chart will be reviewed, revised and dated as changes occur.

ADDENDUM 1-015.A

ORGANIZATIONAL CHARTS

(Insert Organization Chart)

PURPOSE

To ensure uniform quality of patient care and service for all patients throughout the organization and to ensure that patients have access to the resources they need to meet their health care needs.

POLICY

All patients, regardless of race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, or place of national origin have the right to receive the same quality of care throughout the organization and to have access to the home health resources they need to meet their health care needs.

PROCEDURE

1. The care and resources the patient receives, as well as the skill level and training of organization personnel, will be based on the standards of care and practice outlined within this manual, as well as on the patient's health care needs.
2. The organization will maintain a clinical/service record review process to assure that the organization policies and procedures are adhered to by all clinical/service personnel, including direct and contract personnel.
3. The organization will not discriminate against an individual based on whether or not the individual has executed an Advance Directive.

EXPERIMENTAL RESEARCH AND INVESTIGATIONAL STUDIES**Policy No. 1-017.1****PURPOSE**

To set forth guidelines for the organization's participation in research or administration of experimental medications and treatments.

POLICY

Internal and external research and experimental procedures, treatments and medications must be approved by the Medical Director and Executive Director/Administrator prior to initiation.

The physician's (or allowed practitioner's) orders for such procedures, treatment, and medications must be accompanied by the patient's voluntary consent for such interventions. The patient has the right to refuse to participate in any such activity and will not be discriminated against if participation is refused. Consents will be retained in the patient's file.

A copy of research protocols for internally or externally sponsored studies are maintained on file within the organization.

PROCEDURE

1. The Medical Advisory Committee chairperson will initially approve all experimental procedures, treatments, and medications.
2. The referring physician, who must be a registered investigator for the specific drug or treatment, will provide a complete drug protocol, which outlines drug characteristics, actions, admixture, administrative procedures, side effects and special precautions.
3. The Medical Advisory Committee will review, at the least, the following:
 - A. Experimental protocols in relation to the organization's mission and ability to provide the necessary care and services
 - B. The safety and practicality of home administration
 - C. A way to conduct reviews and identify who will be involved
 - D. The relative risks and benefits to the subject
 - E. Specific parameters for protection of participants and confidentiality of personal information
 - F. Mechanism for ensuring that research participants and personnel have been fully informed of the purpose of the study and any risks involved

- G. The process for obtaining the subjects' informed consent and the organization's consent to participate in the research
- 4. The patient must receive a complete and written explanation of the treatment or procedure and possible side effects and complications prior to giving consent. Risk associated with the research project must be clearly delineated.
- 5. The patient's informed consent will be documented on the clinical/service record.
- 6. If the patient has begun an experimental procedure, treatment, or medication in another health care setting, and has given consent, the organization will obtain a copy of the informed consent from the primary investigator.
 - A. Personnel providing care for the patient (procedure, treatment, or medication) will follow applicable organization policy and practice per physician (or allowed practitioner) orders and be qualified to administer the investigational medication.
 - B. The patient's physician will be informed of the patient's participation in the research or clinical trial
 - C. Personnel monitoring a patient's response to the procedure, treatment, or medication will report that response per physician (or allowed practitioner) orders.
- 7. A payer source must be established and documented prior to initiation.
- 8. Knowledge gained from participation in research protocols will be integrated into clinical practice as applicable.
- 9. In the event a patient is admitted on treatments that are atypical, the staff are permitted to perform or monitor under the direction of the physician.

PURPOSE

To define a process for the development and monitoring of an organizational/strategic plan that is consistent with the organization's mission and patient needs.

POLICY

Organization's Name's leaders will be responsible for developing and implementing an effective organization wide planning process that includes:

1. Establishing the organization's mission
2. Clearly defining long-range, strategic, and operational plans
3. Ongoing monitoring to ensure that the organization's mission is maintained over time

The leaders will plan and monitor the organization's care and services to be consistent with its mission and patient needs. The planning process includes:

1. The needs of individuals served, personnel, internal and external resources
2. Clinical/service practice guidelines and relevant literature
3. Sound business practices
4. Resources needed to provide and support care and services
5. Recruitment, retention, development, and continuing education needs for all personnel
6. The data needed to measure the performance of the processes and outcomes of care and services
7. Results of organization quality assessment performance improvement activities

Organization's Name will notify in writing within 10 days of a written request or any changes in administrator, other key management personnel, changes in ownership, name, location and/or address, or mailing address to applicable regulatory and accrediting bodies as required by law and regulation.

Organization leadership will communicate the organization's plans to personnel on an ongoing basis. All personnel will have an opportunity to communicate with leadership regarding operational and clinical issues on a routine, scheduled basis.

The written strategic plan will be developed and approved by the Governing Body at least every three (3) years. Periodic review and revisions of the strategic plan will be documented in the Governing Body meeting minutes. Changes to the plan will be incorporated into the planning document as addenda.

PROCEDURE

1. Annually, the organization's leadership will plan for home care/service delivery by:
 - A. Allocating resources through the budgeting process
 - B. Directing planning activities through an annual review and evaluation of the strategic plan
 - C. Developing policies and procedures to effectively meet the needs of patients, physicians, personnel, and other identified organizations
 - D. Developing and/or updating a marketing plan based on current assessed community needs
2. The planning process will involve:
 - A. Assessing the strengths, weaknesses, opportunities, and threats (S.W.O.T.) of the organization with an emphasis on:
 1. Analysis of service area demographics
 2. Current knowledge of market penetration
 3. New or changing consumer and community needs
 4. Analysis of routinely collected data
 5. Personnel input
 6. Determination of organizational priorities
 - B. Recruitment, retention, development, and continuing education needs for all personnel
 - C. Development of long-range, strategic, and operational goals which include all care and services to be provided, based on the organization's mission (See "[Organizational Goals and Objectives](#)" Addendum 1-018.A.)
 1. Allocation of resources through the budgeting process
 2. Determination of time frames for implementation and expected results

- 3. Assignment of responsibility to specific senior managers.
- D. Development of policies and procedures, both administrative and clinical, which direct the activities of all organization personnel in the provision of care and service
- 3. The planning process will be monitored by leadership through a quarterly review of:
 - A. Organization goals and objectives
 - B. Organization strategic plan and evaluation of the need for adjustments
 - C. Patient satisfaction surveys and complaint forms
 - D. Performance improvement results
 - E. Policies and procedures (annually)
- 4. Minutes of the management meetings and other committees, as well as minutes from the Governing Body and appropriate oversight committees, will reflect the planning process.
- 5. The Governing Body and senior management will communicate the organization's plan to personnel on an ongoing basis. All personnel will have an opportunity to communicate with senior management regarding operational and clinical issues on a routine, scheduled basis.

ADDENDUM 1-018.A

ORGANIZATIONAL GOALS AND OBJECTIVES

(Insert Organization-Specific Goals and Objectives)

PURPOSE

To ensure a timely and appropriate response to changes in the home care environment.

POLICY

Organization's Name will routinely monitor and assess internal and external factors having a potential or actual impact on the organization.

PROCEDURE

1. Utilizing operational and financial models, senior management will monitor and analyze internal and external factors or barriers that have the potential for causing changes in operations or other adverse conditions. Factors to be considered include:
 - A. Risk identification and prevention
 - B. Mergers, acquisitions, and re-organizations
 - C. Development of safeguards and controls
 - D. Impact of health care trends
 - E. Resource management
 - F. Cash flow/profit margins
 - G. Product life cycles
 - H. Business interruptions
2. Senior management will collect information regarding external trends through membership in home care professional organizations, monitoring current industry literature, and networking with other health care professionals.
3. Internal trends will be monitored by senior management through review of clinical, service, financial, and operational reports generated by the organization.

CONTINGENCY PLAN IF ORGANIZATION CLOSES**Policy No. 1-020.1****PURPOSE**

To provide guidelines for a contingency plan for Organization's Name closing.

POLICY

There will be a plan and related guidelines to facilitate a smooth transfer of patients still requiring service if Organization's Name closes.

PROCEDURE

1. In the event that Organization's Name discontinues operations, patients will be discharged, if their condition allows, with physician's (or allowed practitioner's) order, or transferred with a transfer record to another facility or home care service, following organization policies and procedures.
2. All physicians of active patients will be notified at least ten (10) days in advance of organization closing.
3. Verbal and written notification will be made at least ten (10) days in advance of organization closing to the State Licensing Organization.

All administrative records will be retained for at least three (3) years from the date the last cost report (as applicable) was filed. Clinical/service records will continue to be retained for seven (7) years by Organization's Name. Pediatric clinical/service records will be retained for seven (7) years past the age of majority.

PURPOSE

To ensure that administration, documentation, communication, coordination and retrieval of information at branch locations is treated in the same manner as that in the parent location.

POLICY

Branch locations will comply with all applicable policies and procedures as that of the parent organization and for the protection and appropriate use of confidential patient information. All branches and all personnel furnishing services to patients must be licensed, certified, or registered, as applicable, in accordance with the state licensing authority.

PROCEDURE

1. Organization's Name will maintain an exact copy of the organization's policies and procedures manuals in the branch locations and are available and accessible to staff. Other resource materials such as clinical procedures and patient education materials available at the parent location are available to staff at the branch locations.
2. A clinical supervisor that possesses the experience, education and qualifications to oversee all care and services provided by the branch shall be available during all operating hours. When the clinical supervisor is unavailable one or more health professionals that possess the experience, education and qualifications to oversee all care and services provided by the branch shall be available during all operating hours
3. Organization personnel at all locations will be introduced to applicable policies and procedures during the orientation process. Changes in policies and procedures will be communicated to personnel at all locations through the use of personnel meetings, newsletters, or other available means of communication.
4. Branch locations will maintain patient clinical/service records in the identical manner as the parent location including entries into the record and protection of protected health information (PHI).
5. Personnel assigned to branch locations will attend personnel meetings, educational inservices, quality improvement meetings and patient case conferences to ensure that coordination of care and communication of significant information is accomplished.

6. Supervision of the branch will be accomplished in the following manner: the parent organization administrator/executive director or clinical director shall conduct an onsite visit to the branch location at least monthly. The clinical director (parent location) will meet, in-person, with the branch location clinical supervisor at least weekly or more often as needed to discuss patient care or personnel issues. The parent organization administrator/executive director or clinical director shall communicate by telephone or electronically with the branch location clinical supervisor daily to demonstrate supervision and administrative control of the branch location.
7. Administrator/Executive Director or designee will establish and maintain agreements for office management such as office space, utilities, insurance, IT support and vendor contracts. The parent organization will be responsible for the staffing, patient census and other issues affecting the operation of a branch location. The branch clinical supervisor will resolve patient care issues with input from the parent Clinical Director.
8. If the organization wants to establish a branch location, the Department will be notified in advance of establishing that office and provide the following information:
 - A. Description of the services to be provided (Cannot offer services different from those offered by the parent) and if they will be provided directly or under contract
 - B. The geographic area to be served (counties, cities, zip codes)
 - C. A description of how the parent will supervise the branch and the organizational lines under the parent
 - D. Address and phone number of the branch
9. If organization intends to add a location, it must notify CMS, and, if deemed, it should notify ACHC, in writing of the proposed location and if the location will participate in Medicare and/or Medicaid.

CONTRACTED SERVICE PROVIDERS

Policy No. 1-022.1

PURPOSE

To ensure that contracted services are provided by trained and qualified professionals and paraprofessionals.

POLICY

Organization's Name will verify and document the training and qualifications for professional and paraprofessionals who provide services to the organization's patients via contractual agreement. (See "[Written Agreement for Contracted Services](#)" Policy No. 1-024.)

PROCEDURE

1. Documentation of training and professional qualifications may be maintained by Organization's Name or by the contracted organization. If maintained by the contracted organization, verification by Organization's Name will occur at least annually.
2. Documentation will be maintained for at least the following items:
 - A. Successful completion of an approved training course
 - B. Demonstration of skills competency
 - C. Completion of organization's orientation
 - D. Current personnel records containing the documentation required by Organization's Name

PURPOSE

To specify the contents of a Medicare Certified Home Health written agreement by defining the nature and scope of services provided by clinicians and others not directly employed by the organization.

POLICY

Senior management will be responsible for the availability of care and services to meet the needs of the patients served. When the organization provides care and services through another source, the patients will be entitled to the same level of performance from that source as from the organization itself. These contracted services will be defined by a written agreement, before individuals from that source will be permitted to provide services on behalf of the organization.

PROCEDURE

1. The written agreement between the organization and the contract service/individual/organization will define the nature and scope of services.
2. The following Medicare COP requirements for written agreements will be included in all home health contracts in addition to those requirements as listed in "[Annual Organization Evaluation](#)" (see Policy No. 5-004):
 - A. Patients will be accepted for care only by Organization's Name.
 - B. Primary organization maintains control of, supervises, coordinates, and evaluates care provided. Methods to ensure Primary organization control are described within the contract.
 - C. Procedures for scheduling visits and periodic patient evaluation.
 - D. Mechanism to ensure that contracted clinicians possess current professional licensure and/or certification, as applicable.
 - E. Mechanism for orienting contracted personnel to the primary organization's policies, procedures and processes.
 - F. Mechanism for contracting parties to participate in the development of patient's plan of care.

- G. Timeframes for placement of contracted staff and contingency staffing plans.
 - H. Confidentiality of all protected health information.
3. When home health aide services are provided under contract, Organization's Name retains the following responsibilities:
- A. To ensure the quality of care provided by the home health aide.
 - B. Perform home health aide supervision as per regulation.
 - C. To ensure that the home health aides have successfully completed mandated training and competency requirements.
4. The above elements of the written agreements will be used for all contracted services. Any deviation from the approved format must be approved by the Executive Director/Administrator.
5. The organization of individual providing services under arrangement may not have been:
- A. Denied Medicare or Medicaid enrollment;
 - B. Been excluded or terminated from any federal health care program or Medicaid;
 - C. Had its Medicare or Medicaid billing privileges revoked; or
 - D. Been debarred from participating in any government program
6. The organization's annual evaluation process as defined in "[Annual Organization Evaluation](#)" (see Policy No. 5-004) will additionally, monitor, evaluate, and audit the contracted services to ensure compliance with the Medicare Condition of Participation and organization policies and procedures. A copy of the [Home Health Contracted Services Review](#) (see Addendum No. 1-023.A) checklist will be utilized annually to assess compliance and filed in the vendor's file.

ADDENDUM 1-023.A

HOME HEALTH CONTRACTED SERVICES REVIEW

HOME HEALTH CONTRACTED SERVICES REVIEW

The following components must be included in all home health contracts.

	Contract Requirements	ACHC Standard	Yes	No
1.	Specific services or products to be provided by the contracted organization			
2.	Contractor adheres to organization's policies and procedures			
3.	Education, training, and qualifications required of contracted employees			
4.	Mechanism for contractor to participate in Performance Improvement activities, as applicable			
5.	Procedures for documenting and submitting notes that verify provision of contracted services			
6.	Procedures for submission of invoices and methods for reimbursement for contracted services provided			
7.	Effective date of the contract			
8.	Terms for renewal and/or termination			
9.	Patients are accepted for care only by Organization's Name			
10.	Methods for Organization's Name to control, coordinate and evaluate contracted services			
11.	Procedures for scheduling visits and reporting missed visits			
12.	Procedures for periodic evaluation of patient care			
13.	Mechanism to ensure contracted personnel maintain current licensure and/or certification, as applicable			
14.	Responsibility for development of the patient's plan of care			
15.	Timeframes for placement of contracted staff			
16.	Contingency staffing plans			
17.	Maintenance of confidentiality for protected health information			
18.	Contract signed and dated by authorized principles of each party			

Review of contract completed by: _____

Date: _____

PURPOSE

To specify the contents of a written agreement by defining the nature and scope of services provided by clinicians/technicians and others not directly employed by the organization.

POLICY

Senior management will be responsible for the availability of qualified care and services to meet the needs of the patients served. When the organization provides care and services through another source, the patients are entitled to the same level of performance from that source as from the organization itself. These contracted services will be defined by a written agreement before individuals from that source will be permitted to provide services on behalf of the organization. (See "[Contracted Service Providers](#)" Policy No. 1-022.)

Written agreements are signed and dated by authorized individuals of each organization. (See "[Home Health Contracted Services](#)" Policy No. 1-023.)

Written agreements are reviewed annually.

PROCEDURE

1. The written agreement between the organization and the contract service/individual/organization will define the nature and scope of services.
2. The organization shall ensure that all services furnished under arrangement provided by other entities or individuals meet the requirements of the Medicare Conditions of Participation and accreditation standards. The agency, organization, or individual providing services under arrangement may not have been:
 - A. Denied Medicare or Medicaid enrollment
 - B. Been excluded or terminated from any federal health care program or Medicaid
 - C. Had its Medicare or Medicaid billing privileges revoked
 - D. Been debarred from participating in any government program.
3. The written agreement will stipulate the following:
 - A. Service to be provided
 - B. Contractor is required to perform work in accordance with the primary organization's applicable policies and procedures

- C. Contractor assures that all personnel providing care have the education, training, and qualifications specified by Organization's Name
 - D. Mechanisms for the contractor to participate in performance improvement activities
 - E. Procedures for scheduling visits, and periodic patient evaluation
 - F. Procedures for submission of required patient related documentation that verifies the provision of services in accordance with the written service contract
 - G. Procedures for the submission of invoices and related information and reimbursement for care provided
 - H. Effective dates of the contract including terms of renewal or termination
4. The above elements of the written agreements will be used for all contracted services. Any deviation from the approved format must be approved by the Executive Director/Administrator.
5. The organization will retain responsibility for evaluating services, maintaining professional management responsibility, and ensuring continuity of care through its performance improvement program and/or corporate compliance program. If during the annual vendor evaluation of services it is found that services are not being provided per the written agreement, those services will not be billed to avoid improper billing.
6. As part of the organization's annual evaluation process, the Executive Director/Administrator, with the assistance of other organization personnel will monitor, evaluate, and audit the contracted services to ensure that they are being provided according to the contract, and ACHC standards. In addition, the review will:
- A. Formally assess the quality of services provided by the contracted provider
 - B. Determine pertinence of agreement to current practice
 - C. Extend or modify the terms of the agreement
 - D. Negotiate new terms as necessary
 - E. Terminate the contract, if necessary
7. Validation of the annual contract evaluation will be documented and includes
- A. Date of review
 - B. Participating parties
 - C. Continuing relevance of the contract to the provision of care
8. Contractual agreements for provision of services by Organization's Name to another entity will delineate the responsibilities of all parties.

ADDENDUM 1-024.A

**WRITTEN AGREEMENT FOR
HOME CARE SERVICES**

(Model)

**WRITTEN AGREEMENT FOR
HOME CARE SERVICES (MODEL)**

THIS AGREEMENT between Organization's Name (hereinafter "Organization") and the supplier of health care services identified as the Contractor on Attachment A ("Contractor"),
WITNESSETH:

1. PURPOSE. The purpose of this Agreement is to make available to patients of the Organization the health care services of the Contractor.
2. TERM. The term of this Agreement will begin and end on the date set forth on Attachment A.
3. TERMINATION PRIOR TO EXPIRATION OF TERM. This Agreement may be terminated by the Organization at any time, effective upon written notice to the Contractor, if, in the sole and unreviewable opinion of the Organization: (i) services provided by the Contractor are not consistent with applicable professional standards, the Organization's standards, or standards of third-party payers; or (ii) Contractor has committed one (1) or more breaches of this Agreement which endanger Organization patients or indicate inability or unwillingness of Contractor to fulfill its obligations hereunder. The Contractor may terminate this Agreement upon 60 days written notice to the Organization.
4. ORGANIZATION AND CONTRACTOR MANAGEMENT.
 - A. To the extent required by applicable rules of the United States Department of Health and Human Services (hereinafter "HHS Rules"), and to no greater extent, general authority over the activities performed on the Organization's behalf by the Contractor will be retained by the Organization Executive Director/Administrator or designee.
 - B. Day-to-day supervision and control of individuals performing services under this Agreement (including the Contractor, if the Contractor is an individual) will be the sole responsibility of the Contractor, and may not be delegated.
 - C. The Organization may refuse to accept services provided by any individual supplied by the Contractor, if the Organization finds in its sole and unreviewable opinion that the services provided by that individual do not meet the standard required of the Contractor in this Agreement. The Organization bears no authority or responsibility with respect to the hiring, training, or supervision of any individual performing Contractor's obligations under this Agreement.
 - D. Consistent with HHS Rules, only the Organization may accept a patient for care under this Agreement. No payment will be made under this Agreement for Contractor's services to any patient not accepted by the Organization as an Organization patient and assigned to the Contractor by the Organization.
5. PLANNING AND DELIVERY OF SERVICES. The Contractor will provide those patient care services, and will perform those other activities, described in detail on Attachment B.

**WRITTEN AGREEMENT FOR
HOME CARE SERVICES (MODEL) *continued***

- A. The Contractor will assure that every individual who performs services to Organization patients under this Agreement:
1. Adheres to all laws of the State applicable to the services provided;
 2. Is fully trained for the duties required of him or her, and maintains, without interruption or citation, the licensure or certification described on Attachment A hereto, and reports to Organization any adverse action against the individual;
 3. Conforms to all Organization policies pertaining to the qualifications, health, and conduct of patients and family/caregivers;
 4. Learns and adheres to all HHS Rules, and the objectives, policies, procedures, and programs of the Organization;
 5. Attends an orientation program specific to the Organization.
- B. All services performed by Contractor under this Agreement will be performed in a manner consistent with the requirements of HHS and other third-party payers, physician (or allowed practitioner) orders, Organization policies, organizations billing policies and any applicable plans of care developed through the Organization's care planning process.
- C. Continuity of care is important to the quality of the Contractor's services. When a patient is assigned to the Contractor, the Contractor will promptly assign an individual of the Contractor's choice to be the primary person designated to provide care to the Organization patient. The Contractor will not thereafter substitute primary family/caregivers without good cause and without two (2) weeks prior written notice to the Organization.
- D. HHS Rules require that each Organization patient have a plan of treatment/plan of care, and that all services be provided consistent with such plan. The Contractor will participate with the Organization in the development and any revision of a patient's plan of treatment, based on appropriate initial and ongoing patient assessment.
- E. Consistent with HHS Rules, the Contractor will provide patients with such patient rights information as the Organization will direct. The Contractor will document such communication of patient rights in the patient's records.
- F. The Contractor will make and submit to the Organization clinical and progress notes with respect to Organization patients. Records and notes of a patient visit will meet all standards imposed by HHS Rules or other third-party payers and will be filed with the Organization within five (5) working days of the day of the visit. The Organization will have no obligation to provide workspace, clerical help, supplies, or equipment for the preparation of notes, but may require that notes be submitted on a form acceptable to the Organization. No payment will be made for an episode of care until the required notes and records of the episode of care have been provided to the Organization.

**WRITTEN AGREEMENT FOR
HOME CARE SERVICES (MODEL) *continued***

- G. Visits with patients will be scheduled by the Contractor, consistent with patient needs, a plan of treatment, physician (or allowed practitioner) orders, and Organization policies pertaining to evening, weekend, and holiday patient contracts. Deviations in visit schedules including missed visits from any of the above principles will occur only for good cause and will be documented in writing to the Organization. The Organization may not determine the hours of work of the Contractor or its employees.
- H. The Contractor will consult with the organization nursing personnel, as needed, to provide adequate patient progress evaluation. The consultation will include the attendance of the Contractor at patient care conferences. The Contractor will assist the Organization in the coordination, supervision, and evaluation of patient care, including plans for patient discharge from service.

6. EVALUATION OF CONTRACTOR PERFORMANCE.

- A. HHS Rules require the Organization to ensure that all services to Organization patients are within acceptable professional standards. Organization personnel will accompany the Contractor on visits to patients from time to time, for the purpose of monitoring Contractor's compliance with HHS requirements.
- B. Organization personnel will conduct reviews at least annually of the records of the services of the Contractor for the purpose of determining whether the services provided meet the requirements of HHS Rules and other third-party payer requirements.
- C. The Contractor will participate in the Organization's performance improvement activities when services provided by Contractor are selected for study.

- 7. **BILLING AND PAYMENT.** The Contractor's sole compensation for services to Organization patients will be that described in Attachment C. The Contractor will not bill any patient or any patient's responsible party or third-party payer for services provided under this Agreement.
- 8. **INSURANCE.** The Organization provides no insurance of any kind for injuries or losses to, or caused by, the Contractor, its servants, employees, agents, or subcontractors. The Contractor will provide such workers' compensation insurance as may be required by law, for any person who performs any portion of the duties of the Contractor under this Agreement. The Contractor will provide general liability $\pm$ A for any person who performs any portion of the duties of the Contractor under this Agreement. The Contractor will provide proof of insurance covering each individual performing services under this Agreement before such individual provides services, and at any time upon the Organization's request. In the event any individual or organization asserts a claim against the Organization, based wholly or partly upon the Contractor's actions or failure to act, the Contractor will indemnify the Organization for all of the Organization's costs incurred as a result thereof, including payment of any settlement, judgment, award or other payment, as well as actual fees, costs, and attorney fees incurred in the defense of the claim.

**WRITTEN AGREEMENT FOR
HOME CARE SERVICES (MODEL) *continued***

9. PERFORMANCE BY AGENTS, EMPLOYEES, INDEPENDENT CONTRACTORS, ETC. Except as provided on Attachment B, Contractor will perform all services required of it under this Agreement personally or through individuals who are employees of Contractor, and who meet all applicable requirements of this Agreement, and all applicable HHS Rules. Organization will have no authority to select Contractor's employees.
10. RELATIONSHIP OF THE PARTIES. This Agreement creates a relationship of independent contracting parties, and it does not comprise either party as the employee, agent, coventurer, or employer of the other. The Contractor will select its own employees, and will perform all services required of it by its own methods, without supervision of the Organization except as set forth in this Agreement. The Contractor will exercise independent judgment in the performance of its assigned tasks under this Agreement. The Contractor will not, and will assure that its employees do not, represent themselves as employees of the Organization, but will identify themselves to Organization patients as the Contractor or Contractor's employees, respectively.
11. NONEXCLUSIVITY. This Agreement will not be exclusive as to Contractor or Organization. Contractor may market its services to the general public, but will not market its services to an Organization patient to whom it has provided services under the Agreement until six (6) months after the termination of this Agreement.
12. EQUIPMENT AND SUPPLIES. Except as provided in Attachment D, the Organization will provide no equipment or supplies to the Contractor for the Contractor's performance of services under this Agreement.
13. NONDISCRIMINATION. The Contractor will not discriminate against any Organization patient on grounds of race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, or place of national origin.
14. RECORDS. Until the expiration of four (4) years after the last date on which services are furnished pursuant to this Agreement, the Contractor will make available upon written request of the Secretary of Health and Human Services, or upon request of the Comptroller General, or any of their duly authorized representatives, this Agreement, and any books, documents, or records that are necessary to certify the nature and extent of the services provided by Contractor under this Agreement, in compliance with Part 420, Subpart C of Chapter 42 of the Code of Federal Regulations. All patient records will be held in the strictest confidence, and will not be disclosed except as may be allowed or required by law.
15. THIRD PARTIES AND ASSIGNMENT. This Agreement is for the benefit of the Organization and the Contractor, and no other person will be construed to be a beneficiary thereof. This Agreement may not be assigned.

**WRITTEN AGREEMENT FOR
HOME CARE SERVICES (MODEL) *continued***

16. COMPLETE AGREEMENT. This written Agreement (with its attachments) reflects the complete agreement between the parties. Any previous written, verbal or implied contractual relationship between the parties is hereby rescinded. No verbal undertakings or representations not set forth herein will be binding on either party. No agent, employee, or representative of either party has authority subsequently to modify the terms of this Agreement, except in a writing signed by the party to be charged.
17. PRONOUNS. Masculine, feminine, and neuter pronouns in this Agreement will be deemed to include each other, as the context and application of this Agreement may require.
18. CONSTRUCTION. This Agreement will be construed in accordance with the State laws, and will be interpreted as if mutually drafted by Organization and Contractor.
19. ADDITIONAL PROVISIONS. The terms set forth in Attachment D, if any, will apply between the parties. When such a provision conflicts with a provision in paragraphs 1–18, the provision in Attachment D will control.

IN WITNESS WHEREOF, the parties have set their hands unto this Agreement this

_____ day of _____, _____.

By _____ By _____

CONTRACTOR

By _____

Its _____

ATTACHMENT A

1. The name and address of the Contractor is:
2. Contractor's EIN: _____ – _____ NPI#: _____
3. This Agreement will be effective from and including _____, through and including _____, unless terminated under the provisions hereof.
4. Notices:

To Organization:

ATTENTION: _____

Executive Director/Administrator

To Contractor: _____

A notice required to be in writing will be effective when delivered to the address above or, if mailed, at 9:00 a.m. on the next business day after deposit in the U.S. mail, certified mail/return receipt requested, postage attached.

5. The Contractor will maintain workers' compensation insurance (when required by law) and general liability and malpractice insurance having the following limits, covering all individuals performing services under this Agreement:

Malpractice: Per occurrence \$1,000,000; aggregate \$1,000,000

General Liability: Per occurrence \$1,000,000; aggregate \$1,000,000

6. All individuals performing services to Organization patients under this Agreement will maintain State licensure or registration as a _____.

ATTACHMENT B

**SERVICES PROVIDED BY CONTRACTOR
DISCIPLINE SPECIFIC JOB DESCRIPTION**

ATTACHMENT C

CHARGES AND REIMBURSEMENT OF CONTRACTOR

ATTACHMENT D

ADDITIONAL TERMS

*(Attach additional sheets as necessary.
Number paragraphs beginning with 20)*

SCOPE OF SERVICES**Policy No. 1-025.1****PURPOSE**

To describe Organization's Name operations including the geographical service area.

POLICY

Organization's Name will operate an office that will provide a safe and adequate location related to space, facilities, and administrative services.

Organization's Name will be open from 8:00 a.m. to 5:00 p.m., Monday through Friday, except designated holidays or other days decided by the Executive Director/Administrator. Designated organization personnel will be available to patients on an on-call basis during non-office hours. Organization's Name services will be available 24 hours a day, seven (7) days a week.

Organization's Name will provide care to persons living within the following Counties:

_____	_____
_____	_____
_____	_____
_____	_____
_____	_____

Home care services are provided on an intermittent basis to patients in their place of residence.

Scope of Services (See "[Organization-Specific List of Procedures and Treatments](#)" Addendum 1-025.A)

1. Professional nursing services are provided in accordance with the patient's plan of care, under the supervision of a registered nurse and include:
 - A. Initial and ongoing comprehensive assessments of the patient's needs, including OASIS assessments at appropriate points in time
 - B. Initiating the plan of care and revising as necessary
 - C. Providing those services and/or treatments requiring substantial and specialized nursing skill
 - D. Counseling and educating the patient and family/caregiver regarding the disease process, self-care techniques, and prevention strategies

- E. Initiating appropriate preventive and rehabilitative nursing procedures
 - F. Preparing clinical and progress notes
 - G. Coordination of services
 - H. Referral to other services as needed
 - I. Informing the physician (or allowed practitioner) and other staff of changes in the patient's needs
 - J. Evaluating the effectiveness and outcomes of care
 - K. Supervising licensed practical/vocational nurses and paraprofessionals providing services
 - L. Assigning home health aides to specific patients
 - M. Planning for discharge from service
 - N. Participating in inservice programs
2. Licensed practical/vocational nurses supplement the nursing care needs of the patient as provided by the registered nurse. These include:
- A. Providing services in accordance with organization policies
 - B. Preparing clinical and progress notes
 - C. Assisting the registered nurse or physician (or allowed practitioner) in performing specialized procedures and duties
 - D. Assisting the registered nurse in carrying out the plan of care
 - E. Assisting the patient in learning appropriate self-care techniques
3. Home health aide duties include:
- A. Assisting with personal hygiene
 - B. Assisting with ambulation and exercise
 - C. Assisting with medications that are ordinarily self-administered (per state regulations)
 - D. Reporting changes in the patient's condition

- E. Providing nutritional support
 - F. Other supportive tasks as assigned
4. Personal care aide duties include:
- A. Assisting with personal hygiene
 - B. Nutritional support
 - C. Environmental support
5. Homemaker/chore services personnel provide patients with environmental support.
6. Rehabilitative therapies are provided according to the patient's plan of care by a registered/certified physical or occupational therapist in accordance with the patient's plan of care and include:
- A. Initial and ongoing assessments to determine the level of functioning, including OASIS assessments at appropriate points in time
 - B. Developing and revising the plan of care in consultation with the physician (or allowed practitioner) and other care team members
 - C. Goal setting
 - D. Providing therapeutic treatments
 - E. Educating the patient and/or /family
 - F. Evaluation of equipment needs to increase functional level
 - G. Preparing clinical and progress notes
 - H. Coordinating services in consultation with home health staff
 - I. Evaluating the effectiveness and outcomes of care
 - J. Supervising therapy assistants and home health aides, as appropriate
 - K. Participating in inservice programs
 - L. Planning for discharge

7. Licensed and/or certified assistants' (PTA, COTA) duties under the direction of a physical or occupational therapist will include:
 - A. Performing services planned, delegated, and supervised by the therapist
 - B. Providing services in accordance with organization policies
 - C. Preparing clinical and progress notes
 - D. Participating in teaching the patient and family/caregiver
 - E. Participating in inservice programs
8. Speech/language pathology services are provided according to the patient's plan of care by a qualified speech/language pathologist and/or audiologist and include:
 - A. Initial and ongoing assessments to identify the patient's level of functioning and ability to communicate, including OASIS assessments at appropriate points in time
 - B. Goal setting
 - C. Developing the plan of care in consultation with the physician (or allowed practitioner) and other care team members and revising it as appropriate
 - D. Preparing clinical and progress notes
 - E. Consultation and coordination of services with other disciplines
 - F. Patient/family education
 - G. Evaluating effectiveness and outcomes of care
 - H. Informing the physician (or allowed practitioner) and other personnel of changes in the patient's needs
 - I. Discharge planning
 - J. Participation in inservice programs
9. Social work services are provided according to the patient's plan of care by a qualified social worker, or by a qualified social work assistant under the supervision of a qualified social worker, and include:
 - A. Assessing psychosocial status
 - B. Goal setting

- C. Developing the plan of care in consultation with the physician (or allowed practitioner) and other care team members and revising it, as appropriate
 - D. Assisting other team members in understanding the social and emotional factors related to the health problems
 - E. Working with the family, as needed
 - F. Preparing clinical and progress notes
 - G. Evaluating effectiveness and outcomes of care
 - H. Utilizing and providing patient/family with community resources
 - I. Discharge planning
 - J. Consultation and coordination of services with other disciplines
 - K. Participating in inservice programs
10. Nutrition services are provided by a registered dietitian and include:
- A. Assessing the nutritional needs of patient per organization policy and revising the plan of care, as needed
 - B. Providing patient and family/caregiver education, as required
 - C. Consulting with staff regarding special dietary regimens and nutritional requirements
 - D. Preparing clinical and progress notes
 - E. Goal setting
 - F. Coordinating services with other involved disciplines
11. Physician (or allowed practitioner) services are provided by a licensed Doctor of Medicine, Osteopathy, or Podiatry and include:
- A. Diagnosing and treating medical conditions
 - B. Prescribing medications and therapeutic regimens
 - C. Developing and/or authorizing the plan of care
 - D. Approving additions or modifications to original plan of care recommended by professional home care staff
 - E. Performing periodic review for necessary revisions and changes

- F. Submitting signed orders for plans of care and changes in accordance with required time frames
 - G. Reviewing and/or preparing clinical and progress notes
 - H. Staff consultation as indicated
12. Medical supplies and appliances.
13. All services provided will be guided by the state board practice standards.

Note: An organization that furnishes outpatient physical therapy or speech language pathology services must meet all of the applicable conditions of the Medicare Conditions of Participation 484 and the additional health and safety requirements for outpatient services outlined in 485.

ADDENDUM 1-025.A

ORGANIZATION-SPECIFIC LIST OF PROCEDURES AND TREATMENTS

*(Insert organization-specific list of procedures and treatments provided, e.g. Wound Vac.,
Anodyne therapy, Vital Stim)*

LISTING OF SERVICES PROVIDED**Policy No. 1-026.1****PURPOSE**

To assure that at least one (1) of the Medicare home health qualifying services is provided directly by organization staff and that all services, whether provided directly or under contractual agreement, will adhere to organization policies and procedures.

POLICY

At least one (1) qualifying service will be provided (nursing, physical therapy, occupational therapy, speech therapy, medical social services, or home health aide services) directly with 100% organizational personnel. A second qualifying service and additional services may be provided under arrangement with another organization. All services provided, whether directly or under contractual agreement, will be guided by organization policies and procedures.

PROCEDURE

1. Services will be provided as listed below:

Service	Directly	Under Contract
Non-Skilled		
Home Health Aide		
Nurse Assistant		
Skilled		
Registered Nurse		
Licensed Practical/Vocational Nurse		
Physical Therapist		
Occupational Therapist		
Speech Therapist		
Medical Social Worker		
Nutritionist/Registered Dietitian		
Medical supplies and appliances		

2. Job descriptions for each position will be maintained by the organization.
3. The organization shall make available to the public, accurate information regarding the services offered by the organization and any limitations related to:
 - A. The scope of services provided
 - B. The types of specialty services provided
 - C. The duration of services provided
 - D. The frequency of services provided
4. Annually the Executive Director/Administrator shall review:
 - A. The scope of services, including any specialty services,
 - B. The duration & frequency of services provided

SCOPE OF THE BEHAVIORAL HEALTH PROGRAM**Policy No. 1-027.1****PURPOSE**

To provide guidelines for the patient receiving behavioral health services.

POLICY

A patient admitted to the behavioral health program will meet established admission criteria and will have a specific behavioral health assessment performed by a qualified clinician. (See "Scope of Assessments/Qualifications" (HR) Policy No. 1-010.)

The organization will maintain a current program description including a listing of the scope of services.

PROCEDURE***Admission Criteria***

2. In addition to the established routine admission criteria for Organization's Name, behavioral health patients will also meet the following criteria:
 - A. Patient must function in a manner that he/she would be deemed safe in a home environment.
 - B. Patient must not pose a threat to him/herself or to others.
 - C. Patient must be able and willing to comply with the behavioral plan of care.

Assessment Process

1. The clinician providing behavioral health services will follow the standardized plan of care for the behavioral health patient.
2. Assessments of the patients receiving behavioral health care will include, but will not be limited to, the following emotional and behavioral indicators:
 - A. Emotional and behavioral functioning
 - B. Mental status examination including orientation, memory, reasoning and judgment
 - C. Evaluations of language, self-care, social-affective, visual motor, and cognitive functioning

3. The psychosocial assessments and reassessments may include as indicated, but will not be limited to:
 - A. Social role status, including social and community activities
 - B. Financial resources including resources needed to manage functional limitations, pay for care/services and maintain current independent status
 - C. Sexual orientation
 - D. Environment and the home
 - E. Spiritual orientation
 - F. Social, peer-group, and environmental setting
 - G. Patient's family circumstances and relationships, including:
 1. Current living situation including responsible party, emergency contact
 2. Social, ethnic, cultural, emotional, and health factors
 - H. Alcohol and drug use
 - I. Use and side effects of psychoactive drugs
4. The clinician will document all initial and ongoing assessments in the clinical record.
5. Changes in the assessments from visit to visit will be documented and communicated to the appropriate physician/psychiatrist. Communication with the physician will be documented in the clinical record.

Discharge Criteria

1. Organizational discharge criteria will apply to the behavioral health patient without exception or addition.

Staff Qualifications

1. Registered nurses providing behavioral health care will:
 - A. Meet all federal and state regulations regarding qualifications to provide care
 - B. Have one (1) to two (2) years behavioral health experience
 - C. Have completed the organization's orientation and competency assessment for behavioral health

FINANCIAL RESPONSIBILITY AND MEDICARE WRITTEN NOTICES**Policy No. 1-028.1****PURPOSE**

To outline the process by which patients, families and caregivers will understand their financial responsibility for home health services.

POLICY

Upon admission, the admitting clinician will inform the patient and/or his/her representative of his/her payment responsibilities for home health services. The patient will be informed of any subsequent changes in his/her financial responsibility.

PROCEDURE

1. Insurance coverage and patient's responsibility for copayment will be discussed, disclosed, and presented in writing to the patient and family/caregiver. The actual costs for care, if any, will be presented in writing to the patient and family/caregiver. If copay responsibilities are not known, the clinician will provide the patient and family/caregiver with total organization charges until more accurate information can be obtained.
2. If more information is needed for verification of coverage, the clinician will discuss this with the Clinical Supervisor and may alert social services if the patient's financial situation is unclear. The clinician will notify the billing department if a tailored payment plan is required.
3. Patients who incur financial liability must be notified in writing within 30 calendar days from the date the organization is notified of any changes from payers.
4. Patients shall be advised of any changes in the financial responsibility information when they occur. The organization shall advise the patient and representative, if any, of these changes as soon as possible, in advance of the next home health visit.
5. Medicare patients must be provided with timely, accurate, and comprehensible written notices in any case where a reduction or termination of services is to occur, or where services are to be denied before being initiated. The ABN and HHCCN process includes a standard written notice in which the agency must insert appropriate language depending on the reason for any reduction or termination of home health services.
6. An Advance Beneficiary Notice of Noncoverage (See "[Advance Beneficiary Notice of Noncoverage](#)" Addendum 1-028.A) must be provided to Medicare patients whenever the home health agency believes that Medicare will not cover an item(s) and/or service(s). (ABN is a financial liability notice.)

7. HHAs may be required to provide an ABN when a triggering event occurs. Triggering events are:
 - A. Initiation: When an HHA expects that Medicare will not cover any item(s) and/or service(s) delivered under a planned course of treatment from the start of a spell of illness, OR before the delivery of one-time item(s) and/or service(s) that Medicare is not expected to cover.
 1. Occurs at the start of home health care and may also occur when a service is added to an existing home health plan of care. An ABN must be issued to the beneficiary prior to receiving care that is usually covered by Medicare, but in this instance, it is not covered or may not be covered by Medicare because:
 - a. The care is not medically reasonable and necessary
 - b. The beneficiary is not confined to home
 - c. The beneficiary does not need skilled nursing care on an intermittent basis
 - d. The beneficiary is receiving custodial care only
 - B. Reduction: When an HHA reduces or stops some item(s) and/or service(s) during a spell of illness, while continuing others, including when one home health discipline ends but others continue.
 1. Involves any decrease in services or supplies, such as frequency, amount, or level of care, provided by the HHA and/or care that is part of the plan of care
 - C. Termination: When a home health agency expects that Medicare coverage will end for all items and services in total.
8. If the beneficiary chooses to receive items/services in question and Medicare does not cover the home care, HHAs may use may use the ABN to shift liability for the non-covered care to the beneficiary. (ABNs are not used in managed care; however, when beneficiary transitions to Medicare managed care from original Medicare during a home health episode, ABN issuance is required when there are potential charges to the beneficiary). If the beneficiary does not want the item or service that is being initiated, reduced or terminated, no ABN is required.
9. Home health agency exceptions to the ABN notification requirements are:
 - A. Initial assessment, in cases where beneficiaries are not admitted for which the HHA does not charge
 - B. Care that is never covered by Medicare
 - C. Telehealth monitoring used as an adjunct to covered care

- D. Noncovered items/services that are part of care covered in total under Medicare bundled payment
10. Refer to the instructions found in the manual revision attachment to Change Request (CR) 8403, *Medicare Claims Processing Manual*, Chapter 30, Financial Liability Protections, section 50.15.4 and its subsections for specific ABN and Form CMS-R-131 related information, instructions, and additional requirements.
 11. A Home Health Change of Care Notice (see "Home Health Change of Care Notice, HHCCN Form CMS-10280 Addendum 1-028.B) must be provided to a Medicare beneficiary at two points in time, for reasons not related to Medicare coverage. These two triggering events:
 - A. Reduction: When an HHA reduces or stops an item and/or service during a spell of illness while continuing others, including when one home health discipline ends but others continue. Examples:
 1. Based on HHA reasons such as a temporary staffing shortage
 2. Based on physician orders
 - B. Termination: When an HHA ends delivery of all services. Examples:
 1. Based on agency reasons such as staffing, closure of agency, concerns for staff safety
 2. Based on failure to meet face to face encounter requirements
 3. Based on physician orders to discontinue care or lack of orders to continue care
HHCCN must be given PRIOR to discontinuation of services.
 12. Refer to the instructions found in the manual revision attachment to Change Request (CR) 8403, *Medicare Claims Processing Manual*, Chapter 30, Financial Liability Protections, section 60 and its subsections for specific HHCCN and Form CMS-10286 related information, instructions, and additional requirements.
 13. When the home health agency plans to stop furnishing all home health services to a patient because it expects that Medicare will not continue to pay for the services or there is a discharge for cause, they must provide a completed Generic Expedited Determination Notice entitled "Notice of Medicare Provider Non-Coverage," Form CMS-10123 to the patient 48 hours prior to terminating services. The patient or his/her representative must be given the opportunity to request an expedited determination from a QIO. (See "[Notice of Medicare Provider Non-Coverage](#)," Addendum 1-028.C.)
 14. After reading the, "Notice of Medicare Provider Non-Coverage," Form CMS-10123 the patient or his/her representative must sign and date the form indicating they have received the notice and understand they can appeal the decision by contacting a QIO.

15. The second or detailed notice, called "Detailed Explanation of Non-Coverage" Form CMS 10124, is given to a patient or his/her representative when QIO review is requested in order to provide more explanation on why coverage is ending. (See "[FFS Expedited Review Detailed Notice](#)" Addendum 1-028.D.)
16. CMS has provided explicit instructions including, but not limited to, when and how to fill out all required forms, the delivery of notices, time frames for delivery, and who pays for care while the QIO is reviewing the determination. (See "[Additional CMS Recourses for Expedited Notices](#)" Addendum 1-028.E) Organization leadership is responsible for educating staff on current CMS requirements regarding the ABN (CMS-R-131), HHCCN (CMS – 10280), FSS ED Notices and FFS Detailed Notice.
17. All written and verbal notifications of the patient's financial responsibility will be documented in the clinical and billing records. Original versions of the completed ABN, HHCCN, whether annotated or signed, and copies of signed Expedited Notices will be kept in the patient's record at the agency.

ADDENDUM 1-028.A

**ADVANCE BENEFICIARY NOTICE OF NONCOVERAGE (ABN)
FORM: CMS-R-131**

*For the most current version of CMS required form,
access the following website to print the form
and obtain directions on how to complete the form:*

<https://www.cms.gov/medicare/medicare-general-information/bni/abn>

ADDENDUM 1-028.B

HOME HEALTH CHANGE OF CARE NOTICE (HHCCN)

Form: CMS-10280

*For the most current version of CMS required form,
access the following website to print the form
and obtain directions on how to complete the form:*

<https://www.cms.gov/Regulations-and-Guidance/Legislation/PaperworkReductionActof1995/Downloads/CMS-10280.zip>

ADDENDUM 1-028.C

GENERIC EXPEDITED DETERMINATION NOTICE

“Notice of Medicare Provider Non-Coverage”

Form: CMS-10123

*For the most current version of CMS required form,
access the following website to print the form
and obtain directions on how to complete the form:*

<http://www.cms.hhs.gov/BNL/>

(FFS ED Notices)

ADDENDUM 1-028.D

FFS EXPEDITED REVIEW DETAILED NOTICE

"Detailed Explanation of Non-Coverage"

Form: CMS-10124

*For the most current version of CMS required form,
access the following website to print the form
and obtain directions on how to complete the form:*

<http://www.cms.hhs.gov/BNl/>

(FFS ED Notices)

ADDENDUM 1-028.E

ADDITIONAL CMS RESOURCES FOR EXPEDITED NOTICES

*For the most current information from CMS on the Expedited Review Process,
access the following websites:*

Medicare Claims Processing Manual, Chapter 30
Financial Liability Protections, Section 60

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Downloads/clm104c30.pdf>

Generic and Detailed Notices in English and Spanish, Word version of notices, Instructions on
use of forms, Q&As

<http://www.cms.hhs.gov/BNI/>

MLN Matters Article on New Expedited Review Process

<http://www.cms.hhs.gov/MLNMattersArticles/downloads/MM5009.pdf>

CMS maintained Directory of QIO's

www.cms.hhs.gov/qio

PROGRAM PLANNING**Policy No. 1-029.1****PURPOSE**

To define a process for program planning including new program development or the revision or termination of an existing program.

POLICY

Decisions to implement new programs and/or revise or terminate existing programs will be made based on documented information regarding the financial impact to the organization and the overall impact for the community the organization serves.

PROCEDURE

1. Development of new programs will include at a minimum:
 - A. Detailed program design (How will the program function? What population will it serve, etc.?)
 - B. Identification of resources and costs (Will new personnel with new skills be required? Will the organization have to seek outside funding, etc.?)
 - C. Development and implementation of a marketing program (How will the community find out about the new program?)
 - D. A pilot testing period (Length of time for pilot, patient population involved, geographic location, etc.)
2. Revision or termination of an existing program will include at a minimum:
 - A. A review of the cost impact (Will revisions increase revenues? Will termination of a program cost referrals in other programs or will it save the organization money?)
 - B. A review of the benefits (Will the revision better serve the community? Would termination of a program free up resources better used for another program?)
 - C. A review of consequences (What will happen if the revision or termination does occur? What will happen if it does not occur?)

PURPOSE

To define the methodology for development of an effective marketing plan.

POLICY

Organization's Name will annually develop a marketing plan with the intent to identify opportunities for growth and methods for attaining that growth.

The marketing plan will support the strategic plan, be consistent with the organization's mission, and comply with state and federal regulations.

PROCEDURE

1. Senior management in conjunction with marketing managers and/or community liaison representatives will be responsible for the development of the marketing plan.
2. Information including competitive activities, community/consumer needs and feedback, referral source information, growth or decline in existing programs, and patient satisfaction feedback will be gathered on an ongoing basis to be utilized in the development of the marketing plan.
3. Marketing activities may include but will not be limited to:
 - A. Meetings with current and potential referral sources
 - B. Securing third-party payer contracts
 - C. Personnel participation in community activities and events
 - D. Personnel joining and taking an active role in professional organizations
 - E. Senior management personnel serving on local, regional, and/or state boards or committees associated with the home care industry
4. All new types of marketing activities must be approved by the Executive Director/Administrator.
5. Marketing representatives will receive periodic training regarding regulation of marketing practices including, at a minimum:
 - A. Federal anti-kickback statutes
 - B. Federal False Claims Act

- C. The so-called "Stark Laws" and regulations
 - D. Any applicable state statutes
6. Utilization reports will be monitored to ensure that the marketing plan is effective in increasing referrals/admissions.
 7. Market penetration will be monitored on an ongoing basis and periodically measured to compare the organization's performance against the potential market.

SECTION TWO

Program/Service Operations

Policy No.

Availability of Services

2-001

Patient Bill of Rights

2-002

Admission Criteria and Process

2-003

Addendum: Face-to-Face Encounter Procedure*

2-003.A

Addendum: Sample Acceptance to Service Form

2-003.B

Admission Documents

2-004

Informed Consent/Refusal of Treatment

2-005

Addendum: Sample Informed Consent for Medical Photography

2-005.A

Physician Licensure Verification

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Complaint/Grievance Process

2-008

Fostering Internal Communication

2-009

Interface of Patient Data and Management Systems

2-010

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2-011

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2-012

Principles of Information Management

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2-014

Addendum: [Notice of Privacy Practices*](#)

2-014.A

[Minimum Necessary Uses of PHI](#)

2-015

[Minimum Necessary Disclosures of PHI](#)

2-016

[Uses and Disclosures of PHI](#)

2-017

[Authorization for Use or Disclosure of PHI](#)

2-018

[Minimum Necessary Requests For PHI](#)

2-019

[Privacy of Health Information of Deceased Individuals](#)

2-020

[Patient Requests for Privacy Restrictions](#)

2-021

[Patient Requests for Confidential Communications](#)

2-022

[Patient Requests for Access to PHI](#)

2-023

[Patient Requests to Amend PHI](#)

2-024

[Patient Requests for Accounting of PHI Disclosures](#)

2-025

SECTION TWO

Program/Service Operations

Policy No.

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2-026

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2-027

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2-028

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2-030

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2-030.A

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2-030.B

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2-031

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2-033

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2-034

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2-034.A

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2-034.B

Do Not Resuscitate/Do Not Intubate Orders

2-035

Cardiopulmonary Resuscitation

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Assessment of Possible Abuse/Neglect

2-037

Addendum: Organization List of Private and Public Community Agencies
That Provide or Arrange for Assessment of Suspected or

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SECTION TWO

Program/Service Operations

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2-044

Addendum: [Social Media and Blog Guidelines](#)

2-044.A

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2-061

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2-062

[Whistleblower Protection](#)

2-063

AVAILABILITY OF SERVICES

Policy No. 2-001.1

PURPOSE

To define the availability of services to the community.

POLICY

Care and services will be available to the Organization's Name patients twenty-four (24) hours per day, seven (7) days per week.

Personnel will be available to accept referrals to home care services twenty-four (24) hours a day, seven (7) days per week.

The start of care assessment visit must be performed either within forty-eight (48) hours of the referral, within forty-eight (48) hours of the patient's return home, or the start of care date ordered by the physician (or allowed practitioner).

Additional discipline initial assessments will be provided within five (5) business days of the start of care or date of the physician's (or allowed practitioner's) order.

Routine visits will be performed on weekends or after regular business hours when dictated by the type of care required (e.g., daily wound care, every twelve (12) hour medication administration), when specifically ordered by the physician (or allowed practitioner), or upon the patient's preference.

PATIENT BILL OF RIGHTS**Policy No. 2-002.1****PURPOSE**

To encourage awareness of patient rights and provide guidelines to assist patients in making decisions regarding care and for active participation in care planning.

POLICY

Each patient will be an active, informed participant in his/her plan of care. To ensure this process, the patient will be empowered with certain rights and responsibilities as described. If a state court has not adjudged a patient to lack legal capacity to make health care decisions as defined by state law, the patient may designate someone to act as his/her representative to exercise the patient's rights. This representative, on behalf of the patient, may exercise any of the rights provided by the policies and procedures established by the organization.

If the patient has been adjudged to lack legal capacity to make health care decisions as established by state law by a court of proper jurisdiction:

1. The rights of the patient may be exercised by the person appointed by the state court to act on the patient's behalf OR
2. The patient may exercise his or her rights to the extent allowed by court order

To assist with fully understanding patient rights, all policies will be available to the organization personnel, patients, and his/her representatives as well as other organizations and the interested public.

PROCEDURE

1. The patient will be informed verbally and in writing during the initial evaluation visit, in advance of furnishing care of their rights.
2. The Patient Bill of Rights statement defines the right of the patient to:
 - A. Exercise and understand his or her rights and responsibilities as a patient and not to be subject to discrimination or reprisal for exercising these rights.
 - B. Have his or her property treated with respect, consideration, and recognition of patient dignity and individuality.
 - C. Voice grievances regarding treatment or care that is (or fails to be) furnished, or regarding the lack of respect for property by anyone who is furnishing services on behalf of the organization and must not be subjected to discrimination or reprisal for doing so.

- D. Receive an investigation by the organization of complaints made by the patient or the patients family or guardian regarding treatment or care that is (or fails to be) furnished, or regarding lack of respect for the patient's property by anyone furnishing services on behalf of the organization, and must document both the existence of the complaint and the resolution of the complaint.
- E. Participate in, be informed about and consent or refuse care in advance of and during treatment, where appropriate, with respect to:
 - 1. Completion of all assessments;
 - 2. The care to be furnished based on the comprehensive assessment;
 - 3. Establishing and revising the plan of care
 - 4. The disciplines that will furnish the care;
 - 5. The frequency of visits;
 - 6. Expected outcomes of care, including patient-identified goals, and anticipated risks and benefits;
 - 7. Any factors that could impact treatment effectiveness; and
 - 8. Any changes in the care to be furnished.
- F. The organization shall ensure that the patient receives all services outlined in the plan of care.
- G. The right to be free from mental, physical, sexual and/or verbal abuse, including injuries of unknown source, neglect, misappropriation of property, or exploitation.
- H. Receive appropriate care without discrimination in accordance with physician orders.
- I. Receive Advance Directives information, orally and in writing, prior to or at the time of the first home visit, as long as the information is furnished before care is provided and be advised that the Home Health Agency complies with Subpart 1 of 42 CFR 489 and receive written policies and procedures regarding Advance Directives, including a description of an individual's right under applicable state law and how rights are implemented by the organization.
- J. Confidentiality of the clinical records maintained by the organization and the policies and procedures for disclosure. (See "[Patient Privacy Rights](#)" Policy No. 2-014.)
- K. Be informed, verbally and in writing and before care is initiated of the extent to which:
 - 1. Payment may be expected from Medicare, Medicaid, or any other federally funded or aided program known to the organization

2. Charges for services that will not be covered by Medicare, Medicaid, or other Federally-funded or Federal aid program known to the organization
 3. Charges that the individual may have to pay
 4. Notification of any changes in payment information as soon as possible, in advance of the next home visit, that the organization becomes aware of the change.
 5. Receive proper written notice, in advance of a specific service being furnished, if the organization believes that the service may be non-covered care; or in advance of the organization reducing or terminating on-going care.
- L. To have communication needs met (See Policy No. 2-040 "[Facilitating Communication](#)")
1. The organization shall provide verbal and written notice of the patient's rights and responsibilities in the patient's primary or preferred language and in a manner the individual understands, free of charge, with the use of a competent interpreter if necessary, no later than the completion of the second (2nd) visit from a skilled professional (RN, PT, SLP, OT).
- M. Receive in writing, prior to the start of care, the telephone numbers for the State Home Health Hotline and the ACHC Hotline, including hours of operation, and the purpose of the hotlines to receive complaints or questions about the organization. (Patient will be given ACHC address as well.)
- N. Be informed of organizational ownership and control.
- O. Patient privacy rights related to the collection of the Outcome and Assessment Information Set (OASIS):
1. The right to be informed that OASIS information will be collected and the purpose of the collection
 2. The right to have the information kept confidential
 3. The right to be informed that OASIS information will not be disclosed except for legitimate purposes allowed by the Federal Privacy Act
 4. The right to refuse to answer questions
 5. The right to see, review and request changes on their assessment
- P. Choosing a health care provider, including an attending physician or allowed practitioner and identifying visiting personnel with proper identification.
- Q. The organization's transfer and discharge policies.
- R. The contact information for the agency administrator, including the administrator's name, business address, and business phone number in order to receive complaints.

- S. The names, addresses, and telephone numbers of the following Federally-funded and state-funded entities that serve the area where the patient resides:
1. Agency on Aging
 2. Center for Independent Living
 3. Protection and Advocacy Agency
 4. Aging and Disability Resource Center
 5. Quality Improvement Organizations
3. When additional state or federal regulations exist regarding Patient Rights, the Patient Rights statement must include those components.
 4. The admitting clinician will provide each patient or his/her representative with a written copy of the Patient Bill of Rights on admission.
 5. The Patient Bill of Rights statement will be explained (verbally/orally) and distributed to the patient prior to the initiation of organization services. This explanation will be in a language, communication method or manner he/she can reasonably be expected to understand and free of charge.
 6. The patient will be requested to sign the Patient Bill of Rights form. The original form will be kept in the patient's clinical record. A copy will be maintained by the patient. The patient's refusal to sign will be documented in the clinical record, including the reason for refusal.
 7. The admitting clinician will document that the patient has received a copy of the Patient Bill of Rights.
 - A. If the patient is unable to understand his/her rights and responsibilities, documentation in the clinical note will be made.
 - B. In the event a communication barrier exists, if possible, special devices or interpreters will be made available.
 - C. Written information will be provided to patients in English and the predominant non-English languages of the population served.
 8. When the patient's representative signs the Patient Bill of Rights form, an explanation of that relationship must be documented and kept on file in the clinical record.
 9. Within four (4) business days of the initial evaluation visit, the organization shall provide written notice of the transfer and discharge policies, provide contact information of the administrator, provide written notice of the rights and responsibilities, and obtain signature from the patient or legal representative to confirm that they have received a copy of the notice of rights and responsibilities.

10. The family or guardian may exercise the patient's rights when a patient is incompetent or a minor.
11. Supervisory visits with clinical disciplines will be conducted to ensure these rights are honored and protected according to organization policy.
12. All organization personnel, both clinical and non-clinical, will be oriented to the patient's rights and responsibilities prior to the end of their orientation program, as well as annually.

ADMISSION CRITERIA AND PROCESS**Policy No. 2-003.1****PURPOSE**

To establish standards and a process by which a patient can be evaluated and accepted for admission.

POLICY

A patient will be accepted for care without discrimination on the basis of race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, or place of national origin.

A patient will be accepted for care based on consideration. Consideration will be given to the adequacy and suitability of organization personnel, resources to provide the required services, and the reasonable expectation that the patient's medical, nursing, rehabilitative, and social needs can be adequately met in the patient's place of residence.

While a patient will be accepted for services based on his/her medical needs, the patient's ability to pay for such services, either through state or federal assistance programs, private insurance, or personal assets are factors that will be considered.

The organization reserves the right not to accept a patient who does not meet the admission criteria.

A patient will be referred to other resources if the organization cannot meet his/her needs.

Once a patient is admitted to service, the organization is responsible for providing care and services within its financial and service capabilities, mission, and applicable law and regulations.

Admission Criteria

1. The patient must be under the care of a physician. The patient's physician (or allowed practitioner) must order and approve the provision of any service. A skilled service must be ordered.
2. The patient must desire home care services.
3. Organization's Name will consider for acceptance any patient who is appropriate for home care, regardless of payment source.
4. The patient must reside within the geographical area which the Organization's Name services.

5. The physical facilities and equipment in the patient's home must be adequate for safe and effective care.
6. Services may be provided to a patient insured by Medicare who has a primary need for skilled nursing, physical and/or speech therapy on an intermittent basis and is homebound. (A patient is considered to be homebound if he/she has a condition that restricts his/her ability to leave his/her place of residence except with the aid of supportive devices, the use of special transportation, the assistance of another person, or if he/she has a condition which is such that leaving his/her home is medically contraindicated.)
7. Acceptance for home care services will be realistically based on the patient's willingness and ability to function in a noninstitutional environment, and the willingness, ability, and availability of family/caregiver or significant individuals to participate in the care.
8. Eligibility will not be based on the patient's race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, or place of national origin.

PROCEDURE

1. The organization will utilize referral information provided by the family/caregiver, health care clinicians from acute care facilities, skilled or intermediate nursing facilities, other agencies, and physician offices in the determination of eligibility for admission to the program.
2. If the patient's physician (or allowed practitioner) does not make the request for service, he/she will be contacted for start of care orders prior to the evaluation visit and initiation of services.
 - A. If the patient resides in an assisted living facility, it will be determined the type of state license the facility holds, if any, and the required services the facility is obligated to provide.
 - B. A copy of the patient's services agreement with the facility will be viewed to ensure that home health services ordered and provided are not duplicative of those services or required to be provided by the facility.
3. The Clinical Supervisor will assign clinical organization personnel to conduct initial assessments of eligibility for services within forty-eight (48) hours of acceptance of referral information and/or discharge from referring facility.
 - A. The initial assessment visit must be performed either within forty-eight (48) hours of the referral, within 48 hours of the patient's return home, or on the start of care date ordered by the physician (or allowed practitioner).
 - B. The patient's most critical needs for home care services must be identified during the initial assessment and must be met in a timely fashion.

4. The initial and comprehensive assessment visit may be made by an appropriate rehabilitation skilled professional if rehabilitation therapy services (i.e., PT, OT, or SLP) is the only service ordered by the physician (or allowed practitioner) responsible for the home health plan of care and for Medicare patients, determine eligibility for the Medicare home health benefit, including homebound status.
 - A. An Occupational Therapist may complete the initial and/or comprehensive assessment for Medicare patients when occupational therapy is ordered with another qualifying rehabilitation therapy service that establishes program eligibility.
5. Prospective patient's will be notified that acceptance to service is based on the availability of organization's capacity to provide patient care, including the following factors:
 - A. Patient's geographical location
 - B. Complexity of the referred prospective patient's medical needs and level of care required
 - C. Organization personnel's education and experience
 - D. Organization personnel's skills, special training and their competence to meet patient's needs
 - E. Organization's case load and case mix
 - F. Organization's current staffing levels
 - G. Urgency of identified need for assessment
6. In the event that the time frame for assessment cannot be met, the patient's physician (or allowed practitioner), the referral source, and the patient, will be notified for approval of the delay.
 - A. Such notification and approval will be documented.
 - B. If approval is not obtained for the delay, the patient will be referred to another organization for services.
7. A nurse or therapist will attempt to make an initial contact prior to the patient's hospital discharge if possible or appropriate. The initial home visit will be made within forty-eight (48) hours after the patient's discharge from a facility or as ordered by the physician (or allowed practitioner).
8. During the initial assessment visit, the admitting clinician will assess the patient's eligibility for home care services according to the admission criteria to determine or confirm:
 - A. Anticipated needs of the patient and the level of services required
 - B. Eligibility (meets admission criteria)

- C. Qualifying face-to-face encounter date, if completed within ninety (90) days prior to admission. (See "[Face-to-Face Encounter Procedure](#)" Addendum 2-003.A.)
 - D. Source of payment
9. Upon acceptance into service, the patient will be provided with an organization brochure and various educational materials providing the patient and family/caregiver with sufficient information on:
- A. Nature and goals of care and service
 - B. Hours during which care and service is available
 - C. Access to care after hours
 - D. Care costs/charges to the patient, if any, for care, treatment or service
 - E. Organization mission, objectives, and the scope of care provided both directly and through contractual agreement
 - F. Safety information
 - G. Infection control information
 - H. Emergency preparedness plans
 - I. Available community resources
 - J. Complaint/grievance process
 - K. Written information regarding the availability and indications for use of the state and ACHC Home Health Hot Line telephone numbers, the hours of operation and that the purpose of the hotline is to receive complaints or questions about local home health agencies
 - L. Advance Directives
 - M. Other organization personnel involved in care
 - N. Mechanism for notifying the patient and family/caregiver of changes in care and any related liability for payment as a result of those changes
 - O. Notice of privacy practices
10. Patient rights and responsibilities will be explained and a written copy will be provided to the patient and family/caregiver. If a face-to-face encounter has not been completed prior to admission, the clinician will explain the requirement that a face-to-face encounter visit with their physician or (or allowed practitioner) must be completed within thirty (30) days of admission.

11. The admitting clinician will document that the above information has been furnished to the patient and family/caregiver, and they will also document any information not understood by the patient and family/caregiver.
12. The patient and family/caregiver, after review, will be given the opportunity to either accept or refuse services.
13. The patient or his/her representative will sign the required forms indicating acceptance of services and receipt of patient rights and privacy information.
14. Refusal of services will be documented in the clinical record. Notification of the Clinical Supervisor, physician (or allowed practitioner), and referral source will follow with appropriate documentation in the clinical record.
15. The admitting clinician will consult with the Clinical Supervisor concerning the patient's condition following the initial visit. Based on the clinical personnel's assessment of the patient's eligibility for admission, the patient will be admitted for services or referred to alternate sources for care.
16. If the patient is accepted for home health care, an initial plan of care will be developed in consultation with the physician (or allowed practitioner) and the patient and then submitted to the physician for signature.
17. The initial written assessment will be completed within twenty-four (24) hours of the initial assessment/admission visit.
 - A. All documentation needed to develop the plan of care will be completed and turned into the office no later than the next business day.
18. A comprehensive assessment must be completed within five (5) calendar days of the patient's start of care. (See "[Initial and Comprehensive Assessment](#)" Policy No. 4-018.)
 - A. Each patient must receive a patient-specific comprehensive assessment that identifies the need for home care and that meets the patient's medical, nursing, rehabilitative, social, and discharge planning needs.
 - B. Outcomes and Assessment Information Set (OASIS) data must be collected on all patients receiving skilled services except antepartum and postpartum patients, patients under the age of eighteen (18), and patients with payer source other than Medicare or Medicaid.
 1. OASIS data collection is not required for patients who are receiving only personal care or support services (receiving only homemaker services).
 2. The OASIS data will be collected during the comprehensive assessment.
 3. The assessment tool must include the exact use of the current versions of the OASIS data set.
19. The time frames apply for weekend, holiday, and weekday admissions.

20. A clinical record will be initiated for each patient admitted for home health services.
21. If a patient does not meet the admission criteria or cannot be cared for by the organization, the Clinical Supervisor should be notified and appropriate referrals to other sources of care made on behalf of the patient.
22. The following individuals will be notified of non-admits:
 - A. Patient
 - B. Physician (or allowed practitioner)
 - C. Referral source (if not MD)
23. A record of non-admits will be kept for statistical purposes, referencing the date of referral, date of assessment, patient name, services required, physician, reason for non-admit, referral to other health care facilities, etc.
24. In the instance where a patient does not meet the stated criteria for admission to the program, the Executive Director/Administrator in consultation with the Medical Director may decide upon exceptions, with the request of the referring party and/or the patient.
25. In the instance where continued care to a patient contradicts the recommendations of an external or internal entity performing a utilization review, the Executive Director/Administrator will be notified.
26. All care, service, and discharge decisions must be made in response to the care required by the individual, regardless of the external or internal organization's recommendation.
27. The patient, caregiver as appropriate, and physician will be involved in deliberations about the denial of care or conflict of care decisions.
28. A record of conflict of care issues and outcomes will be kept for statistical purposes, referencing the date of the conflict of care issue, the patient name, the external or internal organization recommendations and reasons, and complete documentation of organization decision and patient care needs.

ADDENDUM 2-003.A

FACE-TO-FACE ENCOUNTER PROCEDURE

(Insert agency-specific procedure)

ADDENDUM 2-003.B

SAMPLE ACCEPTANCE TO SERVICE FORM

ACCEPTANCE TO SERVICE

Patient Name: _____ Date: _____

Patient Phone Number: _____ Medical Record #: _____

Patient Address: _____

Name of Patient's Primary Caregiver: _____

Relationship to Patient: _____

Phone Number if different than patient #: _____

Name of Emergency Contact: _____

Relationship to Patient: _____

Phone Number: _____

Name of Case Manager: _____

Name of Physician: _____ Phone Number: _____

Home Health Services Offered by Organization's Name:

_____ Nursing Services

_____ Home Health Aide Services

_____ Physical Therapy Services

_____ Nurse Assistant Services

_____ Occupational Therapy Services

_____ Homemaker/Companion Services

_____ Speech Therapy Services

_____ Nutritionist/Registered Dietitian Services

_____ Medical Social Worker Services

_____ Transportation Services

Services Requested by Patient:

_____ Nursing Services

_____ Home Health Aide Services

_____ Physical Therapy Services

_____ Nurse Assistant Services

_____ Occupational Therapy Services

_____ Homemaker/Companion Services

_____ Speech Therapy Services

_____ Nutritionist/Registered Dietitian Services

_____ Medical Social Worker Services

_____ Transportation Services

Frequency and Duration of Requested Services:

Home Health Service Limitations:

_____ Unable to provide requested services: _____

_____ Unable to provide requested services due to service duration.

_____ Unable to provide requested services due to service frequency.

_____ Unable to provide requested services due to staffing levels

_____ Unable to provide requested services due to current patient volume and patient complexity.

_____ Unable to provide requested services due to skilled staff availability.

Patient Referral Needed: Yes _____ No _____

Patient Referred to: _____

Referral Address: _____

Referral Phone Number: _____

Patient Accepted for Services to Organization's Name: Yes _____ No _____

Personnel Signature: _____

Client Signature: _____

Date: _____

PURPOSE

To ensure organizational compliance with the Patient Bill of Rights and regulatory requirements.

POLICY

Organization's Name will provide written admission documents prior to or at initiation of home care services including:

1. Information regarding billing policies and payment procedures at the time of admission and any subsequent changes within 30 days
2. Information addressing organizational ownership and control
3. A copy of the organization's policy on patient Advance Directives including a description of an individual's right under state law (whether statutory or as recognized by the courts of a state) and how such rights are implemented by the organization
4. Information describing the organization's grievance procedure which includes the names of contacts, phone numbers, hours of operation, and mechanism(s) for communicating problems
5. Information addressing any beneficial relationships between the organization and referring entities.
6. Information addressing the organization's policies and procedures for accessing and or disclosure of clinical records
7. Information, when requested, regarding the organization's liability insurance
8. Information addressing the availability, purpose, and appropriate use of State, Medicare, and ACHC hotline numbers
9. Information addressing emergency management

PROCEDURE

1. Appropriate documents will be included in an admission folder bearing the Organization's Name.
2. The admission information will be reviewed with the patient and/or family at the start of care visit.
3. The admission information will be left in the patient's home.

INFORMED CONSENT/REFUSAL OF TREATMENT**Policy No. 2-005.1****PURPOSE**

To obtain written consent for care during the admission process and to communicate the organization's process for informing patients and family/caregivers regarding services as well as involving patients in the care/service planning process.

POLICY

Upon admission and throughout the course of care/service, the patient and family/caregiver will be:

1. Given information, in an understandable language, to make informed decisions regarding the care/service being provided.
2. Encouraged to participate in the care/service planning process, including planning for transfer, referral, and discharge.
3. Allowed to refuse all or part of his/her care/service to the extent permitted by law; the expected consequences of such actions must be explained.
 - A. Specific consent for recording, photographing, or filming care, treatment, or services will be obtained as needed. (See "[Sample Informed Consent for Medical Photography](#)" Addendum 2-005.A.)
 - B. Form NTC 12-01 informs patients of their right to confirm the license status of their physical therapist and provides information regarding the licensing board. Patients or patient representatives are required to sign and date an acknowledgement confirming receipt of the form. Completed acknowledgements must be retained in the patient's medical record.
For the Form NTC 12-01, go to www.ptbc.ca.gov/laws/prop_regs/ntc_12_01.pdf
4. During the admission visit, the patient or authorized representative will sign the organization's written consent for care/service.

PROCEDURE

1. During the admission visit and follow-up visits, the patient and family/caregiver will be given information (verbally and/or in writing) that describes:
 - A. The services and/or disciplines anticipated to be involved in the care/service of the patient
 - B. The nature and purpose of any technical procedure, including written information when available

- C. The potential benefits and effects of the procedure, including who will perform the procedure
- 2. When appropriate, the family/caregiver will be utilized in the care, treatment, and service of the patient. This may include:
 - A. Assisting with ordered treatments—with physician (or allowed practitioner) approval
 - B. Carrying out activities specified in the plan of care/service
 - C. Encouraging the patient with designated activities
 - D. Performing activities when organization personnel are not present
- 3. Every attempt will be made to include, to the extent possible, available family/caregiver(s) in rendering care and meeting patient-specific goals of care.
- 4. The patient may refuse all or part of his/her care/service except a face-to-face encounter visit at appropriate timeframes.
- 5. The patient will be informed of the expected consequences whenever any treatment/care/service is refused. Documentation of such refusal and physician notification will be made part of the clinical record and includes:
 - A. Date and time of visit or phone contact
 - B. Specific care, treatment, or service being refused
 - C. Description of what was explained to the patient regarding consequences of decision
 - D. Date and time of physician contact
 - E. Action to be taken per physician (or allowed practitioner) orders
 - F. Patient response after any explanations
- 6. If the patient is determined to be a danger to himself/herself or others (i.e., suicidal) or if the patient refused treatment and does not verbalize clearly that he/she understands the consequences of such refusal, the clinician should:
 - A. Notify his/her Program Director immediately
 - B. Notify the attending physician immediately
- 7. If the physician is not available or does not take action, the Program Director will notify the organization Executive Director/Administrator and/or Medical Director, as well as the Ethics Committee.

ADDENDUM 2-005.A

SAMPLE CONSENT FOR MEDICAL PHOTOGRAPHY

(Insert agency policy on the following: Timing of photographs, validating competence, type of equipment used, guidelines on utilization of cell phones as imaging device, type of patient identification allowed, information on maintaining photographs and releasing of copies to patient upon request)

SAMPLE CONSENT FOR MEDICAL PHOTOGRAPHY

Purpose: For medical records

I understand that photographs, videotapes, digital, or other images may be recorded to document my care, and I consent to this. I understand that Organization's Name will retain the ownership rights to these photographs, videotapes, digital, or other images, but that I will be allowed access to view them or obtain copies. I understand that these images will be stored in a secure manner that will protect my privacy and that they will be kept for the time period required by law or outlined in Organization's Name policy. Images that identify me will be released and/or used outside the organization only upon written authorization from me or my legal representative. I have been provided the opportunity to ask questions concerning medical photography and understand that refusal to consent will not affect my medical care. If the patient is under 18 years of age, I verify that I am the parent or guardian of patient _____, and that I will sign for the patient.

_____ I consent to allow medical photographs for **only the** purpose listed above.

_____ I do not provide consent to allow recording or saving of medical photographs.

Date _____ Time _____

Place: _____ Street Address
_____ City, State, Zip code

Signature of patient: _____ (Guardian/relative)

Witness: _____

Registered Nurse: _____

PHYSICIAN LICENSURE VERIFICATION**Policy No. 2-006.1****PURPOSE**

To ensure that all physicians or allowed practitioners are properly licensed to practice medicine in the organization's service areas.

POLICY

Orders will only be received from physicians (or allowed practitioner) licensed to provide care in the state(s) served by the organization.

PROCEDURE

1. The organization will verify that all referring physicians or allowed practitioners are licensed to practice in the state(s).
2. The physician (or allowed practitioner) license must be verified when the individual provides orders to the organization for the first time and annually thereafter.
3. Physician or allowed practitioner licensure will be verified by:
 - A. Ascertaining that the individual is a practitioner "in good standing" with the appropriate State Board of Licensure
 - B. Contacting the local Medical Society or State Board of Licensure
 - C. Accessing Medical Society or State Board of Licensure information on the Internet
 - D. Receiving updated notice from the Board of Licensure listing probation, suspended, or revoked licenses
4. The organization will maintain documentation that verification was done.
5. Verification will be reviewed at least annually or upon expiration of license.

FINANCIAL RESPONSIBILITY**Policy No. 2-007.1****PURPOSE**

To outline the process by which patients, families and caregivers will understand their financial responsibility for services.

POLICY

Upon admission, the admitting clinician/technician will inform orally and in writing the patient and/or his/her representative of his/her payment responsibilities for services. The patient will be informed of any subsequent changes in his/her financial responsibility.

PROCEDURE

1. Insurance coverage and patient's responsibility for copayment will be advised orally and presented in writing to the patient and family/caregiver. The approximate costs for care/service, if any, will be presented in writing to the patient and family/caregiver. If copay responsibilities are not known, the clinician/technician will provide the patient and family/caregiver with total organization charges until more accurate information can be obtained.
2. If more information is needed for verification of coverage, the clinician/technician will discuss this with the Program Supervisor and may alert social services (as available) if the patient's financial situation is unclear. The clinician/technician will notify the billing department if a tailored payment plan is required.
3. Patients who incur financial liability must be notified orally and in writing within 30 calendar days from the date the organization is notified of any changes from payers.
4. Medicare patients must be provided with timely, accurate and comprehensible written notices in any case where a reduction or termination of services is to occur, or where services are to be denied before being initiated. See "[Financial Responsibility and Medicare Written Notices](#)" (Policy No. 1-028) for specific CMS guidelines.
5. All written and verbal notifications of the patient's financial responsibility will be documented in the clinical/service and billing records.

COMPLAINT/GRIEVANCE PROCESS**Policy No. 2-008.1****PURPOSE**

To set forth guidelines for the resolution of patient concerns, dissatisfaction, or complaints and to protect patient and family rights.

POLICY

Any difference of opinion, dispute, or controversy between a patient or family/caregiver or patient representative and Organization's Name concerning any aspect of services or the application of policies or procedures will be considered a grievance.

The Executive Director/Administrator will be informed of situations that may become detrimental to good patient relations, and will be committed to maintaining a consistently high level of patient relations. This grievance procedure will be included in the Patient Bill of Rights document given to each patient upon admission.

The organization will investigate complaints regarding treatment or care, mistreatment, neglect, or verbal, mental, sexual, and physical abuse, including injuries of unknown source, and/or misappropriation of patient property by anyone furnishing services on behalf of the organization. The organization shall be responsible for asking the necessary questions to determine cause of injuries.

PROCEDURE

1. The organization staff member receiving the complaint will discuss, verbally and in writing, the grievance with a supervisor within five (5) days of the alleged grievance. The supervisor will investigate the grievance within five (5) days after receipt of such grievance and will make every effort to resolve the grievance to the patient's satisfaction. Response to the patient regarding the complaint will occur within ten (10) days of receipt.
2. If the grievance cannot be resolved to the patient's satisfaction, the patient or his/her representative is to notify, verbally or in writing, the Executive Director/Administrator. The grievance must state the problem or action alleged and the date the supervisor was notified. The Executive Director/Administrator or designee will then investigate the grievance and contact the patient or his/her representative regarding the grievance in an attempt to resolve the differences. The Executive Director/Administrator will respond to the patient within ten (10) days of notification of failure to resolve the complaint.
3. If the patient feels his/her grievance has not been resolved after working with Organization's Name personnel, he/she will be informed of his/her right to notify the state or ACHC via the respective toll-free telephone numbers.
4. Complaints and any action taken will be documented on a complaint form.
5. Corrective action will be specific and related to the complaint.

6. Resolution information will be communicated in writing to the patient or his/her representative filing the complaint.
7. Risk management personnel will be notified of any complaints which may involve litigation by the clinician/technician involved completing an organization incident report or unusual occurrence form and forwarding a copy to the Risk Management Department.
8. Complaints received on patient satisfaction surveys (mail) will be documented on a complaint form and addressed as outlined above.
9. All complaints from patients who believe their privacy rights have been violated will be forwarded for review to the designated organization personnel or office specified in the organization's Notice of Privacy Practices.
10. All complaints will be logged, tracked, trended, and filed in the performance improvement office.
11. The Performance Improvement Coordinator will prepare a quarterly report summarizing all complaints received that quarter.
12. Reports may include:
 - A. Number of complaints received
 - B. Type of complaints received
 - C. Action and resolution of complaints
13. The Performance Improvement Committee will review patient grievance trends on a quarterly basis. Identified trends will be followed through the established performance improvement process.
14. All organization personnel (clinical and non-clinical) will be informed of this process during a formal orientation process

FOSTERING INTERNAL COMMUNICATION**Policy No. 2-009.1****PURPOSE**

To outline the mechanism for fostering communication between and among individuals and components of the organization.

POLICY

Senior management, together with the Governing Body leadership, individually and jointly will develop and participate in systematic and effective mechanisms to:

1. Foster communication between and among individuals within the organization
2. Coordinate internal activities
3. Communicate with other related organizations

PROCEDURE

1. The organization will participate in various corporate activities including but not be limited to:
 - A. Senior management meetings
 - B. Human resource functions
 - C. Continuing education programs
 - D. Performance improvement teams and activities
2. The organization, when applicable, participates with personnel from the parent organization in coordinating and integrating care and services provided, including discharge planning, information management, human resources, risk management, legal, etc.
3. The Executive Director/Administrator will participate with senior management of the parent company in activities such as strategic planning, managed care contracting, etc.
4. The organization receives periodic updates from the parent organization, through interoffice communication, flyers, newsletters, formal letters, etc.

**INTERFACE OF PATIENT DATA
AND MANAGEMENT SYSTEMS**

Policy No. 2-010.1

PURPOSE

To define processes to link and combine clinical/service information with other internal systems.

POLICY

Organization's Name will have processes (electronic or manual) to combine data and information from various sources. This information can be combined to facilitate comparison and decision-making. The information from various systems will be organized, analyzed, interpreted and accessed on an ongoing basis by organization personnel for use in appropriate organization committees, reports or other forums.

Information systems include clinical/service information systems, operational systems, financial information systems, instructional systems, and communication systems.

CLINICAL/SERVICE DATA COLLECTION**Policy No. 2-011.1****PURPOSE**

To define who has authority to make entries into the clinical/service record and to provide guidelines for valid data collection.

POLICY

Home care personnel providing patient care and supervisory functions have authority to make entries into the clinical/service record. Documentation in the clinical/service record will be timely, detailed, accurate, and reflect the care or services provided. The record format will be reviewed and updated as necessary.

Note: *Times* means actual time that an event occurred, which is not necessarily the time when the documentation was entered into the record. The date and time requirement applies to all entries in the record and the medical record should accurately reflect a clear account of the patient's entire course of care. The clinical record should tell a linear story of the course of the patient's care that is managed and delivered by the organization

PROCEDURE

1. A clinical/service record will be initiated and maintained for each patient receiving care or services according to organization policies found in this manual and will include at a minimum:
 - A. Patient consent, authorization, and elections forms as applicable
 - B. Pertinent medical history
 - C. Physician (or allowed practitioner) orders and evidence of physician oversight activities
 - D. Changes to the plan of care/service
 - E. Dates, times, and types of interventions, assessments, and coordination of care/service
 - F. Patient response to care and services in analyzed and documented as measurable goals
 - G. Patient and family education
 - H. Reasons for interruptions in the provision of care and services
 - I. Current patient status and progress toward goals

J. Outcomes of care/service

2. Entries to the clinical/service record will be made only by organization personnel and/or by contract personnel who have a written agreement with the organization.
3. Entries into the clinical/service record will be clear, concise, and specific statements of fact.
4. Entries into the clinical/service record will be legible.
5. Entries will be made using black ink only.
6. Entries into the clinical/service record will be made on the day care/service is provided to the patient. All documentation will be turned into the office at the end of the workday.
7. The clinical/service note will include:
 - A. What care/service was provided
 - B. Treatment and/or invasive procedures performed
 - C. Patient response to treatment and/or procedures
 - D. The date the service was provided (month, day, year)
 - E. Signature of clinician and his/her credentials
8. Signature authorization of clinical/service documentation will include the staff member's name and credentials (as specified on professional licenses and/or certification documents).
9. Initials will be used on forms where authentication signature space is designated on the form.
10. All entries will reflect the date care was provided, including the month, day, and year.
11. Home health aides (HHA) will only make entries on the aide visit form and the case conference form.
12. Late entries and amendments will be documented stating "late entry or amendment for visit of (date and time of visit)" and include:
 - A. The date the entry is made (month, day, year)
 - B. The date and documentation that was originally omitted or amended
 - C. The signature and title of the staff member making the late entry or amendment

13. Errors in documentation will be corrected as follows:
 - A. Draw a line through the entry, date and initial.
 - B. Do not erase, use correction fluid, or deface a document.
14. Amendments, correction or late entries within an electronic health record must:
 - A. Distinctly identify any amendment, correction or late entry
 - B. Provide a reliable means to clearly identify the original content, the modified content, and the date and authorship of each record modification
15. The information management system will be designed to assure that:
 - A. Data is collected and entered into the record in a systematic manner.
 - B. Organization personnel use consistent definition of data as specified in organization policies.
 - C. Data is relevant, as determined through ongoing, as well as quarterly, clinical record reviews.
 - D. Data is complete, as determined through ongoing, as well as quarterly, clinical record reviews.
16. Personnel authorized to use a computer to authenticate documentation will sign a computer confidentiality statement, verifying that no one else will be allowed to use their computer password. Each staff member will be responsible for the security of his/her computer password. (See "[Computer Access to Information](#)" Policy No. 2-032.)
17. The organization, at least annually, will verify the accuracy of coded data, through the review of records by a Registered Record Administrator or an Associate Record Technician.

PURPOSE

To define personnel authorized to access information.

POLICY

Only authorized personnel will have access to the clinical/service, financial, administrative and personnel records. Any release of information contained in the clinical/service and financial records other than that required by law must be authorized in writing by the patient.

The organization will maintain backup patient, financial, administrative and personnel files at a safe location off-site for use in the event a disaster prevents access to files in the office.

PROCEDURE

1. Access to patient information files (clinical/service record and billing) will be limited to organization personnel involved in the care of the patient and may include:
 - A. Executive Director/Administrator
 - B. Program Director(s)
 - C. Clinical Supervisor
 - D. Personnel (direct and through contract) including RN, LPN/LVN, HHA, PT, OT, SLP, MSW, registered dietician, pharmacist, PTA, and COTA
 - E. Office personnel having a need to know in order to perform their functions and processes including intake personnel, schedulers, billers and other support personnel
2. Personnel will only have access to information on those patients to which they are actively providing care and service, such as shift nurses, on-call, relief, etc.
3. Office personnel will only have access to information on those patients to which they are actively providing service, such as working on bills, following up on complaints, etc.
4. Access to records for individuals from outside the organization, such as surveyors, reviewers and/or consultants, will be approved by the Executive Director/Administrator or Program Director on a per occurrence basis providing they have appropriate identification.
5. Any organization committee may review the clinical/service record at meetings as requested by the Executive Director/Administrator or Program Director.

6. Review of the clinical/service record for the purposes of performance improvement activities and clinical/service record review will be performed according to organization policy.
7. Access to financial, administrative and personnel records information files will be limited to organization personnel involved in the appropriate departments and may include:
 - A. Executive Director/Administrator
 - B. Program Directors (e.g. CFO, COO, CIO, Clinical Director, etc.)
 - C. Clinical supervisors/managers
 - D. Department supervisors/managers
 - E. Office personnel having a need to know in order to perform their functions and processes including finance staff and billers, administrative staff, human resources staff and other support personnel

(See "[Minimum Necessary Uses of PHI](#)" Policy No. 2-015 and "[Minimum Necessary Disclosures of PHI](#)" Policy No. 2-016.)

PRINCIPLES OF INFORMATION MANAGEMENT**Policy No. 2-013.1****PURPOSE**

To develop guidelines for the collection, analysis, and use of data/information to assure that the necessary expertise and tools are available for the analysis and transformation of data into information.

POLICY

The information management system for the collection and management of administrative and clinical/service information will utilize established standards and data elements for the collection and processing of required information.

Data collection will be structured, routine, and timely to meet the information needs of the organization.

PROCEDURE

1. On an annual basis, the principles of information management will be reviewed with all organization personnel, as appropriate to their job responsibilities. The review will include:
 - A. Security and confidentiality of information
 - B. Measurement instruments, statistical tools and data analysis methods
 - C. Data collection methodologies for unbiased data collection
 - D. Interpretation of data
 - E. Use of information, indicators, and measures to assess and improve systems and processes over time
2. The education and training of management personnel on information management will be conducted by any of the following methods:
 - A. Group presentation by internal/external experts
 - B. Departmental meetings
 - C. Attendance at outside seminars
 - D. Newsletters
 - E. Periodicals

3. The education and training of organization personnel will be recorded according to organization policy.

PATIENT PRIVACY RIGHTS**Policy No. 2-014.1****PURPOSE**

To encourage awareness of patient privacy rights and Organization's Name legal duties with respect to these rights and the use and disclosure of protected health information (PHI).

POLICY

Organization's Name will respect and safeguard all protected health information of the patients it serves.

Each patient will be provided with information about his/her privacy rights at the time of admission to Organization's Name.

To assist with fully understanding patient privacy rights and responsibilities, all policies will be available to the organization personnel, patients, and their representatives as well as other organizations and the interested public.

DEFINITION

Protected health information (PHI) is any information about health status, provision of health care, or payment for health care that can be linked to a specific individual in ANY form (verbal, written, electronic). This is interpreted rather broadly and includes any part of a patient's medical record or payment history. Examples of identifiers are: names, all geographical identifiers smaller than a state (except for the initial three digits of a zip code), dates (other than year) directly related to an individual, phone numbers, fax numbers, email addresses, Social Security numbers, Medical record numbers, Health insurance beneficiary numbers, account numbers, certificate/license numbers, vehicle identifiers and serial numbers including license plate numbers, device identifiers and serial numbers, web Uniform Resource Locators (URLs), Internet Protocol (IP) address numbers, biometric identifiers, including finger, retinal and voice prints, full face photographic images and any comparable images, and any other unique identifying number, characteristic, or code except the unique code assigned by the investigator to code the data.

PROCEDURE

1. The patient will be provided with information about his/her privacy rights in the organization's Notice of Privacy Practices, which will be given to the patient during the admission visit. The patient's privacy rights include:
 - A. A right to adequate notice of the uses and disclosures of protected health information that may be made by Organization's Name. (See "[Notice of Privacy Practices](#)" Addendum 2-014.A.)

- B. A right to request privacy protection for protected health information. (See "[Patient Requests for Privacy Restrictions](#)" Policy No. 2-021 and "[Patient Requests for Confidential Communication](#)" Policy No. 2-022.)
 - C. A right of access to inspect and retain a copy of his/her protected health information. (See "[Patient Requests for Access to PHI](#)" Policy No. 2-023.)
 - D. A right to request that the organization amend protected health information or a record about the individual in a designated record set for as long as the protected health information is maintained in the designated record set. (See "[Patient Requests to Amend PHI](#)" Policy No. 2-024.)
 - E. A right to receive an accounting of disclosures of protected health information made by Organization's Name in the six (6) years prior to the date on which the accounting is requested. (See "[Patient Requests for Accounting of PHI Disclosures](#)" Policy No. 2-025.)
- 2. Organization's Name will make a good faith effort to obtain the patient's/representative's written acknowledgement of receipt of this notice. A separate signature/initials line for this acknowledgement may be located on the consent form. If an acknowledgement cannot be obtained, the admitting clinician will document his/her efforts to obtain the acknowledgement and the reason why it was not obtained in the clinical note.
 - 3. The notice will be promptly revised and distributed whenever there is a material change to the uses or disclosures, the individual's rights, organization's legal duties, or other privacy practices stated in the notice. A material change to any term of the notice will not be implemented prior to the effective date of the revised notice, unless required by law.
 - 4. Organization's Name will prominently post the notice and make the notice available through its website.
 - 5. The patient's legal representative may exercise the patient's rights when a patient is incompetent or a minor.
 - 6. When a patient has questions about his/her privacy rights, requests additional information, or would like to exercise one (1) of these rights, he/she will be referred to the appropriate individual or office designated by Organization's Name on the Notice of Privacy Practices.

ADDENDUM 2-014.A

NOTICE OF PRIVACY PRACTICES

GUIDELINES FOR DEVELOPMENT OF A NOTICE OF PRIVACY PRACTICES

A Notice of Privacy Practices should be written in plain language and contain the following elements:

1. The following statement placed as a header or otherwise prominently displayed: "THIS NOTICE DESCRIBES HOW MEDICAL INFORMATION ABOUT YOU MAY BE USED AND DISCLOSED AND HOW YOU CAN GET ACCESS TO THIS INFORMATION. PLEASE REVIEW IT CAREFULLY."
 2. Information regarding uses and disclosures:
 - A. A description, including at least one (1) example, of the types of uses and disclosures that the organization is permitted to make for each of the following purposes: treatment, payment and health care operations *
 - B. A description of each of the other purposes for which the organization is permitted or required to use or disclose protected health information without the patient's consent or authorization *

* **Note:** When use or disclosure for any purpose in 2A or 2B is prohibited or materially limited by any other applicable law, the notice will reflect the more stringent law.

The descriptions will include sufficient detail to put the patient on notice of the uses and disclosures that are permitted or required by applicable law.

 - C. A statement that other uses and disclosures will be made only with the patient's or legal representative's written authorization and that the patient may revoke the authorization at any time. (See "[Authorization for Use or Disclosure of PHI](#)" Policy No. 2-018.)
3. Separate statements of uses and disclosures related to the following activities:
 - A. That the organization may contact the patient to raise funds for the organization
 - B. That the organization may contact the patient to provide appointment reminders or information about treatment alternatives or other related benefits and services that may be of interest to the patient

GUIDELINES FOR DEVELOPMENT OF A NOTICE OF PRIVACY PRACTICES

Continued

4. A statement of the patient's rights with respect to protected health information and a brief description of how the patient may exercise those rights:
 - A. The right to request restrictions on certain uses and disclosures of protected health information, including the statement that Organization's Name is not required to agree to a requested restriction
 - B. The right to receive confidential communications of protected health information
 - C. The right to inspect and copy protected health information
 - D. The right to amend protected health information
 - E. The right to receive an accounting of disclosures of protected health information
 - F. The right to opt out of receiving fundraising communications
 - G. The right to restrict disclosures of protected health information to a health plan where the individual paid out of pocket in full
 - H. The right to obtain a paper copy of the notice upon request (see "[Patient Privacy Rights](#)" Policy No. 2-014)
5. A statement of the organization's duties with respect to maintaining privacy of protected health information:
 - A. That the organization will, as required by law, maintain the privacy of protected health information and to provide patients with notice of its legal duties and privacy practices.
 - B. The organization will notify affected individuals following a breach of unsecured protected health information.
 - C. That the organization will abide by the terms of the notice currently in effect.
 - D. That the organization reserves the right to change the terms of its notice and make the new notice provisions effective for all protected health information that it maintains. The notice will describe how the new notice will be provided to individuals.
6. A statement that the patient may complain to Organization's Name and to the Secretary of the Department of Health and Human Services if he/she believes his/her privacy rights have been violated, instructions on how to file the complaint, and a statement that the patient will not be retaliated against for filing a complaint
7. The name, or title, and telephone number of a person or office to contact to file a complaint or to obtain further information about matters covered in the notice
8. The date on which the notice is first in effect, which will not be earlier than the date on which the notice is printed or otherwise published

MINIMUM NECESSARY USES OF PHI**Policy No. 2-015.1****PURPOSE**

To assure that patients' right to privacy is protected by limiting the protected health information (PHI) available to personnel for use.

POLICY

Organization's Name personnel will only have access to the minimum necessary protected health information to accomplish the intended purpose of the use.

PROCEDURE

1. Organization's Name will identify those personnel or classes of personnel who need access to protected health information to carry out their duties.
2. Organization's Name will specify in writing the category or categories of protected health information to which access is needed for personnel or classes of personnel. Organization's Name will also specify any conditions for access.
3. Organization's Name will not permit personnel or classes of personnel to access an entire clinical record, except when the entire clinical record is specifically justified as the amount that is reasonably necessary to carry out their duties.
4. Personnel will receive training related to the protected health information they may access and any conditions to that access. Training will be provided during orientation, whenever job duties change requiring access to different categories of protected health information, and at other times, as needed.
5. Reasonable efforts will be made to ensure that personnel access only the minimum necessary information needed to carry out their duties.

MINIMUM NECESSARY DISCLOSURES OF PHI**Policy No. 2-016.1****PURPOSE**

To assure that patients' right to privacy is protected by limiting the protected health information (PHI) disclosed.

POLICY

Organization's Name will limit the amount of protected health information, which is disclosed to the amount reasonably necessary to achieve the purpose of the disclosure.

PROCEDURE

1. For all disclosures, Organization's Name will:
 - A. Develop criteria to limit the protected health information disclosed to the amount reasonably necessary to achieve the purpose of the disclosure.
 - B. Review requests on an individual basis in accordance with the criteria.
2. Organization's Name may rely, if such reliance is reasonable under the circumstances, on a requested disclosure as the minimum necessary for a stated purpose when:
 - A. Making permitted disclosures to public officials, if the public official represents that the information requested is the minimum necessary for the stated purpose(s).
 - B. The information is requested by another covered entity.
 - C. The information is requested for the purpose of providing professional services by a professional who is a member of the organization's workforce or a business associate and the professional represents that that the information requested is the minimum necessary for the stated purpose(s).
 - D. Making disclosures for research purposes, as permitted by law.
3. Organization's Name will not disclose an entire clinical record, except when the entire clinical record is specifically justified as the amount that is reasonably necessary to achieve the purpose of the disclosure.

USES AND DISCLOSURES OF PHI**Policy No. 2-017.1****PURPOSE**

To safeguard protected health information (PHI) against unauthorized use.

POLICY

Protected health information will be used and disclosed according to the guidelines set forth in the organization's Notice of Privacy Practices. (See "[Patient Privacy Rights](#)" Policy No. 2-014 and "[Notice of Privacy Practices](#)" Addendum No. 2-014.A.)

PROCEDURE

1. Organization's Name may:
 - A. Use or disclose protected health information to the patient.
 - B. Use or disclose protected health information to carry out its own treatment, payment or health care operations.
 1. Patients will not be discussed by clinical or non-clinical personnel outside of the context of professional conversation regarding patient's condition and care.
 2. Comments and conversations relating to patients made by physicians, nurses or other organization personnel will be made in confidential settings. It will be standard, acceptable and necessary practice to share information with other members of the care team. The decision to share information can be aided by considering the intent of the discussion.
 3. Patient information and clinical record documents will not be left in open, public areas during business hours and will be secured after business hours. (See "[Safeguarding/Retrieval of Clinical Record](#)" Policy No. 2-031.)
 - C. Disclose protected health information for treatment activities of a patient's health care provider.
 - D. Disclose protected health information to another covered entity or health care provider for its payment activities.
 - E. Disclose protected health information to another covered entity for health care operations activities of the entity or for the purpose of health care fraud and abuse detection or compliance. Each entity must either have or had a relationship with the patient who is the subject of the protected health information being requested and the protected health information pertains to such relationship.

2. A patient may request a restriction of uses and disclosures of his/her protected health information. (See "[Patient Requests for Privacy Restrictions](#)" Policy No. 2-021.)
3. Organization's Name will obtain a valid authorization from the patient to use or disclose protected health information. (See "[Authorization for Use or Disclosure of PHI](#)" Policy No. 2-018):
 - A. In psychotherapy notes
 - B. For marketing activities
 - C. For other uses and disclosures as required by law
4. The organization and any agent acting on behalf of Organization's Name in accordance with a written contract, shall ensure the confidentiality of all patient identifiable information contained in the clinical record, including OASIS data and may not release patient identifiable OASIS information in the public.
5. Law enforcement inquiries
 - A. Police or investigative agencies' requests for information will not be complied with unless the patient or his/her legal representative has given specific authorization for release of information or a court order or subpoena is presented.
 - B. Exception: If Organization's Name is acting as an organization of the police department to assist them in gathering data or treating a patient they have referred.
6. Request for original record by the court under subpoena

The Clinical Records Supervisor will designate a staff member to carry the original record to the court designated location. The staff member will stay with the record at all times. The court will copy the record and the staff member will return to organization with the original record.

AUTHORIZATION FOR USE OR DISCLOSURE OF PHI**Policy No. 2-018.1****PURPOSE**

To delineate the process for obtaining patient authorizations to use or disclose protected health information (PHI). To ensure that Organization's Name use or disclosure of protected health information is consistent with the authorization obtained.

POLICY

The organization will obtain a valid authorization from the patient or his/her legal representative prior to using or disclosing protected health information, as required by federal and state laws. Authorizations will be obtained to use or disclose protected health information in psychotherapy notes and for marketing activities. Organization's Name will not condition the provision of treatment on obtaining an authorization, except as allowed by law.

PROCEDURE

1. The designated organization personnel will prepare the Authorization For Use or Disclosure of Information Form.
2. A valid authorization will contain the following elements and will be written in plain language:
 - A. A description of information to be used or disclosed that identifies information in a specific and meaningful way
 - B. Name or other specific identification of the person(s) or class of person(s), authorized to make the requested use or disclosure
 - C. Name or other specific identification of the person(s) or class of person(s), to whom the organization may make the requested use or disclosure
 - D. A description of each purpose of the requested use or disclosure
 - E. An expiration date or expiration event that relates to the patient or the purpose of the use or disclosure
 - F. Signature of the patient and date. If the authorization is signed by a personal representative of the patient, a description of the representative's authority to act for the patient must also be provided
 - G. A statement of the ability or inability of the organization to condition treatment, payment, admission or eligibility for benefits on the authorization

3. The clinician will explain the authorization form to the patient and family/caregiver, or legal representative.
4. The patient and family/caregiver, or his/her legal representative will be asked to sign and date the authorization.
5. The authorization form will be filed in the patient's clinical record and a copy will be given to the patient.
6. The patient has the right to refuse to sign the authorization form. If the authorization form is not signed, the clinician will document his or her efforts to obtain the signature and the reason why it was not obtained in the clinical note.
7. The clinician will notify the Clinical Supervisor whenever the patient refuses to sign the authorization form.
8. The designated organization personnel will carefully review the signed authorization form prior to each use or disclosure of protected health information to ensure that planned use or disclosure is consistent with the authorization.
9. The patient may revoke in writing an authorization at any time. The revocation will be effective for uses or disclosures on or after the date of the revocation.

MINIMUM NECESSARY REQUESTS FOR PHI

Policy No. 2-019.1

PURPOSE

To assure that patients' right to privacy is protected by limiting the protected health information (PHI) requested from other covered entities.

POLICY

Organization's Name will limit the amount of protected health information, which is requested to the amount reasonably necessary to achieve the purpose for which the request is made.

PROCEDURE

1. For all requests, Organization's Name will:
 - A. Develop criteria to limit the protected health information that is requested to the amount reasonably necessary to achieve the purpose for which the request is made.
 - B. Review requests on an individual basis in accordance with the criteria.
2. Organization's Name will not provide an entire clinical record, except when the entire clinical record is specifically justified as the amount that is reasonably necessary to accomplish the purpose of the request.

**PRIVACY OF HEALTH INFORMATION
OF DECEASED INDIVIDUALS
Policy No. 2-020.1****PURPOSE**

To assure that the deceased patient's rights to privacy of protected health information is safeguarded.

POLICY

Organization's Name will use and disclose deceased patient's protected health information in accordance with its policies and procedures and all applicable laws.

PROCEDURE

1. Organization's Name will treat an executor, administrator, or other person who has the authority to act on behalf of a deceased patient or his/her estate, under applicable law, as a personal representative with respect to protected health information.
2. The organization is permitted to disclose decedent's PHI to a family member or other individual who was involved in the care or payment for care of the decedent prior to death so long as the disclosure is not inconsistent with any prior expressed preference of the decedent of which the covered entity is aware.
3. The deceased patient's personal representative will have the same privacy rights as all other patients.
4. Organization's Name will limit the amount of protected health information, which is used or disclosed to the amount reasonably necessary to achieve the purpose of the disclosure.
5. Health information regarding a person who has been deceased for more than 50 years is excluded from the definition of PHI.
6. Organization's Name may disclose protected health information to:
 - A. Funeral directors, as necessary and consistent with applicable law, for them to carry out their duties with respect to the decedent. The protected health information may be disclosed prior to and in reasonable anticipation of the patient's death when this is necessary for funeral directors to carry out their duties.
 - B. A coroner or medical examiner for the purpose of identifying a deceased person, determining a cause of death, or other duties as authorized by law.
 - C. A law enforcement official if Organization's Name has a suspicion that the death may have resulted from criminal conduct.

7. When a deceased patient's personal representative has questions about his/her privacy rights, requests additional information, or would like to exercise one (1) of these rights, he/she will be referred to the appropriate individual designated by Organization's Name.

**PATIENT REQUESTS FOR
PRIVACY RESTRICTIONS
Policy No. 2-021.1**

PURPOSE

To delineate the process for patients to request a restriction of uses and disclosures of protected health information. To ensure that Organization's Name use or disclosure of protected health information is consistent with the restrictions to which it has agreed.

POLICY

Organization's Name will permit a patient to request restrictions on the uses and disclosures of protected health information to carry out treatment payment or healthcare operations. The patient may also request restrictions on information shared with family members, other relatives, other persons responsible for the patient's care and any other person identified by the patient.

Organization's Name will ensure that it adheres to the restrictions to which it has agreed, except in any case where the restricted information is needed to provide emergency treatment. Organization's Name will document all restrictions to which it has agreed.

PROCEDURE

1. All requests for restrictions to uses and disclosures of protected health information will be made in writing to the position or office designated by Organization's Name.
2. The request will be reviewed to assess what impact it will have on the organization's ability to carry out treatment, payment and health care operations. Organization's Name is not required to agree to a restriction. Exception: If a patient pays for treatments out of pocket and requests the organization not to share protected health information with their health plan the organization will comply with that request.
3. All agreements to restrictions will be documented in the patient's clinical record.
4. The organization personnel responsible for authorizing the restriction will communicate the agreement to the Clinical Supervisor responsible for the patient's care.
5. Organization's Name may terminate its agreement to a restriction under the following circumstances:
 - A. The patient agrees to or requests the termination in writing.
 - B. The patient verbally agrees to the termination and the agreement is documented in the clinical record by a designated employee of Organization's Name.

- C. Organization's Name informs the patient that it is terminating its agreement to a restriction. The termination of an agreement will only be effective with respect to protected health information created or received after it has informed the patient.
6. In the event that a patient requires emergency treatment, Organization's Name will disclose only the restricted protected health information needed by the treating health care provider. Organization's Name will request that health care provider not further use or disclose the restricted protected health information.
7. A disclosure of restricted protected health information for emergency treatment will be included in any accounting of disclosures of protected health information provided to the patient. (See "[Patient Requests for Accounting of PHI Disclosures](#)" Policy No. 2-025.)
8. Upon request, the organization will agree to restrict disclosure of protected health information about the individual to their health plan if the disclosure is for the purpose of carrying out payment or healthcare operations and is not otherwise required by law; and the protected health information pertains solely to a healthcare item or service for which the individual, or person other than the health plan on behalf of the individual, has paid for in full.
9. The patient's legal representative may exercise the patient's rights when a patient is incompetent or a minor.

**PATIENT REQUESTS FOR
CONFIDENTIAL COMMUNICATIONS
Policy No. 2-022.1**

PURPOSE

To delineate the process for patients to request to receive communications of protected health information by alternative means or at alternative locations.

POLICY

Organization's Name will permit a patient to request to receive confidential communications of protected health information by alternative means or at alternative locations. Organization's Name will accommodate reasonable requests if the patient clearly states that the disclosure of all or part of that information could endanger him/her.

PROCEDURE

1. All requests to receive confidential communications of protected health information by alternative means or at alternative locations will be made in writing to the position or office designated by Organization's Name. Requests will specify the alternative address or alternative method of communication.
2. The request will be reviewed to determine whether a reasonable accommodation can be made.
3. A description of the authorized accommodation will be documented in the patient's clinical record.
4. The organization personnel responsible for authorizing the restriction will communicate the agreement to the Clinical Supervisor responsible for the patient's care. The Clinical Supervisor will communicate the restriction to personnel involved in the patient's care.
5. The patient's legal representative may exercise the patient's rights when a patient is incompetent or a minor.

PATIENT REQUESTS FOR ACCESS TO PHI**Policy No. 2-023.1****PURPOSE**

To delineate the process for patients to request to inspect and obtain a copy of their protected health information (PHI) to delineate Organization's Name legal responsibilities.

POLICY

Organization's Name will permit patients to request to inspect and obtain a copy of his/her protected health information in a designated record set, for as long as the information is maintained in the designated record set.

Organization's Name will provide timely access to the protected health information in the form or format requested by the patient whenever possible. Organization's Name reserves the right to deny the patient access to all or part of his/her protected health information, as required or permitted by law.

PROCEDURE

1. All requests by a patient to inspect and obtain a copy of protected health information will be made in writing to the position or office designated by Organization's Name. The titles of the persons or offices responsible for receiving and processing requests for access by patients will be documented.
2. The request will be reviewed by designated organization personnel to determine whether it will be approved or denied, in accordance with all applicable laws.
3. Organization's Name reserves the right to deny access to protected health information, without providing the patient an opportunity for review in the following circumstances:
 - A. The information is contained in psychotherapy notes.
 - B. The information is compiled in reasonable anticipation of, or for use in, a civil, criminal, or administrative action or proceeding.
 - C. The information was received from someone other than a health care provider under a promise of confidentiality and access would be reasonably likely to reveal the source of information.
 - D. Other circumstances permitted by law.

4. Organization's Name reserves the right to deny a patient access to protected health information, with the right to a review of the denial, in the following circumstances:
 - A. A licensed health care professional determines that the access requested is reasonably likely to endanger the life or physical safety of the patient or another person.
 - B. The protected health information makes reference to another person, other than a health care provider, and the access requested is reasonably likely to cause substantial harm to such other person.
 - C. The request for access is made by the patient's personal representative and a licensed health care professional has determined that access is reasonably likely to cause substantial harm to the patient or another person.
 - D. Other circumstances permitted by law.
5. Organization's Name will act on the patient's request for access no later than 30 days after receipt of the request.
 - A. When the organization grants the request for access, in whole or in part, it will:
 1. Inform the patient of the acceptance of the request.
 2. Provide the patient with access to the protected health information. If the protected health information is maintained electronically and the individual requests an electronic copy the organization must provide the individual with access to the protected health information in the electronic form and format requested, if it is readily reproducible in that form or format, or, if not, in a readable hard copy form or other form or format mutually agreed upon by Organization's Name and the patient.
 3. Provide the patient with a summary of the protected health information requested in lieu of providing access or provide an explanation of the protected health information to which access has been provided. The patient must agree to the summary and/or explanation and agree in advance to any fees charged by Organization's Name to prepare the summary or explanation.
 4. Arrange with the patient to inspect or obtain a copy of the protected health information during normal business hours or to mail a copy of the information at the patient's request. Organization's Name will impose a reasonable, cost-based fee for copying and postage, as allowed by law.
 - B. When the organization denies the request for access, in whole or in part, it will:
 1. Provide the patient with a timely denial written in plain language that contains:
 - a. The basis for the denial.

- b. A statement of the individual's review rights, if applicable, and how the patient can exercise his/her review rights.
 - c. A description of how the patient may complain to Organization's Name, including the name or title and telephone number of the person or office designated in Organization's Name Notice of Privacy Practices. (See "[Notice of Privacy Practices](#)" Addendum 2-014.A.)
 - 2. Provide the patient, to the extent possible, with access to any other protected health information requested, after excluding the information which Organization has grounds to deny access.
 - C. If Organization's Name does not maintain the protected health information that is the subject of the patient's request for access, and Organization's Name knows where the requested information is maintained, it will inform the patient where to direct the request for access.
- 6. If the requested protected health information is not maintained or accessible to Organization's Name on-site, the organization will take action by no later than 60 days from the receipt of the request.
 - 7. If Organization's Name is unable to act on the patient's request for access to protected health information in the time lines specified above, it will provide the patient with a written statement that includes the reasons for the delay and the date by which Organization's Name will complete its action on the request. Organization's Name may extend the time for such actions by no more than 30 days.
 - 8. If the patient requests a review of a denial for access to protected health information, Organization's Name will designate a licensed health care professional, who was not directly involved in the denial, to review the decision to deny access.
 - A. Organization's Name will promptly refer the request for review.
 - B. The designated reviewer will determine, within a reasonable period of time, whether or not to deny access based on applicable laws.
 - C. Organization's Name will promptly provide a written notice to the patient of the designated reviewer's determination and take other action, as required by the determination.
 - 9. The patient's legal representative may exercise the patient's rights when a patient is incompetent or a minor.
 - 10. If the patient requests access to only their clinical record, the clinical record will be made available to the patient, free of charge, upon requests at the next home visit, or within 4 business days (whichever comes first).

PURPOSE

To delineate the process for patients to request amendments to their protected health information (PHI) maintained in their designated record sets. To delineate the legal responsibilities of Organization's Name.

POLICY

Organization's Name will permit a patient to request amendments to his/her protected health information maintained in his/her designated record set. Organization's Name will provide prompt action to a patient's request for amendment of protected health information.

Organization's Name reserves the right to deny the patient's request for amendment, in whole or in part, as required or permitted by law.

PROCEDURE

1. All requests for amendments to protected health information will be made in writing to the position or office designated by Organization's Name. The patient will provide a reason to support the requested amendment.
2. The request will be reviewed by designated organization personnel to determine whether it will be approved or denied, in accordance with all applicable laws.
3. Organization's Name reserves the right to deny a request for an amendment to protected health information, if the protected health information:
 - A. Was not created by Organization's Name
 - B. Is not part of the designated record set
 - C. Cannot be accessed by the patient (See "[Patient Requests for Access to PHI](#)" Policy No. 2-023.)
 - D. Is accurate and complete
4. Organization's Name will act on the patient's request for access no later than 60 days after receipt of the request. If Organization's Name is unable to act on the patient's request for access to protected health information in the time lines specified above, it will provide the patient with a written statement that includes the reasons for the delay and the date by which Organization's Name will complete its action on the request. Organization's Name will extend the time for such actions by no more than 30 days.

- A. When the organization grants the request for an amendment, in whole or in part, it will:
1. Make the appropriate amendment to the protected health information or record.
 2. Inform the patient of the acceptance of the request and obtain the patient's identification of and agreement to have Organization's Name notify relevant persons with which the amendment needs to be shared.
 3. Make reasonable efforts to inform and provide the amendment within a reasonable time to:
 - a. Persons identified by the patient as having received protected health information about the individual and need the amendment.
 - b. Persons including business associates, that Organization's Name knows have the protected health information that is the subject of the amendment and that may have relied, or could potentially rely, on the information to the detriment of the patient.
- B. When the organization denies the request for access, in whole or in part, it will:
1. Provide the patient with a timely, written denial in plain language which contains:
 - a. The basis for the denial.
 - b. The patient's right to submit a written statement disagreeing with the denial and where to file the statement.
 - c. A statement that, if the patient does not submit a statement of disagreement, the patient may request that Organization's Name provide the individual's request for amendment and the denial with any future disclosures of the protected health information that is the subject of the amendment.
 - d. A description of how the patient may complain to Organization's Name, including the name or title and telephone number of the person or office designated in Organization's Name Notice of Privacy Practices. (See "[Patient Privacy Rights](#)" Policy No. 2-014 and "[Notice of Privacy Practices](#)" Addendum 2-014.A.)
 5. Organization's Name will permit the patient to submit a written statement, of reasonable length, disagreeing with the denial of all or part of the requested amendment and the basis for such disagreement.
 6. Organization's Name may prepare a written rebuttal to the patient's statement of disagreement. When a rebuttal is prepared, a copy will be provided to the patient.

7. Organization's Name will, as appropriate, identify the protected health information in the designated record set that is the subject of the disputed amendment and will append or otherwise link the patient's request for an amendment, Organization's Name denial of the request, the patient's statement of disagreement, if any, and the Organization's Name rebuttal, if any.
8. When Organization's Name provides subsequent disclosures of the protected health information to which the disagreement relates, it will:
 - A. Include the material appended or an accurate summary of the information if the patient submitted a statement of disagreement.
 - B. Include the patient's request for an amendment and Organization's Name's denial if the patient has not submitted a statement of disagreement.
9. Organization's Name will amend a patient's protected health information in its designated record set when it is informed by another covered entity of an amendment.
10. The patient's legal representative may exercise the patient's rights when a patient is incompetent or a minor.

**PATIENT REQUESTS FOR
ACCOUNTING OF PHI DISCLOSURES
Policy No. 2-025.1**

PURPOSE

To delineate the process for patients to request and receive an accounting of disclosures of protected health information (PHI) made by Organization's Name and to delineate Organization's Name legal responsibilities.

POLICY

Organization's Name will provide patients with a timely, written accounting of disclosures of their protected health information made by Organization's Name in the six (6) years prior to the date on which the accounting is requested. Organization's Name retains the right to exclude disclosures from an accounting, as required or permitted by law.

PROCEDURE

1. All requests by a patient for an accounting of disclosures of protected health information made by Organization's Name will be made in writing to the position or office designated by Organization's Name. The titles of the persons or offices responsible for receiving and processing requests for an accounting of disclosures will be documented.
2. A patient may request an accounting of disclosures for a period of six (6) years or less.
3. The request will be reviewed by designated organization personnel and acted upon, in accordance with all applicable laws.
 - A. Organization's Name will provide the patient with the accounting requested no later than 60 days after receipt of the request.
 - B. If Organization's Name is unable to act on the patient's request for an accounting of protected health information in the time lines specified above, it will provide the patient with a written statement that includes the reasons for the delay and the date by which Organization's Name will complete its action on the request. Organization's Name will extend the time for such action by no more than 30 days.
4. The written accounting of disclosures will include:
 - A. The date of the disclosure

- B. The name of the entity or person who received the protected health information and, if known, the address of the entity or person
 - C. A brief description of the protected health information disclosed
 - D. A brief statement of the purpose of the disclosure
 - E. Any other information required by law
- 5. Organization's Name will provide the first accounting to the patient in any 12-month period without charge. Organization's Name may impose a reasonable cost-based fee for each subsequent request in the same 12-month period.
 - 6. Organization's Name will maintain a copy of the written accounting provided to the patient for a period of three (3) years.
 - 7. The patient's legal representative may exercise the patient's rights when a patient is incompetent or a minor.

PURPOSE

To delineate the uses of protected health information (PHI) for fundraising.

POLICY

Organization's Name will only use or disclose patients' demographic information, dates of service, department of service, treating physician, outcome information and health insurance status for the purpose of raising funds for its benefit. Organization's Name will obtain an authorization prior to using or disclosing any other protected health information.

PROCEDURE

1. Organization's Name will include a statement in its Notice of Privacy Practices that it may contact the patient to raise funds for its services. (See "[Patient Privacy Rights](#)" Policy No. 2-014 and "[Notice of Privacy Practices](#)" Addendum 2-014.A.)
2. Organization's Name will include a description in all of the fundraising materials it sends to patients that describes how the patient can opt out of receiving any further fundraising communications.
3. Organization's Name will make reasonable efforts to identify those patients who request to opt out and ensure that they do not receive additional fundraising communications
4. Organization's Name will obtain a valid authorization from the patient prior to using or disclosing any protected health information other than demographic information and dates of service for its fundraising activities. (See "[Authorization for Use or Disclosure of PHI](#)" Policy No. 2-018.)

PURPOSE

To delineate the uses of protected health information (PHI) for marketing.

POLICY

Organization's Name will obtain a valid authorization from patients prior to using or disclosing any protected health information for marketing purposes.

Definitions

Marketing activities:

1. Communications about a product or service that encourages patients to purchase or use the product or service
2. An arrangement between Organization's Name and any other organization whereby Organization's Name discloses patients' protected health information, in exchange for direct or indirect remuneration, for the other organization to make a communication about its own product or services that encourages patients to purchase or use that product or service

Non-marketing activities:

1. To describe a health related product or service that is provided by Organization's Name
2. For treatment of the patient
3. For case management or care coordination of the patient or to direct the patient to alternative treatments therapies, healthcare providers or settings of care

PROCEDURE

1. Organization's Name will obtain a valid authorization from the patient prior to using or disclosing any protected health information for its marketing activities, except if the communication is in the form of:
 - A. A face-to-face communication between Organization's Name and a patient
 - B. A promotional gift of nominal value provided by Organization's Name
2. When the marketing involves direct or indirect remuneration to the organization from a third party, the authorization will state that such remuneration is involved. (See "[Authorization for Use or Disclosure of PHI](#)" Policy No. 2-018.)

PURPOSE

To assure that organization personnel understand the organization's policies and procedures with respect to protected health information.

POLICY

Organization's Name will train all of its personnel on its policies and procedures related to protected health information.

All personnel will be required to sign a Confidentiality Agreement that will include reference to the confidentiality, privacy and security of patient and organizational information.

PROCEDURE

1. All personnel will receive training during orientation, as necessary and appropriate to carry out his/her assigned duties.
2. All personnel whose duties are affected by a material change in the organization's privacy and security policies and procedures will receive additional training within a reasonable period of time.
3. All personnel who have a material change in their duties will receive additional training appropriate to carry out their new duties.
4. All training provided to organization personnel will be documented and documentation will be maintained for a period of six (6) years.

For Security Training Requirements, see "[Security Awareness and Training](#)" Policy No. 2-051.

SANCTIONS FOR PRIVACY AND SECURITY VIOLATIONS**Policy No. 2-029.1****PURPOSE**

To assure that appropriate sanctions are applied for failure to comply with privacy and security policies and procedures.

POLICY

Organization's Name will apply appropriate sanctions against organization personnel who fail to comply with its privacy and security policies and procedures.

PROCEDURE

1. Any occurrence of failure to comply with the organization's privacy and security policies and procedures will be documented and forwarded to the individual or office designated by Organization's Name.
2. The designated individual will review the complaint, undertake further investigation, as needed, and recommend appropriate sanctions, including possible termination. There are three levels of violations:

A. Level 1: Carelessness

1. This level of violation occurs when an employee unintentionally or carelessly accesses, modifies, destroys or discloses to another person an individual's PHI and there is not a legitimate reason for such access or disclosure. Examples included are not limited to:
 - a. Discussing an individual's PHI in a public place
 - Leaving a copy of an individual's PHI in a public area
 - Leaving a computer unattended in an accessible area with an individual's PHI unsecured
 - b. Disregarding the guidelines for remote or wireless access
2. Carelessness does not include accessing PHI by mistake.
3. Employee will be subject to sanctions based on the severity and past incidence of HIPAA policy violations. Sanctions may range in severity from a written warning for a first or second offense to termination for repeated offenses

B. Level 2: Intentional and unauthorized accessing of PHI

1. This level of violation occurs when an employee intentionally accesses PHI for purposes other than conducting work on behalf of the organization or other authorized purpose. Examples included are not limited to:
 - a. Accessing and reviewing a public personality's record
 - b. Accessing the medical record of a friend, relative or co-worker without proper authorization
 - c. Using another employee's user ID and password
 - d. Writing a user ID or password in an exposed area
2. Employee will be subject to sanctions based on the severity and past incidence of HIPAA policy violations. Sanctions may range in severity from a final written warning for a first offense, to an unpaid suspension or termination of employment for second or repeated offenses.

C. Level 3: Intentional and unauthorized disclosure or destruction of patient information

1. This level of violation occurs when an employee accesses and discloses PHI without required authorization or maliciously destroys PHI. Examples include but are not limited to:
 - a. Introducing viruses within the organization's information systems with malicious intent
 - b. Destroying or altering PHI without authorization
 - c. Accessing an individual's PHI for personal gain, regardless of whether any PHI was disclosed, for example, compiling a mailing list for personal use or to be sold
2. Employee will be subject to appropriate sanctions based on the severity of the violation e.g. the significance of the disclosure, whether PHI was disclosed to an outside entity, etc.), whether there is a past incidence of HIPAA policy violations. Violations of HIPAA [policies at this level may result in immediate termination of employment.
3. The CEO/Executive Director/Administrator will be responsible for applying appropriate sanctions.
4. All sanctions that are applied will be documented and documentation will be maintained for a period of six (6) years.
5. Depending on the nature of the violation, the employee may be subject to civil and/or criminal penalties.

PURPOSE

The purpose of the Identity Theft Prevention Program (Program) is to:

1. Identify the relevant identity theft risks
2. Respond appropriately to any risks that are detected to prevent and mitigate identity theft

POLICY

Organization's Name will implement an Identity Theft Prevention Program.

PROCEDURE

1. Organization's Name will identify risks that would be most relevant to the organization. The risk areas generally fall into the following categories:
 - A. Suspicious Documents
 - B. Suspicious Personal Identifying Information
 - C. Alerts from Others (i.e. patient, identity theft victim or law enforcement)
2. Based on the identified risks, Organization's Name will determine how the risks will be detected for new and existing patient accounts.
3. In the event that Organization's Name personnel detect any identified identity theft risks, the staff will follow the appropriate steps, as determined by the organization, to resolve the issue.
4. All organization employees are trained on the Program during orientation and annual inservices.

ADDENDUM 2-030.A

IDENTITY THEFT RISK ASSESSMENT WORKSHEET

IDENTITY THEFT RISK ASSESSMENT WORKSHEET

Categories	Risk Assessment			
	1= Low Risk Process in place	2= Moderate Risk Process needs revision	3= High Risk No Process in place	N/A
<i>Suspicious Documents</i>				
Has staff ever found altered documents at time of admitting a patient?				
<i>Suspicious Personal Identifying Information</i>				
Does the agency request to see a photo ID at time of admission?				
Does the admitting staff visually check Medicare, Medicaid and insurance cards to verify accuracy of information?				
What does agency do if the patient is unable to present an insurance card?				
Is H&P from referral source compared to information provided by the patient?				
Is referral information matched to payor information on FISS or with insurance company prior to billing?				
<i>Alerts from Others</i>				
Has agency had any experience with identity theft?				
Who handles complaints from patients or payors who identify when service is billed to the wrong party?				
<i>Unusual Use of, or Suspicious Activity Related to the Covered Account</i>				
Can your agency track and verify who has printed or transmitted reports of account information?				
Do the processes for collecting patient payments create any risks?				
Does everyone use the same computer log-on or password?				

Organization's Name

Program/Service Operations



ADDENDUM 2-030.B

IDENTITY THEFT RISK RESPONSE MATRIX

IDENTITY THEFT RISK RESPONSE MATRIX

Identity Theft Risk	How Detected	Response to Risk	Prevention/Mitigation Procedure
Example: Patient has an insurance number but can't produce insurance card for verification.	Admitting staff compares admission paperwork with actual documentation to verify information.	Verify patient identification, i.e. driver's license. Determine why card is not available. Contact manager so that insurance company can be contacted and to receive direction on continuing the admission visit. Manager will contact insurance company and verify information.	Require admitting staff to contact patient/caregiver prior to initial visit and instruct them to have all physical documentation available for the visit.
Example: Notice from a patient, a victim of identity theft, a law enforcement agency, or someone that an account has been opened or used fraudulently.	By mail or telephone to the organization. Billing department, trying to collect on an unpaid bill.	Billing department will put a hold on applicable account. Program administrator will log in the notice, investigate if there was fraudulent activity and respond appropriately to investigation results. Ex. Ask the patient for proof that an identity theft claim has been filed.	Train staff on how to contact program administrator if a notification is received.
Example: A claim is rejected or returned due to an invalid ID number.	Billing department receives rejected claim via remittance advice or other mechanism.	Check source document and open other paperwork to check for data entry error. Re-verify insurance ID number by contacting insurance company. Contact patient team if patient is still active and if discharged	Require admitting staff to contact patient/caregiver prior to initial visit and instruct them to have all physical documentation available for the visit. During orientation and inservice training, emphasize the need for accuracy in documentation.

Organization's Name

Program/Service Operations

		contact patient directly.	
Agency Example:			

SAFEGUARDING/RETRIEVAL OF CLINICAL/SERVICE RECORD**Policy No. 2-031.1****PURPOSE**

To establish a procedure for the protection of patient clinical/service record information from loss or unauthorized use.

POLICY

The organization will safeguard the clinical/service record against loss, destruction, tampering, or unauthorized use through the development of processes, policies and procedures. The clinical/service record will be safeguarded in compliance with Health Insurance Portability and Accountability Act (HIPAA) requirements.

PROCEDURE

1. All physical patient clinical/service records will be maintained in locked, waterproof file cabinets in a record room:
 - A. The files will be locked at night.
 - B. The clinical/service record room will be locked at night.
 - C. The clinical/service record room will not be left unattended during working hours.
 - D. If the room is left unattended during business hours, the door will be locked.
 - E. Only authorized personnel will have access.
2. All clinical/service records will be returned to the clinical/service record room prior to the office closing. Clinical/service information and documents such as intake information, field charts, minutes of patient care meetings, clinical/service notes, unsigned plans of treatment being reviewed, verbal orders, and data being retrieved from records for performance improvement will be secured after business hours.
3. The original clinical/service record for active patients will remain in the office at all times unless required under subpoena.
4. Any copies made of patient information will include measures to secure the confidentiality of the material and be in compliance with the HIPAA privacy rule.
5. Clinicians/technicians providing intermittent care may maintain copies of the clinical/service documents for continuity of care, use in care coordination and/or supervision of ancillary personnel.
6. Copies of parts of the record, if taken out of the office, must be secured .

- A. Records will be transported in a covered, opaque, waterproof container.
 - B. Once placed in a vehicle, they will be in an inconspicuous location, such as the trunk.
 - C. The vehicle will be locked at all times.
7. Selected documents may be kept in the patient's home in a home clinical/service record while the case is active.
- A. Elements of the clinical/service record necessary to provide continuity of care include the plan of care/service, home health aide assignment sheet, medication profile, clinical/service notes, etc.
 - B. All home clinical/service record documents and supplies will be picked up by the organization personnel and returned to the office when care ends.
8. A patient's clinical record, whether hard copy or electronic form, shall be made available to a patient, free of charge, upon request at the next home visit, or within four (4) business days, whichever comes first.
9. Organization personnel will return any and all clinical/service record contents or copies of such to the office for destruction no later than one business day after the time of discharge. All exceptions must be approved by the clinical supervisor.
10. If electronic medical record computers/devices are utilized, access to information procedures should be followed. (See "[Access to Information](#)" Policy No. 2-012.) Safeguard procedures assure current acceptable professional standards and follow state laws that may be more prescriptive and:
- A. Minimum necessary information should be on the device.
 - B. Passwords will not be taped to computer.
 - C. Passwords will not be shared with other persons.
 - D. Device should be programmed with a time-out feature.
 - E. When using the device outside the office, face screen away from other individuals.
11. Patients have the right to access their clinical/service records and are informed of the process for requesting access at the time of admission. (See "[Patient Requests for Access to PHI](#)" Policy No. 2-023.)
12. Records will be retained according to organizational policy, local, state, and federal regulations.
- A. Records of adult patients will be retained for a minimum of five (5) years after discharge.
 - B. Records of minors will be retained for a minimum of seven (7) years after reaching the age of majority.

- C. Records involved in litigation will be retained until after settlement.
 - D. Records of patients who received Medicare services will be retained for a minimum of five (5) years past the month of filing of the applicable cost report or until the cost report is settled.
13. If the agency stores records in an off-site location, the following requirements shall apply:
- A. Timely accessibility of stored records on a 24 hour basis, seven days a week
 - B. Records are organized and systematically maintained
 - C. Protection of the clinical records from loss, destruction or unauthorized use
14. Security of information will be ensured for electronic and manual systems and include issues such as:
- A. Disaster recovery plan and continuity of business
 - 1. Plans for scheduled and unscheduled interruptions
 - 2. Contingency procedures for operations interruptions, such as alternate telephone source and universal power supply for critical computer systems
 - 3. Plans for minimal interruptions due to scheduled downtime
 - 4. An emergency service plan
 - 5. A backup system plan, either computerized or manual
 - 6. Data retrieval processes, including retrieval from storage and information presently in the system, retrieval of data in the event of system interruption and backup of data
 - B. Theft
 - C. Vandalism
 - D. Fire and flood recovery
15. Any confirmed, suspected, or potential violation of security or confidentiality will be reported to the CEO/Executive Director/Administrator. Violations will be considered a serious incident requiring immediate investigation and response. Violations of security will be reported and monitored through the organization's performance improvement plan.
- A. A thorough analysis will be conducted, assessing the need for process improvements or increasing security or confidentiality measures.

- B. As a result of this review, action will be taken as necessary to improve care. The Program Director or designee will identify issues with documentation and based on the review, if the issue:
 - 1. Is applicable to an individual, the individual will be counseled
 - 2. Is applicable to the organization as a whole, refer the issue to the senior management team for review
 - C. Any areas demonstrating a pattern or trend will be analyzed by the Performance Improvement Committee for development of recommendations and actions.
16. Breach notification requirements as outlined in the Health Information Technology for Economic and Clinical Health (HITECH) Act will be followed. (See "[Breach Analysis](#)" Policy No. 2-046 and "[Breach Notification](#)" Policy No. 2-047.)

COMPUTER ACCESS TO INFORMATION**Policy No. 2-032.1****PURPOSE**

To protect patient data processed by the computerized management information system.

POLICY

The computer system operator/user will hold all information in strictest confidence in the processing, storage and discarding of all data. Only authorized personnel will have access to written and computer data.

PROCEDURE

1. Authorized personnel will be assigned passwords and/or access codes and complete a signed statement that no one else will be allowed to use his/her computer key. Signed personnel security statements and pass code identification files will be secured and maintained in the Executive Director/Administrator or designee's office. Information accessible via remote terminals will be filtered through these passwords and security checks, creating an audit trail to identify individual users when necessary.
2. Electronic signature, when utilized, will be in compliance with individual state law. Signatures should conform to the following criteria:
 - A. Unique to the person using it
 - B. Capable of verification or authentication
 - C. Under the sole control of the person using it
3. If the drug order is verbal or given by or through electronic transmission, it must be given only to a licensed nurse, nurse practitioner (where appropriate), pharmacist or physician. The individual receiving the order must record it, sign it immediately and have the prescribing person sign it in accordance with state and federal regulations.
4. Data will be protected by safeguards that prevent unauthorized access to information and/or modification of existing data.
5. Computerized programs will control access to information by authorized personnel based on the type of password and position in the organization.
6. Date and time of entries will be designated by the computer's internal clock.
7. Automated controls will be in place to prevent a change in entry.

8. Correction of data may only be made by the author.
9. Hardcopies of automated data will be retrievable only by designated personnel.
10. A system for validation will be in place.
11. An automated and confidential system for backup and storage of data in a controlled and safe environment will be in place.
12. In the event that an outside vendor is used, a statement to maintain confidentiality will be obtained.

PURPOSE

To specify the contents of written contracts between Organization's Name and its business associates.

POLICY

The organization will have written contracts with its business associates to assure that patients' rights to privacy are safeguarded when protected health information (PHI) is disclosed.

PROCEDURE

1. A business associate is a person or entity, not a member of the organizations workforce, who performs functions or activities on behalf of Organization's Name that involve access to protected health information; or a subcontractor that creates, receives, maintains, or transmits protected health information.
2. All contracts between the organization and its business associates will:
 - A. Establish the permitted and required uses of protected health information by the business associate
 - B. Limit the business associate's use or further disclosure as follows:
 1. To the individual
 2. For treatment, payment, or health care operations as permitted by Privacy and Security Rules
 3. In compliance with a valid authorization
 4. Uses and disclosures required by law
 5. Other disclosures as permitted by Privacy and Security Rules
 - C. Provide that the business associate will:
 1. Not use or further disclose the information other than what is permitted or required by the contract or by law.
 2. Have administrative, technical and physical safeguards in place to protect PHI.
 3. Conduct risk assessments related to the PHI handled and document such assessments.
 4. Comply with all Privacy and Security Rules.

5. Maintain liability for violations due to the acts or omissions of an agent of the business associate.
 6. Ensure that any agents, including subcontractors, to whom it provides protected health information received from, or created or received by the business associate on behalf of, Organization's Name agrees to the same restrictions and conditions that apply to the business associate.
 7. Report to Organization's Name any use or disclosure of the information not provided for by its contract of which it becomes aware.
 8. Make protected health information available for access by patients. (See "[Patient Requests for Access to PHI](#)" Policy No. 2-023.)
 9. Make protected health information available for amendments, and incorporate any amendments to protected health information. (See "[Patient Requests to Amend PHI](#)" Policy No. 2-024.)
 10. Make available the information required to provide an accounting of disclosures. (See "[Patient Requests for Accounting of PHI Disclosures](#)" Policy No. 2-025.)
 11. Make its internal practices, books and records relating to the use and disclosures of protected health information received from or created or received by the business associate on behalf of Organization's Name available to the Secretary of the U.S. Department of Health and Human Services for purposes of determining the organization's compliance.
 12. At the termination of the contract, return or destroy all protected health information received from, created by, or received by the business associate on behalf of Organization's Name, that the business associate still maintains in any form. If return or destruction is not feasible, limit further uses and disclosures to those purposes that make return or destruction of the information infeasible. The business associate may not return any copies of such information.
3. In response to complaints or other evidence that a business associate has violated its agreement, Organization's Name will:
 - A. Investigate any complaint received or other credible evidence of a violation;
 - B. Take reasonable steps to discontinue any material breaches or violations;
 - C. Terminate the agreement if attempts to cure a material breach are unsuccessful; and,
 - D. In the event termination of the agreement is not feasible, report the violation to the Secretary of HHS through OCR.
 4. A sample business associate agreement can be located at:
<http://www.hhs.gov/ocr/privacy/hipaa/understanding/coveredentities/contractprov.html>

PURPOSE

To support the implementation of the Patient Self-Determination Act within the framework of state and federal law and organization policies.

POLICY

Organization's Name recognizes that all adult persons have a fundamental right to make decisions relating to their own medical treatment, including the right to accept or refuse medical care. It is the policy of Organization's Name to encourage individuals and their family/caregivers to participate in decisions regarding care, treatment, and services. Valid Advance Directives, such as living wills, Durable Powers of Attorney, and DNR (Do Not Resuscitate) or DNI (Do Not Intubate) orders will be followed to the extent permitted and required by law. In the absence of Advance Directives, Organization's Name will provide appropriate care according to the plan of care/service or as authorized by the attending physician. Organization's Name will not determine the provision of care/service or otherwise discriminate against an individual based on whether or not the individual has executed an Advance Directive.

Definitions

1. **Adult:** A person 18 years or older, or a person legally capable of consenting to his/her own medical treatment.
2. **Advance Directives:** A document in which a person states choices for medical treatment.
3. **Attending Physician:** The physician who is primarily responsible for the medical care of a patient while receiving home care services.
4. **DNR (Do Not Resuscitate):** A medical order to refrain from cardiopulmonary resuscitation if the patient's heart stops beating.
5. **DNI (Do Not Intubate):** A medical order to refrain from inserting life-sustaining breathing and/or feeding tubes, if the need arises.
6. **Patient Self Determination Act:** A federal statute enacted as part of the 1990 Omnibus Budget Reconciliation Act (OBRA) (PL 101-508) which requires, among other things, that health care facilities provide information regarding the right to formulate Advance Directives concerning health care decisions.
7. **Patient Representative:** A person appointed to make decisions for someone else. He/she may be formally appointed (as in a durable power of attorney for health care) or, in the absence of a formal appointment, may be recognized by virtue of a relationship with the patient (such as the patient's next of kin or close family/caregiver).

8. **Terminal Condition:** An incurable condition caused by an injury, disease, or illness, which regardless of the application of life-sustaining procedures, would within reasonable medical judgment produce death, and where the application of life-sustaining procedures only postpones the moment of death of the patient.
9. **POLST/MOLST:** Physician (Medical) Orders for Life-Sustaining Treatment is a physician order that helps give seriously ill patients more control over their end-of-life care. It does not replace an Advance Directive. There are a number of states which have established a POLST/MOLST program. Visit www.polst.org.

PROCEDURE

1. Upon admission, the clinician will provide information, orally and in writing, regarding a patient's right to make decisions concerning health care, which include the right to accept or refuse medical or surgical treatment, even if that treatment is life-sustaining, the right to execute Advance Directives, applicable organization policies including a description of applicable state law. Written information designed for this purpose will be provided to the adult patient. The clinician will document in the clinical/service record that the information was provided and document all discussions concerning Advance Directives.
2. If the patient lacks a decision-making capacity, the admitting clinician will provide information and direct inquiry about Advance Directives to the patient's representative. The clinician/technician will document that the patient representative received information and his/her name and responses will be noted in the clinical record.
3. If conditions are such that it is not practical to provide information to the patient or his/her representative at the time of admission, such information will be provided as soon as feasible after admission.
4. During the admission/evaluation visit, the admitting clinician will ask the patient or his/her representative whether or not he/she has completed an Advance Directive, Durable Power of Attorney (DPOA), living will, or DNR/DNI order. If an Advance Directive has been completed, the clinician/technician will ask for a copy of the Advance Directive so it will be placed in the clinical/service record. If a copy is not immediately available, the patient will be informed that it is his/her responsibility to provide a copy of the Advance Directive to the organization as soon as possible.
 - A. Verify that the photocopy is an exact copy of the original and mark the top of the copy ("Copy From Original"). Document in the clinical/service record the date of the request and to whom the request was given.
 - B. On the last page of the document, indicate "Provided By" and record the name of the person who is presenting the document to you. Document in the clinical/service record the date of the request and to whom the request was given.
 - C. Indicate "Received By" and sign and date the document. Document in the clinical/service record the date of the request and to whom the request was given.

5. If a copy of the patient's Advance Directive is not available to the organization, the clinician/technician will discuss the contents of the Advance Directive with the patient and/or patient representative and document the contents of the Advance Directive in the clinical/service record and communicate the contents to other home care providers.
6. When applicable, the admitting clinician/technician will document on the clinical/service record and notify the attending physician verbally if the patient has executed an Advance Directive.
7. The patient will be encouraged to participate in all aspects of decision-making regarding home care and treatment. Statements by a competent patient of his/her desire to accept or refuse treatment will be documented in the patient's clinical record.
8. All clinicians/technicians providing care/service for the patient will:
 - A. Review the Advance Directive and report any discrepancies between the Directive and current treatment/service plan to the attending physician, Program Director, and the patient
 - B. Utilize available educational materials to answer the patient's questions about Advance Directives, durable power of attorney, or living wills
 - C. Encourage the patient to discuss questions and concerns with appropriate individuals such as the physician, family/caregiver, and his/her selected advocate
 - D. Assist the patient who wants to develop an Advance Directive by obtaining a form and providing access to the outside individuals as necessary to execute the directive
9. An Advance Directive will be implemented as follows:
 - A. The Durable Power of Attorney for an Advance Directive is effective *only* when the patient is unable to participate in his/her own medical treatment decisions.
 - B. The attending physician and another physician or a licensed psychologist must document in the patient's clinical record that the patient is unable to participate in medical treatment decisions.
 - C. The patient's designated advocate can then make medical treatment choices based on the Advance Directive. The patient advocate may make a decision to withhold or withdraw treatment that allows the patient to die. This is done *only* if the patient expressed, in a clear and convincing manner, that the advocate is authorized to make such a decision, and acknowledges that such a decision would or could allow the patient's death.

- D. Executing and implementing an Advance Directive is a process, not a one (1)-time event. On an ongoing basis, personnel will keep the patient, family/caregiver, and patient's representative up to date concerning the patient's medical condition. They will discuss the patient's preferred course of treatment as his/her condition changes. The discussions will be documented in the clinical/service record.
- 10. Educational information about Advance Directives and Organization's Name's policies and procedures regarding Advance Directives will be provided to the medical, nursing, and allied health professionals, as well as home care personnel and volunteers during the orientation period.
- 11. In order to educate the community about Advance Directives, Organization's Name will participate in community forums, as appropriate, and make written materials available regarding Advance Directives.
- 12. If the state and/or region has endorsed the use of POLST/MOLST, the organization will utilize the state and regional specific forms and guidelines. (For more information: <https://polst.org/programs-in-your-state/>)

ADDENDUM 2-034.A

ADVANCE DIRECTIVE INFORMATION STATEMENT

ADVANCE DIRECTIVE INFORMATION STATEMENT

PATIENT INFORMATION REGARDING ORGANIZATION POLICIES AND PROCEDURES

You have received a copy of the Patient Bill of Rights document in addition to several other forms/handouts. These materials were reviewed with you on the initial visit by organization personnel. In addition, there are other organization policies and procedures reviewed with you that are related to your rights as a patient receiving service from this organization. These policies and procedures are summarized below:

ADVANCE DIRECTIVES/WITHHOLDING OF RESUSCITATIVE SERVICES

You were asked during the initial visit if you had executed an Advance Directive. If you have not executed an Advance Directive, you have also been provided with literature relative to your rights under federal and state law to execute such a document.

Organization policy states that:

"The organization recognizes that all persons have a fundamental right to make decisions relating to their own medical treatment, including the right to accept or refuse medical care. It is the policy of the organization to encourage individuals and their families to participate in decisions regarding care and treatment. Valid Advance Directives, such as living wills, Durable Powers of Attorney and DNR (Do Not Resuscitate) or DNI (Do Not Intubate) Orders will be followed to the extent permitted and required by law. In the absence of Advance Directives, the organization will provide appropriate care according to the plan of treatment authorized by the attending physician. The organization will not condition the provision of care or otherwise discriminate against an individual based on whether or not the individual has executed an Advance Directive."

It is also the organization's policy that:

"In the event of cardiac or pulmonary arrest, cardiopulmonary resuscitative measures will be promptly initiated unless a Do Not Resuscitate/Do Not Intubate (DNR/DNI) order has been written by the physician in charge and documented in the patient's clinical record."

GUIDELINES

1. A DNR and/or DNI decision is to be made by the attending physician in consultation with the patient or other legally responsible person when, in the judgment of the physician, the patient suffers from an incurable terminal illness, death is reasonably imminent in all medical probability, and resuscitation will do nothing to relieve the underlying disease condition, nor the probability of death. This order must be written in the patient's clinical record as any other treatment order.
2. The DNR/DNI order will be re-evaluated under the following conditions:
 - A. There is a significant change in patient condition
 - B. At the request of the patient guardian

ADVANCE DIRECTIVE INFORMATION STATEMENT

GUIDELINES *(continued)*

3. It will be the responsibility of the nurse to communicate to the primary attending physician any change in the patient's condition which may make a continuing DNR/DNI order questionable, so that the physician (or allowed practitioner) may re-evaluate the appropriateness of the order.
4. The order may be revoked at any time, verbally or in writing, by the competent patient, the incompetent patient's guardian, or by the attending physician (or allowed practitioner).
5. The DNR/DNI order(s) will be kept in the patient's clinical record and a copy will be kept in the patient's home.
6. Any organization staff member informed of or provided with a revocation of consent will immediately record the revocation request in the patient's clinical record, cancel the order, and notify the physician (or allowed practitioner) responsible for the patient's care of the revocation and cancellation.

ETHICS

It is also this organization's policy that if an organization staff member informs management that he/she cannot implement an Advance Directive or DNR/DNI order on the basis of personal belief or conscience, then that organization staff member will be reassigned.

As a consequence of the complex technical and ethical issues arising today in the provision of care at home, the organization has ethics advisors. These advisors assist the organization in responding to the challenges confronting health care providers who are involved in difficult treatment choices and care decisions. Care decisions may involve ethical issues regarding the withholding or withdrawal of treatment. You, or your representative, have the right to participate in any discussions concerning ethical issues arising from your care.

If you have any questions concerning your rights, these related policies, or other organization policies, please discuss them with your nurse or call the office and ask to speak with the Clinical Supervisor.

ADDENDUM 2-034.B

DURABLE POWER OF ATTORNEY FOR HEALTH CARE

(Note: Place copy of state statute/information here)

DO NOT RESUSCITATE/DO NOT INTUBATE ORDERS**Policy No. 2-035.1****PURPOSE**

To facilitate a patient's choice regarding the extent to which emergency medical care will be instituted.

POLICY

The organization will follow patient's Advance Directives completed according to the requirements of the jurisdiction of the state in which the patient resides. (See "[Advance Directives](#)" Policy No. 2-034.)

The organization supports the patient's right of autonomy to make choices regarding his/her care, and encourage the patient to discuss this issue with his/her significant others. In the event that the patient is without the capacity to make treatment decisions for himself/herself, this decision shall be made by an appropriate surrogate.

A written Do Not Resuscitate (DNR) order signed by the patient's physician (or allowed practitioner) must be on file in the patient's clinical record and admission folder in the patient's home. If there is no DNR order or valid Advance Directives and the patient expires in the presence of a CPR-trained staff person, CPR will be initiated according to the American Heart Association's Basic Life Support (BLS).

PROCEDURE

1. A DNR/DNI decision will be made by the attending physician in consultation with the patient or other legally responsible person when, in the judgment of the physician, the patient suffers from an incurable medical condition, death is reasonably imminent in all medical probability, and a life threatening condition exists in which resuscitation would not be expected to render substantial improvement in the ultimate outcome.
2. The order will be written only by the attending physician (or allowed practitioner).
3. Upon receipt of a DNR/DNI order, the following documentation will occur in the clinical record:
 - A. A summary of the medical situation, including the Case Manager's discussion with the attending physician and a statement of the therapeutic plan for comfort care.
 - B. An account of the discussion with the patient and/or legal representative, preferably by the attending physician, or an explanation as to why a discussion has not occurred.

- C. The DNR/DNI order will be clearly identified in the clinical record.
- D. A copy of the DNR/DNI order will be kept in the patient's home.
- 4. DNR/DNI status will be communicated as follows:
 - A. The Case Manager will immediately advise the Clinical Supervisor.
 - B. The Case Manager will notify other personnel involved in the case within 24 hours and document this notification in the clinical record.
 - C. The Case Manager will immediately notify the home health aides' office:
 - 1. If the patient is new to home health aide service, "Do Not Resuscitate" will be written in the special instruction section of home health aide assignment sheet.
 - 2. If the patient has ongoing home health aide service, a new home health aide assignment sheet will be created with "Do Not Resuscitate" written in special instruction section.
 - 3. On the home health aides' weekly schedules, the letters "DNR" will be written next to the names of the patients with Do Not Resuscitate orders.
 - 4. When home health aides receive revised assignments via telephone, they will be told which patients have DNR status.
- 5. The order will be recertified every 60 days or upon request of concerned parties.
- 6. The DNR/DNI order will be re-evaluated under the following conditions:
 - A. When there is a significant change in patient condition, it will be the responsibility of the clinician, within the standard of practice, to communicate to the attending physician (or allowed practitioner) any change in the patient's condition that impact the DNR/DNI order.
 - B. At the request of the patient or his/her representative.
- 7. The DNR/DNI orders may be revoked at any time verbally or in writing by:
 - A. The competent patient
 - B. The incompetent patient's legal representative
 - C. The attending physician in consultation with a competent patient or an appropriate surrogate decision maker

8. Organization personnel informed of or provided with a revocation of DNR/DNI by the patient or patient's representative will immediately record the revocation request in the patient's clinical record, cancel the order, and notify the physician (or allowed practitioner) responsible for the patient's care.
9. If the patient is not capable of making his/her decisions regarding medical care, a decision should be reached after consultation between the physician and one (1) of the following, according to the hierarchy of decision makers:
 - A. A court appointed guardian
 - B. A proxy designated by a durable power of attorney for health care authorized according to law
 - C. A spouse
 - D. An adult child
 - E. A parent
 - F. An adult sibling
 - G. A nearest relative
10. All communication between organization personnel and the patient and family/caregiver regarding resuscitation of the patient will be documented in the clinical record.

CARDIOPULMONARY RESUSCITATION**Policy No. 2-036.1****PURPOSE**

To outline the responsibilities of organization personnel in initiating cardiopulmonary resuscitation.

POLICY

All clinical personnel will be CPR certified in accordance with the American Heart Association's (AHA) guidelines for Basic Life Support (BLS) upon hire and as directed thereafter by the AHA. Online CPR certification is not accepted.

In the event of an arrest, witnessed or not, and in the absence of Advance Directives, 911 (or emergency rescue squad) will be called and BLS initiated and followed per AHA guidelines. (See "[Advance Directives](#)" Policy No. 2-034, and "[Do Not Resuscitate/Do Not Intubate Orders](#)" Policy No. 2-035.)

If it is determined by the organization that cardiopulmonary resuscitation will not be initiated, there will be written communication of this in the patient admission documents.

ASSESSMENT OF POSSIBLE ABUSE/NEGLECT
Policy No. 2-037.1**PURPOSE**

To provide guidelines for identification of suspected abuse victims for care and referral to community resources.

POLICY

The organization will report all suspected cases of abuse, neglect, or exploitation in compliance with appropriate state statutes to appropriate protection organizations. The organization will investigate complaints regarding treatment or care, mistreatment, neglect, or verbal, mental, sexual, and physical abuse, including injuries of unknown source, and/or misappropriation of patient property by anyone furnishing services on behalf of the organization. The organization shall be responsible for asking the necessary questions to determine cause of injuries.

Definitions

1. ***Abuse/Domestic Violence:*** Any act that constitutes a violation of the prostitution, criminal, or sexual conduct statutes; the intentional and nontherapeutic infliction of pain, injury, or any persistent course of conduct intended to produce mental or emotional distress.
2. ***Abandonment:*** Evidence that the patient has been "dropped off" by someone, or the family unit has no intention of coming back for the patient. Other high-risk situations: drug or alcohol addiction in the family, isolation of the patient, history of untreated psychiatric problems, evidence of unusual family stress, excessive dependence of the patient on the caretaker.
3. ***Neglect:*** Failure of a caretaker to supply the patient with the necessary food, clothing, shelter, health care, or supervision; or the absence or likelihood of an absence of the necessary food, clothing, shelter, health care, or supervision for a patient.
4. ***Exploitation:*** Unexplained loss of social security or pension checks, any evidence that material goods are being taken in exchange for care, any evidence that personal belongings of patient (house, jewelry, car) are being taken without consent or approval of patient.
5. ***Caregiver:*** An individual or facility responsible for all or some of the care voluntarily or by agreement, such as a relative, friend, or organization personnel.
6. ***Report:*** Any report received by the local welfare organization, police department, county sheriff, or licensing organization; a verbal and/or written statement of abuse and/or neglect that states the following:
 - A. What has happened
 - B. To whom it happened
 - C. When it happened

D. Where it happened

E. Who was responsible for the abuse or neglect

7. **Individual Mandated to Report:** A professional or the professional's delegate who is engaged in the care of patients, or in education, social services, law enforcement, or in any of the related occupations, who had knowledge of the abuse or neglect of a patient, or who has reasonable cause to believe that a patient is being or has been abused or neglected, or who has knowledge that a patient has sustained a physical injury that is not reasonably explained by the history of injuries provided by the caretaker or caregiver(s) of the patient. (Note that specific reporting requirements vary from state to state.)

Note: Any individual who is legally mandated to report suspected abuse and/or neglect and who intentionally fails to report such suspected abuse/neglect is guilty of a misdemeanor and liable for damages caused by failure.

An individual who willingly makes a false abuse and/or neglect report is liable for civil action for any damages suffered by individuals who were reported as suspects in such abuse and/or neglect.

Occurrences may include but are not limited to:

Physical Abuse/ Domestic Violence	Neglect/Failure to Provide	Exploitation	Verbal Abuse	Sexual Abuse
Neglect	Physical care	Money misuse	Verbal assault	Sexual molestation
Malnutrition	Emotional care	Property misuse	Verbal humiliation	Sexual assault
Bruise	Medical care	Money theft	Verbal threat	Sexual exploitation
Welt	Acceptable environment	Property theft		Sexual injury
Abrasion	Adequate supervision			
Puncture wound				
Laceration				
Burn				
Dislocation				
Fracture				
Internal Injury				
Hitting				
Slapping				
Exposure to adverse weather conditions				

PROCEDURE

1. Any clinician who discovers, within the course of rendering care, any suspected abuse or neglect will immediately, including after-hours, report such occurrences to his/her immediate supervisor. This includes:
 - A. Any knowledge of abuse or neglect
 - B. Any knowledge of patient self-abuse or self-neglect
 - C. Reasonable cause to suspect abuse or neglect
 - D. Reasonable cause to suspect self-abuse or self-neglect
 - E. Any knowledge that a patient has sustained an injury that is not reasonably explained by the patient's history of injuries
2. If patient is in immediate jeopardy, the proper authorities will be contacted by the clinician immediately as required by state or local law.
3. The clinician will submit a verbal report of the suspected abuse/neglect to the proper authorities in accordance with state law and complete a suspected abuse and/or neglect investigation report within 24 hours of the incident, with the addition of the following information:
 - A. Name of alleged perpetrator
 - B. Relationship to patient, if any
 - C. Information regarding suspected abuse/neglect:
 1. Date of occurrence
 2. Witness to occurrence, if any
 3. Nature of occurrence (as indicated above)
 4. Description of the abuse/neglect
4. The suspected abuse/neglect investigation report with specific information relating to the occurrence is forwarded to the Clinical Supervisor, who will:
 - A. Notify the physician and referral source (as appropriate) or direct the clinician to do so
 - B. Review the documentation for completeness and objectivity
 - C. Forward the information to the Executive Director/Administrator within 24 hours of report

5. The Executive Director/Administrator will:
 - A. Review all reports (incident report and follow-up investigation)
 - B. Conduct any further review if necessary
 - C. Document additional information
 - D. Submit reports to the proper authorities within 24 hours of receipt
6. Reports, reviews, and investigations of suspected abuse/neglect will be held in strictest confidence.
7. The organization will maintain a list of private and public community agencies that provide or arrange for assessment of suspected or alleged abuse/neglect victims. (See ["Organization List of Private and Public Community Agencies That Provide or Arrange For Assessment of Suspected or Alleged Abuse/Neglect Victims"](#) Addendum 2-037.A.)
8. The clinician will make a referral to a community organization whenever necessary.

California:

1. Reports of suspected or alleged instance of abuse shall be made by telephone or through a confidential internet reporting tool, immediately or as soon as practicably possible.
 - A. If reported by telephone, a written report shall be sent, or an internet report shall be made through the confidential internet reporting tool within two (2) working days.
 - B. The report shall be made to the Adult Protective Services Agency or the Local Law Enforcement Agency.
2. If a mandated reporter has knowledge or reasonably suspects, that types of elder or dependent adult abuse for which reports are not mandated have been inflicted upon an elder or dependent adult, or that their emotional well-being is endangered in any other way, may report the known or suspected instance of abuse.
 - A. The report shall be made to the county Adult Protective Services Agency.
 - B. If the conduct involves criminal activity, it shall be reported immediately to the appropriate law enforcement agency.
3. If two (2) or more mandated reporters are present and jointly have knowledge or reasonably suspect that types of abuse of an elder or a dependent adult for which a report is or is not mandated have occurred, and there is agreement among them, the telephone report or internet report, may be made by a member of the team selected by mutual agreement, and a single report may be made and signed by the selected member of the reporting team.

- A. Any member who has knowledge that the member designated to report has failed to do so shall thereafter make the report
4. Written abuse reports shall be submitted in one of the following ways:
- A. On a State Department of Social Services Form distributed by the county Adult Protective Services Agency.
 - B. Through a confidential internet reporting tool, as determined by the county Adult Protective Services Agency.
5. Reports shall contain:
- A. The name, address, telephone number and occupation of the person making the report,
 - B. The name, address and age of the elder or dependent adult,
 - C. The present location of the elder or dependent adult,
 - D. The names and addresses of family members or any other adult responsible for the elder's or dependent adult's care,
 - E. The nature and extent of the elder's or dependent adult's condition,
 - F. The date, time and place of the incident,
 - G. Any statement relating to the incident made by the victim,
 - H. The name of any individuals believed to have knowledge of the incident,
 - I. The name of the individuals believed to have been responsible for the incident and their connection to the victim, and
 - J. Any other information, including information that led that person to suspect elder or dependent adult abuse, as requested by the agency receiving the report.

ADDENDUM 2-037.A

**ORGANIZATION LIST OF PRIVATE AND PUBLIC COMMUNITY
AGENCIES THAT PROVIDE OR ARRANGE FOR ASSESSMENT OF
SUSPECTED OR ALLEGED ABUSE/NEGLECT VICTIMS**

PURPOSE

To provide an organizational process to define and address ethical issues that arise in the care of patients.

POLICY

A group of qualified professionals will be designated by the Governing Body to review ethical issues as they arise. Management meetings, case conferences, performance improvement meetings, or oversight committees can serve as vehicles to consider, discuss and resolve ethical issues. Ad hoc ethics forums may also be established. Representation will include a multidisciplinary team comprised of a member of management, a physician, appropriate clinical personnel, consumer, clergy and an attorney or risk management representative. The organization will develop and maintain resources and provide education programs concerning ethical issues. (See also "[Advance Directives](#)" Policy No. 2-034, "[Do Not Resuscitate/Do Not Intubate Orders](#)" Policy No. 2-035, "[Cardiopulmonary Resuscitation](#)" Policy No. 2-036.)

The patient and family/caregiver or their representative has the right to participate in any discussion concerning a conflict or ethical issue arising from his/her care.

Definitions

Ethical issues in home care include, but are not limited to:

1. Withholding or withdrawal of treatment
2. Unsafe home situations and patient safety
3. Nonadherence to treatment plan or refusal of treatments
4. Choosing to stay in a neglectful or abusive environment
5. Over or under treatment by a physician/family/caregiver
6. Family/caregiver participating in medical decisions
7. Informed consent
8. Confidentiality and patient privacy rights
9. Care of patients without insurance or other payment sources
10. Any issue which causes an ethical conflict or moral dilemma
11. How the organization will address the care of patients that have elected to participate in the End of Life Option Act process

12. Ethical business practices that include issues for marketing of services, admission practices, transfer practices, discharge practices, and billing practices

PROCEDURE

1. The group designated by the Governing Body to review ethical issues will be responsible for:
 - A. Promptly addressing issues as they arise
 - B. Reviewing all aspects of the issue
 - C. Requesting clarification of information where indicated
 - D. Securing outside assistance from "ethical experts" as needed
 - E. Resolving the ethical issue according to applicable law, community standards of practice, appropriate allocation of resources with consideration to the role of interested parties
2. Organization personnel may discuss any ethical concerns with their immediate supervisors. Further discussions may be held during management meetings, case conferences, performance improvement meetings, or oversight committee meetings.
3. Any organization personnel, physicians or other professionals involved in the care of the patient or the patient and family/caregiver may initiate a referral for an ethics consultation by notifying a Program Director or Executive Director/Administrator.
4. Organization personnel, patients, their representatives, and attending physicians may request, in advance, to attend a meeting of the selected committee whenever discussion may be relevant to the care involving an individual patient.
5. Minutes will be maintained for all meetings. To assure confidentiality, any discussions involving individual patient cases or organization personnel will not include names but will utilize identification numbers. Minutes will be kept in the Executive Director/Administrator's office.
6. Issues involving conflicts and ethical concerns will be tracked and reported through performance improvement activities and reported, at least quarterly, to the Governing Body as part of those activities.

NONDISCRIMINATION POLICY AND GRIEVANCE PROCESS**Policy No. 2-039.1****PURPOSE**

To prevent organization personnel from discriminating against other personnel, patients, or other organizations on the basis of race, color, religion, age, sex (sex characteristics including intersex traits, sexual orientation, gender identity and sex stereotypes, pregnancy or related conditions, childbirth and related conditions), disability (mental or physical), communicable disease, or national origin.

POLICY

In accordance with Title VI of the Civil Rights Act of 1964, Section 1557 of the Affordable Care Act (ACA) of 2010 and its implementing regulation, Organization's Name will, directly or through contractual or other arrangement, admit and treat all persons without regard to race, color, or place of national origin in its provision of services and benefits, including assignments or transfers within facilities.

In accordance with Section 504 of the Rehabilitation Act of 1973, Section 1557 of the Affordable Care Act (ACA) of 2010 and its implementing regulations, Organization's Name will not, directly or through contractual or other arrangements, discriminate on the basis of disability (mental or physical) in admissions, access, treatment or employment.

In accordance with the Age Discrimination Act of 1975, Section 1557 of the Affordable Care Act (ACA) of 2010 and its implementing regulation, Organization's Name will not, directly or through contractual or other arrangements, discriminate on the basis of age in the provision of services unless age is a factor necessary to the normal operation or the achievement of any statutory objective.

In accordance with Title II of the Americans with Disabilities Act of 1990, Organization's Name will not, on the basis of disability, exclude or deny a qualified individual with a disability from participation in, or benefits of, the services, programs or activities of the organization.

In accordance with other regulations the organization will not discriminate in admissions, access, treatment, or employment on the basis of gender, sexual orientation, religion, or communicable disease.

PROCEDURE

1. The Section 504/ADA Compliance Coordinator and Section 1557 Civil Rights Coordinator (can be same person) designated to coordinate the efforts of Organization's Name to comply with the regulations will be the Executive Director/Administrator. Contact the Executive Director/Administrator at _____ (insert telephone number.)
2. Organization's Name will identify an organization or person in their service area who can interpret or translate for persons with limited English proficiency and who can disseminate information to and communicate with sensory impaired persons. These contacts will be listed and kept in the policy manual. (See "[Facilitating Communication](#)" Policy No. 2-040.)

3. A copy of this policy will be posted in the reception area of Organization's Name, given to each organization staff member, and sent to each referral source.
4. A nondiscrimination statement (See #5) will be posted in a conspicuous place, such as the reception area of the organization and will be printed on brochures, other printed public materials and in a conspicuous location on the organization's web site accessible from the home page, in English and at least the top 15 non-English languages spoken in the state.
5. The nondiscrimination statement will read: *"Organization's Name complies with applicable Federal civil rights laws and does not discriminate on the basis of race, color, national origin, age, disability, or sex. Organization's Name does not exclude people or treat them differently because of race, color, national origin, age, disability, or sex. Organization's Name provides free aids and services to people with disabilities to communicate effectively with us, such as qualified sign language interpreters and written materials in other formats (e.g. large print, audio, accessible electronic formats). Organization's Name provides free language services to people whose primary language is not English such as qualified interpreters and information written in other languages. If you need these services, contact the Section 504/ADA Coordinator/Section 1557 Civil Rights Coordinator at _____ (insert phone number). If you believe that Organization's Name has failed to provide these services or discriminated in another way on the basis of race, color, national origin, age, disability, or sex you can file a grievance with _____ (insert name and title of ADA/Civil Rights Coordinator) _____ (insert mailing address) _____ (insert telephone number and TTY number if available) _____ (insert fax) _____ (insert email). You can file a grievance in person or by mail, fax, or email. If you need help filing a grievance, _____ (insert name and title of ADA/Civil Rights Coordinator) is available to help you. You can also file a civil rights complaint with the U.S. Department of Health and Human Services, Office for Civil Rights, electronically through the Office of Civil Rights Compliant Portal, available at <https://ocrportal.hhs.gov/ocr/portal/lobby.jsf>, or by mail or phone at: U.S. Department of Health and Human Services, 200 Independence Avenue SW, Room 509F, HHH Building, Washington, DC 2020; 1-800-368-1019, 800-537-7697(TDD)"*
6. Any person who believes she or he has been subjected to discrimination or who believes he or she has witnessed discrimination, in contradiction of the policy stated above, may file a grievance under this procedure. It is against the law for Organization's Name to retaliate against anyone who files a grievance or cooperates in the investigation of a grievance.
7. Grievances must be submitted to the Section 504/ADA Compliance Coordinator/ Section 1557 Civil Rights Coordinator within 60 days of the date the person filing the grievance becomes aware of the alleged discriminatory action.
8. A complaint may be filed in writing, or verbally, containing the name and address of the person filing it ("the grievant"). The complaint must state the problem or action alleged to be discriminatory and the remedy or relief sought by the grievant.

9. The Section 504 Coordinator/Section 1557 Civil Rights Coordinator (or her/his representative) will conduct an investigation of the complaint to determine its validity. This investigation may be informal, but it must be thorough, affording all interested persons an opportunity to submit evidence relevant to the complaint.
10. The Section 504/ADA Compliance Coordinator/ Section 1557 Civil Rights Coordinator will issue a written decision on the grievance no later than 30 days after its filing.
11. The grievant may appeal the decision of the Section 504/ADA Compliance Coordinator/Section 1557 Civil Rights Coordinator by filing an appeal in writing to Organization's Name within 15 days of receiving the Section 504/ADA Compliance Coordinator/Section 1557 Civil Rights Coordinator's decision.
12. Organization's Name will issue a written decision in response to the appeal no later than 30 days after its filing.
13. The Section 504/ADA Compliance Coordinator/Section 1557 Civil Rights Coordinator will maintain the files and records of Organization's Name relating to such grievances.
14. The availability and use of this grievance procedure does not preclude a person from filing a complaint of discrimination on the basis of handicap with the regional office for Civil Rights of the U.S. Department of Health and Human Services.
15. All organization personnel will be informed of this process during their orientation process.
16. Organization's Name will make appropriate arrangements to assure that persons with disabilities can participate in or make use of this grievance process on the same basis as the nondisabled. Such arrangements may include, but will not be limited to, the providing interpreters for the deaf, providing taped cassettes of material for the blind, or assuring a barrier-free location for the proceedings. The Section 504 Coordinator will be responsible for providing such arrangements.

FACILITATING COMMUNICATION**Policy No. 2-040.1****PURPOSE**

To assure that patients, visitors, and personnel with speech, vision, or hearing impairments, as well as those who have a limited command of the English language, have access to appropriate interpretive assistance and other aids at no cost to patient(s) being served. This is in compliance with Section 504 of the Rehabilitation Act of 1973 and Title VI of the Civil Rights Act of 1964 and Section 1557 of the Affordable Care Act (ACA) of 2010.

POLICY

Organization's Name does not discriminate against any person because of language or sensory impediments. Personnel will treat all patients with respect and dignity and will use forms of communication appropriate to meet the patient's needs.

Written materials will be made available in the recognized major languages in the area and state where the organization does business. Written materials will contain the telephone number of the local TDD telephone relay number. Organization personnel will consistently and clearly communicate with patients in a language or form they can reasonably understand. The organization will facilitate communication by using special devices, interpreters or other communication aides.

Definitions

1. ***Hearing Impaired:*** A hearing impaired individual has difficulty hearing or discriminating verbal conversation either in a face to face situation or over the telephone. An individual with this impairment may require a hearing aid, telephone amplifier, TDD or sign language interpreter.
2. ***Communicatively Impaired:*** A communicatively impaired individual has expressive or receptive language deficits that may be present after an illness or injury. This may include individuals with voice disorders, laryngectomy, glossectomy or cognitive disorders.
3. ***Limited English Proficiency (LEP):*** A person with Limited English Proficiency is one whose command of the English language is not sufficient to promote meaningful interaction for service.
4. ***Telecommunication Device for the Deaf (TDD):*** A TDD is a small, typewriter-style instrument that allows a person to make or receive a telephone call directly without using another person to interpret.

PROCEDURE

1. The initial assessment will determine the patient's communication ability. If there is any question regarding the patient's ability to communicate properly, the assessment should stop and an interpreter should be secured.
 - A. If the patient refuses offered communication assistance, the clinician will document the refusal in the patient's clinical record.
2. All patient related forms, such as Consent for Services, Complaint/Grievance Process, etc., will be available in English, and other languages as appropriate. If there is a need to translate these forms to another language, an interpreter will be secured. (The patient will also sign an English language version in the event that the authorization needs to be forwarded to another organization.) For all other patients speaking languages other than English, Organization's Name will secure an interpreter to interpret all organization policies and procedures of Organization's Name relevant to the care of the patient. (See ["Organization List of Interpreters"](#) Addendum 2-040.A for listing.)
3. For visually impaired patients:
 - A. The admitting clinician will read aloud all documents normally provided to the patient and ascertain that the person has heard and understands what was read. The admitting clinician documents this in the clinical/service record.
 - B. A clinician will make available large print patient information that may be available applicable to the disease process, i.e., colostomy, diabetic care, heart disease.
4. For hearing impaired patients:
 - A. The admitting clinician ascertains the patient's preferred methods of communication, i.e., paper and pencil, lip reading, or sign language.
 - B. If the preferred method is sign language, the admitting clinician will contact the resource providing a sign language interpreter and establishes a plan for ongoing communications. (See ["Organization List of Interpreters"](#) Addendum 2-040.A.)
 - C. Obtaining the use of a TDD:
 1. Hearing or communicatively impaired individuals who have access to a TDD instrument can call the Relay Service to enable them to communicate with personnel of the organization.
 2. In the event it becomes necessary for organization personnel to initiate telephone communication with an individual who is hearing or communicatively impaired, Relay Service can be utilized.
 3. Direct Access to a TDD instrument will be available.
 4. Local TDD Number: _____ (insert telephone number.)

5. For patients who have limited English proficiency (LEP) and require an interpreter:
 - A. The organization will maintain a list of organization personnel who speak languages other than English. (See "[Organization List of Interpreters](#)" Addendum 2-040.A to assign an interpreter to non-English speaking patients.)
 - B. If an interpreter in the required language is not available, a telephone interpreting service may be used. Directions for the use of this service can be accessed by calling that service.
 - C. Face-to-face interactions with the patient who is hearing or communicatively impaired, will be facilitated by utilizing sign language, as listed under "American Sign."
 - D. If a certified sign language interpreter is required, the resource list should be consulted. As much advance notice as possible is preferred.
 - E. Family members or friends of the patient will not be used as interpreters unless specifically requested by the individual and **after** the patient has understood that an offer of an interpreter at no charge has been made. Such an offer and response will be documented in the patient's record. If the patient chooses to use a family member or friend as an interpreter, issues of competency of interpretation, confidentiality, privacy, and conflict of interest will be considered. If the family member or friend is not competent or appropriate for any of these reasons, competent interpreter services will be provided to the patient. Children will **not** be used to interpret, in order to ensure confidentiality of information and accurate communication.
 - F. Document all language assisted care in the patient's clinical record.
 - G. If applicable, a copy of the telephone utilization report from the interpreting service will be maintained to substantiate the use of the interpreting service.
6. For communicatively impaired patients (e.g. personal with disabilities):
 - A. Patients with speech, expressive or receptive language deficits should have a consult with a speech therapist to determine appropriate, effective use of auxiliary assistive devices such as flash cards, communication board, accessible websites, other services, etc.
 - B. Physician approval for the consultation will be obtained.
7. For patients with different beliefs, backgrounds, and religions that may impact the patient's lifestyle, habits, view of health, healing, terminal illness, dying and bereavement:
 - A. During orientation: care of the whole person/beliefs/religions will be discussed.
 - B. Annual staff education: new emerging cultures/beliefs and access to organization interpreters will be discussed.

ADDENDUM 2-040.A

ORGANIZATION LIST OF INTERPRETERS

(Include interpreters for sign language, verbal and/or sensory impaired, and non-English speaking patients.)

(Note: Include Telephone Interpreting Service number, if applicable.)

PURPOSE

Organization's Name is committed to prevention, detection, and to taking all appropriate action to assure compliance with all legal and regulatory statutes and to promote honest and ethical behavior in all work-related activities.

POLICY

Organization's Name has established this plan to ensure that quality patient care is provided in a manner that fully complies with all applicable state and federal laws and regulations. It is the policy of Organization's Name that (1) all employees are educated about the applicable laws and trained in matters of compliance, (2) there is periodic auditing, monitoring and oversight of compliance with those laws, (3) there exists an atmosphere that encourages and enables the reporting of non-compliance without fear of retribution, (4) responsibility is not delegated to persons with a propensity to act in a non-compliant manner, and (5) mechanisms exist to investigate, discipline and correct non-compliance.

The plan provides for the existence of a Corporate Compliance Officer (CCO) who has ultimate responsibility and accountability for compliance matters. **However, each individual employee of Organization's Name remains responsible and accountable for his or her own compliance with applicable laws.** Confirmed acts of non-compliance will be disciplined, including termination.

PROCEDURE

Assignment of Corporate Compliance Officer

1. There shall be appointed a Corporate Compliance Officer, reporting to the Organization's Name Administrator/CEO and Governing Body (Compliance Committee of the Board).
2. The CCO oversees the education of personnel regarding proper compliance, the auditing and monitoring of the statutes of compliance, and the reporting, investigation, discipline and correction of non-compliance. It is also his/her responsibility to ensure programs are in place to guarantee that significant discretionary authority is not delegated to persons with a demonstrated or suspected propensity for improper or unlawful conduct. It is not expected that the CCO will have the knowledge or expertise necessary to ensure compliance with all laws and regulations that affect Organization's Name. He/she is responsible, however, for the overall programs and must ensure that qualified, knowledgeable personnel assist in monitoring and educational functions.
3. The CCO reports on the compliance plan to the Quality/ Performance Improvement (QI or PI) or Compliance Committee (at least quarterly) and The Governing Body (at least annually). The report includes but is not limited to:

- A. The level of compliance or non-compliance found as a result of monitoring and auditing (both internal and external)
 - B. The success of efforts to improve compliance, including training and education
 - C. The non-delegation of discretionary authority to those with the propensity to act improperly
 - D. Corrective or disciplinary action taken with respect to those found to be non-compliant.
4. The CCO may appoint such staff as deemed necessary to assist in the performance of the responsibilities outlined above.

Employee Reporting

- 1. All employees have the responsibility to comply with applicable laws and regulations and to report any acts of non-compliance.
- 2. Any employee who perceives or learns of an act of non-compliance should either speak to his/her supervisor, call the CCO, call the Compliance Hot Line at (insert your number here) or place information in the Corporate Compliance lockbox located in an easily accessible area. Supervisors are required to report these issues through established management channels and/or the CCO. Reports may be made anonymously, although giving a name and phone number generally makes investigating reports easier and more effective. All employees are encouraged to call the hot line if they have any question about whether their concern should be reported. A written record of every report received will be kept for a period of six (6) years. Every effort will be made to preserve the confidentiality of reports of non-compliance (although calls made anonymously will always preserve the autonomy of the caller). All employees must understand, however, that circumstances may arise in which it is necessary or appropriate to disclose information. In such cases, disclosures will only be made as necessary.
- 3. All employees are required to report acts of non-compliance. Any employee found to have known of such acts, but who failed to report them, will be subject to discipline.
- 4. No employee shall, in any way, retaliate against another employee for reporting an act of non-compliance. Acts of retaliation should also be reported to the hot line/lockbox and will be investigated by the CCO or his/her designee. Any confirmed act of retaliation shall result in discipline.

Investigation of Non-Compliance

- 1. The CCO or their designee(s) will investigate every report of non-compliance whether reported through the hot line or otherwise. Investigations will be done promptly and will consist of interviewing personnel, examining documents, and consulting with legal counsel, if necessary.

2. The CCO or their designee(s) have full authority to interview any employee and review any document (subject to state and federal laws on patient confidentiality) he/she deems necessary to complete the investigation.
3. A written record of each investigation will be created and maintained by the CCO. He/she will make every effort to preserve the confidentiality of such records and will make any necessary disclosures on a "need-to-know" basis only. (See Addendum 2-041.A Sample Compliance Report)
4. The CCO will report the results of each investigation considered significant to the Administrator/CEO. He/she will recommend a course of discipline and/or other corrective action. Sanctions for non-compliance may be imposed.

Corrective Action or Discipline (See HR Policy No. 1-037 "Progressive Discipline Policy")

1. Every confirmed act of non-compliance may result in corrective action or discipline. The sanction for a single act of non-compliance will be decided by the CCO. The Administrator/CEO may advise on sanctions for severe or repeated instances of non-compliance. Sanctions may include, but are not limited to, a requirement to follow a certain process or procedure in the future, restitution, and/or discipline including termination.

Training (See HR Policy No. 1-022 "Orientation," Policy No. 1-023 "Personnel Development," and Policy No. 1-019 "Standards of Conduct/Ethical Behavior")

1. The CCO will monitor the education of employees concerning the existence of the compliance plan, the contents of the plan, and the need to abide by the specific laws and regulations. The CCO will ensure that employees receive a copy of the Standard of Conduct. He/she will inform employees of changes in the laws or regulations periodically and systematically through written communications and in-service training.
2. All current and new employees will have access to the plan. A copy will appear in the Employee Handbook. All new employees will be oriented to the plan and all employees will receive annual inservice training regarding the plan.

Monitoring and Auditing

1. The CCO and Compliance Committee will conduct an annual risk assessment to identify possible risk areas that need to be addressed by the annual compliance plan. The OIG identified risk areas will be addressed. The CCO and Compliance Committee will determine the areas that need continued monitoring and will develop tools to monitor those areas/processes.
2. The CCO will be responsible for monitoring employees' compliance with applicable laws and regulations. He/she will ensure that the level of compliance with the Conditions of Participation is audited at least annually. He/she will arrange as well for external auditing as deemed necessary.

3. If the CCO discovers that a team's/department's or individual's level of compliance is unacceptable, he/she may impose a plan of corrective action, which may include future monitoring of an individual, team/department or specific process on a more frequent basis. Corrective actions and sanction for acts of non-compliance will be managed as outlined previously.
4. Annual audit and monitoring plans will be developed based on topics addressed in the annually published OIG work plan, CMS fraud alerts, previous audit findings and areas identified internally as needing improvement.

A. Audit topics may include, but are not limited to:

1. Signed Bill of Rights
2. Signed Physician orders
3. Adherence to plan of care
4. Home Health Aide supervision
5. C0F0S0
6. One-time nursing visit
7. Therapy utilization
8. Homebound status
9. Face-to-face encounter visit made
10. Therapy reassessments made at appropriate timeframes

B. Billing Accuracy

1. All claims for services submitted to health care programs (Medicare, Medicaid, commercial insurance, etc.) for reimbursement will accurately reflect the services ordered and performed. All billing information will be provided to the appropriate payer using accurate information including patient name and address, date(s) of services, date of birth, and service identifiers (CPT-4 codes, HCPCS, Revenue codes or Rate codes).
 - a. Billing Codes (CPT-4, HCPCS, Rate or Revenue Codes) used to bill will accurately describe the service performed and will be payer-specific.
 - b. The physician's order will not be altered in any manner (increasing or decreasing the number and/or types of services) without the written consent of the ordering physician.
 - c. Billing Code accuracy is reviewed at the initiation of a service.

- d. Intentionally or knowingly upcoding a service to maximize reimbursement is forbidden and will result in disciplinary action.

Approved Billing Practices

1. Organization's Name bills:
 - A. For medically necessary services
 - B. For services ordered and rendered
 - C. Medicare for patients that meet eligibility criteria
 - D. For services rendered that are supported by documentation in the patient's charts
 - E. For services provided by qualified and licensed clinical personnel
2. Organization's Name will utilize the following billing/accounting practices:
 - A. Identify and return overpayments promptly to the appropriate payor
 - B. Allow for posting of defined costs to the cost report
 - C. Work credit-balances in a timely fashion for all payers

Effectiveness of Compliance Program

1. The effectiveness of the Compliance Program will be evaluated and reported to the Governing Body, senior management and all employees. The following will be utilized to determine effectiveness:
 - A. Documentation that employees were adequately trained
 - B. Reports from the hotline, including nature and results of any investigation
 - C. Documentation of corrective action, including disciplinary action taken and policy improvements introduced
 - D. Modifications to the Compliance Program
 - E. Self-disclosures, if applicable
 - F. Results of the auditing and monitoring efforts

ADDENDUM 2-041.A

SAMPLE COMPLIANCE REPORT

(Insert organization reporting form)

COMPLIANCE REPORT
(CONFIDENTIAL)

Report Date: _____

Received by: _____

Made by: _____

Method of Reporting:☐

In Person

☐

Telephone Conference

☐

Letter

Contact/Drop Box

Full description of Issue: (use back of form if necessary)

Action Taken by Corporate Compliance Officer:☐

Not a compliance issue—redirect individual to appropriate manager. Discuss with Administrator/CEO.

☐

Begin investigative process (describe process below).

☐

Share on a need-to-know basis. List individuals who have knowledge of incident.

☐

Contact legal counsel.

Comments:

Resolutions:☐Re-education effort
Policy/Procedure☐

Disciplinary action

☐

Changes in

Date completed: _____
completed:

Date completed: _____

_____ Date

☐

Disclosure to appropriate agency:

Date completed: _____

Comments:

PURPOSE

Organization's Name is committed to prevention, detection, and to taking all appropriate action to assure compliance with all legal and regulatory statutes and to promote honest and ethical behavior in all work-related activities.

POLICY

Organization's Name Governing Body will designate a Corporate Compliance Officer (CCO) who is responsible for the day to day oversight of the Compliance Plan and coordination of all compliance initiatives.

PROCEDURE

1. The primary duties of the Compliance Officer include but are not limited to the following:
 - A. To develop and monitor the implementation of the Compliance Program.
 - B. To develop annual work programs and protocols which address key initiatives to be undertaken based on the organization's policies, perceived areas of risk and governmental focus on compliance issues.
 - C. To periodically review and recommend revising the Compliance program as necessary to meet the changing needs of the organization in its business and regulatory environment.
 - D. To oversee the coordination of billing, human resources, contracting and other department-specific compliance issues and plans with appropriate senior management to assure compliance with the Compliance Plan and compliance issues.
 - E. To oversee training and monitoring of new and existing employees with the respective education departments or operating departments to assure that employees receive timely and accurate information and sign affirmation statements in a timely manner.
 - F. To oversee that significant outside contractors comply with the organization's Standards of Conduct
 - G. To oversee compliance with the organization's contracting policies.
 - H. To develop methodologies to address any issues which arise from audits and other oversight measures on a timely basis.
 - I. To assure that monitoring activities are conducted in a non-threatening basis.

- J. To assure a confidential communications process for solicitation, evaluation and response to complaints and problems. (Respond to Hotline issues).

PURPOSE

Organization's Name is committed to prevention, detection, and to taking all appropriate action to assure compliance with all legal and regulatory statutes and to promote honest and ethical behavior in all work-related activities.

POLICY

An ongoing evaluation process will be established utilizing existing and new monitoring procedures to assure compliance. An annual evaluation of the compliance program's intent and effectiveness will also be completed. Reports will be provided to Administration and the Governing Body. This policy includes key areas for monitoring, and is not intended to be all-inclusive of monitoring activities. Monitoring controls identified are categorized by functional areas and include, but are not limited to, those issues currently targeted by the Officer of the Inspector General. This list will be updated as necessary.

PROCEDURE

1. Administration/Financial

- A. All employees will be informed of the Standards of Conduct and report any suspected violations to their supervisor, or the Corporate Compliance Officer to reasonably ensure that all activities are in compliance with the Standards of Conduct (See Addendum 2-043.A "[Kickbacks, Inducements and Self-Referral](#)"). An annual audit of the Corporate Compliance Plan will be completed.
- B. The organization will have an annual financial audit conducted by its certified public accountants to examine, on a test-basis, evidence supporting the proper handling and reporting of amounts, and disclosures relating to, its financial activity.
- C. Organization's Name will conduct an annual review of the Cost Report to assure proper allocation of costs to specific programs, assess the allowableness of G&A expenses, marketing and advertising expenses and proper recording of related expenses.
 1. Time sheets are reviewed to determine accuracy and completeness. Payroll staff will follow-up on any unusual program code or cost center allocations.
 2. The organization will perform periodic audits of expenditures to determine compliance with organization's policies. All expenses are monitored to evaluate accuracy and validity.

- D. Organization's Name will conduct and document annual reviews with its third party contractors to reasonably ensure that activities are in compliance with the Standards of Conduct and related policies.
- E. The organization will conduct an annual review of the Conflict of Interest policy with all employees and Governing Body
- F. The organization will conduct an annual review of compliance with the terms, conditions and covenants contained in its financing loan agreements.
- G. Organization's Name will conduct and document an annual review of its billing practices to reasonably ensure that all activities are in compliance with the Standards of Conduct and organization policies, third party billing requirements, and that billing occurs for only those services provided, documented, there are signed physician orders, and face-to-face encounter was made.

2. Human Resources (HR) Department

- A. Organization's Name will conduct an annual audit to assure that the Standards of Conduct and the agency personnel policies and procedures are being followed. The audit will include, but is not limited to:
 - 1. Pre-employment interview sheets completed, reference checks completed and documented.
 - 2. Licensure, certification and/or registration of professional staff is verified and maintained throughout the employee's tenure.
 - 3. Criminal background completed on all new employees. Drivers' license checks on field staff and driving positions.
 - 4. Systems exist and are effective in tracking performance evaluations, professional licenses, inservice education (including mandatories).
 - 5. Process exists and is effective for investigation/follow-up on work-related injuries.
 - 6. OIG/GSA exclusion checks are completed and documented.

3. Clinical/Documentation Compliance

A. Patient's Eligibility and Appropriate Documentation

The homebound and skilled care/medical necessity requirements for Medicare coverage are assessed utilizing several methods:

- 1. Supervisory case conferences with staff
- 2. Supervisory visits with staff

3. Audits of Start of Care and subsequent recertification documentation

B. Patient Abandonment/Discriminatory Admission and Discharge Practices

1. Organization's Name will ensure that patients will be admitted and discharged according to the organization policy.

C. Physician's orders

Mechanisms are currently in place to assure timely signed physician orders prior to billing.

1. Order tracking on a daily basis by the Health Information Management/Department of Medical Records. A physician signature log will be maintained.

2. Staff will send regular follow-up notices to physicians that do not return signed orders in a timely manner.

3. Physician License Verification—A mechanism exists whereby Organization's Name knows that a physician is licensed to practice and whether they have been precluded from the Medicare/Medicaid Program.

D. Payer Changes

A process is in place to assure that payer changes occur on a timely basis and that meets the requirement for patient notification (ABN-Advance Beneficiary Notice) within the required time frames.

E. Home Health Aide Services

This includes the following:

1. Appropriateness of level of worker for tasks needed to be completed
2. Personal care necessary, ordered and provided for payer coverage
3. Care Plan matches orders, aide schedule matches orders and services provided match documentation, orders and bill generated.
4. Aide supervisory visits are performed and documented every 14 days.

F. Medical Records

A process is in place to assure that medical records are appropriately secure and that record retention meets the time frames required by regulation and accrediting bodies.

G. Denial Tracking

Denials for services rendered and bills are tracked and data compiled quarterly to determine any needed corrective action with respect to re-education, specific caregiver problems or any other trends.

H. Marketing/Sales

1. Marketing materials will be reviewed annually for accuracy in regard to applicable statutes and regulations
2. Marketing and sales staff will be oriented to anti-kickback and Stark regulations

I. Information Technology

1. Systems are evaluated to ensure privacy and security of PHI
2. Device integrity is maintained through appropriate software updates

J. Billing and Intake

1. Processes are in place to ensure the following does not occur:
 - a. Duplicate billing/improper billing
 - b. Billing for services not rendered
 - c. Billing for services provided by unqualified or unlicensed personnel
 - d. Billing for service without signed physician orders

ADDENDUM 2-043.A

KICKBACKS, INDUCEMENTS AND SELF-REFERRAL

KICKBACKS, INDUCEMENTS AND SELF-REFERRAL**Policy No. 2-043.A****PURPOSE**

To identify marketing/sales/community awareness practices that are allowable and consistent with the Organization's Compliance Program, Anti-Kickback statute, Stark Law, Civil Monetary Penalties Law and the OIG Program Guidance for Home Health agencies.

POLICY

Organization's Name employees, contractors and representatives that interact with patients, potential patients, relatives of patients, physicians, hospitals, discharge planners, contractors, and assisted living facilities will interact within the rules and regulations governing such relationships for all business lines.

PROCEDURE

1. The organization will ensure that:
 - A. No item or service of value will be given or received in exchange for a business decision or to induce referrals
 - B. No soliciting, accepting, or offering of any gifts or gratuity of more than of nominal value by or between organization employees, to or from clients/patients
 - C. There is no soliciting or receiving of items of value in exchange for providing a supplier access to clients'/patients' medical records and other information needed to bill any payor
 - D. The organization will not enter into any agreement with a supplier of goods and services whereby the organization receives discounts on certain items and services in return for making referrals for other types of services
 - E. The organization will not enter into any joint ventures with entities that supply goods and services to the organization designed to induce referrals
 - F. There are no financial agreements with vendors whereby the organization receives non-covered items (e.g. disposable diapers) at below market prices or for no charged in return for ordering Medicare-reimbursed products from those vendors
 - G. There are no incentives, financial arrangements, contracts, or joint ventures with referral or potential referral, sources designed to induce referrals
 - H. Coinsurance and deductible amounts are not waived, and that a good faith effort is made to determine that a client/patient is in financial need prior to abandoning collection efforts

- I. The organization and all representatives do not engage in improper solicitation or high-pressure marketing with patients
2. The Corporate Compliance Officer will periodically review marketing materials and marketing/sales documentation to ensure compliance with the anti-kickback statutes.
3. This policy will be reviewed at orientation and during annual mandatory education.

PURPOSE

To provide organization employees with guidelines for participation in social media, including Organization's Name hosted social media and non-hosted social media in which the employee's affiliation is known, identified, or presumed.

POLICY

Organization's Name recognizes the value of online social media sites and blogs as vital resources to positively promote the organization's mission, values, operational goals, marketing, and recruitment activities.

DEFINITIONS

Blog: a blog is a website maintained by an individual or organization with regular entries of commentary, descriptions of events, or other materials such as graphics or video.

Podcast: a collection of digital media files distributed over the Internet, often using syndication feeds, for playback on portable media players and personal computers.

Protected Health Information (PHI): individually identifiable information (oral, written, or electronic) about a patient's physical or mental health, the receipt of health care, or payment for that care.

RSS feeds or Syndication feeds: a family of different formats used to publish updated content such as blog entries, news headlines or podcasts and "feed" this information to subscribers via email or by an RSS reader.

Social media: includes but are not limited to blogs, podcasts, discussion forums, online collaborative information and publishing systems that are accessible to internal and external audiences (i.e., Wikis), RSS feeds, video sharing, and social networks like Facebook, Twitter, Instagram and LinkedIn.

Wiki: allows users to create, edit, and link Web pages easily; often used to create collaborative sites (called "Wikis") and to power community Web sites.

PROCEDURE

1. This policy applies to employees using social media while at work. It also applies to the use of social media when away from work, when the employee's organization affiliation is identified, known, or presumed. It does not apply to content that is non-home care related or is otherwise unrelated to the organization.

2. The organization will establish a social media oversight committee to develop and manage the social media and blog communication strategy. Participants may include representatives from Marketing, Human Resources, Information Technology, Corporate Compliance, and Education. The Compliance/Privacy Officer will chair this committee.
3. Employees may not post any material that is obscene, defamatory, profane, libelous, threatening, harassing, abusive, hateful, embarrassing or discriminatory manner on the basis of age, race, sex or sexual orientation to another person or entity when posting to organization-hosted sites.
4. Employees may not post content that fails to comply with any and all applicable local, state, and federal laws. Employees must abide by the copyright laws by ensuring that they have permission to use or reproduce copyrighted photos, graphics, text, video, or other material owned by others.
5. All uses and disclosures of patient identifying health information (PHI) is strictly prohibited without the express written authorization for the use and disclose of the information from the patient/patient's representative.
6. On non-organization hosted sites, employees may not disclose any confidential or proprietary information about the organization, represent that they are communicating the organization's views or do anything that might create the impression that they are communicating on behalf of the organization.
7. The social media oversight committee shall serve as a resource for questions and concerns regarding the appropriate use of social media and blogs by the employees. ("[Social Media and Blog Guidelines](#)" Addendum 2-044.A.)
8. The inappropriate use of social media or blogs by the employee that conflicts with the organization's mission and values, violates administrative policies and procedures, and/or compromises the privacy and security of confidential patient health or proprietary business information shall be subject to corrective action, up to and including termination. In addition, breach of confidential patient health information may also be subject to legal proceedings and/or criminal charges.

Disclaimer: This policy will not be applied in a way that interferes with an employee's rights under the National Labor Relations Act.

ADDENDUM 2-044.A

SOCIAL MEDIA AND BLOG GUIDELINES

SOCIAL MEDIA AND BLOG GUIDELINES

These guidelines are suggestions of dos and don'ts of social media and blogs. This list is not inclusive.

1. Get approval: Do not announce organization news on a social media site or blog. Do not reference clients, patients, or partners without their approval.
2. Don't betray our patient's trust: Disclosing confidential patient Protected Health Information (PHI) in an inappropriate manner is a federal offense. Even acknowledging the care of a patient is an unacceptable disclosure of PHI.
3. Don't cheat your employer: Social media sites may be addictive in nature. Employees should not be checking their Facebook updates or other sites when they are supposed to be doing their job.
4. Use a disclaimer: If you publish a blog, post a comment, or share an image and it has something to do with the work you do, make it clear that what you say is your view and opinions and not necessarily the views and opinions of the organization.
5. Respect copyright laws.
6. Don't jeopardize your reputation and/or future employment opportunities: You should consider everything you post online begins to build a lifetime record of you.
7. Be accurate: Respect the facts and link to the trusted sources that validate your opinions.
8. Be professional: Employees are reminded that statements made in the confines of private blogs and chat rooms must treat the organization and its employees, clients, and competitors with respect.

PURPOSE

To establish a procedure to prevent, detect, contain and correct privacy violations involving protected health information.

POLICY

The organization will identify risks and safeguard protected health information against loss, destruction, tampering, or unauthorized use.

PROCEDURE

1. Organization's Name will designate a privacy official responsible for responsible for developing and implementing its privacy policies and procedures.
2. Organization's Name will identify a contact person responsible for receiving complaints and providing individuals with information on the organization's privacy practices.
 - A. The Notice of Privacy Practice will include information for individuals to file complaints regarding privacy both to the organization and to the Secretary of Health and Human Services.
 - B. Organization's Name will not retaliate against a person for exercising rights provided by the Privacy Rule, for assisting in an investigation by HHS or another appropriate authority, or for opposing an act or practice that the person believes in good faith violates the Privacy Rule.
 - C. Organization's Name will not require an individual to waive any right under the Privacy Rule as a condition for obtaining treatment.
3. Organization's Name will train all employees and volunteers on the privacy policies and procedures as necessary and appropriate for them to carry out their functions.
4. Organization's Name will conduct an accurate and thorough assessment annually of the potential risks and vulnerabilities to the confidentiality, integrity, security and availability of electronic protected health information. If the organization makes any changes to hardware, software, etc. that affects ePHI, a risk assessment will be completed to identify potential vulnerabilities and will mitigate risks based on that assessment.

5. Organization's Name will implement security measures sufficient to reduce risks and vulnerabilities to a reasonable and appropriate level to comply with HIPAA security standards.
6. Organization's Name will apply appropriate sanctions against workforce members who fail to comply with the organization's security policies and procedures.
7. Organization's Name will mitigate, to the extent practicable, harmful effect it learns was caused by use or disclosure of protected health information by its employees, volunteers or business associates in violation of its privacy policies and procedures or the Privacy Rule. Potential steps to mitigation include:
 - A. Identify the cause of the violation and amend privacy policies and technical procedures, as necessary to assure it does not happen again.
 - B. Limit the further distribution of improperly disclosed information.
 - C. Follow breach notification protocol.
8. Organization's Name will maintain, until six (6) years after the later of the date of their creation or last effective date, its privacy policies, procedures, its privacy practices notices, disposition of complaints, and other actions, activities, and designations that the Privacy Rule requires to be documented.

PURPOSE

To comply with the Health Insurance Portability and Accountability Act (HIPAA) Privacy, Security, and Enforcement Rules and state regulations.

POLICY

Organization's Name will conduct an analysis for breach notification when there is an impermissible use or disclosure of protected health information.

PROCEDURE

1. An impermissible use or disclosure of protected health information is presumed to be a breach unless the organization has demonstrated that there is a low probability that the protected health information has been compromised.
2. Organization's Name will conduct an analysis of any acquisition, access, use, or disclosure of protected health information in a manner not permitted by Privacy and Security Rules for use in determining breach notification requirements.

The analysis will include:

- A. The nature and extent of the protected health information involved, including the types of identifiers and the likelihood of re-identification;
- B. The unauthorized person who used the protected health information or to whom the disclosure was made;
- C. Whether the protected health information was actually acquired or viewed; and
- D. The extent to which the risk to the protected health information has been mitigated.
- E. Other areas as required by state regulations.

PURPOSE

To comply with the Health Insurance Portability and Accountability Act (HIPAA) Privacy, Security, and Enforcement Rules and state regulations.

POLICY

Organization's Name will comply with all federal and state requirements for notification following a breach.

PROCEDURE

1. Affected individuals and the California Department of Public Health will be notified of any unlawful or unauthorized access to, or use or disclosure of, a patient's medical record information as soon as possible but no later than fifteen (15) business days from the date of the discovery of the breach. (Federal guidelines are no later than sixty (60) calendar days)
 - A. A breach is considered discovered when the organization becomes aware of the breach.
 - B. The discovery standard applies to employees and agents of the organization, including business associates.
2. Any notification to affected individuals will include the following, to the extent possible:
 - A. A brief description of what happened, including the date of the breach and the date of the discovery of the breach, if known;
 - B. A description of the types of unsecured protected health information that were involved in the breach (such as whether full name, social security number, date of birth, home address, account number, diagnosis, disability code, or other types of information involved);
 - C. Any steps individuals should take to protect themselves from potential harm resulting from the breach;
 - D. A brief description of what the organization is doing to investigate the breach, to mitigate harm to individuals, and to protect against any further breaches; and
 - E. Contact procedures for individuals to ask questions or learn additional information, which will include a toll-free telephone number, an e-mail address, Web site, or postal address.
3. Notification to affected individuals will occur as follows:

A. Written Notice:

1. Affected individuals should receive written notification by first-class mail to the individual at the last known address, or if the individual agrees, by electronic mail. Agreements to receive notification by electronic mail must be in writing. The notification may be provided in one or more mailings as information is available.
2. The California Attorney General must be sent a sample notice if more than 500 California residents were required to be notified.
3. If the organization knows the individual is deceased and has the address of the next of kin or personal representative, written notification will occur by first-class mail to either the next of kin or personal representative of the individual.

B. Substitute notice.

1. In the case of insufficient or out-of-date contact information, a substitute form of notice reasonably calculated to reach the individual shall be provided.
2. In the case of insufficient or out-of-date contact information for fewer than 10 individuals, then substitute notice may be provided by an alternative form of written notice, telephone, or other means.
3. In the case in which there is insufficient or out-of-date contact information for 10 or more individuals, then such substitute notice shall:
 - a. Be in the form of either a conspicuous posting for a period of 90 days on the home page of the Web site of the organization, or conspicuous notice in major print or broadcast media in geographic areas where the individuals affected by the breach likely reside; and
 - b. Include a toll-free phone number that remains active for at least 90 days where an individual can learn whether the individual's unsecured protected health information may be included in the breach.

C. In any case deemed by Organization's Name to require urgency because of possible imminent misuse of unsecured protected health information, the organization may provide information to individuals by telephone or other means, as appropriate, in addition to notice provided under paragraph 3.A. of this section.

4. The organization shall delay the reporting of any unlawful or unauthorized access to, or use or disclosure of, a patient's medical information beyond fifteen (15) business days if a law enforcement agency or official provides the organization with a written or oral statement that compliance with the reporting requirements would likely impede the law enforcement agency's investigation that relates to the unlawful or unauthorized access to, and use or disclosure of, a patient's medical information and specifies a date upon which the delay shall end, not to exceed sixty (60) days after a written request is made, or thirty (30) days after an oral request is made.

5. If the statement of the law enforcement agency or official is made orally, the organization shall do the following:
 - A. Document the oral statement, including, but not limited to, the identity of the law enforcement agency or official making the oral statement and the date upon which the oral statement was made.
 - B. Limit the delay in reporting the unlawful or unauthorized access to, or use or disclosure of, the patient's medical information to the date specified in the oral statement, not to exceed thirty (30) calendar days from the date that the oral statement is made.
6. The organization shall submit a report that is delayed no later than fifteen (15) business days after the date designated as the end of the delay.
7. For a breach of unsecured protected health information involving more than 500 California residents, Organization's Name shall, following the discovery of the breach, notify prominent media outlets serving the State or jurisdiction without unreasonable delay and in no case later than 60 calendar days after discovery of a breach.
8. Organization's Name will notify HHS in the manner specified on the HHS Web site of any breaches involving 500 or more individuals.
9. Organization's Name will maintain a log for breaches of unsecured protected health information involving less than 500 individuals. Not later than 60 days after the end of each calendar year the organization will notify HHS, in the manner specified on the HHS web site, of breaches discovered during the preceding calendar year.
10. Organization's Name maintains responsibility for notification of breaches involving business associates, even when notification responsibility has been delegated to the business associate.
11. Organization's Name will review state regulations to assure compliance with both state and federal regulations regarding notification requirements.

Note: California Breach Notification Laws: California Civil Code Section 1798.82 and California Health and Safety Code Section 1280.15.

SECURITY MANAGEMENT PROCESS**Policy No. 2-048.1****PURPOSE**

To establish a process to prevent, detect, contain and correct security violations involving ePHI.

POLICY

Organization's Name will identify risks and safeguard protected health information against loss, destruction, tampering or unauthorized use. ePHI will be safeguarded in compliance with the Health Information Technology for Economic and Clinical Health Act (HITECH) security requirements.

Definitions

1. **ePHI**: Electronic protected health information
2. **ePHI Systems**: A system that creates, accesses, maintains, transmits or receives:
 - A. Primary source ePHI (original source)
 - B. ePHI critical for treatment, payment or health care operations
 - C. Any form of ePHI where the host system is configured to allow access by multiple people

Examples include:

1. A personal computer with a Microsoft Access database containing ePHI that is configured to allow access by more than one person
2. A departmental server containing ePHI
3. A computer system used to create, access, transmit or receive ePHI that is configured to allow access by a vendor/contractor
4. A clinical care system which contains primary source ePHI
5. A billing system which is critical for clinical operations

PROCEDURE

1. Organization's Name will designate a Security Officer responsible for developing and implementing its security policies and procedures. The Security Officer shall take reasonable and appropriate measures to:
 - A. Confirm that ePHI systems are reasonably and appropriately protected and have reasonable and appropriate safeguards in order to safeguard the confidentiality, integrity, or availability of any of ePHI.
 - B. Confirm that Organization's Name is compliant with applicable federal, state and local laws pertaining to security of ePHI.
 - C. Confirm that recently acquired ePHI systems have options that support required and/or addressable implementation specifications of the HIPAA Security Regulations and Organization's Name internal security requirements.
 - D. Guide the development, documentation and dissemination of appropriate security policies and procedures for the users and administrators of ePHI systems.
 - E. Approve and oversee the administration, implementation and selection of security controls for ePHI systems.
 - F. Review reports that workforce members receive security training on a periodic basis.
 - G. Consult with Privacy Officer to confirm that security policies, procedures and controls support compliance with the HIPAA Privacy Regulations.
 - H. Confirm that an inventory of ePHI systems is maintained and updated on a periodic basis.
 - I. Review periodic reports that confirm that a risk analysis of ePHI systems is completed on at least an annual basis as set forth in the Risk Analysis section of this policy.
 - J. Oversee the implementation of an effective risk management program.
 - K. Confirm that the threats and risks to the confidentiality, integrity and availability of ePHI are monitored and evaluated.
 - L. Confirm that records of ePHI systems are developed, monitored and audited to identify security incidents and malicious activity.
 - M. Oversee the development and implementation of an effective security incident response policy and related procedures.
 - N. Confirm that adequate physical security controls exist to protect ePHI.
 - O. Create reports, as necessary, to inform the Governing Body of continuing compliance.

2. Organization's Name's security management process will include the following:
 - A. Commitment to take steps to ensure the confidentiality, integrity and availability of ePHI.
 - B. Institution of security controls, policies and procedures that appropriately and reasonably prevent, detect, contain and correct identified risks to the confidentiality, integrity, and availability of ePHI.
 - C. Periodic reviews and revisions of security controls, policies and procedures.
 - D. Ongoing training and awareness for workforce members on these security controls, policies and procedures.

Risk Analysis

1. Organization's Name will take steps to identify and prioritize the risks to the confidentiality, integrity and availability of ePHI on a periodic basis. A documented risk analysis process shall be used as the basis for the identification, definition and prioritization of potential risks and vulnerabilities to ePHI. The risk analysis shall include at a minimum the following:
 - A. Identifying controls that are in place for each system, such as physical security, passwords and audit capability.
 - B. Identifying risks to the system and ePHI, under the current controls, such as unauthorized access or loss/theft.
 - C. Identifying controls that are needed to address the risks, such as additional locks or system upgrades, and whether/how those controls can be implemented.
2. Organization's Name shall update the risk analysis annually. In addition to the annual risk analysis that Organization's Name completes, the risk analysis shall be updated when environmental or operational changes arise that impact the confidentiality, integrity or availability of ePHI. Such changes include:
 - A. New threats or risks that impact ePHI.
 - B. A security incident that impacts ePHI.
 - C. Changes to information security requirements or responsibilities that impact ePHI. (e.g., new state or federal regulation, new or modified security control has been implemented).
 - D. Changes to organizational or technical infrastructure that impact ePHI (e.g., addition of a new network, new hardware/software standard implemented, new method of creating, receiving, maintaining, or transmitting ePHI).

- E. Hardware and software upgrades.
- 3. The documented risk analysis results shall be reviewed by the Security Officer and appropriate members of the administrative staff and shall be maintained in a secure fashion.

Risk Management

- 1. Organization's Name will implement security measures designed to reduce the risks to ePHI.
- 2. The risk management process implemented shall be based on a documented process that is used as a basis for selection and implementation of the security measures. The risk management process will include the following:
 - A. Assessment and prioritization, on the basis of risks, of ePHI systems.
 - B. Selection and implementation of reasonable, appropriate and cost-effective security measures to manage, mitigate or accept identified risks.
 - C. Security training and awareness on implemented security measures to workforce members.
 - D. Periodic evaluation and revision, as necessary.
- 3. The risk management process is led by the Security Officer and shall be based on the following:
 - A. Risk analysis: Risk analysis is the basis of its risk management activities.
 - B. Risk prioritization: Risks identified in the risk analysis shall be prioritized on a scale from high to low based on the potential impact to ePHI systems. Information on the probability of occurrence shall be based upon the risk analysis. The highest priority shall be given to those risks with unacceptably high-risk ratings. Resources, as available, shall be allocated according to the identified risks.
 - C. Method identification: The appropriate security methods to minimize or eliminate identified risks to ePHI shall be identified. Security methods shall be identified based on the nature, feasibility and effectiveness of the specific security method.
 - D. Cost-benefit analysis: Organization's Name shall identify and define the costs and benefits of implementing or not implementing the identified security methods.
 - E. Security method selection: Based on the cost-benefit analysis, the organization shall select the most appropriate, reasonable, and cost-effective security methods for reducing identified risks to ePHI.

- F. Assignment of responsibility: The selected security methods shall be implemented by the Security Officer and other workforce members who have assigned security responsibility and the appropriate expertise.
 - G. Security method implementation: The selected security methods shall be properly implemented by the responsible workforce members. The Security Officer is responsible for overseeing this implementation.
 - H. Security method evaluation: The selected and implemented security methods shall be evaluated and revised, as necessary.
4. The strategies for managing risk shall be proportionate with the risks to and sensitivity of ePHI. The security measures shall reasonably protect the confidentiality, integrity and availability of ePHI and the risk will be managed on a continuous basis. The following methods are used to manage risk:
- A. Risk acceptance
 - B. Risk avoidance
 - C. Risk limitation
 - D. Risk transference
5. The results of the risk management process shall be documented in writing, reviewed by the Security Officer and maintained by Organization's Name.

Security Sanctions

See "[Sanctions for Privacy and Security Violations](#)" Policy No. 2-029

Information System Activity Review

1. Organization's Name will review, on a periodic basis, records of activity on its information systems that create, receive, maintain or transmit ePHI. Organization's Name will take reasonable and appropriate steps to ensure that ePHI systems have the appropriate hardware, software or procedural auditing mechanisms installed on them to enable review of information system activity on a periodic basis. The risk analysis shall determine the level and type of auditing mechanisms that will be implemented on ePHI systems. Examples of generated reports of information system activity auditable events include:
- A. Failed authentication attempts
 - B. Use of audit software programs or utilities
 - C. Access of particularly designated ePHI (e.g., ePHI regarding VIPs)
 - D. Information system start-up or shutdown

- E. Use of privileged accounts (e.g., system administrator account)
 - F. Security incidents
2. When feasible, these information system activity auditing mechanisms will generate the following information about information systems activity:
- A. Date and time of activity
 - B. Description of attempted or completed activity
 - C. Identification of user performing activity
 - D. Origin of activity (e.g., I/P address, workstation ID)
3. Organization's Name shall review logs of information system activity audit mechanisms implemented on ePHI systems at a frequency determined by the risk analysis. The following factors should be considered with respect to the frequency of reviews of audit mechanisms:
- A. The merit or sensitivity of the ePHI
 - B. The importance of the applications operating on the information systems
 - C. The degree to which the information systems are connected to other ePHI systems and the degree to which that connection poses a risk to the ePHI
4. The information system activity audit mechanism review process shall include:
- A. Definition of what activity is significant
 - B. Procedures for defining how significant activity will be identified and, if appropriate, reported
 - C. Procedures for maintaining the integrity of records of significant activity
 - D. Identification of which workforce members will review records of activity
 - E. Definition of which activity records need to be archived and for what duration
5. Organization's Name shall maintain and follow a specific procedure for conducting information systems activity review, including review of information systems activity and review of auditable events on a periodic basis but at least annually. These procedures shall identify the information systems activity to be reviewed and the auditing mechanism to be used to capture the information systems activity.
6. The audit results shall be retained for six (6) years.

WORKFORCE SECURITY**Policy No. 2-049.1****PURPOSE**

The conduct of workforce members in striving to safeguard ePHI is integral to the compliance efforts under the HIPAA Security Regulations. This policy reflects the organization's commitment to comply with such regulations by instituting safeguards to prevent unauthorized access to ePHI.

POLICY

Organization's Name will implement reasonable and appropriate safeguards to prevent unauthorized access to ePHI while ensuring that properly authorized workforce members' access to ePHI is permitted.

PROCEDURE**Authorization and/or Supervision:**

1. Organization's Name will take reasonable and appropriate steps to ensure that workforce members who have the ability to access ePHI, or work in areas where ePHI might be accessed, will be properly authorized and/or supervised. Organization's Name will use its Minimum Necessary policy, (See "[Minimum Necessary Uses of PHI](#)" Policy No. 2-015) and other policies, as appropriate, as the basis for the type and extent of authorized access to ePHI.
2. Organization's Name will establish a process for granting authorization and access to ePHI, including:
 - A. Procedures for granting different levels of access to ePHI and to areas where ePHI might be accessed
 - B. Procedures for logging and tracking authorization of workforce members access to ePHI and to areas where ePHI might be accessed
 - C. Procedures for logging and tracking authorization of third parties' access to ePHI and areas where ePHI might be accessed
3. Workforce members will not be allowed access to ePHI or to areas where ePHI might be accessed until proper authorization is granted.
4. Only authorized workforce members who have the need for specific information in order to fulfill their respective job responsibilities will be authorized to access ePHI or areas where ePHI might be accessed.

Workforce Members Clearance Procedure:

1. Organization's Name will identify and define the security responsibilities of and supervision required for all defined organizational positions. Security responsibilities include responsibilities for implementing or maintaining security and the protection of the confidentiality, integrity and availability of information systems or processes.
2. Organization's Name will review prospective workforce member backgrounds during the hiring process and will perform verification checks on prospective workforce members. The organization will analyze prospective workforce member access to and expected abilities to modify or change ePHI as one of the bases for the type and number of verification checks conducted. Verification checks may include:
 - A. Confirmation of claimed academic and professional experience and qualifications
 - B. Professional license validation
 - C. Credit check
 - D. Criminal background check
3. Workforce members who access ePHI will sign confidentiality agreements in which they agree not to provide ePHI to or to discuss confidential information with unauthorized persons. The Human Resources department will retain such signed agreements in the workforce members' file.

Termination Procedures:

1. When a workforce member provides notice of his or her intention to end employment, the workforce member's supervisor will give reasonable notice to the person(s) responsible for terminating access to the ePHI systems for the departing workforce member so that access can be terminated when she/he leaves.
2. The Human Resources department will log, track and securely maintain receipts and responses to such termination of access notices, including the following information:
 - A. Date and time of notice of workforce member departure received
 - B. Date of planned workforce member departure
 - C. Description of access to be terminated
 - D. Date, time and description of actions taken

3. When workforce members end employment, all privileges to access ePHI, including both internal and remote information system privileges, will be disabled or removed by the time of departure, or if not feasible, as soon thereafter as possible. When workforce members need to be terminated immediately, the organization will remove or disable their information system privileges prior to notification of termination, when feasible. Information system privileges include workstations and server access, data access, network access, email accounts and inclusion on group email lists.
4. Physical access to areas where ePHI is located will be terminated as appropriate.
5. Workforce members will have their ePHI information system privileges disabled after their access methods or user IDs have been inactive for a period of six months or as determined by the Security Officer. The Security Officer will review privileges that are disabled due to inactivity and take the necessary steps to determine the cause of the inactivity. If inactivity is due to causes other than termination/resignation, designated staff will complete a review and take measures to terminate, limit, suspend or maintain the workforce member's access, as appropriate.
6. Department supervisors/managers will ensure that cryptographic (encryption) keys are available if departing workforce members have used cryptography on ePHI.
7. A workforce member who ends employment will not retain, give away or remove from the organization premises any ePHI. At the time of his/her departure, workforce members will provide ePHI in his/her possession to his/her supervisor. Organization's Name reserves the right to pursue any and all remedies against workforce members who violate this provision. Departing workforce members' supervisors will determine the appropriate handling of any ePHI that the departing workforce members possess, in accordance with policy (See ["Device and Media Controls"](#) Policy No. 2-057).
8. Organization's Name will deactivate or change physical security access codes used to protect ePHI of departing workforce members, when known.
9. Human Resources will track and log the return of equipment and property that contains or allows access to ePHI with the workforce member's name, date and time equipment and property was returned and identification of returned items, and will securely maintain the tracking and logging information. The equipment and property that may contain, or allow or enable the workforce member to access, ePHI include:
 - A. Portable computers
 - B. Personal Digital Assistants (PDAs)
 - C. Name tags or name identification badges
 - D. Access cards
 - E. Building, desk or office keys

INFORMATION ACCESS MANAGEMENT**Policy No. 2-050.1****PURPOSE**

To ensure workforce members' access to ePHI is appropriate and complies with the minimum necessary rule under HIPAA.

POLICY

Organization's Name will take reasonable steps to manage access to ePHI appropriately.

PROCEDURE**Access Authorization**

1. Organization's Name will implement a documented process for granting and authorizing appropriate access to ePHI, to include where feasible:
 - A. Procedure for permitting appropriate levels of access to ePHI based on job role
 - B. Procedure for logging and tracking authorization of such access to ePHI
 - C. Procedure for reviewing and revising on a periodic basis, authorization of access to ePHI

Access Establishment and Modification

1. Organization's Name will create a documented process for establishing, documenting, reviewing and modifying access to ePHI in accordance with the Access Authorization operational specification in this policy, that include:
 - A. Procedure for establishing and documenting different access levels to ePHI
 - B. Procedure for documenting establishment of access to ePHI
 - C. Procedure for reviewing on a regular basis workforce members access privileges to ePHI
 - D. Procedure for modifying the access privileges of workforce members to ePHI, as appropriate, based on the periodic reviews
2. Organization's Name will properly authorize and train workforce members to access ePHI and will document that process, including:
 - A. Definition and classification of permitted access methods (e.g., user ID and password, two factor authentication, log-on procedure)

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- B. Definition and classification of length of time access will be permitted (e.g., indefinite, a fixed period for temporary workforce members, or a fixed limited period based on business need)
 - C. Procedure for granting and changing a workforce member's access method
 - D. Procedure for managing access rights in a networked and distributed environment
 - E. Procedure for appropriate logging and tracking of activities by an authorized workforce member on ePHI
3. Security controls or methods that establish access to ePHI shall include:
- A. The disabling or removing of access methods for workforce members who no longer require access to ePHI.
 - B. Confirmation that redundant user identifiers (i.e., user IDs) are not created.
 - C. User identifiers (i.e., user IDs) that enable workforce members to be uniquely identified. Workforce member privilege levels will not be reflected in the structure of user IDs.
 - D. Organization's Name will implement appropriate operational measures approved by the Security Officer when a legacy system is in place that permits only common or shared identifiers.
4. Organization's Name will log and track modifications of access rights and securely maintain the tracking and logging information. Tracking and logging shall provide the following information:
- A. Date and time of modification
 - B. Identification of workforce member whose access is being modified
 - C. Description of modified access rights
 - D. Reason for modification of access rights

SECURITY AWARENESS AND TRAINING**Policy No. 2-051.1****PURPOSE**

To provide and document security awareness and training for workforce members in order that those persons can properly carry out their functions while appropriately safeguarding ePHI.

POLICY

Organization's Name workforce members shall take HIPAA Security training annually. Additional training will be provided within a reasonable period of time after material changes to HIPAA security policies and procedures.

PROCEDURE

1. Organization's Name will provide annual training and supporting reference materials to workforce members, as appropriate, to carry out their functions with respect to the security of ePHI. The method of delivery of such training may include on-site or remote training. After the training has been conducted, Human Resources will maintain such records as it deems appropriate that confirm that a workforce member received training. Training should include:
 - A. Awareness of and familiarity with security policies, specifications, and procedures
 - B. The secure usage of ePHI, as set forth in Protection from Malicious Software, Log-in Monitoring and Password Management operational specifications in this policy
 - C. Risks to the confidentiality, integrity and availability of ePHI
 - D. Legal and business responsibilities for protecting ePHI
2. The Security Officer will determine the necessity of additional training needed if there is a material change in HIPAA regulations or organizational policies and procedures.
3. Organization's Name will make its security policies and procedures available for reference to its workforce members.
4. The organization will provide security information and awareness reminders and updates to workforce members.

Security Reminders

1. The Security Officer will be responsible for taking reasonable steps to ensure that workforce members, including those who work remotely, receive security information and awareness reminders quarterly and as needed, including:
 - A. Information on security risks and how to follow security policies and procedures

- B. How to use ePHI systems in a manner that reduces security risks, and on selected security topics, including:
 - 1. Organization's security policies and procedures
 - 2. Organization's security controls and processes
 - 3. Significant risks to ePHI systems
 - 4. Legal and business responsibilities for protecting ePHI systems
- C. When any of the following events occur:
 - 1. Substantial revisions are made to security policies or procedures
 - 2. Substantial new security controls are implemented
 - 3. Significant changes are made to existing security controls
 - 4. Substantial changes are made to legal or business responsibilities
 - 5. Substantial threats or risks arise against ePHI systems
- 2. Means of providing security information and awareness reminders and updates may include, but are not limited to, email reminders, posters, letters, workforce member meetings, security days, screen savers, information system sign-on messages, newsletter articles and information posted to a website.

Protection From Malicious Software

- 1. The Security Officer will develop, implement and periodically review a process for guarding against, detecting and reporting malicious software that pose risks to ePHI. Malicious software prevention, detection, and reporting procedures will include:
 - A. Anti-virus software installed and updated on ePHI systems
 - B. Procedures for workforce members to report suspected or confirmed malicious software
 - C. Plan for recovering from malicious software attacks
 - D. Process to examine electronic mail attachments and downloads before they can be used on ePHI systems
- 2. Workforce members will not bypass or disable anti-virus software installed on ePHI systems unless properly authorized to do so.

3. Organization's Name will provide training and awareness to its workforce members about guarding against, detecting and reporting malicious software. Training and awareness on protection from malicious software will include, for example, the following topics:
 - A. How to discover malicious software
 - B. How to report malicious software
 - C. How to discover malicious software fraud
 - D. How to not download or receive malicious software including not opening or launching email attachments that may contain malicious software
 - E. How to use anti-virus software appropriately

Log-In Monitoring

1. Organization's Name will develop, implement and periodically review a process for monitoring log-in attempts to ePHI systems and reporting log-in discrepancies. The log-in process may include, for example, the following attributes:
 - A. Notification displays upon log-in stating that the system must only be accessed by an authorized workforce member
 - B. Help messages that could assist an unauthorized user must not be provided during the log-in process
 - C. Limitations on the number of unsuccessful log-in attempts are implemented
 - D. The system does not state which part of the log-in information is correct or incorrect if there is an error
 - E. Prior to successfully completing the log-in process, information system or application identifying information is not provided
 - F. Limit the time allowed for the log-in procedure
 - G. Record failed log-in attempts
 - H. After the specific pre-determined number of failed log-in attempts, a time period is documented before permitting further log-in attempts or any further attempts are rejected until a designated workforce member has given authorization
 - I. Upon completion of a successful log-in, the date and time of the previous successful log-in by the workforce member are displayed

2. Organization's Name will provide training and awareness periodically and as needed to all workforce members regarding the procedures for monitoring log-in attempts and reporting discrepancies regarding their access or log-in attempts. The log-in monitoring training and awareness will include the following topics:
 - A. How to detect a log-in discrepancy
 - B. How to report a log-in discrepancy
 - C. How to successfully use the secure log-in process

Password Management

1. Organization's Name will develop, implement and review a process for appropriately creating, changing and safeguarding passwords used to verify users' identities and to obtain access to ePHI. The password management procedure may include, for example:
 - A. Require and force regular password changes (e.g., every 30/60/90 days)
 - B. Require and force the use of individual passwords to maintain accountability
 - C. Permit workforce members to select and change their own passwords
 - D. Require unique passwords that meet the standards defined by Organization's Name (e.g., no password re-uses for a minimum period of time)
 - E. Require passwords not to be displayed in clear text when inputting into ePHI systems
 - F. Require passwords to be given to workforce members in a secure manner, through a pre-defined process
 - G. Require changing of default vendor passwords immediately following installation of hardware or software
 - H. Prohibit the use of "Admin" or "Administrator" as login for administrator accounts or of "Demo" for demonstration logins
2. Organization's Name will require its workforce members to use the following standards when possible to create strong, secure passwords:
 - A. A minimum length of passwords is eight (8) characters
 - B. A combination of numeric, non-alphanumeric, and alphabetical characters and of capital and lowercase letters
 - C. Passwords that are not easily guessable or obtained by using personal information such as names, pet's name, license plate, birthday, etc.

3. Organization's Name will provide its workforce members with training and awareness on appropriately creating, changing and safeguarding passwords used to verify users' identities. Password management training and awareness will include the following requirements for access to ePHI systems:
 - A. The organization's password standards and guidelines
 - B. The process for changing temporary passwords when assigned for new log-in
 - C. The importance of avoiding maintaining passwords in a paper record

SECURITY INCIDENT PROCEDURES**Policy No. 2-052.1****PURPOSE**

Awareness of, response to and creation of reports about security incidents in the context of its operations are integral parts of Organization's Name efforts to comply with the HIPAA Security Regulations.

POLICY

Organization's Name will implement a process for promptly identifying, reporting, tracking and responding to security incidents, and will conduct training awareness on those security incident procedures.

PROCEDURE**Response and Reporting**

1. Organization's Name will include the following in its process for promptly identifying security incidents, as appropriate:
 - A. Risk analysis of ePHI systems
 - B. On the basis of the risk analysis, identify what events constitute a security incident
 - C. Process for identifying a security incident
2. The Security Officer is responsible for security incident reporting and response and will perform an investigation when evidence shows that a security incident has occurred and will respond promptly to the security incident.
3. Organization's Name will have a procedure for workforce members to report a security incident to the Security Officer. A workforce member will not prohibit or otherwise attempt to hinder or prevent another workforce member from reporting a security incident to the Security Officer and will cooperate fully with security incident investigations.
4. Organization's Name will include training and awareness for workforce members in its process for promptly identifying, reporting, tracking, and responding to security incidents.
5. Organization's Name will mitigate, to the extent practicable, harmful effects of security incidents that are known and document those security incidents and their outcomes.
6. When performing a risk assessment, the Security Officer will assess the probability that the electronic protected health information has been compromised based on considerations that include at least the following four (4) factors:
 - A. The nature and extent of the protected health information involved, including the types of identifiers and the likelihood of re-identification

- B. The unauthorized person who used the protected health information or to whom the disclosure was made
- C. Whether the protected health information was actually acquired or viewed
- D. The extent to which the risk to the protected health information has been mitigated

PURPOSE

To ensure that Organization's Name is prepared for emergencies or disasters, in such that ePHI is protected and available for the continuation of service provision.

POLICY

Organization's Name will take appropriate steps to establish and implement emergency response procedures in order to prepare for and respond to emergencies and disasters that may damage or otherwise disable ePHI systems, and by taking reasonable and appropriate steps to ensure that critical data including applications, operating systems, database software and other software supporting packages and tools will survive a disaster or other emergency.

PROCEDURE

Data Backup Plan:

1. Organization's Name will take reasonable and appropriate steps to back-up and store ePHI stored on ePHI systems and to create exact and retrievable copies of ePHI.
2. Organization's Name will create and implement a plan for creating and maintaining backup data from all electronic media associated with ePHI that:
 - A. Defines who is responsible for taking reasonable steps to ensure the backup of ePHI
 - B. Defines a backup schedule
 - C. Specifies the ePHI systems that are to be backed up
 - D. Defines where backup media is to be stored and workforce members who may access the stored backup media
 - E. Defines where backup media is to be kept secure before it is moved to storage
 - F. Defines who may remove the backup media and transfer it to storage
 - G. Defines restoration procedures to restore ePHI from backup media to the appropriate ePHI systems
3. Organization's Name will implement a backup procedure that will:
 - A. Generate up-to-date copies of ePHI that can be recovered in the event that ePHI systems are damaged by or during a disaster or other emergency

- B. Complete periodic testing of its restoration procedures for ePHI systems to confirm the effectiveness of those procedures and that the ePHI can be restored in the time set forth in the Disaster Recovery Plan section of this policy.
- C. Document the retention period for backup media that contain backup copies of ePHI
- D. Store backup copies of ePHI, complete records of the backup copies and document restoration procedures in a remote and secure location within sufficient distance from the site
- E. Provide access to authorized workforce members for timely retrieval of the backup information stored at the remote location
- F. Provide physical, environmental and technical security for the backup media stored at the remote location that will be consistent with the security provided to ePHI onsite

Disaster Recovery Plan:

- 1. Organization's Name will take steps to maintain a detailed Disaster Recovery Plan to recover ePHI that is lost, damaged or corrupted in the event of a disaster or other emergency.
- 2. The Disaster Recovery Plan will include:
 - A. The conditions under which the Disaster Recovery Plan may be activated
 - B. Workforce members' roles and responsibilities in executing the Disaster Recovery Plan
 - C. Recommended procedures that contain the actions to be taken to restore ePHI and to return ePHI systems to normal operations, within a defined timeframe.
 - D. Documented order in which ePHI will be restored and the ePHI systems will be returned to operation
 - E. Documented reporting and notification procedures to the Security Officer or designated workforce members
 - F. In the event of a disaster or other emergency, procedures for permitting appropriate specified workforce members physical access to the facilities and to any backup media on which ePHI is stored whether onsite or offsite, in order to carry out the recovery plan
 - G. Procedures that specify how and when the plan will be tested and maintained
 - H. An Emergency Mode Operation Plan as set forth in the Emergency Mode Operation Plan section of this policy

3. Organization's Name will provide periodic training and awareness on the Disaster Recovery Plan to all workforce members. The Security Officer will determine the frequency of the training and awareness in accordance with the policy (See "[Security Awareness and Training](#)" Policy No. 2-051).
4. Organization's Name will provide current copies of the Disaster Recovery Plan and training and awareness on that plan to the appropriate workforce members on a periodic basis, as well as keep copies of the plan off-site.

Emergency Mode Operation Plan:

1. Organization's Name will implement a detailed Emergency Mode Operation Plan designed to allow the continuation of critical operations processes, while permitting necessary access to and use of ePHI, during and immediately following an emergency.
2. The Emergency Mode Operation Plan will:
 - A. Define and categorize reasonably foreseeable emergencies that could have an impact on the confidentiality, integrity, and availability of ePHI systems
 - B. Include a procedure that specifies how the organization will react to emergencies that impact the confidentiality, integrity and availability of ePHI
 - C. Include a procedure to follow during and immediately following an emergency that outlines how security processes and controls to ensure the confidentiality, integrity and availability of ePHI will be maintained
 - D. Include a procedure authorizing workforce members to enter the organization and any offsite location where backup storage media are stored to maintain the security processes and controls that protect the confidentiality, integrity and availability of ePHI while functioning in emergency mode
 - E. Identify and document processes and controls that protect the confidentiality, integrity and availability of ePHI while functioning in emergency mode

Testing and Revision Procedure:

1. Organization's Name will take appropriate steps to perform testing of its Contingency Plan to assess its sufficiency and determine whether the plan is accurate and up-to-date, and by making necessary revisions on a periodic basis (at least annually).
2. The Security Officer will receive the documented testing results and recommend revisions, as necessary, to the Contingency Plan to address any issues identified in the testing of the disaster recovery.

3. Organization's Name will modify its Disaster Recovery Plan to make it sufficient and keep the plan accurate and up-to-date. Events that result in a revision of the plan will include:
 - A. Disaster recovery role and responsibility changes, including changes to contact information
 - B. Changes to the physical or technical infrastructure
 - C. Changes in threats to ePHI systems
 - D. Results of testing that indicate that the plan needs to be modified to ensure that it is sufficient, accurate and up-to-date

Applications and Data Criticality Analysis:

1. Organization's Name will analyze and document the criticality of ePHI and ePHI systems. The purpose of this criticality analysis is to document the impact to its services, processes and operating objectives if a disaster or other emergency causes ePHI systems to become unavailable for a documented amount of time. The criticality analysis will serve as the basis for the prioritization of ePHI and ePHI systems.

PURPOSE

Periodic evaluations of security safeguards, especially in response to environmental or operational changes, are necessary to re-affirm that ePHI continues to be protected.

POLICY

Organization's Name will take steps to conduct annual technical and non-technical evaluation of its security safeguards, including policies, controls and processes, in response to environmental or operational changes affecting the security of ePHI.

PROCEDURE

1. Organization's Name will undertake an annual technical and nontechnical evaluation of its security safeguards to determine the extent of compliance with the standards implemented under the HIPAA Security Regulations. Re-evaluations will be conducted in response to environmental or operational changes occurring since the last evaluation. Changes that may trigger a re-evaluation of Organization's Name's security safeguards include:
 - A. Known security incidents
 - B. Significant new threats or risks to security of ePHI
 - C. Changes to organizational or technical infrastructure
 - D. Changes to information security requirements or responsibilities
 - E. New security technologies that are available and new security recommendations
2. The evaluations will be completed by a team designated by the Security Officer.
3. Each evaluation shall include reasonable and appropriate activities, such as:
 - A. A review of the security policies and procedures to evaluate their appropriateness and effectiveness at protecting against any reasonably anticipated threats or hazards to the confidentiality, integrity and availability of ePHI.
 - B. A gap analysis to compare the security policies and procedures against actual practices.
 - C. An identification of threats and risks to ePHI and ePHI systems, as set forth in Organization's Name's risk analysis.
 - D. An assessment of security controls and processes as reasonable and appropriate protections against the risks identified for ePHI systems.

- E. Testing and evaluation of security controls and processes to determine whether they have been implemented properly and whether those controls and processes appropriately protect ePHI. An authorized workforce member shall be designated to conduct the testing.
- 4. The evaluation process and results shall be documented by the responsible workforce member(s) in a report that is provided to the Security Officer and Privacy Officer.
- 5. Following each evaluation, the organization will update its security policies, procedures, controls, and processes if the results of the evaluation show that such updates are needed.
- 6.

FACILITY ACCESS CONTROLS**Policy No. 2-055.1****PURPOSE**

To establish policies and procedures to limit physical access to electronic information systems and to protect facilities where ePHI is stored from unauthorized physical access, tampering and theft.

POLICY

Organization's Name will take appropriate steps to protect ePHI systems, as well as the facilities in which they are located, from unauthorized physical access, tampering, theft and physical damage while taking reasonable and appropriate steps to ensure that access by properly authorized workforce members is granted.

PROCEDURE**Contingency Operations**

1. Organization's Name will take reasonable steps to ensure that in the event of a disaster or emergency, while operating in emergency mode, appropriate workforce members can enter its facilities to take the necessary actions as indicated in its procedures as set forth in the Disaster Recovery Plan and Emergency Mode Operation Plan (See "[Contingency Plan](#)" Policy No. 2-053).
2. Organization's Name will develop a procedure to allow authorized workforce members or business associates access to its facilities to support restoration of lost data. The organization shall define workforce member's roles in its Disaster Recovery Plan and Emergency Mode Operations Plan (The Plan) and address facilities, ePHI systems and electronic media involved. The Plan should define how the actions taken by such workforce members are tracked and logged and how unauthorized accesses can be detected and prevented.
3. Only authorized workforce members shall be permitted to administer or modify processes and controls that protect the security of ePHI in the event of an emergency.

Facility Security

1. Organization's Name will develop, document and implement a facility security plan that describes how it seeks to protect its facility and ePHI systems from unauthorized access, tampering or theft.
2. The facility security plan should address such elements as the following:
 - A. Identification of ePHI systems to be protected from unauthorized access, tampering or theft

- B. Identification of processes and controls used to protect ePHI systems from unauthorized access, tampering or theft
 - C. Actions to be taken if unauthorized access, tampering or theft attempts have been made against ePHI systems
 - D. Identification of the workforce member's responsibilities within the facility security plan
 - E. Maintenance schedule that specifies and documents how and when the plan will be reviewed and tested and a process for maintaining and revising the facility security plan
3. Organization's Name will distribute the facility security plan to the necessary workforce members, provide appropriate training and maintain an appropriate number of copies of the facility security plan off-site.

Access Control and Validation Procedures

1. Organization's Name will define, document and implement a procedure for controlling and validating physical access to facilities that house ePHI systems, to include the following elements:
- A. Provide workforce members access rights to highly sensitive areas only as needed in order to accomplish a legitimate business task
 - B. Define and document roles or functions that require physical access rights to the facilities
 - C. Periodically review and, where necessary, revise access rights to the facilities and ePHI systems
 - D. Track, log and maintain in a secure manner physical access to the facilities
2. The organization will instruct workforce members on the following:
- A. Do not attempt to gain physical access to sensitive facilities containing ePHI systems for which they have not been given proper authorization to access. Do not allow unauthorized individuals access to restricted areas.
 - B. Immediately report to an appropriate authority, such as the Security Officer, the loss or theft of any device (e.g., card) that enables physical access to facilities.
 - C. Carry or display an identification badge when at facilities containing ePHI systems. Visitors to sensitive facilities shall show proper identification and state their reasons for needed access prior to gaining access.

Maintenance Records

1. Organization's Name will document repairs and modifications to the physical elements of its facilities that are related to the physical security of the facility and ePHI systems and store the documentation in a secure manner. Such documentation should include such elements as:
 - A. Date and time of repair or modification
 - B. Description of physical component prior to repair or modification
 - C. Reason(s) for repair or modification (including any damage and any related security incident)
 - D. Person(s) performing repair or modification
 - E. Outcome of repair or modification
2. The Security Officer will be notified when a repair or modification is made that impacts the physical security of the facilities.

WORKSTATION USE AND SECURITY**Policy No. 2-056.1****PURPOSE**

To ensure that workstations and other computer systems that access, store or transmit ePHI are used in a secure and appropriate manner.

POLICY

Organization's Name will specify and maintain reasonable and appropriate safeguards to maximize the security of ePHI by delineating proper workstation use and security.

PROCEDURE

1. Organization's Name will define the authorized purposes of each workstation or class of workstations that can access ePHI.
2. Organization's Name will take reasonable steps to ensure that workforce members understand which purposes and functions are authorized on their workstations and do not use workstations for unauthorized purposes or to perform unauthorized functions. Workforce members will be encouraged to report any unauthorized activity at a workstation.
3. Workforce members will be instructed not to share passwords with others, except to assure business continuity. If any workforce member suspects misuse of user IDs or passwords, they are required to promptly report that misuse to the Security Officer.
4. Organization's Name will document and implement procedures that indicate the manner in which workstations accessing ePHI are located in physically secure areas and display screens are positioned or protected, in order to minimize the risk of access by unauthorized individuals and prevent unauthorized viewing of ePHI. Documentation and procedures include:
 - A. Workforce members are to activate their workstation locking software when they leave their workstations unattended
 - B. Workforce members are to log off from their workstations when their shift is complete
 - C. Steps are taken to ensure that workstations removed from the facility are protected with security controls equivalent to on-site workstations
 - D. Organization's Name will implement additional precautions for portable workstations (e.g. laptops, PDAs, portable medical equipment) that store ePHI

- E. Workforce members will not store ePHI on unencrypted workstations
- F. Workforce members must immediately report the theft or loss of any workstation containing ePHI (including those owned by the individual) to the Security Officer

DEVICE AND MEDIA CONTROLS**Policy No. 2-057.1****PURPOSE**

To establish requirements regarding tracking devices and media containing ePHI into, out of and within the organization.

POLICY

Organization's Name will control its hardware and electronic media through the entire lifecycle, from initial receipt to final removal.

PROCEDURE**Disposal**

1. Organization's Name will take reasonable and appropriate steps to dispose of ePHI when it is no longer needed, in a manner so as to permanently, completely and irreversibly delete ePHI and to prevent future access by unauthorized individuals. Organization's Name will log the disposal of devices and media. The log will include the following information:
 - A. Date and time of disposal
 - B. Who performed the disposal
 - C. Brief description of media or information systems disposed of
 - D. Reason for disposal
 - E. Verification of data removal or destruction prior to disposal
2. Organization's Name will take reasonable and appropriate steps to remove ePHI from hardware and media, prior to final disposal or re-use.

Media Re-use

1. Organization's Name will take steps to remove or overwrite ePHI on its electronic media before the media are re-used for any purpose in order to prevent unauthorized access to the ePHI.

Accountability

1. Organization's Name will take steps to establish and maintain records of movements of hardware and electronic media on which ePHI is or was stored.
 - A. A reconciliation of the inventory should occur annually.
 - B. The inventory should include a description of the device or media, serial number if available, to whom the device or media is assigned, where it is typically maintained, when it was assigned and when it was returned.

Data Backup and Storage

1. Organization's Name will take steps to ensure that exact, retrievable backup copies of ePHI are made before movement of equipment when needed and will define and document an appropriate retention period for the backup copies of ePHI. (See "[Contingency Plan](#)" Policy No. 2-053).

ACCESS CONTROLS: TECHNICAL SAFEGUARDS**Policy No. 2-058.1****PURPOSE**

Although access controls, as required by the HIPAA Security Regulations, may differ under normal and emergency conditions, they are integral to the safeguarding of electronic protected health information.

POLICY

Organization's Name will ensure that ePHI systems support, and are installed with technical safeguards, to control and restrict access to such ePHI systems to persons and software programs that are authorized to have such access.

PROCEDURE**Unique User Identification:**

1. Organization's Name will ensure that access to ePHI systems is granted to workforce members using unique user identifiers (i.e., user IDs) that:
 - A. Identify individual workforce members (i.e., no redundant user IDs)
 - B. Permit activities performed on ePHI systems to be traced to the individual workforce member through the unique identifier
2. Unique user identifiers shall not give any indication of workforce members' privilege levels.
3. The procedure for assigning unique user identifiers (user IDs) can include any or all of the following, or another appropriate method:
 - A. Workforce members' names
 - B. Exclusive numbers (e.g., PIN, password)
 - C. Biometric identification
4. The Security Officer will review and approve the use of any group user identifiers to gain access to ePHI systems.

Emergency Access Procedure:

1. Organization's Name will take reasonable and appropriate steps to establish, implement, and document an emergency access procedure delineating the necessary steps to enable authorized workforce members to obtain access to necessary ePHI during a disaster or other emergency.

2. All appropriate workforce members will receive periodic training and awareness on the emergency access procedure.
3. All appropriate workforce members will receive a current copy of the emergency access procedure and keep an appropriate number of copies at a secure off-site location.
4. The Security Officer will be responsible for approving the emergency access procedure, updating it from time to time as necessary and confirming that the procedure appropriately balances the need for emergency access to ePHI during a disaster or other emergency with the need to protect the confidentiality, integrity and availability of ePHI.

Automatic Logoff:

1. Electronic sessions will be terminated and workforce members will be logged out of ePHI systems after 15 minutes, requiring the user to be identified and authenticated again in order to regain access and continue the session.
2. The Security Officer will determine when it is not reasonable or appropriate to implement electronic automatic logoff mechanisms on certain ePHI systems and will approve equivalent alternative mechanisms (e.g., screen/session locking, screensaver implemented after period of time).
3. Workforce members will be instructed to terminate electronic sessions on ePHI systems when such sessions are completed and to log off from or lock their workstations or other ePHI systems when their shifts are completed or when they expect to be away from their workstation or other ePHI system for an extended period of time (e.g., lunch, meetings, and breaks).

Encryption and Decryption:

1. Based on the risk analysis, Organization's Name will determine when to implement encryption for ePHI and ePHI systems and the type and quality of the encryption algorithm and cryptographic key length.
2. Based on the results of the risk analysis, the Security Officer will approve the encryption mechanism used.
3. When encryption is used, Organization's Name will:
 - A. Protect its cryptographic keys against modification and destruction and protect its private keys against unauthorized disclosure
 - B. Implement a process for managing the cryptographic keys used to encrypt ePHI stored or maintained on ePHI systems
 - C. Periodically determine activation and deactivation dates for its cryptographic keys

4. The Security Officer will maintain documentation defining when encryption is utilized to protect ePHI stored or maintained on ePHI systems and how such encryption is implemented.

Secure Text Messaging Platform:

1. Texting of personal information or any ePHI **must** be accomplished through a secure text messaging platform.
2. The platform should include, at a minimum, the following components
 - A. Secure sign-on process;
 - B. Encrypted messaging;
 - C. Delivery and read receipts;
 - D. Date and time stamp;
 - E. Customized message retention timeframes; and
 - F. Specified contact list of individuals authorized to receive and record orders
3. The Security Officer will monitor compliance with the secure texting platform and include in the annual risk assessment.

HIPAA SECURITY AUDIT CONTROLS**Policy No. 2-059.1****PURPOSE**

To ensure that Organization's Name's systems containing ePHI are regularly audited for HIPAA compliance.

POLICY

Organization's Name will review information system activity on an ongoing basis.

PROCEDURE

Where feasible, information systems shall have the appropriate hardware, software or procedural auditing mechanisms to generate reports of auditable events. Organization's Name will review the audit mechanism on a periodic basis.

1. Organization's Name will maintain and implement a process for audit log maintenance, including:
 - A. Identification of employees who review logs (e.g., network logs and application-level logs)
 - B. Frequency of log review (at least annually)
 - C. Procedure for determining how auditable events are identified during audit log review and reported to the appropriate manager
 - D. Retention period of logs (See "[Record Retention](#)" Policy No. 4-064)

PURPOSE

To safeguard ePHI, it is important to corroborate that ePHI has not been altered or destroyed in an unauthorized manner as required pursuant to the HIPAA Security Regulations.

POLICY

Organization's Name will take appropriate steps to protect the integrity of ePHI that it creates, receives, maintains or transmits from unauthorized modification or destruction.

PROCEDURE

Mechanism to Authenticate Electronic Protected Health Information

1. Organization's Name will take reasonable and appropriate steps to implement electronic mechanisms to prove that ePHI has not been altered or destroyed in an unauthorized manner, including:
 - A. Which ePHI will be authenticated
 - B. Which electronic mechanisms will be reasonable and appropriate
2. The Security Officer will approve the electronic mechanisms that have been implemented to protect ePHI from unauthorized alteration or destruction and to authenticate the integrity of ePHI, and will take reasonable and appropriate steps to ensure that the electronic mechanisms are reviewed and that integrity incident reports are generated from the electronic mechanisms.
3. Organization's Name will take reasonable and appropriate steps to train workforce members regarding the electronic mechanism(s) that have been implemented to confirm the integrity of ePHI.

PERSON OR ENTITY AUTHENTICATION**Policy No. 2-061.1****PURPOSE**

To ensure the authenticity of a person or entity prior to granting access to ePHI as required by the HIPAA Security Regulations.

POLICY

Organization's Name will maintain a documented authentication process for verifying the identity of any person or entity prior to granting them access to ePHI.

PROCEDURE

1. Organization's Name will use appropriate authentication methods to confirm that only properly authenticated and authorized persons or entities access ePHI. Appropriate access methods may include:
 - A. Unique user identifiers (user IDs)
 - B. Security identifiers (password)
 - C. Password systems
 - D. Personal Identification Number (PIN) systems
 - E. Security token systems
 - F. Biometric identification systems
 - G. Telephone callback systems
 - H. Digital signatures
2. The authentication processes may include:
 - A. Documented procedures for granting persons and entities authentication credentials or for changing an existing authentication method
 - B. Uniquely identifiable authentication identifiers in order to track the identifier to a workforce member
 - C. Documented procedures for detecting and responding to any person or entity attempting to access ePHI without proper authentication

- D. Removing or disabling authentication credentials in ePHI systems for persons or entities that no longer require access to ePHI
 - E. Periodic validation that no redundant authentication credentials have been issued or are in use
 - F. Protection of authentication credentials (e.g., passwords, PINs) with appropriate controls to prevent unauthorized access
 - G. When feasible, masking, suppressing or otherwise obscuring the passwords and PINs of persons and entities seeking to access ePHI so that unauthorized persons are not able to observe them
- 3. Access methods for authentication to ePHI systems shall not be built into logon scripts. Exceptions may be made only after review and approval by the Security Officer.
 - 4. Organization's Name will limit authentication attempts to its ePHI to no more than ten (10) attempts. Authentication attempts that exceed the limit may result, as appropriate, in:
 - A. Disabling relevant account for an appropriate period of time
 - B. Logging of event
 - C. Notifying appropriate management
 - 5. The Security Officer shall take reasonable and appropriate steps to ensure that workforce members are provided training and awareness about the authentication methods used.

PURPOSE

To ensure the confidentiality, integrity and availability of electronic protected health care information (ePHI), technical security measures to guard against unauthorized access.

POLICY

Organization's Name will take appropriate steps to implement security measures to protect the integrity of ePHI while it is being transmitted over electronic communications networks.

PROCEDURE

Integrity Controls

1. When the risk analysis determines it to be necessary, Organization's Name will use integrity controls to ensure that the value and state of its ePHI is maintained during transmission and that ePHI is protected against unauthorized alteration or destruction during transmission over electronic communications networks.
2. The Security Officer will approve the implemented integrity controls and will take reasonable and appropriate steps to confirm effective implementation of the integrity controls, to review and update them as necessary.
3. Protected health information stored, whether intentionally or not, in a photocopier, facsimile and other devices is subject to the HIPAA Privacy and Security Rules.
4. The Security Officer will determine whether it is appropriate to accept the use of the external portable medium on their systems when an individual requests to receive PHI in a particular electronic form and format. If the Security Officer determines there is an unacceptable level of risk, the individual may opt to receive an alternative form of the ePHI.
5. Workforce members will receive training and awareness regarding integrity controls implemented to protect ePHI from unauthorized alteration or destruction during transmission over electronic communications network.

Encryption

1. When deemed necessary, encryption will be used to protect ePHI during transmission over electronic communications networks.
2. The Security Officer will approve the encryption methods used to protect ePHI during transmission over an electronic communications network.

3. The Security Officer will implement a process for managing and protecting the cryptographic keys used to encrypt its ePHI against modification or destruction, will protect its private keys against unauthorized disclosure and will implement a process for managing the cryptographic keys used to encrypt ePHI transmitted over electronic communications networks.
4. Organization's Name will provide workforce members with training and awareness regarding encryption methods implemented to protect ePHI.

WHISTLEBLOWER PROTECTION**Policy No. 2-063.1****PURPOSE**

To encourage and enable employees and others to raise serious concerns internally so that the organization can address and correct inappropriate conduct and actions. The organization requires board members, directors, officers, employees and volunteers to observe high standards of business and personal ethics in the conduct of their duties and responsibilities. As employees and representatives of the organization, we must practice honesty and integrity in fulfilling our responsibilities and comply with all applicable laws and regulations.

POLICY

It is the responsibility of all board members, officers, directors, employees and volunteers to report concerns about violations of the organization's code of conduct or suspected violations of law or regulations that govern the organization's operations.

This whistleblowing policy is intended to cover protections for employees that raise concerns regarding the organization such as:

- Incorrect financial reporting
- Unlawful activity
- Activities that are not in line with organizational policies, including the Code of Conduct
- Activities, which otherwise amount to serious improper conduct

Note: See Policy No. 2-041 "[Corporate Compliance Plan](#)," Policy No. 2-042 "[Corporate Compliance Officer](#)" and Policy No. 2-043 "[Internal Control Systems/Accountabilities](#)" for further detail.

PROCEDURE**1. No Retaliation**

- A. An employee who retaliates against someone who has reported a violation in good faith is subject to discipline up to and including termination of employment. (See Policy No. 2-041 "[Corporate Compliance Plan](#)")

2. Reporting Procedure

- A. Employees should share their questions, concerns, suggestions or complaints with their supervisor.
- B. If the employee is not comfortable speaking with their supervisor/manager or is not satisfied with their supervisor's/manager's response, the employee is encouraged to speak with the Compliance Officer or designee.

- C. Supervisors and managers are required to report complaints or concerns about suspected ethical and legal violations in writing to the organization's Compliance Officer or designee, who has the responsibility to investigate all reported complaints. If a supervisor or manager fails to report suspected violations, disciplinary actions up to and including termination may occur.
- D. Employees with concerns or complaints may also submit their concerns in writing directly to their supervisor/manager or the Executive Director or the organization's Compliance Officer or designee.

3. Compliance Officer

- A. The organization's Compliance Officer is responsible for ensuring that all complaints about unethical or illegal conduct are investigated and resolved. (See Addendum No. 2-041.A ["Sample Compliance Report"](#))
- B. The Compliance Officer will advise the Executive Director and/or the Board of Directors of all complaints and their resolution and will report at least annually to the Board of Directors on compliance activity relating to accounting or alleged financial improprieties.

4. Accounting and Auditing Matters

- A. The Organization's Compliance Officer shall immediately notify the Audit Committee/Finance Committee of the Board of Directors any concerns or complaint regarding corporate accounting practices, internal controls or auditing and work with the committee until the matter is resolved.

5. Acting in Good Faith

- A. Anyone filing a written complaint concerning a violation or suspected violation must be acting in good faith and have reasonable grounds for believing the information disclosed indicates a violation.
- B. Any allegations that prove not to be substantiated and which prove to have been made maliciously or knowingly to be false will be viewed as a serious disciplinary offense.

6. Confidentiality

- A. Violations or suspected violations may be submitted on a confidential basis by the complainant.
- B. Reports of violations or suspected violations will be kept confidential to the extent possible, consistent with the need to conduct an adequate investigation.

7. Handling of Reported Violations

- A. The organization's Compliance Officer will notify the person who submitted a complaint and acknowledge receipt of the reported violation or suspected violation.

- B. All reports will be promptly investigated and appropriate corrective action will be taken if warranted by the investigation.

SECTION THREE

Fiscal Management

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*Requires state or organization-specific information.

PURPOSE

To provide a written financial plan to assure sufficient allocation of resources and finances for organization operations.

POLICY

On an annual basis, senior management, including at least the Chief Executive Officer, Chief Financial Officer, and program directors, will prepare a budget and operating plan to assure that adequate monies are available to carry out the programs and services designed to meet the needs of the patient population being served. These documents will be reviewed and approved by the Governing Body.

Factors used to develop the annual budget may include:

1. Strategic and operational plans, both short- and long-term
2. Assumptions upon which the budget is built include:
 - A. Information from revenue and expense centers
 - B. Budgetary variances
 - C. Documented trends regarding availability of adequate funds
 - D. Availability of external funding
 - E. Funds restricted by granting institutions
 - F. Reserves for unanticipated expenses
3. Applicable data from:
 - A. Program evaluation findings
 - B. Governing Body initiatives
 - C. Other sources that address adequacy of fiscal and other resource allocations, including revenue, capital, and expenses
4. Information that indicates a need to refine fiscal allocations for the provision of care/service
5. Processes used for measuring and improving performance of each office, department, or service relative to the approved budget

6. Survey of the marketplace serving key patient populations

PROCEDURES

1. Senior management will collaborate with representatives from all departments and programs to develop, implement, and monitor the annual operating budget.
2. The Chief Financial Officer will prepare a budget process outline and time frame annually as well as a long-term capital expenditure plan.
3. The Chief Financial Officer or designee will prepare a draft schedule of revenues and expenses for the coming year and proposed capital expenditures, based on statistical data from current and prior periods.
4. The draft budget and capital expenditure plan will be reviewed by senior management and other designated personnel. Consideration will be given to:
 - A. Appropriateness of the plan for providing care/service to meet the patient's needs
 - B. Strategic plans that affect, involve, or influence the provision of care and service
 - C. Revenue, capital, and expense budgets that directly/indirectly relate to organization personnel's ability to provide care and service
 - D. Operational plans that directly/indirectly affect the organization personnel's ability to provide appropriate, effective, efficient, safe, timely, and continuous patient care and service with respect and caring
 - E. Policies that directly/indirectly affect organization personnel and the care/service they provide
5. Changes will be proposed to the Chief Financial Officer, based on the review by senior management and other designated personnel.
6. The Executive Director/Administrator will present the budget and capital expenditure plan to the appropriate review committees. Final approval will be by the Governing Body.
7. The Governing Body will review, revise as necessary, and approve the annual budget. These actions will be documented in the Governing Body meeting minutes.

PURPOSE

To ensure compliance with Medicare Home Health Conditions of Participation, (42 CFR 484.100 (f)) in regard to Capital Expenditure planning.

POLICY

On an annual basis, senior management, including at least the Executive Director/Administrator, CFO, and program directors, will develop a three (3) year Capital Expenditure plan which includes the current year.

The overall Capital Expenditure plan and annual operating budget is reviewed and revised annually as directed by the Governing Body. (See "[Annual Operating Budget](#)" Policy No. 3-001.)

PROCEDURES

1. The plan will contain anticipated sources of financing for and the objectives of each anticipated expenditure of more than \$600,000 that would be considered capital items.

FINANCIAL MANAGEMENT AND CONTROL**Policy No. 3-003.1****PURPOSE**

To assist the organization in the provision of operational feedback to senior management and the Governing Body and to accumulate the financial information and statistics required to complete the Medicare Cost Report.

POLICY

Financial and operational tools and reports will be utilized to facilitate financial oversight and appropriation of sufficient funds to maintain organization operations and meet Medicare cost report requirements.

PROCEDURE

1. Monthly reports utilized in the oversight of the organization's operations may include:
 - A. Financial reports with documented variances
 - B. Admissions and discharges by payer source
 - C. Units of service by discipline and payer
 - D. Patients by diagnostic category and length of stay
 - E. Bad debt and indigent care
 - F. Compliance with regulatory body requirements
 - G. Denials of payment by source
 - H. Cost per episode/visit/product
 - I. Billing by payer
 - J. Accounts receivable by payer
 - K. Aged receivables by payer
 - L. Bad debt allowances
 - M. Accounts payable by vendor
 - N. Aged payables by vendor

2. Monthly financial statements will contain key financial ratios and show a reasonable match between revenue and expense line items, including at least:
 - A. Cost per unit analysis
 - B. Days of revenue and receivables
 - C. Cash flow/debt ratio
 - D. Net income/operating revenues
 - E. Reconciliation of budget to actual results of operations
3. Senior management will review the reports for variances and trends to evaluate the organization's performance and to make current and informed decisions ensuring financial success.
4. Customary financial controls will be strictly adhered to, including:
 - A. A definition of internal audit procedures and annual review of the financial plan and budget
 - B. An financial review by a qualified individual will be conducted annually which will include identification of recommendations and a written report
 - C. Adherence to established financial policies and procedures
 1. Segregation of duties
 2. Reconciliation of control accounts
 3. Approval levels for disbursements and adjustments
 4. Collection of accounts receivable
 5. Budgeting
 6. Receipt of funds
 7. Disbursement of funds
 8. Cash and asset account reconciliation
 9. Cash management.
5. Senior management will provide monthly financial summary reports to the Governing Body for review.

6. Annually, senior management will collect the needed data for the Medicare cost report which includes:
 - A. Number of patients served
 - B. Number of patient visits by discipline and payor source
 - C. Unduplicated number of patients served by payor
 - D. Standard fee structure for all services
 - E. Square footage by Medicare cost center
 - F. Patient visits by discipline by CBSA code
 - G. Medical supply charges by payor source
 - H. Related party transactions
 - I. Segregation of non-reimbursable activities – cost and utilization statistics
 - J. FTEs by discipline
 - K. Up-to-date access to IACS (Individuals Authorized Access to CMS Computer Services)
7. The CFO will be responsible for filing the annual Medicare cost report per Medicare guidelines.
8. The organization will ensure full compliance with applicable statutes, regulations, and Federal health care program requirements to ensure the following regarding the cost report:
 - A. Costs are not claimed unless they are reimbursable, reasonable, and are based on appropriate and accurate documentation
 - B. Allocations of costs to various cost centers are accurately made and supportable by verifiable and auditable data
 - C. Unallowable costs are not claimed for reimbursement
 - D. Accounts containing both allowable and unallowable costs are analyzed to determine the unallowable amount that should not be claimed for reimbursement
 - E. Costs are properly classified
 - F. Medicare fiscal intermediary prior year audit adjustments are implemented and are either not claimed for reimbursement or if claimed for reimbursement, are clearly identified as protested amounts on the cost report

- G. All related parties are identified on the cost report and all related party charges are reduced to the cost to the related parties
- H. Home Health Chain: Allocations from the home office cost statement to individual home health agency cost reports are accurately made and supportable by verifiable and auditable data
- I. Management fees are reasonable and necessary, and do not include unallowable costs, such as certain acquisition costs associated with the purchase of a home health agency
- J. Any return of overpayments, including those resulting from an internal review or audit, are appropriately reflected in cost reports
- K. The organization will promptly notify the Medicare fiscal intermediary, or other payor, in writing of errors discovered after the submission of the cost report, and, where applicable, after the submission of a home health agency chain's home office cost statement

PURPOSE

To assist the organization in managing its financial resources effectively.

POLICY

Organization's Name is committed to maintaining a fiscally strong operation in order to continue its services to the community and to maintain its responsibility to the people who are employed by the organization.

PROCEDURE

1. Senior management will collect, analyze, and use financial information on a regularly scheduled basis to ensure that there is sufficient funding to maintain organizational operations.
2. A positive cash flow will be maintained by basing financial decisions on information gained through the astute use of cash forecasting tools.
3. Contingency plans will be maintained in the unlikely event of operating shortfalls.
4. In the event that the organization would become financially challenged, a profitability review of each program will be completed by senior management.
5. Reduction or suspension of services or termination of unprofitable programs may be considered in financially challenging times.
6. The Executive Director/Administrator will present program change recommendations to the appropriate oversight committees for review. Final approval will be by the Governing Body.

PURPOSE

To provide senior management with necessary financial information for effective fiscal decision-making.

POLICY

Organization's Name will maintain and utilize a management information system to generate key financial reports on a monthly basis.

Monthly financial reports will include key financial ratios and will directly compare revenue and expense line items.

PROCEDURE

1. The director for Information Management will supervise production of the organization's monthly financial reports. Senior management will review these reports for accuracy.
2. Senior management will identify and explain variances and trends identified through financial report review.
3. Senior management will compare monthly financial results with budgeted goals and explain identified variances.
4. Significant variance or trend information will be utilized by senior management to reforecast the organization's financial performance and goals on a go-forward basis.
5. Summary reports will be submitted to the Governing Body at least quarterly.
6. Year-to-date financial performance information will be annualized and utilized to prepare the organization's annual budget and capital expenditure for the upcoming year.
7. End-of-year financial data will be closely analyzed and compared to historical performance to identify trends, which may influence senior management's decisions regarding program development, revisions in existing programs, cessation of existing programs, or redesign of the organization's marketing plan.

PURPOSE

To define the process by which fees are determined for each service provided by the organization.

POLICY

The organization will maintain a competitive fee structure for services to ensure an acceptable profit margin. Charges will be based upon current reimbursement rates.

Any variation from established fees (e.g., managed care contractual discounts) will require approval by the Executive Director/Administrator, the Finance Committee, and, ultimately, the Governing Body.

PROCEDURE

1. Senior management will collaborate to establish a fee for each service provided based on actual costs per discipline/service and current market fee analysis.
2. Cost of services vs. fees for services will be monitored regularly to ensure that costs do not exceed fees.
3. Recommended revisions in the organization's fee structure will be sent to the Governing Body for approval.
4. The Chief Financial Officer, in conjunction with the Executive Director/Administrator, will have the authority to negotiate discounts with contracted third-party payers. Such discounts will follow Governing Body-established guidelines.
5. Fee structure will be made available to patients/families, upon request, and staff through team education.
6. Fee structure will be made available to the public and referral sources upon request.

PURPOSE

To identify the criteria to be applied when accepting patients for charity care.

POLICY

Patients without third-party payer coverage and who are unable to pay for medically necessary care will be accepted for charity care admission, per established criteria.

Organization's Name will establish objective criteria and financial screening procedures for determining eligibility for charity care.

The organization will consistently apply the charity care policy.

PROCEDURE

1. When it is identified that the patient has no source for payment of services and requires medically necessary care/service, the patient must provide personal financial information upon which the determination of charity care will be made.
2. A social worker, as available, will meet with the patient to determine potential eligibility for financial assistance from other community resources.
3. The Executive Director/Administrator, with the appropriate program director, will review all applicable patient information, including financial declarations, physician (or allowed practitioner) orders, initial assessment information, and social work notes to determine acceptance for charity care.
4. All documentation utilized in the determination for acceptance for charity care will be maintained in the patient's billing record.
5. When financial declarations reveal the patient is able to make partial payment for services, the Executive Director/Administrator, with the appropriate program director, will determine the sliding-fee schedule to be implemented.
6. The revised sliding-fee schedule will be presented to the patient for agreement and signature.
7. After acceptance for charity care, the patient's ability to pay will be reassessed every 60–90 days.

8. When the organization is unable to admit the patient or to continue charity care, every effort will be made to refer the patient for appropriate care/service with an alternate provider.
9. The referral source will be advised of acceptance, non-acceptance, continuation, or discharge from charity care.

PURPOSE

To ensure the accuracy of billed charges.

POLICY

Prior to finalization of an invoice, all charges will be verified against visit notes or delivery tickets as appropriate.

PROCEDURE

1. Personnel will submit documentation (paper or electronic) on a daily basis verifying the number of visits or deliveries completed per day.
2. Documentation will be verified. Discrepancies will be discussed with the clinician to ensure that the information is correct prior to billing.
3. Charges for services will be generated from the daily documentation, once its accuracy has been verified.
4. The weekly pre-billing register will be verified against visit/delivery documentation. Discrepancies will be researched and the pre-billing register corrected as indicated.
5. Upon verification of the pre-billing register, final bills will be produced and transmitted to the appropriate payer.

BILLING AND COLLECTIONS**Policy No. 3-009.1****PURPOSE**

To define responsibilities and processes for billing and claims submission of appropriate parties for services provided.

POLICY

At the time of admission, third-party payer information will be collected and verified. The patient is informed at this time of any financial responsibility he/she may incur.

All applicable federal and state regulations regarding the billing of Medicare/Medicaid will be strictly followed, as indicated.

Commercial third-party payer requirements will be known and followed to expedite the collection process.

PROCEDURE**Billing and Claims Submission**

1. Medicare claims will be produced and billed per current regulations. Signed and dated physician plans of care and other orders will be received prior to final claim billing. Personnel responsible for Medicare billing and collection functions will be instructed to regularly monitor changes posted as Program Memorandum at www.cms.gov.
2. Medicaid claims will be produced and billed per current state regulations. Personnel responsible for Medicaid billing and collection functions will be instructed to regularly monitor changes posted on the applicable state web site.
3. Non-government claims (commercial insurance) will be produced and billed per each company's billing guidelines. A copy of billing guidelines for each contracted payer will be kept current and available for billing personnel. Billing guidelines for non-contracted payers will be obtained at the time of insurance coverage verification.
4. When questions arise regarding billing and coding practices, personnel will seek clarification from management. If management cannot resolve a particular question, outside resources (i.e. MAC, legal counsel or a consultant) should be contacted for assistance. The inquiry and answer should be properly documented in writing and be sent to the Corporate Compliance Officer.
5. Billing staff should only submit claims when there is appropriate and complete clinical documentation to fully support the claim. The patient documentation should include the following:
 - A. Length of time spent conducting the service,

- B. The individual (name and credentials) providing the services,
- C. Where appropriate, the person(s) supervising the provider of services, and
- D. Information to fully support the services rendered, as well as codes and diagnoses to be utilized for each claim.

Collections

1. Insurance copayments will be billed directly to the patient, upon receipt of third-party payment. The copayment or deductible will not be routinely waived. If the patient does not have the ability to pay, the charity care procedure will be implemented.
2. Payments will be posted daily. Billing staff will not post payments to accounts for which they submitted claims.
3. Unpaid claims ("aging") will be reviewed monthly. Follow-up will include at least:
 - A. Thirty (30) days unpaid – send second statement showing outstanding balance.
 - B. Sixty (60) days unpaid – send third statement showing outstanding balance and a reminder note stating the account is past due.
 - C. Ninety (90) days unpaid – send first collection letter.
 - D. One hundred-twenty (120) days unpaid – send second collection letter and contact by telephone to ascertain reason for delay in payment.
 - E. More than 120 days unpaid – send final collection letter and make second telephone contact.
4. Documentation of all collection efforts will be maintained in the billing record.
5. At the time an account ages to greater than 120 days, the Chief Financial Officer will be informed and will become responsible for further action.
6. The Chief Financial Officer will review all previous attempts to collect the account and make a recommendation to the Executive Director/Administrator to either send the account to a professional collection service, if applicable, or to write the balance off as bad debt.

Other Procedures Regarding Billing

1. Bill only for items or services actually rendered
 - A. Never submit a claim without adequate documentation the services billed were actually provided.

- B. Such documentation should include the date and time of service provision, the identity of the person to whom the service was rendered, a description of the service rendered, and the identity of the person providing the service.
- 2. Bill only for medically necessary services.
 - A. Claims should only be submitted for services that are medically necessary and that were ordered by a physician or other appropriate licensed provider.
 - B. Should any question arise regarding the medical necessity of a service, adequate documentation to substantiate the medical necessity must be provided and reviewed prior to submitting the claim.
 - C. Medicare claims for services must be "reasonable and necessary" according to the standards for Medicare reimbursement.
- 3. Check all billing codes for accuracy
 - A. Claims should only be submitted when the correct billing code(s) has been assigned to the service as outlined by the payor including Medicare and Medicaid.
 - B. Care should be taken to ensure that all Medicare claims are submitted with the correct HHRG (Home Health Resource Group)
 - C. When questions arise regarding the proper code (s) to utilize, personnel will seek clarification from the appropriate management personnel. If management cannot resolve a particular question, outside resources (i.e. MAC, legal counsel or a consultant) should be contacted for assistance prior to claim submission
- 4. Check to ensure that there is no duplicate billing
 - A. Check the claim to ensure that no more than one (1) claim is submitted for the service rendered.
- 5. Do not submit claims for improperly referred clients
 - A. Claims will not be submitted if it is found that the referral was due to any violation of the anti-kickback statute.
- 6. Refund credit balances accurately and in a timely manner.
- 7. Discovery of billing errors
 - A. If a billing error is discovered the error should be reported to the Billing Manager who will investigate the cause of the error
 - B. Appropriate steps will be taken to correct the error
 - C. The Corporate Compliance Officer will be notified if necessary

- D. Any overpayment identified as a result of an error found through audits or recognized by an employee or agent (s) of the organization at any level, will be promptly paid to the appropriate payor. Interest will be paid if required.
 - 1. Medicare and Medicaid: Required to report and return the overpayment within "60 days after the date on which the overpayment was identified"
 - 2. Third-party payors: Report and return the overpayment as outlined in the payor's billing and overpayment instructions.

ACCOUNTS RECEIVABLE REVIEW**Policy No. 3-010.1****PURPOSE**

To provide an accurate account and review of the organization's billing and collection efforts.

POLICY

An accounts receivable report is reviewed monthly by senior management to ensure the proper billing and collection of accounts.

Organization's Name strives to maintain its accounts receivable days at 90 days or less.

PROCEDURE

1. An accounts receivable report will be generated for review on a monthly basis.
2. Accounts remaining unpaid after 60 days will be identified for follow-up attempts to collect.
3. A summary report of additional collection efforts will be prepared by billing personnel for each account open after 90 days.
4. The aging summary report will be submitted monthly with the accounts receivable report to senior management for review.
5. Senior management will forward the summary collections report and any available additional information regarding collection efforts to the Governing Body on a regular basis.
6. Collection efforts will be tracked and trended monthly and compared to the previous year's performance. Significant variances from the previous year will be reviewed for identification of the variance cause.

PURPOSE

To define the mechanism for processing uncollectible accounts.

POLICY

After all efforts to collect payment have been exhausted, the account may be written off as bad debt.

PROCEDURE

1. When an account has been determined by the billing department supervisor to be uncollectible, a request for write-off will be completed, and copies of supporting documentation will be attached. Required information to accompany the request for write-off includes:
 - A. Date of admission and discharge
 - B. Total patient charges
 - C. Amount requested for write-off
 - D. Reasons for write-off
 - E. Copies of all documentation with the delinquent account
 - F. Other information as indicated
2. The request for write-off and supporting documentation will be forwarded to the Chief Financial Officer for approval.
3. The Chief Financial Officer has the authority to approve the write-off of bad debt up to \$1,000.
4. Bad debt write-off greater than \$1,000 requires the approval of the Chief Financial Officer and the Executive Director/Administrator.
5. The request for write-off and accompanying support documentation will be returned to the billing supervisor within five (5) business days.
6. When approved, bad debt write-off will be entered into the financial system designated with the appropriate general ledger code.

7. When a request for bad debt write-off is not approved due to the need for additional information, that information will be collected and forwarded to the Chief Financial Officer within five (5) business days.
8. All associated documentation will be retained by the organization for the same period of time as the clinical record.

CONTRACTUAL ALLOWANCES**Policy No. 3-012.1****PURPOSE**

To outline the contractual allowance process.

POLICY***Definition***

1. **Contractual Allowance:** The difference between billed charges and actual reimbursement.
 - A. Monthly contractual allowances, as permitted via managed care contracts, will be projected during the annual budgeting process.
 - B. Actual contractual allowances will be reviewed against projected contractual allowances for identification of significant variances.

PROCEDURE

1. Contractual allowances will be posted at the same time as the receivable.
2. The contractual allowance will be determined by subtracting the allowed reimbursement from the organization's published charges.
3. Senior management will review variances in actual contractual allowances as compared to projected contractual allowances.
4. The Chief Financial Officer will investigate significant variances and make appropriate recommendations.

PURPOSE

To ensure that cash receipts are properly recorded.

POLICY

The cash receipts system will adhere to proper accounting internal controls.

PROCEDURE

1. Mail will be received, opened, and date-stamped by the office manager, and checks/cash will be forwarded to the finance department.
2. A daily check/cash log will be prepared, and it will include an itemized list of each check and cash payment received.
3. The check/cash log will be totaled and forwarded, along with the calculator tape, to designated personnel.
4. Billing personnel will total the checks and cash received and retain the attached calculator tape.
5. A person who has not prepared either of the two (2) original calculations will resolve any discrepancies between the totals.
6. Copies will be made of each check and any cash received and filed in the appropriate account binder.
7. All cash receipts will be deposited on the day received. A copy of the validated deposit slip will be returned to the organization.
8. The validated deposit slip, copies of checks/cash, and remittance advice records will be forwarded to and retained by the accounts receivable department.

**PURCHASING AUTHORIZATION
AND ACCOUNTS PAYABLE
Policy No. 3-014.1****PURPOSE**

To establish a cost-effective system for securing routine supplies necessary for the efficient operation of the organization.

To define the mechanism for payment of accounts.

POLICY

Purchases of routine operating supplies and non-capital equipment will be made in the most cost-effective manner and will not exceed budgeted allowances for such items.

Accounts payable (invoices) containing proper documentation of supplies or services will be paid within 60 days.

PROCEDURE

1. During the annual budget preparation process, each department head will project the type of supplies and non-capital equipment required for the upcoming year along with the approximate monthly cost.
2. Upon approval of the budget by senior management and the Governing Body, each department director will be authorized to purchase budget-approved items in the amounts and at the cost approved.
3. The department director may execute purchase orders for budget-approved purchases.
4. Open purchase orders will be filed until the order is received by the organization, at which time the employee receiving the order verifies the merchandise against the purchase order.
5. When the order is confirmed as correct and complete, the purchase order will be forwarded to accounts payable for processing.
6. Accounts payable personnel compare the purchase order to the invoice and process payment.
7. The Executive Director/Administrator must approve items not included in the approved budget but subsequently identified as necessary to the operation of the department. A requisition for non-approved purchasing must be completed and sent to the Executive Director/Administrator.

8. The approved requisition will be returned to the appropriate department director, who may then initiate the purchase process. The approved requisition will be retained with the purchasing order.

FIXED ASSETS AND DEPRECIATION**Policy No. 3-015.1****PURPOSE**

To establish guidelines to differentiate between a current expense and a fixed asset equipment purchase.

To establish depreciation schedules for fixed assets.

POLICY

Per IRS regulations, all fixed assets placed into service after 1986 will be depreciated using the Modified Accelerated Cost Recovery System (MACRS). Fixed assets placed into service prior to 1987 will be depreciated using the Accelerated Cost Recovery System (ACRS). Exceptions may include the use of straight-line depreciation, as directed by the IRS (e.g., computer software).

Definition

1. **Fixed Asset:** A tangible item with a useful life in excess of one (1) year.

PROCEDURE

1. An item's first year depreciation will be based on the number of full months the item was in service.
2. Examples of fixed assets and associated depreciation methods and schedules are included in the following table.

Fixed Asset	Depreciation Method	Depreciation Schedule
Furniture and fixtures	MACRS/ACRS	7 years
Computer equipment	MACRS/ACRS	5 years
Fax machines	MACRS/ACRS	5 years
Software*	Straight-line	3 years
Medical Equipment	MACRS/ACRS	5 years
Leased Capital Equipment	MACRS/ACRS	Lifetime of the lease

* Computer software can be straight-line depreciated, if it meets all of the following tests:

- It is readily available for purchase by the general public.
- It is subject to a nonexclusive license.
- It has not been substantially modified.

PURPOSE

To define the process for ensuring an accurate payroll processing system.

POLICY

Each classification of non-salaried personnel will document hours worked on a daily basis and submit documentation to their respective supervisor on a biweekly basis.

Personnel will be paid for hours worked or approved benefit time, in accordance with organization policy and current wage and hour law.

PROCEDURE

1. Personnel paid by the hour will record actual hours worked on a daily basis.
2. Every two (2) weeks this documentation must be submitted to the appropriate supervisor for review and approval.
3. When benefit time is requested, the request must accompany the submission of documented time worked.
4. Approved time records will be forwarded to the payroll department for processing.
5. All time will be allocated to the employee's primary work center, unless indicated otherwise on the time record.
6. In keeping with state and federal laws and regulations, the organization will withhold required taxes and any other deductions necessary. The withheld amounts will be promptly forwarded by the organization to be appropriate government entity, per published guidelines.
7. Direct deposit of paychecks will be available for all personnel.
8. Those not participating in the direct deposit program may have their checks mailed to their home, or they may receive their checks in person at a predetermined time.

ALLOCATION OF TIME WORKED**Policy No. 3-017.1****PURPOSE**

To outline the process for allocation of worked time to the proper cost center.

POLICY

When personnel perform services for more than one (1) cost center, a detailed record will be maintained to ensure allocation of time to the correct cost center.

PROCEDURE

1. Personnel performing services for multiple cost centers will document hours worked for each cost center on a biweekly basis.
2. The supervisor will review and approve the documentation. Each supervisor of the affected cost centers will receive a copy of the approved time sheet allocating hours to the various areas.
3. Time allocation documentation will be submitted to the payroll for calculation of hours per cost center.
4. Time allocation records will be retained per current CMS guidelines, as applicable.

PURPOSE

To provide guidelines for the acceptance and use of donated funds.

POLICY

Organization's Name will accept donations from individuals who wish to support the home health program and services. Most donations are in memory of patients who have been served by the program. Funds will be utilized to promote the home health program. Funds will be utilized in a constructive and consistent manner.

PROCEDURE

1. When a surviving family/caregiver inquires as to the procedure regarding donation funds to the organization, the following information can be given:
 - A. The donor forwards a check with a note or card identifying in whose name the donation is being made. The check is made out to Organization's Name.
2. All donated funds received will be forwarded to the development department. This department will keep a record of all donations, make a copy of the check and correspondence for its files, and send an acknowledgment to the family/caregiver and a thank-you letter to the donor. The Executive Director/Administrator will sign a card expressing appreciation, which will be included in the card sent to the donor. The donated check will be forwarded to the business office.
3. Request for use of donated funds can be made by any organization team member.
4. All expenditures of the donated funds will be approved by the Executive Director/Administrator. Criteria for use or request:
 - A. To purchase items that increase comfort of patients and families/caregivers
 - B. To pay for personal care services that are needed but not covered under the Medicare/Medicaid benefit
 - C. To provide for special supplies or equipment that patients cannot afford and which will allow the patients to stay at home or return home
5. A report on the amount of donations and their usage will be submitted biannually to the Executive Director/Administrator. At any time, information about monies will be available from the Executive Director/Administrator.

CERTIFICATES OF INSURANCE**Policy No. 3-019.1****PURPOSE**

To ensure adequate and appropriate operation-related insurance coverage.

POLICY

Organization's Name employs individuals who provide home-based services. Thus, it accepts the responsibility for the provision of all employer personnel-related insurance, such as workers' compensation coverage, malpractice, and liability.

The organization has an active Governing Body requiring protection for decisions made on behalf of the organization and its mission.

PROCEDURE

1. Workers' compensation coverage will be carried on all organization personnel. This insurance coverage will be in effect for all personnel while they are on duty, whether in an institution or in a patient's home.
2. Malpractice insurance will be carried for all licensed and non-licensed personnel in the amount of \$1,000,000.
3. General liability insurance in the amount of \$1,000,000 will be carried for all organization personnel.
4. All contract personnel will be required to carry their own professional liability insurance level of \$1,000,000 per occurrence and \$3,000,000 aggregate. Verification of coverage will be maintained in their personnel file.
5. Property and casualty insurance will be maintained in an amount to cover asset value.
6. Fidelity bonds will cover the value of liquid assets.
7. Director's and officer's liability coverage will be maintained at \$250,000. (Owners as principals will be excluded.)
8. Copies of the Certificate(s) of Insurance will be available to patients upon request.
9. Certificates of Insurance will be kept on file in the Executive Director/Administrator's office for inspection.

**OASIS DATA TRANSMISSION
Policy No. 3-020.1****PURPOSE**

To outline the OASIS data transmission requirements.

POLICY

The organization will adhere to all OASIS data transmission requirements as outlined in the Medicare Conditions of Participation, Reporting of OASIS Information 42 CFR 484.45. OASIS data will be reported electronically in accordance with 42 CFR 484.55.

PROCEDURE

1. The organization will encode and electronically transmit completed OASIS data to the national internet Quality Improvement Evaluation System (iQIES) for each applicable patient within thirty (30) days of completing the assessment of the patient.
2. OASIS data is collected and completed by the qualified clinician as part of the comprehensive assessment at the required time points (i.e. start of care, resumption of care, follow-up, transfer to inpatient facility with or without discharge, discharge to community, and death at home.)
 - A. All encoded OASIS data shall accurately reflect the patient's status at the time the assessment was completed.
 - B. The organization may take up to thirty (30) calendar days after the date of completion of the comprehensive assessment to enter the OASIS data into their computers using internet Quality Improvement Evaluation System (iQIES) or software that conforms to all CMS data transmission specifications available on the OASIS website.
 - C. All OASIS data items must be complete, i.e. locked, in order to accurately compute the information necessary for billing Medicare patients.
3. The encoded OASIS data must accurately reflect the patient's status at the time of assessment.
 - A. The organization will conduct clinical and data entry audits to verify that collected OASIS data are consistent with reported OASIS data.
 - B. The validation logs in the iQIES will be reviewed for accuracy purposes and kept for twelve (12) months.

- C. Discrepancies identified will be corrected following organization approved processes.
4. The organization must electronically transmit accurate, complete and encoded OASIS data for each patient to the iQIES (internet Quality Improvement and Evaluation System) ASAP (Assessment and Submission and Processing System) or CMS OASIS contractor at least monthly.
 5. The organization must electronically transmit data using electronic communications software that complies with the Federal Information Processing Standard (FIPS 140-2, issued May 25, 2001) from the HHA or the HHA contractor to the CMS collection site.
 6. The organization must transmit data that includes the CMS assigned branch identification number, as applicable.
 7. The organization must encode and transmit data using the software available from CMS or software that conforms to CMS standard electronic record layout, edit specifications, and data dictionary, and that includes the required OASIS data set.
 8. A tracking mechanism will be utilized by the organization to ensure accuracy and timeliness of OASIS data and transmission.
 9. The organization will contact iQIES for transmission instructions in the case that there is an inability to transmit OASIS data.

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CARE PLANNING PROCESS**Policy No. 4-001.1****PURPOSE**

To provide clinical direction to the clinicians providing direct patient care.

POLICY

A written plan of care will be initiated within five (5) days of start of care and updated at least every 60 days or as patient's condition warrants. The patient plan of care will be developed or revised within five (5) working days of initiation of each service or of the reassessment of the patient. All clinicians involved in the patient's care, either directly or indirectly, will contribute to the plan of care. The patient and family/caregiver will participate in decisions regarding the plan of care whenever possible. The care planning process will be documented on the plan of care, individualized discipline specific care plans (if applicable), clinical notes, medication profiles, care conference/summary forms, and discharge/clinical summaries.

The plan of care will be reviewed more frequently than every 60 days when a patient elects a transfer to another home health organization, a significant change occurs in the patient's condition, or a patient is discharged and readmitted during the same 60-day period.

PROCEDURE

1. At the time of the initial assessment, the clinician, along with other involved disciplines, will develop the patient plan of care based upon the patient's identified needs and will review it with the patient and family/caregiver. A Case Manager will be assigned to coordinate all activities involved in the patient's care.
2. All clinicians will consider the conclusions of initial and ongoing assessments in their care planning process, including but not limited to:
 - A. Individualized patient needs and resultant problems related to care, functional status, and family/caregiver support system
 - B. Changes in patient's condition
 - C. Clinical drug monitoring, as appropriate
 - D. Pain and symptom management, as appropriate
 - E. Psychosocial and spiritual needs of patient and family/caregiver, as appropriate
 - F. Patient treatment choices
 - G. Patient and caregiver education and training that the organization will provide specific to the patient's care needs.

3. Based on the assessment and conclusions, the plan of care will include, but will not be limited to:
 - A. Patient specific interventions and education
 - B. Identified patient problems and needs, including functional limitations, activities permitted, nutritional requirements
 - C. Reasonable, measurable, and individualized goals and outcomes identified by the organization and the patient
 - D. Specific services and treatments to be provided including services furnished via telecommunication systems (if appropriate)
 - E. Actions to be taken to meet the patient goals
 - F. Type, frequency, and duration of above actions
 - G. Equipment and supplies
 - H. Information related to any advanced directives
 - I. All pertinent diagnoses
 - J. The patient's mental, psychosocial, and cognitive status
 - K. All medications
 - L. Safety measures to protect against injury
 - M. A description of the patient's risk for emergency department visits and hospital re-admission and all necessary interventions to address the underlying risk factors.
 - N. Patient and caregiver education and training to facilitate timely discharge
 - O. Prognosis
 - P. Rehabilitation potential
 - Q. All patient care orders, including verbal.
 - R. Information related to any advance directives; and
 - S. Any additional items the HHA or physician may choose to include.

4. In addition to the plan of care, if a patient is accepted into the organization's telemedicine program (if applicable), the plan of care will incorporate elements of the telemedicine program.
 - A. The telehealth plan of care will be developed based on specific telemedicine interventions being provided, but will not be limited to:
 1. Disease management
 2. Remote vital signs
 3. Interactive home telehealth
 - B. The telehealth plan of care will include the telehealth encounter frequency and how the patient will be monitored should there be a discontinuation of the telehealth program.
5. The care planning decisions will be reflected in the specific services that will be provided and the associated actions planned and implemented to meet individualized patient problems and goals.
6. The plan of care will be based upon the physician's (or allowed practitioner's) orders and will encompass the equipment, supplies, and services required to meet the patient's needs.
7. Patient receiving physical therapy or speech therapy only will have a plan of care initiated by the primary physical therapist or speech therapist within 48 hours of completion of the initial assessment.
8. The plan of care will be revised as frequently as deemed necessary by the clinicians based on the ongoing assessments of the patient. Revision dates will be noted on the plan of care.
9. The frequency of the review of the plan of care will be based on changes in the patient's health status, needs, and the environmental factors affecting care. The clinicians will be responsible to revise the plan of care or update the plan at least every sixty (60) days. (See "[Reassessments/Recertification](#)" Policy No. 4-020.)
10. Changes in the plan of care will be noted with the following documentation:
 - A. Assessment
 - B. Plan of care with clinical outcome goals
 - C. Care plan with specific services/actions to be taken
 - D. Clinical notes
 - E. Verbal orders

11. Clinicians will inform the patient's physician (or allowed practitioner) of any changes that suggest a need to alter the plan of care. Changes must be written, dated, and signed by the professional making the changes.
12. The Case Manager will sign the plan of care and document other clinical personnel who reviewed it.
13. Problems and/or needs related to patient's condition, desires, abilities, family/caregiver support systems, and relevant medication monitoring will be included in the plan of care.
14. Services to be provided will be based on the prioritized needs of the patient. Each patient will be monitored for his/her response to care or services provided against established patient goals and patient outcomes to determine if goals have been achieved.
15. Care decisions and services to be provided will be made as a result of the care planning process, analysis of initial and ongoing assessments, and analysis of patient response to care against goals and outcomes.
16. The Clinical Supervisor or designee will review the plan of care for all patients.
17. The organization shall provide the patient and caregiver with a copy of written instructions outlining:
 - A. Visit schedule, including the day of the week, frequency of visits by personnel and personnel acting on behalf of the organization.
 - B. Patient medication schedule/instructions, including medication name, dosage and frequency and which medications will be administered by the organization's personnel and personnel acting on behalf of the organization
 1. Instructions will be written in a plain language the patient can understand and that does not use medical abbreviations.
 - C. Any treatments to be administered by the organization's personnel and personnel acting on behalf of the organization, including therapy services.
 - D. Any other pertinent instruction related to the patient's care and treatments that the organization will provide, specific to the patient's care needs.
 - E. Name and contact information of the organization's clinical manager.

PHYSICIAN PARTICIPATION IN PLAN OF CARE
Policy No. 4-002.1**PURPOSE**

To provide guidelines for the physician's (or allowed practitioner's) participation in home health care services.

POLICY

A physician (or allowed practitioner) will direct the care of every home health care patient admitted for service. The attending physician (or allowed practitioner) will certify that medical, skilled, rehabilitative, and social services provided by the organization are medically necessary and meets the requirements to be covered by Medicare. The attending physician (or allowed practitioner) will participate in the care planning process by initiating, reviewing and revising therapeutic and diagnostic orders. The care will be provided in compliance with the therapeutic and diagnostic orders and accepted standards and practice.

Definition

Verbal Order: A physician (or allowed practitioner) order that is spoken to appropriate personnel and later put in writing for the purposes of documenting as well as establishing or revising the patient's plan of care.

PROCEDURE

1. Physician, dentist, podiatrist or any other licensed practitioner within his or her scope of practice orders will be individualized, based on patient's needs, and include:
 - A. Patient diagnoses
 - B. Treatments and/or procedures needed, including type, frequency, duration, and goals
 - C. Medications to be administered and/or monitored
 - D. A description of equipment and related supplies provided by the organization
 - E. A description of any medical, physical, psychosocial, or environmental precautions, limitations, and activities permitted
2. When services are provided on the basis of a physician's (or allowed practitioner's) verbal orders, a registered nurse, or other qualified practitioner responsible for furnishing or supervising the ordered services, in accordance with state law shall document the orders in the patient's clinical record, and sign, date, and time the orders.

3. Orders are reviewed for appropriateness and accuracy before care, treatment and services are provided. The organization may assist the physician in determining medical necessity and formulating an appropriate plan of care, if requested by the physician (or allowed practitioner).
4. The attending physician's (or allowed practitioner's) verbal orders will be obtained at the time the plan of care is established.
5. The attending physician (or allowed practitioner) will sign the plan of care/treatment within 30 days of the start of care.
6. The attending physician's (or allowed practitioner's) recertification will be obtained in intervals of at least every 60 days when the patient's plan of care is reviewed, the patient recertified, and more often, if warranted.
7. At initial certification, the attending physician (or allowed practitioner) must perform the face-to-face encounter and compose a brief narrative that describes how the patient's clinical condition as seen during the encounter supports the patient's homebound status and need for skilled care. This encounter can occur up to 90 days prior to start of care or within 30 days after the start of care. (See "[Face-to-Face Encounter Procedure](#)" Addendum 2-003.A.)
8. The total plan of care is renewed more frequently when:
 - A. A patient elects a transfer to another home health agency
 - B. A significant change occurs in the patient's condition
 - C. A patient is discharged and readmitted during the same 60-day episode
9. Orders will be reviewed and revised by the patient's physician (or allowed practitioner) based on:
 - A. Changes in the care or service being provided
 - B. Changes in the patient's physical and psychosocial condition
 - C. The patient's response to care
 - D. The patient's outcome related to treatment care or services
 - E. Changes in diagnosis or treatment, including procedures, medications and equipment
10. The attending physician (or allowed practitioner) will recertify the need for continuing home health care services and provide an estimate of how long the services will be needed no sooner than five (5) days prior to the recertification date for patients receiving OASIS reassessments.
11. All original plans of care/treatment forms will be filed in the patient's clinical record.

12. If a patient changes physician, the clinicians will not continue services until obtaining new written orders from the physician (or allowed practitioner) assuming the patient's care.
13. If a patient is under the care of more than one (1) physician, the nurse will be responsible to the designated primary physician. All other physicians (or allowed practitioner) should be made aware of the services being provided to the patient. The referring physician will be informed of the involvement of additional consulting physicians, when applicable.
14. If a patient is referred by a hospitalist, who treated the patient during hospitalization, an attending physician (or allowed practitioner) who will follow the patient and be responsible for ongoing care must be determined and documented. (The hospitalist can provide the face-to-face encounter certifying that the patient's clinical condition supports homebound status and need for skilled services.)
15. The organization will educate physicians (or allowed practitioner), as appropriate, regarding the scope of their duty to certify patients for home health services to be reimbursed by Medicare.

VERIFICATION OF PHYSICIAN ORDERS**Policy No. 4-003.1****PURPOSE**

To ensure that accurate physician (or allowed practitioner) orders are obtained in accordance with applicable law and regulation.

POLICY

Orders will be obtained from a licensed physician (or allowed practitioner) for care and services to be provided to home health patients.

Verbal orders will be spoken to, taken and documented only by professional, licensed home health personnel (RN or qualified therapist). Orders received via text message will not be accepted.

A qualified individual will review each order or prescription before care is provided. The sole exception for verification will be with emergency orders or prescriptions where a delay for verification would likely result in an adverse result for the patient.

PROCEDURE

1. An order or prescription will be verified when there is a question or discrepancy in the order/prescription and when the order is communicated by someone other than the physician or his/her agent. The order or prescription reviewed may be the original order, a facsimile copy if permitted by law, or the direct transcription of a verbal order.
2. All telephone orders will be received and processed in accordance with state and federal laws and regulations.
3. All telephone orders or verbal orders will be "read back" to the physician (or allowed practitioner) or designee to assure accuracy.
4. Orders will be documented on a form provided by Organization's Name, dated and signed by the professional receiving the order.
5. A copy of the physician's (or allowed practitioner's) order will be kept in the clinical record.
6. The original of the order form will be delivered to the physician (or allowed practitioner) for signature.
7. When the signed order form is returned to Organization's Name, the copy will be removed from the clinical record and destroyed, and the original will be retained in the clinical record.
8. Signed orders will be in the clinical record within 30 days of initiation of care or interim order, unless otherwise specified by applicable state law and regulation.

REHABILITATION CARE PLANNING**Policy No. 4-004.1****PURPOSE**

To provide clinical direction to therapists providing direct patient care.

POLICY

Qualified rehabilitation professionals will develop and implement the rehabilitation care plan with the patient, family, and caregivers. Rehabilitation plans and goals will guide the provision of rehabilitation services and are appropriate to the patient's environment, needs, and severity of disease. Rehabilitation outcomes will promote optimal levels of functioning. Discharge planning from services will be integrated into the functional rehabilitation assessment.

All services will be provided in accordance with accepted standards of practice by or under the direction of a qualified physical, occupational, or speech therapist.

PROCEDURE

1. When rehabilitation needs are identified, the patient's physical status and functional abilities will be evaluated by a rehabilitation professional before instruction and treatments are initiated.
2. Based on this functional assessment, the rehabilitation professional will develop and implement the rehabilitation plan with the patient and family/caregiver. The patient will be encouraged to make choices about his/her participation in rehabilitation. The rehabilitation professional will encourage patient and family/caregiver participation in implementing the rehabilitation plan by:
 - A. Identifying interventions to reach reasonable goals
 - B. Coordinating and collaborating on rehabilitation interventions
 - C. Documenting the patient's treatment choices, response to interventions, progress toward goals and objectives, and changes in condition (See "[Care Planning Process](#)" Policy No. 4-001.)
3. The patient and family/caregiver will receive information regarding potential benefits and risks of rehabilitation services in order to make informed decisions. Their expectations will be considered and documented in the rehabilitation plan.
4. After the initial assessment and as treatment progresses, the patient will be reassessed on an ongoing basis every 30 days. Patient goals will be revised based on reassessment data.

5. Rehabilitation goals and plans will be based on, but not limited to, the following:
 - A. The patient's personal goals and expectations
 - B. Individualized needs for rehabilitation that are consistent with the patient's diagnosis, age, severity of disease, prognosis, and disability
6. Rehabilitation goals and plans are designed to help the patient achieve and maintain his/her optimal level of functioning, self-care, and independence by:
 - A. Managing the patient's specific health problems
 - B. Maximizing the patient's emotional well-being in accordance with the diagnosis, prognosis, and treatment program
7. Discharge planning is initiated early in treatment based on ongoing assessments and stated expectations for achieving treatment goals and objectives. Criteria for discharge or termination of services may vary based on age, disability, treatment setting, and the plan for professional services (See "[Discharge Criteria and Process](#)" Policy No. 4-045.)

Note: An organization that furnishes outpatient physical therapy or speech language pathology services must meet all of the applicable conditions and the additional health and safety requirements for outpatient services outlined in §§485.711, 485.713, 485.715, 485.719, 485.723 and 485.727.

NUTRITION CARE PLANNING**Policy No. 4-005.1****PURPOSE**

To provide guidelines for clinicians in the provision of appropriate nutrition care.

POLICY

Patient with identified nutrition care needs will be referred to an appropriate, qualified registered dietitian, when indicated.

Definitions

1. ***Nutrition Care***: The interdisciplinary nature of nutrition care, including physicians, nurses, dietitians, pharmacists, and others as appropriate; interventions and counseling on appropriate nutrition intake by integrating information from nutrition assessments.
2. ***Assessment***: The comprehensive approach to defining nutrition status, using a nutrition screening, and interdisciplinary assessment process.
3. ***Education***: The role of nutrition care and treatment.
4. ***Coordination***: The communication of nutrition status and recommended nutrition care planning among all disciplines involved in the care of the patient.

PROCEDURE

1. During the initial assessment, the admitting clinician will assess the patient's nutritional status. The nutritional assessment will include information as identified in "[Nutritional Assessment](#)" (see Policy No. 4-022.)
2. Based on the results of the assessment, food and nutrition therapies will be ordered as part of the plan of care prescribed by the physician (or allowed practitioner).
3. The admitting clinician will arrange for a nutritional consultation when:
 - A. The patient is at moderate or high nutrition risk.
 - B. Nutrition care planning is complex.
 - C. There is need for a further nutrition assessment by a specially trained and educated clinician.
 - D. Organization personnel are not able to provide complete nutrition therapies.

4. During routine home visits, clinicians will continually educate patients, when appropriate, regarding:
 - A. Proper conditions of sanitation to protect food and nutrition therapies from contamination and spoilage
 - B. Proper temperatures of food storage, utilizing appropriate thermometers and maintaining temperature records, when appropriate
 - C. The control of lighting, ventilation, and humidity to prevent condensation of moisture and growth of molds, when appropriate
 - D. Thorough cleaning and sanitizing of all work surfaces, supplies, and equipment after each use
 - E. Appropriate hand washing prior to and during preparation of food/nutrition solutions
5. Ongoing monitoring to determine the extent to which goals are achieved and patient's nutrition needs/problems are met/resolved will include the following activities, when appropriate:
 - A. Monitoring patient's response to food/nutrition therapy
 - B. Reviewing the appropriateness of choice of food/nutrition therapy, regimen, and route
 - C. Communicating conclusions verbally and/or in writing to other home care team members
 - D. Identifying/monitoring the patient who is not receiving adequate nutrition intake
 - E. Identifying/monitoring the comfort levels of patient who chooses not to take food/nutrition therapy
 - F. Increasing the monitoring level for transitional feedings from parenteral to enteral or oral to enteral, as applicable
6. Potential problems for patient's nutritional status will be reported to the clinician's immediate Clinical Supervisor and, when appropriate, the patient's physician.
7. When a patient has been prescribed a special diet, the clinician will provide the patient and family/caregiver with written and verbal information regarding the diet.

8. The effectiveness and appropriateness of nutrition therapies will be monitored on an ongoing basis through evaluation of goals, outcomes of therapies, and during case conferences. This evaluation will be communicated to all disciplines involved in the care of the patient.
9. Clinical staff that may be administering enteral or parenteral feedings will have a demonstrated competency on file and will have participated in additional education. (See "Scope of Assessments/Qualifications" (HR) Policy No. 1-010.)

HOME HEALTH AIDE PLAN OF CARE
Policy No. 4-006.1**PURPOSE**

To define the process for the development and use of the home health aide plan of care.

POLICY

Each patient receiving home health aide services will have an individualized plan developed by an appropriate professional and utilized to direct the care performed by the assigned aide.

PROCEDURE

1. The patient's Case Manager or other appropriate skilled professional (physical therapist, speech-language pathologist, or occupational therapist), upon initialization of aide services, will develop the written home health aide plan of care, consistent with the comprehensive plan of care and physician (or allowed practitioner).
2. The home health aide plan of care will be individualized to the specific patient and will include at least:
 - A. Type of services/procedures to be provided, such as hands on personal care, the performance of simple procedures as an extension of therapy or nursing services, assistance in ambulation or exercises, and assistance in administering medications ordinarily self administered.
 - B. Frequency of visits (use of PRN or per patient choice are not acceptable)
 - C. Diagnosis/prognosis, if relevant to care
 - D. Functional limitations
 - E. Patient's mental status
 - F. Activities permitted
 - G. Nutritional requirements
 - H. Specific procedure to be performed, including amount, frequency, and duration
 - I. Safety measures, including use of specific equipment
 - J. Instructions for completion of documentation
 - K. Reporting changes in patient's condition and needs

L. Allergies

3. The Case Manager, or other appropriate skilled professional, will review the home health aide plan of care with the aide assigned to the case. The Case Manager, or other appropriate skilled professional, and assigned aide will sign the home health aide plan of care.
4. The home health aide plan of care will be revised at least every sixty (60) days based upon a professional reassessment of the patient and at any time the patient's change of condition warrants revision.
5. The Case Manager, or other appropriate skilled professional, will review changes to the home health aide plan of care with the assigned aide. The Case Manager, or other appropriate skilled professional and aide will initial the plan to indicate this review has occurred.
6. The home health aide plan of care will be reviewed with the patient to ensure understanding of the aide's role in the home.
7. A current copy of the home health aide plan of care will be maintained in the patient's home.
8. The registered nurse or other appropriate clinician who is familiar with the patient, the patient's plan of care and the written patient care instructions will complete a supervisory assessment of the home health aide no less frequently than every fourteen (14) days to ensure care is provided according to plan. The home health aide does not need to be present during the supervisory assessment.
9. Home health aides are members of the interdisciplinary team and shall report changes in the patient's condition to a registered nurse or other appropriate skilled professional, and shall complete appropriate documentation in compliance with organizational policies and procedures.

ORIENTATION OF ASSIGNED HOME HEALTH AIDE
Policy No. 4-007.1**PURPOSE**

To define the process for the communication of patient information and assignment of responsibilities to paraprofessional personnel caring for the patient.

POLICY

When making patient care assignments, consideration will be given to the needs of the patient, the home health aide's competencies, and the specific care that is to be provided.

Home health aides will receive patient information in the form of an aide assignment prior to caring for the patient. This will include information about the physical, psychosocial, and environmental aspects of care.

Patient care communication may include verbal or written instruction and demonstration. The communication about assigned responsibilities may include on-site orientation when appropriate, and at the very least, telephone contact prior to caring for the patient.

PROCEDURE

1. The personal care and support services provided will be based on the initial and ongoing assessments of patient needs as conducted by a nurse or therapist in the patient's home.
 - A. The nurse or therapist will be responsible for the initial assessment and assignment of the home health aide. Assessments are updated every 60 days and as the patient's condition changes.
 - B. The functional status, psychological status, and availability of support will be considered in determining the frequency of visits and plan of care.
2. The home health aide will review the duties to be performed and the arrangements for providing services as stated in the plan of care, and he/she will discuss this with the nurse or therapist.
 - A. An orientation or placement visit is scheduled in the patient's home by the clinician, whenever feasible.
 - B. A home health aide assignment sheet is completed, reviewed with the home health aide, and signed by either the nurse or therapist and the home health aide. Return demonstration will be requested, as appropriate.

3. Each patient receives care in accordance with the plan of care and related instructions.
 - A. The home health aide assignment sheet correlates with the orders on the plan of care (485).
 - B. The aide will complete an aide clinical note on each patient. This is returned to the organization on a weekly basis and incorporated in the patient record. If electronic documentation is utilized, notes will be sent daily.
 - C. When a health problem is identified or a significant change in a patient's physical condition is noted, the aide will report this information to the coordinator and/or a Clinical Supervisor in the office.
4. When a change in home health aide or substitution of aide is required:
 - A. Orientation will occur on-site unless the organization personnel are already familiar with the patient.
 - B. The coordinator will complete a home health aide service change report that is included in the clinical record.

SUPPORT/CHORE WORKER SERVICE PLAN
Policy No. 4-008.1**PURPOSE**

To define the process for the development and use of the support/chore worker service plan.

POLICY

Each patient receiving support/chore worker services will have an individualized plan developed and utilized to direct the care performed by the assigned worker.

PROCEDURE

1. The patient's Case Manager or designee, upon initialization of support/chore worker services, will develop the support services plan.
2. The support services plan will be individualized to the specific patient and will include at least:
 - A. Type of services required
 - B. Schedule of services
 - C. Specific duties to be performed including amount, frequency, and duration
 - D. Other items pertinent to the service needs of the patient
3. The Case Manager will review the support services plan with the worker assigned to the case. The Case Manager and assigned worker will sign the service plan.
4. The support services plan will be revised at least every six (6) months based upon a professional reassessment of the patient and at any time the patient's change of condition warrants revision.
5. The Case Manager will review changes to the support services plan with the assigned worker. The Case Manager and worker will initial the plan to indicate this review has occurred.
6. The support services plan will be reviewed with the patient to ensure understanding of the worker's role in the home.
7. A current copy of the support services plan will be maintained in the patient's home.
8. The Case Manager or other appropriate clinician will supervise the worker to ensure care is provided according to plan.

DISCHARGE PLANNING**Policy No. 4-009.1****PURPOSE**

To promote patient independence, safety, and use of community resources prior to patient discharge from the organization.

POLICY

Discharge planning will be initiated for every patient upon admission to the organization. Patients will not be discharged without appropriate preparation. The patient's continuing care needs will be assessed on an ongoing basis, as well as at discharge. Information will be provided to assist the patient in planning his/her discharge, including referral and transfer.

PROCEDURE

1. During the initial assessment, the clinician will:
 - A. Assess the following and identify:
 1. Anticipated date of discharge
 2. Resources available, including persons and finances
 3. Anticipated changes in living situation
 4. Areas that might require assistance
 - B. Document the patient discharge potential on the plan of care.
 - C. Provide information regarding the patient discharge potential at case conferences with other team members, as appropriate.
2. Clinicians will assist patients regarding their discharge by:
 - A. Consulting with the patient and family/caregiver regarding the need for discharge from the organization
 - B. Serving as a referral source for patient and family/caregiver in obtaining follow-up support services
 - C. Consulting with the patient and family/caregiver regarding the provision of discharge information

- D. Participating in a conference with the patient and family/caregiver regarding the patient discharge plans, if requested
- E. Sending a post discharge letter to the patient's physician (or allowed practitioner)
- 3. Clinicians will inform the appropriate Clinical Supervisor in the event that problems arise in discharge planning and obtain appropriate assistance.
- 4. If patients are being transferred to another HHA or being discharged to a SNF, IRF, or LTCH, the organization will assist patients and/or caregivers in selecting a post-acute care provider by using and sharing data that includes but is not limited to HHA, SNF, IRF, or LTCH data on quality measures and data on resource use measures.
- 5. The organization will ensure that the post-acute care data on quality measures and resource use measures are relevant and applicable to the patient's goals of care and treatment preferences.
- 6. The organization will ensure all necessary medical information pertaining to the patient's current course of treatment, treatment preferences, illness and post-discharge goals of care are sent to the receiving facility or health care practitioner to ensure a safe and effective transition of care.
- 7. The organization will comply with all requests from the receiving facility or health care practitioner for clinical information as necessary for the treatment of the patient.
- 8. All communication and information regarding discharge planning will be documented in the clinical record.

(See "[Discharge Criteria and Process](#)" Policy No. 4-045.)

(See "[Patient Education Related to Discharge Planning](#)" Policy No. 4-061.)

CONTINUITY OF CARE
Policy No. 4-010.1**PURPOSE**

To ensure continuity of care for patient while receiving home care services.

POLICY

Consideration will be made at all times to fostering continuity of care by assigning consistent personnel to the patient, whenever possible. This includes limiting the number of identified organization personnel, whenever possible, that are caring for the patient. The exception is for on-call visits and weekend and holiday coverage.

Continuity of care will be ensured by:

1. Periodic communication between team members concerning the patient's progress and special needs as evidenced in case conference reports and clinical notes. This communication will support the objectives outlined in the plan of care.
2. Participation of team members in a case conference that is held at least every 60 days. (See "[Case Conference/Progress Summary](#)" Policy No. 4-011.)

PROCEDURE

1. The clinician will be responsible for:
 - A. Communicating with all personnel caring for the patient including the physician (or allowed practitioner)
 - B. Updating physicians' (or allowed practitioner's) orders and obtaining lab test results as needed
 - C. Updating the plan of care
 - D. Updating his/her own discipline's plan of care
 - E. Communicating changes in orders and findings to the Clinical Supervisor or designee, or other team members as necessary
 - F. Communicating between multiple disciplines to optimize visit schedules for the benefit of the patient and the care to be provided
 - G. Discharge planning
 - H. Scheduling nursing and home health aide visits

- I. Scheduling and conducting home health aide supervisory visits
 - J. Updating the HHA plan of care
2. If the patient is not receiving skilled nursing care and is receiving rehabilitation services, the rehab professional in cooperation with the Clinical Supervisor will be responsible for section 1A – 1J of this policy.
 3. The Clinical Supervisor will review the patient census and staffing levels on a daily basis and will make patient assignments which consider:
 - A. Geographic area
 - B. Patient needs and skills required
 - C. The skill, education, training, and availability of personnel
 - D. Language and communication requirements
 - E. Patient acuity
 - F. The clinician's caseload
 - G. Previous organization personnel assigned to case
 - H. Patient request for personnel
 4. The Clinical Supervisor will consult the Clinical Director or Executive Director/Administrator regarding any daily staffing issues that cannot be resolved.

CASE CONFERENCE/PROGRESS SUMMARY**Policy No. 4-011.1****PURPOSE**

To define the process for case conferences and documenting patient progress in the clinical record.

POLICY

Case conferences will be held at the start of care and at least every 60 days to review and discuss all multidisciplinary cases.

Items of discussion will include, but will not be limited to, the type and frequency of service by each discipline involved, changes in the patient's overall status, problems, possible resolutions, progress towards goals, and any necessary revisions in the plan of care.

Case conferences will include utilization review; therefore, all clinicians—both direct and contract personnel—working with patients will participate in case conferences.

PROCEDURE

1. Each multidisciplinary patient will have a case conference during the month following the certification month.
2. For each patient, the Case Manager will lead the conference and discuss:
 - A. Physical status of the patient
 - B. Clinical implications of diagnoses and treatment prescribed
 - C. Patient treatment choices
 - D. Changes in condition since last conference
 - E. Interventions for all disciplines and patient response
 - F. Teaching plan and its effectiveness
 - G. Progress toward goals
 - H. Indications for recertification
 - I. Discharge plan

3. A record will be kept indicating who participated in the meeting and which patients were discussed. The record will list patients by name and clinical record number.
4. A case conference/progress summary, written by the appropriate discipline(s) seeing the patient, will be placed in the patient's clinical record after each case conference meeting. A copy will be sent to the attending physician (or allowed practitioner).

**MONITORING PATIENT'S RESPONSE/
REPORTING TO PHYSICIAN
Policy No. 4-012.1****PURPOSE**

To provide guidelines for monitoring the patient's response to care, and for reporting to the patient's physician (or allowed practitioner).

POLICY

Clinicians will monitor, document, and report the patient's response to care and treatment provided on each home visit. Progress of goals will be measured at regular intervals.

Clinicians will establish and maintain ongoing communication with the physician (or allowed practitioner) to ensure safe and appropriate care for the patient.

PROCEDURE

1. During each home visit, the clinician will monitor the patient's response to care against the established goals including, but not limited to:
 - A. Care interventions and treatments
 - B. Medications
 - C. Teaching
2. During case conferences as well as the recertification process, the care will be evaluated to determine achievement of goals.
3. The patient's physician (or allowed practitioner) will be contacted on the same day when any of the following occur:
 - A. Significant changes in the patient's condition
 - B. Significant changes in the patient's psychosocial status, family/caregiver support, home environment
 - C. Inability to achieve goals within the specified time frame
 - D. Changes in the patient's expected response to treatment or medications

- E. Changes that have occurred regarding diagnosis, prognosis, or treatment (including procedures, medications, precautions, and limitations)
 - F. When there is any problem implementing the plan of care
 - G. When results are received for relevant laboratory tests ordered
 - H. When a visit is missed for any reason
 - I. Patient is to be discharged from the organization or a specific service is to be discontinued
4. All conferences or attempts to communicate with physician will be documented in the clinical record.
- A. Documentation of the physician notification will include:
 - 1. Date and time contacted
 - 2. Patient name
 - 3. Name of physician notified or his/her representative
 - 4. Reason for notification
 - 5. Physician's response
 - 6. Action taken or orders obtained
 - 7. Professional's signature and title
 - B. Documentation of attempted physician notification will include:
 - 1. Date and time
 - 2. Patient name
 - 3. Name of physician attempting to notify
 - 4. Reason for notification
 - 5. Name of person taking message
5. When unable to contact the patient's physician (or allowed practitioner) for medical consultation warranted by change in patient's condition, the following procedures will be followed:
- A. The nurse or therapist will immediately notify the Clinical Supervisor or designee regarding the need for medical consultation and problems encountered.

- B. An attempt will be made by the Clinical Supervisor or designee to contact the patient's physician (or allowed practitioner).
 - C. If the Clinical Supervisor or designee is unable to contact patient's physician, the Clinical Supervisor or designee will notify the organization's Medical Director of the change in patient condition and the inability to contact patient's physician (or allowed practitioner) to request medical consultation.
- 6. Based on the communication with the physician (or allowed practitioner), a verbal order will be obtained for any change in the plan of care and communicated to all appropriate team members to ensure that care is provided according to the revised plan of care.
 - 7. The organization shall communicate with all physicians involved in the plan of care to avoid duplication or contradictory physician (or allowed practitioner) orders and to assure that all patient needs are being met. All physician orders shall be integrated into the plan of care. The plan of care is reviewed and signed by the physician responsible for the plan of care
 - 8. The plan of care will be updated according to the organization's policies and procedures. (See "[Care Planning Process](#)" Policy No. 4-001.)

60-DAY SUMMARY REPORT**Policy No. 4-013.1****PURPOSE**

To define the process for completion of a 60-day summary of patient care.

POLICY

A 60-day summary is no longer required by the Conditions of Participation but may be completed for a patient at the end of the episode when the patient will be recertified. The summary will be forwarded to all physicians involved in the patient's care.

PROCEDURE

1. Upon determination that a patient requires ongoing care and will be recertified for an additional episode of care, the patient's Case Manager may complete a 60-day summary.
2. Information in the 60-day summary report to the physician may include:
 - A. Reason for admission or last recertification, as applicable
 - B. A summary of care and treatments/procedures provided
 - C. A summary of the patient's response to treatment
 - D. A summary of changes to the plan of care
 - E. Why recertification for ongoing care is necessary
3. A copy of the 60-day summary report will be retained in the clinical record.

PATIENT NOTIFICATION OF CHANGES IN CARE
Policy No. 4-014.1**PURPOSE**

To define the organization requirements for patient notification of changes in care.

POLICY

The patient will be notified within 24 hours of any significant changes in the agreed-upon schedule or plan of care.

PROCEDURE*Visit Schedule*

1. Clinicians will contact the patient the night prior to a visit to verify the approximate time of the visit (within a two-hour time span).
2. Any significant changes will be called into the office (e.g., moving a visit from morning to afternoon).
3. When a significant variation of tentative time for visit (i.e., greater than one (1) hour) is anticipated, the clinical personnel will notify the patient of the change and verify acceptance.
4. When a visit cannot be made because of unforeseen problems, personnel will immediately notify the office.

Plan of Care Changes

1. Whenever the plan of care is changed, including services, frequencies, treatments, etc., the patient will be notified at the time of the visit.
2. Documentation of the notification will include:
 - A. Date and time of notification
 - B. Specific changes in the plan of care
 - C. Patient response or acceptance

Changes in Liability for Payment

1. The patient will be advised verbally and in writing of any changes in the initial information regarding his/her liability for payment within 30 days from the date the organization becomes aware of the changes.
2. Documentation of the notification will be made in the clinical and billing record.

(See "[Financial Responsibility](#)" Policy No. 2-007 and "[Financial Responsibility and Medicare Written Notices](#)" Policy No. 1-028.)

ON-CALL/WEEKEND STAFFING
Policy No. 4-015.1**PURPOSE**

To define the on-call system for addressing 24-hour coverage of services.

POLICY

Patient care needs are the highest priority; therefore, weekend and evening staffing will be scheduled accordingly. Clinical Staff are expected to perform visits on an as-needed basis, including weekends.

There will be on-call staff available after office hours Monday through Friday, and 24 hours a day on weekends. Staff on-call will be:

1. Administrative call by a senior management staff member
2. Clinical call by a registered nurse (all registered nurses will participate in an on-call rotation)

PROCEDURE

1. On admission, each patient will be made aware of the organization's 24-hour availability.
2. The on-call schedule will be developed on a monthly basis by the Clinical Director or designee. The schedule will be forwarded to the answering service and on-call staff.
3. The on-call staff can be reached by calling the home health number. After hours, this number will be forwarded to the answering service.
4. The on-call nurse will be issued a pager and/or a cellular phone to allow for mobility.
5. Reports will be given to the on-call nurse daily Monday through Friday. Supplies and records will be available to the on-call staff through direct access to the office.
6. Clinical staff must respond to a page within 15 minutes and be able to reach a patient within one (1) hour. (There may be rare exceptions, depending on how far away the patient lives and if the staff member is with another patient at the time of the page.)
7. In the event that the patient has an emergency need, the on-call staff member will call 911 on behalf of the patient and report to the patient's home immediately as appropriate.
8. The on-call nurse will report his/her evening and/or weekend patient care activities to the Clinical Director.

9. All on-call activities concerning a patient will be documented on the on-call communication form. If an on-call visit is necessary, the activities of the visit will be documented in the clinical note.

**COORDINATION OF SERVICES
WITH OTHER PROVIDERS
Policy No. 4-016.1****PURPOSE**

To ensure the coordination of services provided by the organization and by other service providers.

POLICY

A Case Manager will be assigned to be responsible for coordinating services provided to the patient by the organization, including services provided directly and through contract. The Case Manager will act as liaison with other organizations or individuals also providing care to the patient to assure effective coordination of related services.

PROCEDURE

1. All referrals to other service providers will be documented in the clinical record.
2. The other service provider will be responsible to contact the physician (or allowed practitioner) for orders for evaluation and treatment.
3. The Case Manager will be responsible for the coordination between service providers, which will include, but not be limited to:
 - A. Organization personnel's understanding of each organization's/individual's responsibility in providing care.
 - B. Initiation of communication with other organizations/individuals when there are significant changes in the patient's overall care.
 - C. Monitoring duplication or conflict of services provided by various organizations or individuals. If anyone is aware of duplication or conflict, attempts to correct the situation are necessary.
4. Issues arising from the above will be communicated to the Clinical Supervisor or designee.
5. Ongoing communication regarding specific patients will be the responsibility of the Case Manager.
6. Ongoing communication regarding issues and concerns with the organizations or individuals providing care will be the responsibility of Organization's Name management team.

INTERNAL REFERRAL PROCESS

Policy No. 4-017.1

PURPOSE

To outline the process to make a referral for additional services.

POLICY

Referrals to other disciplines will be processed as any other referral.

PROCEDURE

1. All internal referrals to other disciplines will be documented within the clinical record.
2. When a clinician identifies that an additional service is needed, he/she will contact the physician to obtain an order and complete an internal referral form.
3. The complete internal referral form will be given to the Clinical Supervisor or scheduler, who will assign the appropriate clinician.
4. The clinician assigned to provide additional services will contact the physician (or allowed practitioner) for orders for evaluation and treatment.

INITIAL AND COMPREHENSIVE ASSESSMENT**Policy No. 4-018.1****PURPOSE**

To provide guidelines for the initial assessment of patients admitted to service and for completing the plan of care.

POLICY

An initial patient assessment will be performed and documented in the patient's clinical record by a registered nurse, physical therapist, occupational therapist or speech therapist. The initial and comprehensive assessment shall be completed within the appropriate timeframes.

Prior to completion of a comprehensive assessment by a therapist, a registered nurse or qualified clinician must conduct a Drug Regimen Review.

The assessment will be patient-specific and comprehensive to include the patient's need for home care, rehabilitative care, social, and discharge planning needs. The assessment will also include the exact use of the current versions of the Outcomes and Assessment Information Set (OASIS). This assessment will measure patient outcomes from data collected at the start of care and at the following defined intervals thereafter:

1. The last five (5) days of every sixty (60) day episode beginning with the start of care date (recertification), unless there is:
 - A. An elected transfer;
 - B. A significant change in condition
 - C. A discharge and return to the same organization during the sixty (60) day episode.
2. Within forty-eight (48) hours of the patient's return home or of the organization's knowledge of the patient's return from a twenty-four (24) hour hospital admission for other than diagnostic testing, or on the physician (or allowed practitioner) ordered resumption date.
3. At discharge

PROCEDURE

1. The registered nurse shall conduct the initial and comprehensive assessment visit to determine the immediate care and support needs of the patient and for Medicare patients, to determine eligibility for the Medicare home health benefit, including homebound status.
 - A. The initial assessment visit shall be conducted either within forty-eight (48) hours of referral or within forty-eight (48) hours of the patients return home, or on the physician's (or allowed practitioner's) ordered start of care date.

- B. The comprehensive assessment shall be completed in a timely manner, consistent with the patient's immediate needs, but no later than five (5) calendar days after the start of care.
- 2. The initial and comprehensive assessment visit may be made by an appropriate rehabilitation skilled professional if rehabilitation therapy services (i.e., PT, OT, or SLP) is the only service ordered by the physician (or allowed practitioner) responsible for the home health plan of care and for Medicare patients, determine eligibility for the Medicare home health benefit, including homebound status.
 - A. An Occupational Therapist may complete the initial and/or comprehensive assessment for Medicare patients when occupational therapy is ordered with another qualifying rehabilitation therapy service that establishes program eligibility.
- 3. During the comprehensive assessment, the clinician will assign the patient a risk categorization for the emergency preparedness plan.
- 4. During the initial and comprehensive patient assessment, all baseline data to be used in measuring the patient's progress towards goals and other relevant information will be documented in the patient's clinical record, including at least the following information, if applicable:
 - A. Outcomes and Assessment Information Set (OASIS) data must be collected on all patients receiving skilled services except antepartum and postpartum patients, patients under the age of eighteen (18), and patients whose care is reimbursed by a private insurance company.
 - 1. OASIS data collection is not required for patients who are receiving only personal care or support services (patients receiving only homemaker services).
 - 2. The OASIS data will be collected during the comprehensive assessment.
 - 3. The assessment tool must also include the exact use of the current versions of the OASIS data set.
 - B. A physical assessment, including any pertinent diagnosis, vital signs, skin, pain status, allergies, nutritional status, and other relevant data related to pertinent physical findings of common symptoms such as constipation, dyspnea, skin breakdown, sleep disorders, nausea and vomiting, anxiety dehydration, etc. (Head to toe assessment.) (See ["Nutritional Assessment"](#) Policy No. 4-022.)
 - C. Patient's functional status, including but not limited to, the degree of self-care, and the amount and level of assistance needed in the following areas: (See ["Rehabilitation Care Planning"](#) Policy No. 4-004 and ["Functional Assessment"](#) Policy No. 4-021.)
 - D. Patient's medical, alcohol, drug and psychosocial history, past medical history or surgical history (including onset/exacerbation), use of emergent care and any data items collected at an inpatient admission or discharge will also be part of the assessment.

- E. The patient's psychosocial/behavioral/neurological status, including emotional barriers to treatment, cognitive limitations, memory, role changes, family dynamics, coping mechanisms, reasoning/judgement and orientation (i.e. depression, suicide risk, response to previous loss.)
- F. The patient's and family/caregiver's educational needs, abilities, motivation, and readiness to learn.
- G. Name and address of the patient's physician (or allowed practitioner).
- H. Name of the hospital, other agencies, and persons involved in the past and present care of the patient.
- I. An evaluation of the home environment for safety and health hazards, adequate living arrangements and assessment of emergency preparedness of the patient.
- J. Presence of any Advance Directives for care or discussions with patient and, as appropriate, family/caregiver, regarding the withholding of resuscitative services or the withdrawal of life-sustaining treatment.
- K. Wishes regarding care, treatment and end-of-life decisions and concerns, legal and others, related to life closure, as appropriate.
- L. Equipment presently in the home and potentially needed by patient.
- M. Any identified symptoms of pain. (See "[Pain Assessment](#)" Policy No. 4-023.)
- N. A review of all medications the patient is currently using in order to identify any potential adverse effects and drug reactions, including ineffective drug therapy, significant side effects, significant drug interactions, duplicate drug therapy and noncompliance with drug therapy. (See "[Medication Profile](#)" Policy No. 4-024.)
- O. Patient and family/caregiver and other available support systems and the type of care the family/caregiver is available, capable, and willing to provide. The patient's involvement with social and community resources will be assessed.
- P. Assess availability and schedules, and the patient's representative, if any.
- Q. Cultural and religious practices.
- R. Laboratory results, health screenings (including TB if appropriate), and immunizations.
- S. Specific, individualized patient needs and problems pertinent to the care being provided.
- T. Identification of responsible party and emergency contact(s).
- U. Communication strengths and barriers, literacy and language.

- V. Financial (money management), economic and community resources available.
 - W. Anticipated discharge needs.
5. The comprehensive assessment shall reflect and determine:
- A. Patient problems/needs/strengths
 - B. The patient's continuing need for home care
 - C. That the patient meets payment eligibility requirements (e.g., homebound status)
 - D. Patient prognosis
 - E. Patient care preferences
 - F. Baseline information to be used to measure the patient's progress toward achievement of desired outcomes
 - G. Plan of care, including type of services, frequency, and duration
 - H. The ability of the organization to adequately meet the patient's medical, nursing, rehabilitation, social and discharge planning needs
 - I. Any information that may be used to demonstrate the patient's progress toward achievement of the goals identified by the patient and the measurable outcomes identified by the organization.
6. A plan of care will be developed from the information gathered during the initial and comprehensive assessment. The patient's physician (or allowed practitioner) is consulted for approval if additions or modifications to the plan of care are required after the assessment is completed.
7. The Clinical Supervisor or designee will be responsible for the review of the plan of care. (See "[Care Planning Process](#)" Policy No. 4-001.)

ONGOING ASSESSMENTS**Policy No. 4-019.1****PURPOSE**

To provide guidelines for assessments of patient during ongoing care.

POLICY

The scope and intensity of ongoing assessments will be determined by the patient's diagnoses, condition, desire for care, response to previous care, and the care setting.

PROCEDURE

1. During each home visit the appropriate clinician will re-evaluate the patient according to the problems identified during the initial visit and thereafter.
2. Using the standards of care identified by the organization, the clinician will reassess the patient for:
 - A. Blood pressure, pulse, respirations, temperature
 - B. Weight (once each week, if indicated by disease process)
 - C. Pain status when applicable
 - D. Breath sounds
 - E. Skin integrity
 - F. Bowel sounds; elimination (urinary and bowel)
 - G. Appetite/diet, nutritional status
 - H. Mental status
 - I. Functional status
 - J. Safety/home environment
 - K. Patient and family/caregiver support
 - L. Progress toward goals and patient needs and problems
 - M. Compliance to treatment and/or medications

3. Re-assessments should focus on:
 - A. Patient's response to care
 - B. Changes in patient condition
 - C. Changes in patient diagnoses
 - D. Changes in patient's care environment or support systems
4. When a patient is receiving only personal care and support services, the reassessment will occur at least every six (6) months or more often, as necessary.
5. Based on each reassessment, the plan of care, including problems, needs, goals, and outcomes will be reviewed and revised accordingly by the clinician.
6. Based on the findings of the reassessment, change/verbal orders will be generated and forwarded to the physician (or allowed practitioner) as needed.
7. The physician will be notified to verify any changes in medications, including over-the-counter medications (which might interact or be duplicative with other patient medications), and treatment/interventions that require physician approval.

(See "[Verification of Physician Orders](#)" Policy No. 4-003.)

REASSESSMENTS/RECERTIFICATION**Policy No. 4-020.1****PURPOSE**

To outline the requirements for ongoing updates to the comprehensive assessment (reassessments) and recertification.

POLICY

The comprehensive assessment must be updated and revised every 60 days beginning with the start of care. A comprehensive assessment, including all required OASIS data elements, must be performed to measure patient progress towards outcomes.

The total plan of care must be renewed at least every sixty (60) days, or more often as warranted by the condition of the patient. Any assessment findings that suggest a need to alter the plan of care will be reported to the patient's physician.

OASIS assessments for recertification, significant change in condition, transfer, resumption of care and discharge will be completed within the mandated time frames.

OASIS data will be encoded within seven (7) days of completion of the OASIS data set and transmitted to the state agency or CMS OASIS contractor at least monthly.

A tracking mechanism will be utilized by the organization to ensure accuracy and timeliness of OASIS data and its transmission.

The recertification statement must indicate the patient's continuing need for home health services.

PROCEDURE

1. Each clinician will follow the policies and procedures for ongoing assessments for each home visit. (See "[Ongoing Assessments](#)" Policy No. 4-019.)
2. For each new episode of care, a comprehensive assessment will be completed no earlier than five (5) days before and no later than one (1) day before the calendar day on which the new episode of care will begin.
3. The update of the comprehensive assessment must, at a minimum, include:
 - A. Completion of all follow-up date items of OASIS and any changes in patient status
 - B. Drug regimen review of all medications for drug interactions, potential adverse effects and drug reactions, duplicative drug therapy, ineffective drug therapy, significant side effects, significant drug interactions

- C. Documentation of changes in the patient's assessment finding
- 4. Each patient will be reassessed using a comprehensive OASIS assessment tool for the review and revision of the plan of care when:
 - A. There is a significant change in the patient's condition, care environment and/or support system
 - B. Transfer of a patient to an inpatient facility
 - C. The patient returns home after an inpatient admission lasting 24 hours or longer for any purpose other than diagnostic testing
 - D. The patient is discharged
- 5. The physician (or allowed practitioner) will be notified of any of the above situations.
- 6. Revisions to the plan of care shall be communicated as follows:
 - A. Any revision to the plan of care due to a change in patient health status shall be communicated to the patient, representative (if any), caregiver, and the physician who is responsible for the plan of care.
 - B. Any revisions related to plans for the patient's discharge shall be communicated to the patient, representative, caregiver, all physicians issuing orders for the plan of care, and the patient's primary care practitioner or other health care professional who will be responsible for providing care and services to the patient after discharge from the organization.
 - C. All communication shall be documented in the clinical record and the plan of care shall be updated and a verbal order sent to the physician.
- 7. Documentation in the clinical record should support the assessment as well as the actions taken in response.
- 8. Three (3) weeks prior to the recertification date, the Clinical Supervisor or designee will alert the clinical personnel regarding the need for a review and update of the plan of care and possible recertification.
- 9. After the reassessment is conducted (no sooner than five (5) days prior to the recertification date for patients receiving OASIS reassessments), a case conference will be held and documented in the clinical record.
- 10. The physician (or allowed practitioner) will be contacted to verify the continued need for care and his/her agreement with the continued plan of care.
- 11. An interim order to continue home care services will be prepared and sent to the physician (or allowed practitioner).

12. The updated plan of care will be prepared and sent to the physician for review and signature in a timely manner.

FUNCTIONAL ASSESSMENT**Policy No. 4-021.1****PURPOSE**

To provide guidelines for the appropriate assessment of patient who may have functional limitations requiring assistance and services from the organization.

POLICY

During the initial assessment, the patient will have his/her functional status assessed for provision of appropriate services. When rehabilitation needs are identified, a qualified rehabilitation professional will perform an in-depth functional assessment prior to the initiation of treatment.

PROCEDURE

1. During the initial assessment, the clinician performing the admission visit will assess the patient's functional status, including but not limited to the following:
 - A. Level of independence in the environment of care (home, school, work) for the following:
 1. Eating
 2. Toileting
 3. Transfers
 4. Walking
 5. Shopping
 6. Cleaning
 7. Laundry
 8. Bathing
 9. Dressing
 - B. Mobility status
 - C. Pain status

- D. Problems with continence
 - E. Ability to operate and maintain equipment
 - F. Communication level and skills
 - G. Memory
 - H. Cognitive level
 - I. Orientation
 - J. Emotional response to current health status
 - K. Dental and oral hygiene
2. Based on the above assessment criteria, the functional level classification should be made:
 - 0 – completely independent
 - 1 – requires use of equipment or device
 - 2 – requires help from another person for assistance, supervision or teaching
 - 3 – requires help from another person in activity
 - 4 – dependent, does not participate in activity
 3. The functional assessment should be conducted through observation of the patient's ability to perform tasks.
 4. Once the level of functioning is determined, the clinician will determine which additional services or further functional assessments the patient may benefit from, including home health aide, therapies (physical, occupational or speech), and medical social worker services.
 5. If another service is needed, the physician (or allowed practitioner) will be contacted to obtain an order. Once approved, a referral will be made to that discipline for the initiation of services.
 6. When rehabilitation or social needs are identified and an order has been received from a physician (or allowed practitioner), the patient's physical status and functional abilities will be evaluated by a qualified rehabilitation/social service professional prior to the initiation of instruction or treatment.
 7. The patient's discipline-specific functional status will be further assessed, including but not limited to, the following:
 - A. Current level of functioning
 - 1. Mobility

2. Restrictions
3. Assistive devices
4. Resources needed to manage function limitations
- B. Self care responsibilities
 1. Environmental
 2. Safety or health hazards
 3. Adequate living arrangements
 4. Available caregivers and other community resources
- C. Independence
 1. Home environment such as bathroom safety, accessibility and safety with mobility (e.g. throw rugs, blocked exits, etc.)
 2. Financial resources available to maintain independence
- D. Quality of life
 1. Fire safety
 2. Security hazards
 3. Medical equipment
 4. Other basic necessities
8. Based on this functional assessment, the rehabilitation professional will develop and implement a rehabilitation plan with the patient and family/caregiver.

NUTRITIONAL ASSESSMENT**Policy No. 4-022.1****PURPOSE**

To provide guidelines for the appropriate assessment of patients who may require a nutritional assessment by a qualified clinician.

POLICY

When the initial and comprehensive assessment indicates an alteration in nutritional status, the clinician will make a referral to a qualified health care professional for further nutritional assessment.

PROCEDURE

1. During the initial assessment, the following information may be obtained, through interview and observation, as part of the baseline data (including by not limited to):
 - A. Diet/appetite (including anorexia)
 - B. Height/weight
 - C. Body mass index
 - D. Digestive disorders
 - E. Dysphasia/swallowing difficulties
 - F. Factors affecting nutritional status, including but not limited to:
 1. Nausea/vomiting
 2. Diarrhea/constipation
 3. Emotional issues
 4. Oral hygiene/dental care
 5. Environment for meals
 - G. Hydration/fluid intake

2. Based on the above, if the patient is determined to have any of the following, the clinician will contact the patient's physician (or allowed practitioner) and obtain follow-up orders or an order for a nutritional consult:
 - A. Nutrition, altered, less than body requirements
 - B. Nutrition, more than body requirements
3. Documentation in the clinical record should reflect the physician (or allowed practitioner) contact and the order for a nutritional consult.
4. The registered dietitian will make entries into the clinical record regarding plans, discussions, and interventions related to the nutritional consult.

PAIN ASSESSMENT**Policy No. 4-023.1****PURPOSE**

To provide guidelines for the appropriate identification and assessment of patient who may experience pain.

POLICY

Each patient will receive at least an initial pain assessment. When pain is identified, a more comprehensive pain assessment will be completed when warranted by the patient's condition. Pain assessments will be age-appropriate and will be documented to facilitate regular reassessment and follow up by clinicians.

PROCEDURE

1. During the initial assessment, the patient will be asked a general screening question regarding current or recent pain as part of the baseline data. Clinicians will consider the patient's personal, cultural, spiritual, and ethnic beliefs when assessing pain or discomfort.
2. When the patient or the clinician identifies pain, the following in-depth pain assessment information should be obtained whenever possible:
 - A. Pain intensity using a rating scale (on a scale of 0 – 10: 0 = no pain; 10 = unbearable pain). Pain intensity should include current pain, worst pain, and least pain using the scale. A separate, age appropriate, pain scale may be used for children.
 - B. Pain location.
 - C. Pain quality, patterns of radiation, and character. Use the patient's own words whenever possible.
 - D. Pain onset, duration, variations, and patterns.
 - E. Alleviating and aggravating factors.
 - F. Present pain management regimen and effectiveness.
 - G. Pain management history to include a medication history, presence of common barriers to reporting pain and using analgesics, past interventions and response, and manner of expressing pain.
 - H. Effects of pain—these include impact on daily life, function, sleep, appetite, relationships with others, emotions, concentration, etc.

- I. The patient's pain goal, including pain intensity and goals related to function, activities, and quality of life.
 - J. Physical exam or observation of the site of pain.
 - K. Secondary symptoms related to pain such as nausea, vomiting, respiratory distress or nutritional compromise.
- 3. If patient is unable to communicate pain using the rating scale, is cognitively impaired, or is a child, the clinician will assess behavioral factors that signal pain or discomfort and include this information in the assessment.
 - 4. For parents or caregivers of a child who exhibits or reports pain, information will be provided regarding the parent's role in interpreting their child's behavioral changes that may indicate pain or discomfort.
 - 5. Pain assessments will be documented in the clinical record, and information will be communicated to other caregivers.

MEDICATION PROFILE**Policy No. 4-024.1****PURPOSE**

To define the use of the medication profile in evaluating a patient's medication regimen, including over-the-counter (OTC) medications, nutrition supplements, herbal remedies, vitamins and minerals.

POLICY

Patient receiving medications administered by the organization will have a current, accurate medication profile in the clinical record.

Medication profiles will be updated for each change to reflect current medications, new, and/or discontinued medications.

PROCEDURE

1. Upon admission to the organization, the admitting clinician will initiate a medication profile to document the current medication regimen, including name, drug classification, date prescribed/discontinued, dose, strength, route, frequency, date and time of administration and any drug or food allergies.
2. A drug regimen review will be performed at the time of admission, when updates to the comprehensive assessments are performed, when care is resumed after a patient has been placed on hold, and with the addition of a new medication.
 - A. The review will be performed by qualified personnel and will identify drug/food interactions, potential adverse effects and drug reactions, ineffective drug therapy, significant side effects, significant drug interactions, duplicative drug therapy, and noncompliance with drug therapy.
3. During subsequent home visits, the medication profile will be used as a care planning and teaching guide to ensure that the patient and family/caregiver as well as other clinicians understand the medication regimen. This includes, but will not be limited to:
 - A. Using the medication profile to evaluate the use of medications in the home setting. The medication profile includes, but is not limited to: all current medications, date prescribed, name of medication, dose, frequency, drug classification, date discontinued, and drug and/or food allergies
 - B. Using the medication profile to teach purpose of medication, dosages, routes, administration times, side effects, and contraindications
 - C. Using the medication profile as a communication tool for other clinicians involved in the care

4. Each patient will receive appropriate written material for specific medications he/she is receiving. The material will contain information on actions of the medication, potential side effects, contraindications the patient should be aware of, and any special instructions when taking the specific medication.
5. Based on review of the medication profile as well as the written material, changes in the plan of care may be required.
6. Any conclusions and findings of patient medication use or monitoring should be communicated to the pharmacist, when appropriate, and other clinicians.
7. Deviations from taking medications as ordered will be documented in clinical notes, and the physician (or allowed practitioner) will be notified.

**IDENTIFICATION OF MEDICATION
FOR ADMINISTRATION
Policy No. 4-025.1****PURPOSE**

To provide general guidelines for the safe administration of medications.

POLICY

Orders for the administration of medications must be given by a physician (or allowed practitioner) and include patient name, patient identifier, the name of the medication, dosage, dilution, route, frequency of administration, and rate of infusion, if applicable, as well as orders for anaphylaxis and laboratory work, when appropriate.

PROCEDURE

1. Prior to medication administration, the nurse should be familiar with the patient's medical history and review present medication regimen, including allergies to foods and drugs.
2. The nurse will review the written physician (or allowed practitioner) orders prior to medication administration.
3. It is the nurse's responsibility to be knowledgeable of the medication to be administered, including indications, normal dosage range, dilution, route of delivery, rate of delivery, precautions, side effects, expected therapeutic effect, proper antidote, and incompatibilities, as applicable.
4. Medications should be properly labeled with the patient's name, an additional patient identifier, name of drug, dosage, diluent, date of preparation, expiration date, initials of preparer, and any special instructions, as applicable.
5. The nurse will review the medication label for name, additional patient identifier, drug, dosage, and prescription.
6. The nurse will validate patient name and listed patient identifier with the patient or family/caregiver, as appropriate.
7. All medications will be checked for stability by visualizing the medication and observing for the following, but not limited to:
 - A. Deterioration, as evidenced by particulate matter
 - B. Discoloration, cloudiness

- C. Dampness
 - D. Intactness, including seals
 - E. Expiration date
 - F. Storage facilities/containers
8. If the medication is not stable for administration, the nurse will hold the medication and contact the appropriate pharmacy for replacements.
 9. Prior to administration, the nurse will verify and/or review information to determine that the medication is not contraindicated for the patient based on the following:
 - A. Known medication allergies
 - B. Known food allergies
 - C. Medication incompatibility for potential interaction
 - D. Patient's physical or mental condition
 - E. Relevant laboratory results
 - F. Previous reactions to medications
 10. If potential contraindications are identified, the nurse will contact the pharmacist and physician involved in the care of the patient for further instructions.
 11. The nurse will document medication teaching, side effects, administration, and other related information in the clinical record.

Note: Except for very limited use related to research studies, the federal government prohibits the use or possession of marijuana for any purpose. Several states, however, have approved the use of marijuana for medical and, in some states, recreational use. Even in those states that permit the possession and use of marijuana, the federal government still prohibits it. Because of this tension between state and federal law, home health and hospice agencies should consult reputable legal counsel with familiarity of state and federal law to determine whether their employees may play a role in the self-administration of marijuana by the patient in the patient's home.

ADMINISTRATION AND DOCUMENTATION OF MEDICATIONS**Policy No. 4-026.1****PURPOSE**

To provide guidelines for the safe administration of medications by licensed personnel.

POLICY

Licensed nursing personnel will administer and document only those medications which have been ordered as a part of the plan of care and have been approved for safe administration and monitoring during a home visit.

Registered nurses may administer oral, rectal, subcutaneous, intramuscular, intravenous, epidural and intrathecal medication. (See "[Intravenous Administration of Medications/Solutions](#)" Policy No. 4-029, and "[Intravenous Administration of Chemotherapy](#)" Policy No. 4-030.)

Licensed practical/vocational nurses may administer oral, subcutaneous, and intramuscular injections. Intravenous medication administration will be per state Nurse Practice Act.

Per practice act guidelines, certification and competency demonstrated for IV administration of identified drugs will be documented in the clinician's personnel file.

Non-licensed personnel may, with instruction, supervise and assist the patient's self-administration of medication. Non-licensed personnel may document assisting the patient in his/her self-administration of medication where not prohibited by state law or regulation.

Organization's Name will make available to organization personnel the current editions of drug reference materials or software such as but not limited to: USP, PDR, Nursing Drug Handbook, or Nursing Drug Guide to be used as drug information and patient education sources for consistency of referenced professional information. These reference materials will provide information concerning drug indications, drug interactions, pharmacology/pharmacokinetics, side/adverse effects, and patient consultation guidance, dosing information, and dosing forms. (See "[Drug Information For The Nurse](#)" Addendum 4-026.C and "[Advice For The Patient—Drug Information in Lay Language](#)" Addendum 4-026.D.)

PROCEDURE

1. As part of the assessment process, a drug history will be taken, including prescribed and over-the-counter medications, and herbal and nutritional supplements. A comparison will be made between the physician's (or allowed practitioner's) orders and the current medications the patient is taking. Any discrepancies or contradictions should be reported to the physician for resolution.

2. The nurse will provide instruction to the patient on a continuing basis. This instruction will include medication administration, route, how medication relates to disease process, contraindications, side effects, and adverse reactions.
3. Licensed nurses will administer medications ordered by the physician (or allowed practitioner) that can be safely administered and monitored during a home visit.
 - A. For those ordered medications that cannot be safely administered and monitored, the physician will be contacted for discussion. (See "[Medications Not Approved for Safe Home Administration](#)" Addendum 4-026.B.)
 - B. Medication administration will occur upon successful completion of the competency skills checklist. This will include identification of precautions and requirements for treatments such as equipment required, assessments for adverse reactions, lab results to be reviewed prior to administration, guidelines for physician notification, and infection control practices.
4. Non-licensed personnel will not administer medications. Where not prohibited by law, non-licensed personnel may assist the patient in the self-administration of medication and complete the corresponding documentation.
5. Nurses who provide intermittent home care visits will document only those medications that they administer, not medications the patient self-administers during the absence of nursing personnel.
6. Medications will be administered within 30 minutes before or after the prescribed times. Deviance from this time frame will be noted on the medication administration record with an explanation made in the clinical notes.
7. Medications refused, held, and/or omitted will be indicated on the medication administration record with an explanation made in the clinical notes. The Clinical Supervisor and attending physician (or allowed practitioner) will be notified. An incident report will be filed as necessary.
8. If, in the judgment of the nurse, it would be beneficial to the patient to document his/her self-administered medications, a patient self-medication checklist may be prepared and left in the home for the patient's use. This document will be used as a service only and will not be incorporated into the clinical record.
9. Influenza and Pneumococcal vaccines may be administered in the home by physician (or allowed practitioner) order or as permitted by law or regulation according to physician-approved organizational protocol(s). The vaccinations may be administered after an assessment of the patient to determine screening contraindications.
10. Experimental drugs will be administered by Organization's Name in accordance with "[Investigational Medications](#)" (see Policy No. 4-040)

11. Organization's Name will not administer blood and blood products in the home.

California Immunization Data Requirements

1. The organization shall disclose to report vaccination information to either the local health departments or the regional immunization information and reminder systems and the State Department of Public Health.
 - A. The name of the patient or client and names of the parents or guardians of the patient or client.
 - B. Date of birth of the patient or client
 - C. Types and dates of immunizations received by the patient or client.
 - D. Manufacturer and lot number of each immunization received.
 - E. Adverse reaction to immunizations received.
 - F. Other nonmedical information necessary to establish the patient's or client's unique identify and record.
 - G. Results of tuberculosis screening.
 - H. Current address and telephone number of the patient or client and the parents or guardians of the patient or client.
 - I. Patient's or client's gender.
 - J. Patient's or client's place of birth.
 - K. Patient's or client's race and ethnicity.
2. Organizations located in Alpine, Amador, Calaveras, Mariposa, Merced, San Joaquin, Stanislaus or Tuolumne counties shall report all vaccine administration information into Healthy Futures/RIDE. All other organizations shall report every vaccine administration into the immunization registry CAIR.
3. If the organization administers vaccines but is not entering records into a California immunization registry, follow the steps below:
 - A. Facilities in Alpine, Amador, Calaveras, Mariposa, Merced, San Joaquin, Stanislaus, or Tuolumne counties should contact the Health Futures/RIDE Help Desk at (209) 468-2292 or support@myhealthyfutures.org.

- B.** Facilities in all other counties should visit the CAIR website or contact the CAIR Help Desk at CAIRHelpdesk@cdph.ca.gov or (800) 578-7889.

Note: If an outside organization administers vaccines at a facility, the organization is also required to report all doses into a California immunization registry.

ADDENDUM 4-026.A

DRUG/CLASSIFICATIONS AND THEIR ROUTES

DRUG CLASSIFICATION AND THEIR ROUTES

Classification	Route	By Whom
Antibiotics, Antimicrobials	PO, IM, Inhalation	RN, LPN
	IV	RN
Cardiovascular Drugs		
• Inotropics & Antiarrhythmics	PO	RN, LPN
• Beta-Adrenergic Blockers	PO	RN, LPN
• Antianginal	PO, SL, Inhalation, Topical	RN, LPN
• Alpha-Adrenergic Blockers	PO	RN/LPN
• Antihypertensives	PO	RN, LPN
Central Nervous System Drugs		
• Analgesics	PO, IM	RN, LPN
	IV	RN
• Anticonvulsants	PO	RN, LPN
• Antidepressants	PO	RN, LPN
• Anti-Parkinson's	PO	RN, LPN
• Antipyretics	PO, Suppositories	RN, LPN
• Antianxiety Agents	PO, IM	RN, LPN
• Antipsychotics	PO, IM	RN, LPN
• Sedatives/Hypnotics	PO, IM	RN, LPN
Autonomic Nervous System Drugs		
• Adrenergics	PO, IM	RN, LPN
	IV	RN
• Cholinergics	PO	RN, LPN
• Anticholinergics	PO	RN, LPN
• Skeletal Muscle Relaxants	PO, IM	RN, LPN
	IV	RN
Respiratory Tract Drugs		
• Antihistamines	PO, IM, SC	RN, LPN
	IV	RN
• Bronchodilators	PO, IM, SC, Inhalation	RN, LPN
• Expectorants, Antitussives	PO	RN, LPN

DRUG CLASSIFICATION AND THEIR ROUTES

Classification	Route	By Whom
Gastrointestinal Tract Drugs		
• Antacids, Antiflatulants	PO	RN, LPN
• Digestive Enzymes	PO	RN, LPN
• Antidiarrheals	PO, SC	RN, LPN
• Laxatives	PO, Suppository, Enema	RN, LPN
• Antiemetics	PO, IM	RN, LPN
	IV	RN
• Antiulcer Agents	PO	RN, LPN
Hormonal Agents	PO, IM	RN, LPN
Agents for Fluid & Electrolyte Balance	PO, IM	RN, LPN
Hematologic Agents		
• Hematinics	PO	RN, LPN
• Anticoagulants	PO, SC	RN, LPN
• Hemostatics	In wound/ulcer	RN, LPN
Antineoplastic Agents	PO	RN, LPN
	IV	RN
Immunomodulation Agents		
• Immunosuppressants	PO	RN, LPN
• Vaccines and Toxoids	PO, SC, IM, Intradermal	RN, LPN
• Immune Serums	IM	RN, LPN
	IV	Not Given
• Biological Response Modifiers	SC, IM	RN, LPN
	IV	RN
Ophthalmic, Otic, Nasal Drugs	Eye, Ear, Nose	RN, LPN
Topical Agents	Topical	RN, LPN
Nutritional Agents	PO, IM	RN, LPN

ADDENDUM 4-026.B

**MEDICATIONS NOT APPROVED FOR
SAFE HOME ADMINISTRATION**

ADDENDUM 4-026.C

DRUG INFORMATION FOR THE NURSE

(Insert Organization Drug Information or indicate where nurse may obtain information as needed)

ADDENDUM 4-026.D

**ADVICE FOR THE PATIENT—
DRUG INFORMATION IN LAY LANGUAGE**

(Insert Organization Patient Drug Information or indicate where staff may obtain information as needed)

(Example: Drug Disposal

<https://www.fda.gov/drugs/safe-disposal-medicines/disposal-unused-medicines-what-you-should-know>)

PATIENT SELF-ADMINISTRATION OF MEDICATION
Policy No. 4-027.1**PURPOSE**

To promote correct administration of medication by patients and families/caregivers.

POLICY

Organization's Name will encourage patient and family/caregiver participation in his/her own care and will provide teaching about the correct administration of medications by patient and family/caregiver as ordered by the attending physician (or allowed practitioner) or as purchased over the counter. Teaching will also include the safe storage of medications.

(See "[Safe/Effective Use of Medications](#)" Policy No. 4-050)

PROCEDURE

1. The clinician will provide an opportunity for the patient to administer his/her own medications. The medications listed at the time of admission and recertification will be considered part of the plan of care.
2. The clinician will:
 - A. Teach the patient the purpose and side effects of medications and the patient's role in identifying and preventing medication errors.
 - B. Assist the patient in setting up medications for the first time.
 - C. Assess patient's ability to self-administer medications correctly and document the patient's response and understanding to teaching.
 - D. Answer questions/concerns expressed by the patient and family/caregiver regarding the patient's self-administration of medications.
 - E. Document information given to the patient regarding the medication, date, and time medications are to be given; teaching of side effects; and any pertinent observations made during the visit, such as patient's needs, outcome, etc., in the patient's clinical record, as appropriate.
 - F. Complete the medication profile form at admission and recertification.
 - G. Assess patient's use of over the counter (OTC) and herbal medications; document with start date.

- H. Instruct patient and family/caregiver regarding safe storage of medications. Consideration should be given to the following:
1. Medications should be stored separately from other poisonous drugs and chemicals.
 2. Medications should be removed from storage during instruction and administration times.
 3. Medications should be kept out of the reach of children, pets, and confused or disoriented patients.
 4. The nurse will plan with the patient and family/caregiver for the safe therapeutic storage of drugs during the assessment process.
 5. Drugs requiring refrigeration should be stored inside the refrigerator.
 6. Urine testing and other diagnostic materials should be stored away from all medications, heat, light, and moisture.

**HOME USE AND DISPOSAL OF
CONTROLLED SUBSTANCES**
Policy No. 4-028.1**PURPOSE**

To assure proper use of, accounting for, and disposal of controlled substances, such as drugs and biologicals.

POLICY

Organization's Name voluntarily adheres to a controlled drug reporting process.

PROCEDURE

1. Controlled substances will be distributed directly to the patient or his/her representative. The dispensing pharmacist is responsible for monitoring the amount of drug issued and the length of time between renewals.
2. In cases where organization personnel are in the home 24 hours a day, a drug count will be made by the licensed personnel at the time of shift change.
3. The accounting process for controlled drugs will be recorded on the narcotic count record, which will be maintained as a part of the clinical record.
4. An informal documentation procedure for the patient and family/caregiver may be outlined for use when organization personnel are not present in the home.
5. When a home care patient no longer has a need for a controlled substance the clinician will instruct the patient and/or the family/caregiver to dispose of the drugs.
6. The clinician will document in the clinical record that the patient and family/caregiver was instructed to dispose of medications and took responsibility to do so.
7. Organization personnel will not dispose of any patient medications.
8. Organizations will monitor for suspected drug diversion, including opioids, depressants, hallucinogens, stimulants, and anabolic steroids.

INTRAVENOUS ADMINISTRATION OF MEDICATIONS/SOLUTIONS Policy No. 4-029.1

PURPOSE

To establish the conditions for administration of intravenous medications/solutions in the home.

POLICY

Registered nurses may establish peripheral intravenous (IV) lines and administer organization-approved IV medications and solutions under the orders of a physician (or allowed practitioner). (See "[Medications Approved/Not Approved for Intravenous Administration](#)" Addendum 4-029.A.)

PROCEDURE

1. A physician (or allowed practitioner) order will be obtained for approved IV medications and solutions.
2. All orders for IV medications and solutions will specify medication name and dosage, diluent type and amount, route, frequency of administration, and rate of infusion.
3. The patient receiving IV medications and solutions (see "[Medications Approved/Not Approved for Intravenous Administration](#)" Addendum 4-029.A) should have received his/her first dose of prescribed medicine in a hospital setting, in a physician's (or allowed practitioner's) office, or under the supervision of a physician or his/her representative prior to admission to Organization's Name, without evidence of allergic reaction, unless prior approval is obtained by consultation with the Clinical Supervisor and Medical Director.
4. When it has been determined that the first dosages of medication and solutions can be given in the home without prior administration, the nurse will remain in the home for at least 30 minutes after discontinuing the IV infusion.
5. IV medications and solutions will only be administered through a peripheral or central venous line.
6. Laboratory work, as indicated for each medication or solution, will be ordered by the physician (or allowed practitioner).
7. Only drugs that are approved by the Food and Drug Administration (noninvestigational) and included in "[Medications Approved/Not Approved for Intravenous Administration](#)" (see Addendum 4-029.A) will be administered unless prior approval is obtained by consultation with the Clinical Supervisor and Medical Director.

8. Drugs that are included on the "[Medications Approved/Not Approved for Intravenous Administration](#)" Addendum 4-029.A will be reviewed and updated at least every twelve (12) months or whenever a new agent or dosage form is used.
9. Only medications and solutions that are prepared by a pharmacy and are properly labeled with patient's name, name of drug, dosage, dilution, date of preparation, expiration date, initials of preparer, and any special instructions will be administered.
10. Patient-specific anaphylaxis kits will be supplied by the home infusion company in the following instances: (See "[Anaphylaxis Protocol](#)" Policy No. 4-037.)
 - A. Patient is receiving an approved first-time dose of medication in the home.
 - B. Patient has numerous medication allergies.
 - C. At physician's (or allowed practitioner's) request/order.
11. A physician (or allowed practitioner) must be notified if any of the following circumstances occur:
 - A. If clinical findings are abnormal
 - B. If laboratory findings are abnormal
 - C. If any allergic or toxic symptoms are exhibited by the patient
 - D. If drug regimen review shows an alternative pharmacotherapeutic plan that could achieve safer, more effective, and more economical patient care
 - E. If anaphylaxis occurs and/or different medication is required to treat
 - F. When proper placement of a central venous catheter is questioned
 - G. If repeated difficulty occurs in establishing a peripheral line

ADDENDUM 4-029.A

**MEDICATIONS APPROVED/NOT APPROVED FOR
INTRAVENOUS ADMINISTRATION**

MEDICATIONS APPROVED FOR INTRAVENOUS ADMINISTRATION

The following list of medications has been approved for administration in the home.

Additional medications may be added on approval.

All orders for IV medications will be reviewed for proper procedure and approved by a qualified Clinical Supervisor.

Insert Organization Approved Intravenous Medication List

**MEDICATIONS NOT APPROVED
FOR INTRAVENOUS ADMINISTRATION**

The following list of medications has not been approved for administration in the home.

Insert Organization Not-Approved Intravenous Medication List

**INTRAVENOUS ADMINISTRATION
OF CHEMOTHERAPY**
Policy No. 4-030.1**PURPOSE**

To delineate the process for the safe administration of intravenous chemotherapy in the home setting.

POLICY

Intravenous chemotherapeutic agents will be administered by a registered nurse certified in the administration of said agents.

Registered nurses who perform IV administration of medications and solutions may do so only under the following conditions:

1. Only antineoplastic agents that are included in Organization's Name IV chemotherapy formulary list will be administered by chemotherapy-certified nurses. (See "[Antineoplastic and Other Hazardous Medications Approved/Not Approved for Administration](#)" Addendum 4-030.A.)
2. Antineoplastic agents that are included on the "[Antineoplastic and Other Hazardous Medications Approved/Not Approved for Administration](#)" Addendum 4-030.A will be reviewed and updated at least every twelve (12) months or whenever a new agent or dosage form is used.
3. The first cycle of an antineoplastic drug will be given in the physician's (or allowed practitioner's) office or hospital setting without evidence of adverse reaction prior to administration in the home setting. (All Taxol is to be given as first dose in hospital.)
4. The physician (or allowed practitioner) ordering the antineoplastic drug for home administration will also order:
 - A. Dose as mg/m^2 and actual calculated dose, dilution, route of administration, absolute dosage, frequency, number of doses of the drug to be administered, length of infusion, and total lifetime dose, when appropriate
 - B. Orders for antiemetic drugs prn when appropriate
 - C. Orders for treatment of extravasation prn if the drug is a vesicant
 - D. Orders for treatment of anaphylaxis prn
 - E. Orders for heparin prn, when appropriate
 - F. Laboratory work as applicable

5. Venous access will be adequate for administration of chemotherapeutic drugs (blood return must be demonstrated prior).
6. The home environment will be compatible with safe administration of IV chemotherapeutics.
7. Organization will be given an opportunity to assess the patient or chart in the hospital, if appropriate.
8. Chemotherapeutic drugs will be prepared by a pharmacy and received in the patient's home, ready for administration. They will be properly labeled with the patient's name, name of drug, dosage, diluent, date and time of preparation, expiration date, initials of preparer, and any special instructions (e.g., refrigerate). If a drug is a vesicant, a "vesicant" label should be attached.

PROCEDURE

1. The registered nurse administering the chemotherapeutic drug will:
 - A. Review all orders for IV medications for proper procedure and protocol.
 - B. Review individual drug protocol before each administration.
 - C. Verify with the physician (or allowed practitioner) that the medication order and the prepared medication from the pharmacy are the same.
 - D. Recheck calculations of drug order and compare dosage to individual drug protocol.
 - E. Review laboratory results. (Refer to each antineoplastic drug to determine laboratory work.)
 - F. Do baseline nursing assessment of patient. (Refer to each antineoplastic drug to review relevant clinical findings.)
 - G. Explain the procedure to the patient and discuss potential side effects, including anaphylaxis.
 - H. Place the patient in a comfortable position and in a well-lighted area.
 - I. Give pretreatment medications, if ordered.
 - J. Select appropriate site for placement of peripheral IV line. When giving vesicant drugs, avoid wrist veins and the antecubital fossa.
 - K. After peripheral line placement, check patency by instilling 5 to 10 ml of sterile normal saline.
 - L. Check patency of the central venous line by aspirating 3 to 5 ml of blood.

- M. If IV site is patent, the needle should be taped so that the insertion site is easily seen.
- N. When giving more than one (1) drug in a series, always give the vesicant drug first.
- O. Observe site continuously for signs of extravasation.
- P. Use volumetric pumps when administering continuous IV infusion.
- Q. Flush catheter/cannula with 5 ml of sterile normal saline between the last administration of chemotherapeutic agents and following the last drug.
- R. When administration of IV chemotherapy is complete, remove all drug-contaminated supplies and dispose of them using hazardous waste standards.
- S. Observe the patient throughout intermittent administration of chemotherapy for symptoms of anaphylaxis or side effects. When patient is receiving continuous IV infusion, instruct patient and family/caregiver in symptoms of anaphylaxis, in side effects, and when appropriate, in symptoms of extravasation and appropriate interventions.
- T. Remain in the home for at least 30 minutes following IV push administration of drug and following the start of IV infusion of drug.
- U. Provide follow-up education:
 - 1. Give date for next chemotherapy administration.
 - 2. Explain how emergency care can be summoned.
 - 3. Arrange, if necessary, for appropriate laboratory work.
- 2. A physician (or allowed practitioner) must be consulted prior to administration of IV chemotherapy in the following circumstances:
 - A. If clinical findings are abnormal (refer to each antineoplastic drug to determine relevant clinical observations).
 - B. If laboratory findings are abnormal (refer to each antineoplastic drug to determine relevant laboratory work).
 - C. When proper placement of central venous catheter is questioned.
- 3. The nurse will follow recommended safety guidelines. Strict adherence to protocol aimed at nurse safety may greatly reduce the possibility of toxic side effects from exposure to chemotherapeutic medications. A spill kit will be available during chemotherapy administration.

- A. Hand hygiene should be performed before and after administration.
- B. Use doubled gloves when handling tubing and administering the drug.
- C. Use aseptic technique/standard precautions throughout procedure.
- D. Luer-Lok syringes and IV tubings should be used in drug administration.
- E. Take action to prevent needle punctures.
- F. Use packaged 4x4-inch gauze pads at syringe or needle tip when expelling excess air.
- G. Use leak-proof, puncture-resistant disposal containers.
- H. Extravasation kit will be available in the home.
- I. Have patience and take adequate time (NEVER RUSH) during administration of drug.
- J. Patient and family/caregiver should be instructed to take necessary precautions with contaminated linen/excreta (e.g., gloves, hand washing).

Site Infiltration/Extravasation Protocol

- A. Signs and symptoms of infiltration during IV administration of vesicant antineoplastic drugs include edema, blanching, pain, redness, stinging, and burning near injection site and proximal to the injection site upward along the vein.
- B. Infiltration/extravasation apply to the following drugs:
 - Dactinomycin
 - Daunorubicin
 - Doxorubicin hydrochloride
 - Mithramycin
 - Mitomycin C
 - Nitrogen mustard
 - Vinblastine sulfate
 - Vincristine sulfate
- C. Discontinue injection of chemotherapeutic agent IMMEDIATELY.
- D. Aspirate 3 to 5 ml of blood from IV (if possible).
- E. Remove IV needle from patient.

- F. Administer drug as ordered by attending physician (or allowed practitioner) to counteract intravasation.
- G. Immediately apply ice bag. Instruct patient to continue using ice bag as much as possible for 24 hours.
- H. Notify physician immediately of extravasation and obtain further physician (or allowed practitioner) orders for care of site.
- I. Instruct patient to call physician if there is pain, infection, or ulceration.
- J. Patient to be seen by physician within 72 hours. Instruct patient to arrange for appointment.
- K. Document in clinical notes. Complete an incident report. Notify appropriate supervisor.

ADDENDUM 4-030.A

**ANTINEOPLASTIC AND OTHER HAZARDOUS MEDICATIONS
APPROVED/NOT APPROVED FOR ADMINISTRATION**

**ANTINEOPLASTIC AND OTHER HAZARDOUS MEDICATIONS APPROVED FOR
ADMINISTRATION**

Insert Organization Approved List of Antineoplastic and Other Hazardous Medications

Additional medications will be added on approval.

**ANTINEOPLASTIC AND OTHER HAZARDOUS MEDICATIONS NOT APPROVED FOR
ADMINISTRATION**

Insert Organization Not Approved List of Antineoplastic and Other Hazardous Medications

FIRST DOSE POLICY**Policy No. 4-031.1****PURPOSE**

To define the process for the safe administration of a first dose medication in the home setting.

POLICY

Whenever possible, the first dose of any IV medication or therapy should be administered under the direct supervision of a physician (or allowed practitioner) in order to assess the safety and appropriateness of the drug and administration techniques.

FIRST DOSING IN THE HOME IS ALWAYS CONSIDERED AN EXCEPTION, AND ALL OTHER OPTIONS SHOULD BE EXPLORED.

Criteria for First Dosing

1. An appropriate and complete physician's (or allowed practitioner's) order must be obtained that specifies the drug, the dose, diluent type and amount, route, frequency, infusion time (if appropriate), any allergic or anaphylactic precautions, and when the last dose was given in the doctor's office or facility and patient response.
2. All attempts to obtain information regarding the patient's past medication allergies or anaphylactic reaction will be made.
3. The patient must be assessed as clinically stable at the time the decision is made to administer the first dose of medication at home and prior to the actual administration of the first dose.
4. The patient must be at least three (3) years old and weigh at least 15 Kg.
5. The patient must be alert, cooperative, and able to respond in such a way as to report symptoms.
6. The patient must reside in an area where there is access to Emergency Medical Services and be given phone numbers and locations of emergency support services.
7. The drug will be administered by an IV qualified registered nurse.
8. The nurse will remain in the home to observe the patient for at least thirty (30) minutes after the administration of the medication to insure the patient as tolerated the medication well.

Some examples of IV therapies that may be suitable for first dose in the home are:

- Antibiotics
- SQ/IV pain management (not anesthetics)
- Chemotherapy
- Hydration
- Parenteral and enteral nutrition
- Specific drugs such as Desferal, GMCSF, Epogen

Therapies NOT suitable for first time dosing are:

- Blood and blood component therapy,
- Dilantin
- Intraspinal pain management and anesthetic pain management
- IgG
- Investigational drugs
- Specific drugs such as Terbutaline, Aminophylline, Amphotericin, Dextron
- Vancomycin
- Inderal
- Haldol
- Innovar
- Digitalis
- Streptokinase
- L-Asparaginase

CRUSHING OF MEDICATIONS**Policy No. 4-032.1****PURPOSE**

To provide guidelines for crushing medications.

POLICY

Certain drugs may not be crushed. As a matter of policy, the organization adheres to the following list of medications as noncrushable medications. (See "[Oral Dosage Forms That Should Not Be Crushed](#)" Addendum 4-032.A.)

The nurse Case Manager will determine if a medication may be safely crushed.

PROCEDURE

1. The nurse will determine whether or not a medication may be crushed based on the drug manufacturer's information contained in the package inserts or information published in a reliable medical drug reference.
2. Should a question arise as to the "crushability" of any medication, the situation should be referred to the consultant pharmacist immediately.
3. If solid form is unsuitable for the patient, the consultant pharmacist will be contacted for alternatives.
4. The nurse will instruct patient and family/caregiver regarding the safe crushing of medication.

ADDENDUM 4-032.A

ORAL DOSAGE FORMS THAT SHOULD NOT BE CRUSHED

(Insert Consultant Pharmacist List Here)

**PULSE RATE DETERMINATION
WITH CERTAIN DRUGS
Policy No. 4-033.1**

PURPOSE

To delineate the process for determining pulse rate for certain drugs.

POLICY #1

The apical pulse rate will be measured and recorded prior to administration of drugs known to affect heart rate.

Rationale

Digital glycosides, beta, and calcium channel blocking agents can slow the heart rate by means of their effects on the sinus node (pacemaker) and the AV (atrioventricular) nodes of the heart. The apical heart rate can aid in assessing the effect of these drugs on the heart. When the heart rate is too slow it may indicate excessive drug effect.

PROCEDURE

1. Apical pulse rates will be measured prior to each administration.
 - A. Doses will not be held unless pulse is < 60 or as otherwise specified by attending physician.
 - B. Physician will be notified if pulse is < 60 so dose may be adjusted or held, if indicated.
2. All pulse rates will be recorded in the clinical notes.
3. If dose is held, nurse will record reason in the clinical notes.
4. Other vital sign measurements (i.e., blood pressure, respiratory rate) may be taken and reported to attending physician.

POLICY #2

Pulse rates and blood pressure will be measured and recorded following the administration of medications.

Insert Organization Approved List***Rationale***

Various nitroglycerin products are being used to treat angina pectoris, as well as refractory congestive heart failure. Nitroglycerin can dramatically drop the patient's blood pressure (hypotension) in some cases and this may result in a reflex tachycardia (increased heart rate) in some cases.

This response is most apt to occur when the use of nitrate products is first initiated or after a dose has been increased. It may be helpful to the patient to monitor the heart rate for a period of time when such a drug is started or when a dose is increased to see how the patient tolerates the drug.

Alpha, beta, and calcium channel blockers may decrease heart rate and blood pressure. The response is most apt to occur when the use of nitrate products is first initiated or after a dose has been increased. It may be helpful to the patient to monitor the heart rate for a period of time when such a drug is started or when a dose is increased to see how the patient tolerates the drug.

PROCEDURE

1. Apical pulse rates will be measured and recorded following the administration of the identified medications.
2. For the first week after an oral or topical nitroglycerin product is initiated, pulse rate and blood pressure will be measured and charted in the nurse's clinical notes. If a dose is increased, pulse monitoring to rule out the development of reflex tachycardia will be completed and recorded in the clinical notes.
3. The classification of significant tachycardia (> 100 beats per minute) will be made by the attending physician (or allowed practitioner).
 - A. Persistent rates > 100 should be reported to the physician.
 - B. Decisions to hold doses or adjust doses due to changes in pulse rates will be made by the physician.
4. The time the pulse is taken will depend on the bioavailability of the product (how fast it is absorbed into the blood stream).

The following guide is recommended to determine the best time to take the pulse after administration or application of the medication:

Insert Organization Approved List

Nitrates

Time After Dose

Calcium Channel Blockers

Alpha Blockers

Beta Blockers

STORAGE OF MEDICATIONS AND NUTRITIONAL PRODUCTS**Policy No. 4-034.1****PURPOSE**

To ensure that medications and nutritional therapy solutions are properly handled and stored.

POLICY

Medications and nutritional therapy solutions will be properly stored in the organization and in the patient's home.

PROCEDURE

1. Storage of medication in the organization will be consistent with applicable law and regulation.
2. Investigational medications and cytotoxic medications will be stored in a secure location and appropriately identified.
3. Medications used for external use and disinfectants will be stored separately from internal and injectable medications.
4. All medications, chemicals, and biologicals will be labeled for contents, with expiration dates clearly identified.
5. Medications and nutritional therapy solutions will be stored under conditions that enhance stability. Elements to be considered include:
 - A. Appropriate storage temperatures utilizing appropriate thermometers and temperature logs
 - B. Protecting solutions from contamination and spoilage
 - C. Controlling lighting, ventilation, and humidity
 - D. Prevention of moisture, condensation, and mold growth
 - E. Thorough cleaning and sanitizing of all surfaces, supplies, and equipment after each use

6. The environment where medications or nutritional therapies are prepared will be appropriate to the therapy preparations, whether in the office or in the patient's home. As appropriate to the setting, areas to consider include:
 - A. Functionally separate areas for sterile product preparation
 - B. An environment suitable to preparation of sterile products
 - C. Safety equipment to protect personnel preparing cytotoxic or hazardous medications
 - D. Clutter free, clean work surface for medication preparation or nutritional therapy solutions
7. Medication and nutrition therapy preparation will only be done by personnel with documented competencies regarding medication and nutrition therapy preparation.

MEDICATION LABELING**Policy No. 4-035.1****PURPOSE**

To promote safe medication administration through appropriate labeling.

POLICY

Medications will be administered only from properly labeled containers. No self-prescribed (patient) medications will be given by Organization's Name personnel.

PROCEDURE

1. All prescriptions should be labeled by the pharmacist with the following: pharmacy name and telephone number, prescription number, patient's name, date of filling, physician's name, name and strength of medication, directions for administration, and expiration date.
2. If necessary, the clinician will obtain additional medication information.
3. Medications will not be given from unlabeled containers.
4. If there is any reason to suspect the drug found in a container is not the drug labeled, the medication will not be given.
5. Labels will be carefully checked for expiration date, and the medication should be checked for stability (i.e., without indication of deterioration, dampness, cloudiness, discoloration, etc.).

ADVERSE DRUG REACTIONS**Policy No. 4-036.1****PURPOSE**

To ensure patient and nurse are instructed to identify adverse reactions to medications and to report them in a timely manner. To ensure that the nurse, in coordination with the pharmacist and physician, implements actions when adverse drug reactions occur, including the reporting to appropriate outside agencies.

POLICY

All organization personnel will be prepared to identify and react to adverse drug reactions. The process for defining, identifying, and reviewing significant adverse reactions will be collaborative in nature among nursing, pharmacy, and others as appropriate. All adverse drug reactions will be reported both internally and externally to appropriate agencies, as needed.

Patients will receive instruction regarding medication side effects, signs and symptoms of adverse reactions, and any necessary emergency response measures.

Definition

1. **Adverse Drug Reaction**: An undesirable or unexpected event that occurs due to administration of a medication, which results in:
 - A. Discontinuation of a drug
 - B. Modification of a dose of medication
 - C. Supportive treatment or additional treatment with prescription medication
 - D. A condition that is life threatening
 - E. Prolonging or requiring hospitalization
 - F. A disability
 - G. Death

The World Health Organization defines an adverse drug reaction as "Any response to a drug which is noxious and unintended, and which occurs at doses normally used in man for prophylaxis, diagnosis, or therapy of disease, or for the modification of physiological function."

PROCEDURE

1. Patient will be verbally instructed and receive written information (medication profiles) regarding identification of potential adverse drug reactions on initiation of therapy. Patient will receive instructions to call the pharmacist and/or nurse immediately if signs and symptoms of adverse drug reactions develop. This instruction will be documented in the clinical record and validated as part of the initial home care training session.
2. For patients who may be receiving antibiotics (particularly IM or IV, antiarthritic, and/or any drugs known to increase risk for anaphylaxis or severe side effects), the physician (or allowed practitioner) may be contacted prior to administration to obtain orders for anaphylaxis protocol.
3. Whenever adverse drug reactions are observed by the clinician and/or reported by the patient, the clinician should advise the patient to hold the next dose until the physician (or allowed practitioner) can be consulted.
4. All adverse reactions will be promptly reported to the patient's physician (or allowed practitioner) in order to minimize patient's health risks and discomfort.
5. If the patient is at risk for further complications of an emergency nature, the clinician will initiate appropriate emergency measures as per organization policy.
6. If the patient shows signs and symptoms of anaphylaxis, the clinician will follow the anaphylaxis protocol. (See "[Anaphylaxis Protocol](#)" Policy No. 4-037.)
7. All adverse reactions will be reported through the incident reporting process. (See "[Incident Reporting](#)" Policy No. 5-005.) Once reported, they will be reviewed and analyzed for any significant trends, patterns, or unusual occurrences that may impact patient care.
8. If the adverse drug reaction results in serious injury, illness, or death, risk management will be notified immediately.
 - A. The FDA's Med Watch Reporting Form will be completed by the pharmacist having knowledge of the ADR, and the form will be mailed/faxed to the FDA as appropriate in serious or unexpected adverse drug reactions. The FDA Med Watch Reporting Form can be obtained from the FDA by calling 1-800-FDA-1088 or at the website www.fda.gov.
9. All adverse drug reactions will be reviewed as part of the performance improvement program.
 - A. Reports will be reviewed and analyzed for trending and pertinent actions that may be required.

- B. Actions to improve performance will be determined based on the trends and/or analysis.
 - C. Any trends or patterns noted will be addressed through the performance improvement activities.
10. As part of the yearly organization evaluation, the adverse drug reporting process will be reviewed for its effectiveness in detecting reactions and the organization's ability to respond and improve the medication administration process. In addition, the organization will assess the usefulness of the definitions used for adverse drug reactions.

ADDENDUM 4-036.A

ADVICE ABOUT VOLUNTARY REPORTING

ADVICE ABOUT VOLUNTARY REPORTING

Report experiences with:

- medications (drugs or biologics)
- medical devices (including in vitro diagnostics)
- special nutritional products (dietary supplements, medical foods, infant formulas)
- other products regulated by FDA

Report **SERIOUS** adverse events. An event is serious when the patient outcome is:

- death
- life-threatening (real risk of dying)
- hospitalization (initial or prolonged)
- disability (significant, persistent, or permanent)
- congenital anomaly
- required intervention to prevent permanent impairment or damage

Report even if:

- you're not certain the product caused the event
- you don't have all the details

Report product problems—quality, performance, or safety concerns such as:

- suspected contamination
- questionable stability
- defective components
- poor packaging or labeling

How to report:

- only fill in the sections that apply to your report
- use section C for all products except medical devices
- attach additional blank pages if needed
- use a separate form for each patient
- report either to FDA or the manufacturer (or both)

Important numbers:

- 800-FDA-0178 to FAX report
- 800-FDA-7737 to report by modem
- 800-FDA-1088 for more information or to report quality problems
- 800-822-7967 for a VAERS form for vaccines

If your report involves a serious adverse event with a device and it occurred in a facility outside a doctor's office, that facility may be legally required to report to FDA and/or the manufacturer. Please notify the person in that facility that would handle such reporting.

Confidentiality: The patient's identity is held in strict confidence by FDA and is protected to the fullest extent of the law. The reporter's identity may be shared with the manufacturer unless requested otherwise. However, FDA will not disclose the reporter's identity in response to a request from the public, pursuant to the Freedom of Information Act.

ANAPHYLAXIS PROTOCOL**Policy No. 4-037.1****PURPOSE**

To provide guidelines for the clinician to follow in the event of a patient's anaphylactic reaction.

POLICY

Organization's Name nurse will implement the anaphylaxis protocol for emergency use in the event of an anaphylactic reaction during administration of IV medications. Emergency medications will be given according to a physician's (or allowed practitioner's) order.

Definition***Signs and Symptoms of Anaphylaxis***

1. Respiratory distress: dyspnea, wheezing, choking, cyanosis
2. Dermatologic changes: urticaria, erythema, angioedema, pruritus
3. Gastrointestinal complaints: nausea, vomiting, abdominal cramps, diarrhea
4. Vascular response: rapidly falling blood pressure, chills, sweating

PROCEDURE***Prior to IV Medication Administration***

1. Obtain baseline nursing assessment and vital signs.
2. Review patient's prior response to medication. Specifically inquire about allergies.
3. Instruct patient as to signs and symptoms to report at once.
4. Have telephone number of paramedics available (use 911, if appropriate).
5. If applicable, have anaphylaxis kit within reach.

During IV Medication Administration

1. Stop flow of drug.
2. Evaluate signs and symptoms rapidly.

3. Administer medications from an anaphylaxis kit according to physician-described directions.
4. Notify paramedics (use 911, if appropriate).
5. Notify physician (or allowed practitioner) and follow orders received.
6. Monitor patient's vital signs. If patient is hypotensive, keep him/her supine and elevate his legs.
7. Remain with patient until paramedics arrive.
8. If cardiopulmonary arrest occurs, begin resuscitation unless patient has current DNR order.

MEDICATION ERROR**Policy No. 4-038.1****PURPOSE**

To provide a process for identifying, reporting, and reviewing medication errors.

POLICY

Any medication error will be reported to the Clinical Director or Clinical Supervisor and the attending physician (or allowed practitioner). The process for defining, identifying, and reviewing significant medication errors will be collaborative among nursing, pharmacy, and others, as appropriate. The medication error rate will be monitored and reported per the appropriate committee.

Definition

1. **Medication Error**: Any error, which includes, but is not limited to the following:
 - A. Patient and family/caregiver does not follow physician (or allowed practitioner) orders or nurse instructions in administering medications—e.g., titrating of IV opiates outside of the physician's (or allowed practitioner's) ordered titration parameters.
 - B. Wrong medication, wrong time, wrong dose, wrong route of administration during intervention by the nurse, extra dose, omission of ordered drug.
 - C. Missing a scheduled administration of a SQ/IM/IV medication for any reason (e.g., staffing difficulties, equipment, supplies).

PROCEDURE

1. If an error is made in medication administration, the personnel making or discovering the error will notify the Clinical Supervisor.
2. If the error is made by a nurse, he/she may directly notify the physician (or allowed practitioner) after notifying the Clinical Supervisor of the error. The Clinical Supervisor will be notified of the physician's (or allowed practitioner's) comments and correction to orders, if applicable.
3. The patient will be observed for any untoward effects of the medication error for the duration of the excretion time in the case of the administration of an improper drug or improper dosage, and notation is made.
4. If appropriate, the pharmacist will be notified for any further actions that may be required.

5. If the error has been one of omission, the physician or Clinical Supervisor will be requested to instruct the nurse as to a corrected medication schedule.
6. Events and actions resulting from the error are objectively outlined in the clinical notes.
7. An incident report will be completed either by the nurse making the error, the nurse discovering the error, and/or the Clinical Supervisor. (See "[Incident Reporting](#)" Policy No. 5-005.)
8. All medication errors will be reviewed as part of the performance improvement program.
 - A. Reports are reviewed and analyzed for trending and pertinent actions that may be required.
 - B. Actions to improve performance will be determined based on the trends and/or analysis.
 - C. Any trends or patterns noted will be addressed through the performance improvement activities.
9. As part of the annual organization evaluation, the medication error reporting process will be reviewed for its effectiveness in detecting errors and the organization's ability to respond and to improve the medication administration process. In addition, the organization will assess, at this time, the usefulness of the definitions used for medication errors.

MEDICATION MONITORING**Policy No. 4-039.1****PURPOSE**

To define the ongoing medication monitoring process for the home care patient.

POLICY

Ongoing patient medication monitoring will use a collaborative approach between clinicians, physicians, pharmacists, patients, and families/caregivers.

The results of medication monitoring will be used to improve the patient's medication regime. The ongoing monitoring will occur in accordance with the established goals of therapy and be used to:

1. Evaluate the continued use of a medication in the current regimen.
2. Evaluate patient adherence to the prescribed medication regimen.

PROCEDURE

1. The clinician will assess the effect of medications on the patient. The assessment will identify potential contraindications, drug interactions, duplicative drug therapy, significant adverse reactions, and noncompliance with drug therapy.
2. The clinical effects of medications will be assessed through direct observation during home visits, review of assessment information, as well as clinical results of diagnostic studies.
3. Review of the information gathered from the above activities will be shared by the clinician with appropriate physicians (or allowed practitioner) and pharmacists, as well as during case conferences with other home care team members.
4. The information obtained through patient medication monitoring will be documented in the patient's clinical record and, if applicable, in the pharmacy record.

INVESTIGATIONAL MEDICATIONS

Policy No. 4-040.1

PURPOSE

To define the organization's policy for use of investigational medications.

POLICY

Experimental drugs will be administered per "[Experimental Research and Investigational Studies](#)" (see Policy No. 1-017.)

WITHDRAWAL OF LIFE-SUSTAINING CARE
Policy No. 4-041.1**PURPOSE**

To outline the responsibilities of organization personnel in withdrawing life-sustaining care.

POLICY

The decision to withdraw life-sustaining care will be made by the patient and family/caregiver (or his/her legal representative) and the physician (or allowed practitioner) in consideration of any Advance Directives. Organization's Name will comply with a written order from the attending physician (or allowed practitioner) to withdraw life-sustaining care.

(See "[Advance Directives](#)" Policy No. 2-034, "[Do Not Resuscitate/Do Not Intubate Orders](#)" Policy No. 2-035, "[Cardiopulmonary Resuscitation](#)" Policy No. 2-036, and "[Ethical Issues](#)" Policy No. 2-038.)

PROCEDURE

1. Upon admission, the patient receiving life-sustaining care and his/her family/caregiver will be informed of the organization policy regarding the withdrawal of this care.
2. All communication between organization personnel and the patient and family/caregiver or the physician (or allowed practitioner) regarding withdrawal of life-sustaining care will be documented in the clinical record.
3. If a decision to withdraw life-sustaining care is made, organization personnel will comply with a written order from the attending physician (or allowed practitioner).

CARE OF THE DYING PATIENT**Policy No. 4-042.1****PURPOSE**

To provide guidelines for the care of the dying patient.

POLICY

The organization recognizes the importance of each patient's and family/caregiver's unique and individual needs within the home care setting. Care provided for the dying patient will be responsive and respectful, and will be planned, implemented, and monitored in order to:

1. Optimize the patient's comfort and dignity
2. Manage pain and symptoms through interventions that alleviate and/or control pain and assess the patient's level of pain control
3. Identify secondary symptoms, determine their response to treatment, and take actions to limit them
4. Consider the psychosocial, emotional, and spiritual needs of the patient and family/caregiver
5. Implement bereavement care that supports the patient's and family/caregiver's coping mechanisms throughout the grief process

PROCEDURE

1. At the start of care, and on an ongoing basis, organization personnel will assess the dying patient for:
 - A. Comfort/pain level and response to pain management plan
 - B. Coping mechanisms, strengths of the patient and family/caregiver unit, and participation in the grief process/bereavement
 - C. Psychosocial, emotional, and spiritual needs
 - D. Presence of secondary symptoms and response to treatment
2. An individualized plan of care will be developed in cooperation with the patient and family/caregiver, physician, and other disciplines (if indicated) which facilitates:

- A. Physical/psychological comfort measures
 - B. Pain management (control or alleviation) according to physician (or allowed practitioner) orders which may include analgesia and noninvasive or nonpharmacological interventions
 - C. Prevention of secondary symptoms, including but not limited to: nausea, vomiting, diarrhea, stomatitis, alopecia, GI disturbances, blood dyscrasia, etc.
 - D. Prompt identification of secondary symptoms if they should occur, and physician notification of such
 - E. Monitoring of the response to treatment of secondary symptoms
 - F. Support for development of the patient's/family's/caregiver's coping mechanisms, including but not limited to, verbalization of feelings, referral to social services, hospice, etc.
 - G. Recognition of the patient's needs related to dignity, self-respect, and personal preferences
 - H. Support for the grieving process
3. With each patient visit, the nurse will follow the plan of care, assess the need for change, update the plan of care, and document accordingly. Aspects of care that must be reflected in the clinical record, when applicable, include:
- A. Pain management—The origin, location, severity (on a scale of 0 –10: 0 = no pain, 10 = unbearable pain), and alleviating and exacerbating factors for pain/discomfort
 - B. Preventative and treatment/measures provided for secondary symptoms and/or pain/discomfort, and the response to treatment
 - C. Psychosocial interventions to facilitate development of coping mechanisms and the grieving process, and the patient's and family/caregiver's response
 - D. Referrals to community resources
4. Case conferences will reflect coordination and communication between various team members relative to the patient's and family/caregiver's evolving physical, psychological, emotional, spiritual and bereavement needs (See ["Case Conference/ Progress Summary"](#) Policy No. 4-011.)

5. Organization personnel will adhere to:
- A. The desires made known by patient through the use of an advance medical directive executed according to state regulations and policy (See "[Advance Directives](#)" Policy No. 2-034.)
 - B. Do Not Resuscitate orders written by the patient's physician (or allowed practitioner) in accordance with state regulations and policy (See "[Do Not Resuscitate/Do Not Intubate Orders](#)" Policy No. 2-035.)

TRANSFER/REFERRAL CRITERIA AND PROCESS**Policy No. 4-043.1****PURPOSE**

To outline the process for transferring or referring a patient to another service provider.

POLICY

When a patient's needs change significantly and he/she requires care that cannot be provided by the organization, a transfer/referral to another service provider will be made.

When the patient's plan of care changes and this change results in a transfer or referral, the patient, his/her representative, as well as his/her primary physician (or allowed practitioner), will be notified and involved in planning decisions.

Definition

Service Provider: Another home health, hospice or health care facility.

Transfer/Referral Criteria

Home health care services for a patient will not be arbitrarily terminated. They may be transferred/referred only for the following reasons, which will be documented in the clinical record:

1. Medical reasons
 - A. A determination of the inappropriateness of continuing the services
 - B. A change in the patient's medical or treatment program resulting in services or care that cannot be provided
2. Other reasons
 - A. Moved out of service area
 - B. No longer a preferred provider

PROCEDURE

1. The organization will assist the patient and/or caregiver in selecting a post-acute care provider by using and sharing data that includes, but is not limited to HHA, SNF, IRF, or LTCH data on quality measures and data on resource use measures.
2. The organization will ensure that the post-acute care data on quality measures and data on resource use measures is relevant and applicable to the patient's goals of care and treatment preferences.

3. When a patient is referred to another organization, service, or individual, the patient will be informed of any financial benefit to Organization's Name.
4. The physician (or allowed practitioner) will be notified and an order will be obtained to transfer the patient.
5. The physician (or allowed practitioner) who writes the patient transfer order will verbally confirm the transfer arrangements and give the appropriate information to the receiving health care provider.
6. The clinician or designee will:
 - A. Inform the patient and family/caregiver of the physician (or allowed practitioner) transfer order.
 - B. Involve the patient and family/caregiver in the transfer.
 - C. Serve as a liaison between the patient, the family/caregiver, and the physician (or allowed practitioner) relative to the transfer arrangements.
 - D. Notify all internal or external providers of care for the patient.
7. All necessary medical information pertaining to the patient's current condition and treatment, post-discharge goals of care and treatment preferences will be sent to the receiving facility or health care practitioner.
8. The organization will comply with requests for additional clinical information necessary for the treatment of the patient.
9. All communication with the receiving provider, physician (or allowed practitioner), and patient will be documented in the clinical record.
10. Within 48 hours of transfer, the clinician will complete a transfer summary. (See "[Transfer Summary](#)" Policy No. 4-044.)
11. Within 48 hours of transfer, the clinical records clerk will send a copy of the transfer summary or clinical record to the receiving provider.
12. A copy of the transfer summary will also be sent to the physician (or allowed practitioner).
13. The clinician will update the comprehensive assessment, including required OASIS data elements, as required by regulation.

TRANSFER SUMMARY**Policy No. 4-044.1****PURPOSE**

To define the requirements for the documentation of a patient transfer to another service provider.

POLICY

All patients transferred from the organization will have a transfer summary completed and filed in the clinical record.

Definition

Service Provider: Another home health, hospice or health care facility.

PROCEDURE

1. Within 48 hours of transfer, the transferring clinician will complete a transfer summary that includes, as appropriate:
 - A. Patient identifying information and emergency contact
 - B. Destination of patient transferred
 - C. Patient's physician (or allowed practitioner) and phone number
 - D. Diagnoses related to transfer and any significant health history
 - E. Transfer orders and instructions
 - F. The reason for transfer
 - G. The physical and psychosocial status at the time of transfer, including specific medical, psychosocial, or other problems requiring interventions or follow-up
 - H. Continuing symptom management needs
 - I. Medication profile
 - J. A summary of the care provided and the progress toward achieving goals
 - K. Any instructions and/or referrals provided to the patient
 - L. The existence of any Advance Directives known to the organization
 - M. Date of face-to-face encounter, if during initial certification

- N. Post discharge goals
 - O. Treatment preferences to the receiving facility or health care practitioner
 - P. The date of transfer, which is the date of the last visit made
 - Q. Contact person receiving report and time report given
2. Completed transfer summaries will be given to a clinical records clerk, who will send a copy to the receiving organization, along with a copy of the clinical record, within two (2) business days of a planned transfer or within two (2) days of becoming aware of an unplanned transfer and will file the original in the clinical record.
 3. A copy of the transfer summary will be sent to the physician (or allowed practitioner) within 48 hours of transfer.
 4. Organization's Name will make available any additional necessary clinical information requested by the receiving facility or health practitioner, such as a copy of the patient's current plan of care or the most recent physician's (or allowed practitioner's) orders.

DISCHARGE CRITERIA AND PROCESS**Policy No. 4-045.1****PURPOSE**

To outline the process for a discharging a patient from service.

POLICY

When the patient's plan of care changes and this change results in discharge or reduction of services, the patient or his/her representative, as well as his/her primary physician (or allowed practitioner), will be notified and involved in planning decisions.

A discharge summary will be completed and filed in the clinical record. (See "[Discharge Summary](#)" Policy No. 4-046.)

Definitions

1. **Termination/Discharge**: Discontinuance of all organization services by the organization.
2. **Reduction of Services**: A change in the patient's service plan in which one (1) or more existing services are discontinued.

Discharge/Reduction of Services Criteria

1. Services will be terminated when the patient meets one (1) or more of the discharge criteria:
 - A. A change in the patient's medical or treatment program that requires a change to a different level of care.
 - B. A change in the patient's condition requires care or services other than that provided by the organization. If the patient's acuity requires another level of care that the organization cannot provide, the organization shall arrange for a safe and appropriate transfer to another organization that can provide the needed level of care.
 - C. If appropriate, the physician (or allowed practitioner) and the organization agree that the goals of home care have been attained or are no longer attainable.
 - D. The patient or family/caregiver refuses or discontinues care.
 - E. The patient or family/caregiver refuses to cooperate in attaining the objectives of home care.
 - F. Conditions in the home are no longer safe for the patient or organization personnel.
 - G. The patient and/or family display disruptive, abusive, and uncooperative behavior.

- H. Family/caregiver has been prepared and is capable of assuming responsibility for care.
- I. The patient moves from the geographic area served by the organization.
- J. The patient's physician (or allowed practitioner) has failed to renew orders, or the patient has changed physicians and orders cannot be obtained from the new physician to support patient's needs.
- K. The physician (or allowed practitioner) gives orders that are not consistent with the stated diagnosis as required by law and fails to give the needed orders when requested by the organization.
- L. If the physician (or allowed practitioner) face-to-face encounter was not completed prior to the initial certification, the patient or family/caregiver refuses to obtain a physician (or allowed practitioner) face-to-face visit within 30 days of start of care.
- M. The patient or payor will no longer pay for the services provided by the organization.
- N. The organization is closing out a particular service or all of its services.
- O. The patient expires.

PROCEDURE

1. The organization will verbally notify the patient of the decision to terminate or reduce services within one (1) visit prior to the time the change in service is to occur (i.e., prior to the last scheduled visit).
2. Prior notice will not be necessary when services are discontinued by the patient or physician (or allowed practitioner); however, action taken must be documented in the clinical record and a discharge summary completed. A copy of the discharge instructions will be mailed to the patient.
3. An update to the comprehensive assessment, including required OASIS data elements, will be completed, as required by regulation.
4. For a patient requiring continuing care, assistance will be given to the patient and family/caregiver in order to manage continuing care needs after the organization services are discontinued. Discharge instructions will be provided.
 - A. Discharge planning will identify needs the patient may have.
 - B. Arrangements for such services will be coordinated by the organization when applicable.

5. The decision to terminate or reduce services must be documented in the clinical record, citing the circumstances and notification to the patient, the responsible family/caregiver or representative, and the patient's physician (or allowed practitioner). Efforts to resolve problems prior to discharge will also be documented in the patient's record. The patient shall be provided contact information for other home health agencies and providers if continued care is needed. The Clinical Supervisor or designee is accountable for the decision and the required documentation. If the decision to terminate services is due to the patient's behavior, the behavior of other persons in the patient's home, or situation, the clinical record must reflect the following:
 - A. Identification of the problems encountered
 - B. Assessment of the situation
 - C. Communication with organization management and the physician responsible for the plan of care
 - D. A plan to resolve the issues
 - E. Results of the plan implementation
6. Each clinician making the final visit for his/her discipline will complete the sections of the discharge notice for discontinuing a discipline. (See "[Discharge Summary](#)" Policy No. 4-046.)
7. If more than one (1) discipline is providing care, the discipline being discontinued will be specified on the interim order.
8. ORGANIZATION DISCHARGE: A discharge summary will be completed for all discharged patients. A copy may be mailed to the primary physician (or allowed practitioner). (See "[Discharge Summary](#)" Policy No. 4-046.)
9. DISCHARGE FOR CAUSE: If the patient is being discharged for cause, the physician(s) or allowed practitioner(s) issuing orders for the plan of care, and the patient's primary care practitioner or other health care professional who will be responsible for providing care and services to the patient after discharge will be notified. For Medicare and Medicare HMO patients, the organization will issue a Notice of Medicare Non-Coverage (NOMNC) at least 48 hours prior to termination.
10. The clinician will update the comprehensive assessment, including required OASIS data elements, as required by regulation.
11. All discharge paperwork will be due in the office within 72 hours of the discharge date. This will include the discharge order, discharge summary, plan of care, medication profile, and OASIS.

12. The discharge record will be organized according to the organization policy regarding clinical record contents. Documentation will be reviewed by the Clinical Supervisor or designee and completed within 30 days of the discharge, at which time it will be removed from the active files.

DISCHARGE SUMMARY**Policy No. 4-046.1****PURPOSE**

To define the requirements for documentation of discontinuation of a service and when patients are discharged from the organization.

POLICY

Each patient discharged from a service and/or from the organization will have a written discharge summary completed and filed in the clinical record.

PROCEDURE

1. Each clinician who provides care will complete a discharge summary at the time his/her discipline is discharged, which may include as appropriate:
 - A. Patient identifying information
 - B. Patient's physician (or allowed practitioner) and phone number
 - C. The date of discharge, the date the physician and patient informed of discharge
 - D. The reason for discharge
 - E. Diagnosis
 - F. The status of problems identified at admission and during the provision of care
 - G. The progress towards goals/desired outcomes
 - H. Treatment preferences
 - I. Medical status at discharge including continuing symptom management needs
 - J. The overall status of the patient
 - K. A summary of the care or services provided
 - L. Instructions given to patient or responsible party
2. The discharge summary and other relevant clinical record documents will be completed and submitted within 72 hours of discharge from service.

3. A copy of the discharge summary and any necessary medical information will be provided to the patient's primary care practitioner or other health care professional who will be responsible for providing care and services to the patient after discharge from the organization within five (5) business days of the patient's discharge.
4. The organization will complete all necessary audits to determine the completeness of the patient's clinical record within 30 days of the last home visit and discharge date. The discharge record will be organized according to policy for clinical records contents and removed from the active files.

CONTENTS OF CLINICAL RECORD**Policy No. 4-047.1****PURPOSE**

To outline the requirement and components of a clinical record.

POLICY

A clinical record will be maintained for each patient receiving care. The clinical record will contain sufficient information to identify the patient, describe the patient's problems and needs, justify care, accurately document care provided and results in detail, and facilitate continuity of care among organization and contract personnel.

(See "[Clinical/Service Data Collection](#)" Policy No. 2-011 and "[Assembly of Clinical Record](#)" Policy No. 4-065.)

PROCEDURE

1. The following information will be available in the clinical record for patients receiving skilled care:
 - A. Patient's name, clinical record number, sex, current address, phone number, date of birth, and his/her legally authorized representative
 - B. Date of admission
 - C. Name, address and telephone number of family/caregiver and patient representative to be contacted in the event of an emergency/death
 - D. Reason for service/admission
 1. Primary and secondary diagnoses and prognosis, admission diagnosis or pertinent health information
 2. Notation of the conditions and diagnoses which are relevant to the plan of treatment, plan of care, or plan for personal care services
 - E. Plan of treatment, plan of care, or plan for personal care services in its entirety
 - F. Laboratory and X-ray reports
 - G. Treatment consent or service authorization forms
 - H. Documentation that a list of patient rights has been made available to each patient, patient's representative or family

- I. Referral source and admission and discharge dates from hospital or other institution, if applicable
- J. Patient database including physical, psychosocial, and environmental data, and family profile
- K. Legal representative, when applicable
- L. Legible, complete, and individualized diagnostic and therapeutic orders authenticated within the time frame defined by the organization or according to law and regulation (See "[Verification of Physician Orders](#)" Policy No. 4-003.)
- M. Relevant diet or dietary restrictions, if any
- N. Allergies or sensitivities. This information shall be given such prominence in the record that it is obvious to any health practitioner or agency personnel who have reasons to provide food or medication to the patient.
- O. Suitability or adaptability of the home to planned services
- P. Safety measures to protect the patient from injury or harm
- Q. Functional limitations related to care and services provided
- R. Signed and dated progress notes for each discipline rendering service. Clinical notes shall be signed and incorporated into the patient's health record at least every seven working days.
- S. Record and findings of initial and ongoing assessments
- T. Procedures and/or treatments rendered
- U. Every dose of medication administered by organization personnel and any adverse drug reaction
- V. Identity of other individuals and organizations known to be involved in patient care
- W. Description of patient's activity restrictions related to care and service
- X. Care planning activity based on the patient's problems, needs, and expected outcomes
- Y. Educational needs of patient and family/caregiver
- Z. Patient and family/caregiver education provided
- AA. Name, address and telephone number of patient's physician, dentist, podiatrist, or other licensed and legal authorized person whose orders or recommendations are being implemented by the home health agency

- BB. Name, address and telephone number of patient's primary care practitioner or other authorized health care professional who will be responsible for providing care and services to the patient after discharge from the organization
- CC. Relevant communication to the patient's physician
- DD. Transfer/discharge summaries and other summary reports sent to the physician (or allowed practitioner), as required by law and regulation and organization policy
 - 1. The discharge statement shall include the date of discharge, reason for termination of services, and condition upon discharge
- EE. Advance Directive information and documentation as required by organization policy
- FF. Consents for care as required by law and regulation and organization policy
- GG. Documentation of the assessment data collected, actions and interventions performed, and patient response to care provided
- HH. Current medication profile including prescription and nonprescription medications, herbal products and home remedies, including dose, frequency, and route of administration, with ongoing updates to the patient's medication regimen, including new, changed and discontinued medications; adverse reactions, significant side effects, drug allergies, and contraindications
- II. Potential drug/food interactions
- JJ. Relevant diagnostic and therapeutic procedures, treatments, and tests and their results
- KK. Documentation of communication to the organization receiving the patient when a patient is transferred
- LL. Medical equipment provided by the organization or related to care and service provided
- MM. Signed documents to include but not limited to: release of information, informed consent, receipt of patient rights statement, receipt of privacy practices, ABNs, etc.
- NN. Documentation of telecommunication services, if appropriate
- 2. The clinical record for patient receiving only personal care and/or support/chore services contains the following information:
 - A. Individual to contact in emergency
 - B. Specific duties to perform
 - C. Documentation of services rendered

- D. Plan of service including schedule of services
- E. Statement of discontinuation of service and notification of others, as appropriate
- F. Documentation of supervision of the aide/chore worker

PURPOSE

To define the process for periodic and ongoing review of the patient's clinical record.

POLICY

Clinical records will be reviewed at least quarterly by qualified organization personnel to assure that documentation entered is reliable, timely, valid, and accurate. The record review shall reflect the scope of the QAPI program to show measurable improvement in indicators for which there is evidence that improvement in those indicators will improve health outcomes, patient safety, and quality of care. The frequency and detail of the data collection shall be approved by the organization's governing body.

PROCEDURE

Ongoing Review

1. Each clinical record will be reviewed on an ongoing basis by the Clinical Supervisor, Performance Improvement Coordinator, Clinical Director, or designees for:
 - A. The timeliness of entries into the clinical record, including following procedures for late entries
 - B. Meets payor requirements for medical necessity, homebound status, qualifying service and face-to-face visit
 - C. Compliance with organizational policy
 - D. Compliance with the established plan of care
 - E. The completeness of clinical records
 - F. The accuracy of clinical records
 - G. The appropriateness of services rendered (e.g. reasonable and necessary services, appropriate level of care and services rendered, medical condition justifies the number of visits provided, etc.)
 - H. The need for continued care

Note: Clinical record reviews will be performed by qualified health professionals of equivalent or higher level of training than the care provider.

2. As a result of this review, action will be taken as necessary to improve care and to ensure proper billing for services. The Clinical Supervisor, Performance Improvement Coordinator, Clinical Director, or designees will identify issues with documentation and, based on the review, if the issue:
 - A. Is applicable to an individual, the individual will be counseled
 - B. Is applicable to the organization as a whole, will refer the issue to the management team, QAPI Committee or Compliance Committee, as appropriate for review

Quarterly Clinical Record Review

1. The clinical record review will consist of a process based on the following guidelines:
 - A. The review will consist of a random sample selection of both active and inactive cases.
 1. The same of ten (10) percent of the total patient census with a minimum of twenty records and a maximum of 100 records every six months
 2. Of the 10% sample, 50% will be active patient clinical records and 50% will be discharged patients.
 - B. The review will encompass a representation of all professional disciplines defined in the scope of services.
 - C. Each professional discipline will participate in review of clinical records for their service.
 - D. No person involved in the care of a patient may participate in the review of that patient's record.
 - E. All records reviewed will be secured for confidentiality.
 - F. All records will be reviewed in the designated area.
 - G. All records will be reviewed using a clinical record review tool to:
 1. Both concurrent and retrospective.
 2. Performed against preset criteria of practice for each discipline providing care. Criteria of practice shall include:
 - A. Appropriateness of the level of care provided to protect the health and safety of patients.
 - B. Timeliness of the provision of care.
 - C. Adequacy of the care to meet patients' needs.

- D. Appropriateness of the specific services provided.
 - E. Compliance with the standards of practice for patient care.
 - F. Accessibility to care.
 - G. Continuity of care.
 - H. Privacy and confidentiality of care.
 - I. Safety of care environment.
 - J. Participation in care by patient and family.
- 3. Determine the adequacy of the plan of care and to determine if further service is necessary and appropriate
 - 4. Determine that data is reliable, valid, and accurate
- 2. The Performance Improvement Coordinator will incorporate the review and analysis of Outcome Based Quality Indicators (OBQI) reports as part of the organization's quarterly clinical record review. The OBQI record review and analysis will focus on case mix and adverse events report data. The record review will include:
 - A. Adverse events with the most clinical relevance to the organization based on the case mix report
 - B. Adverse events with the highest incidence as compared to the reference group
 - 3. Record review findings will be documented and the data collated and analyzed.
 - 4. Results will be utilized for improvements in patient care and incorporated into performance improvement plans and activities.
 - 5. A summary of the results and corresponding analysis will be presented to the following:
 - A. QAPI Committee
 - B. Compliance Committee
 - C. Management Team

PURPOSE

To provide guidelines for the provision of health information and instruction to patients and family/caregivers.

POLICY

Patients and family/caregivers will receive education in verbal, visual, and written format, as appropriate. The scope of teaching will be determined by assessing the needs, abilities, learning preferences, and readiness to learn of the patient and family/caregiver, as well as by the plan of care, goals and established outcomes. Education will be the responsibility of each clinician and will focus on:

1. Facilitating the patient's and family/caregiver's understanding of his/her health status, health care options, and consequences of options
2. Encouraging patient participation in decision-making about health care options
3. Increasing patient and family/caregiver potential to follow the plan of care and to meet established goals
4. Maximizing care skills of the patient and family/caregiver
5. Enhancing the patient's and family/caregiver's knowledge and ability to cope with and manage pain
6. Increasing the patient's and family/caregiver's ability to cope with the health status, prognosis and outcomes
7. Enhancing the patient's and family/caregiver's role in continuing care
8. Promoting a healthy lifestyle
9. Maintaining the patient's health status
10. Assisting the patient's and family/caregiver's ability to cope with death, if applicable

PROCEDURE

1. The clinician will assess, plan, implement, and evaluate the following on each patient and family/caregiver, as appropriate:
 - A. The atmosphere for conducive learning

- B. The pertinent information needed by the patient and family/caregiver in relation to the care being rendered
 - C. The level of knowledge of the patient and family/caregiver in relation to the diagnoses, plan of care, required activities by patient and family/caregiver, lifestyle changes, etc.
 - D. The ability and readiness of the patient and family/caregiver to learn
 - E. Personal, cultural, spiritual, ethical, and religious practices that might affect learning and care expectations and treatment
 - F. Emotional barriers that might affect learning
 - G. Desire and motivation to learn
 - H. Physical and/or cognitive limitations, as well as communication and language barriers to learning
2. Based on the above, the assessments will be used as the basis for planning patient and family/caregiver education. In the event that any barriers to learning exist, these barriers, as appropriate, will be discussed with the patient and family/caregiver as well as the Clinical Supervisor. If they cannot be overcome, the patient's physician will be contacted.
 3. When appropriate, the clinician will use preprinted, organization-approved patient teaching materials. When not available, the clinician will identify available written or visual materials to aid in the education process.
 4. If a patient's condition prevents him/her from participating in instruction, family/caregivers will receive the information/instruction.
 5. If a patient lives alone and there is no one able or willing to receive the instruction, the physician will be informed, and this information will be documented in the clinical record.
 6. Unless otherwise ordered by the physician (or allowed practitioner), the patient and family/caregiver will receive verbal and, as appropriate, written instructions that are in plain language that does not use medical abbreviations on:
 - A. The patient's disease process and prognosis
 - B. The medical regimen
 - C. Medication management and administration
 - D. Food and drug interactions
 - E. Nutrition interventions, modified diets, and oral health

- F. Prescribed treatments
 - G. Pain management
 - H. Consequences of noncompliance
 - I. Basic home safety
 - J. Personal hygiene and grooming
 - K. Infection prevention and control
 - L. Safe, effective use of equipment and supplies
 - M. Environmental and mobility safety
 - N. Emergency preparedness plan
7. Documentation of patient and family/caregiver education will consist of:
 - A. Describing what was taught to the patient (if using preprinted materials, document the name of the handout)
 - B. Describing the patient's response to the teaching, including the level of understanding and the ability to repeat or demonstrate what was taught
 - C. Describing any additional learning needs not currently met
 - D. Describing teaching planned for subsequent visits
 8. When applicable and available, audiotapes, videotapes, books, booklets, etc. will be made available to patients to assist in the educational process.
 9. Community resources will be accessed, as appropriate and available, based on patient and family/caregiver needs.
 10. Patient and family/caregiver education will be interdisciplinary, as appropriate to the plan of care. At case conferences, patient education and learning needs and processes should be discussed by all clinicians involved with the care.
 11. The patient's and family/caregiver's knowledge, skills, and behaviors will be assessed during home visits through an interactive process. Reinforcement of instruction will be provided as needed.
 12. Individual needs and learning styles will be evaluated when identifying appropriate methods and resources for patient education.

SAFE/EFFECTIVE USE OF MEDICATIONS**Policy No. 4-050.1****PURPOSE**

To provide guidelines for the instruction of patients/family/caregivers regarding the safe, effective use of medication.

To promote correct administration of medication by patients and families/caregivers.

POLICY

Patients and family/caregivers will receive information regarding the safe and effective use of medications, in accordance with applicable organization policies.

Organization's Name will encourage patient and family/caregiver participation in his/her own care and will provide teaching about the correct administration of medications by the patient or family/caregiver as ordered by the attending physician (or allowed practitioner) or purchased over the counter. Teaching will also include the safe storage of medications.

PROCEDURE

1. Upon admission, the patient and family/caregiver will be assessed as to his/her knowledge and skill required for safe and effective use of medications.
2. The components of the medication assessment used to determine patient and family/caregiver knowledge and skill related to medication administration will include, but will not be limited to:
 - A. Name, dosage, route, duration, time and usage of medication, intended use as well as expected actions of drug therapy
 - B. Preparation, self-administration, and use of medication, including over-the-counter products
 - C. Safeguards against contamination
 - D. Compounding and administration techniques (if applicable)
 - E. Side effects, adverse reactions/interactions as well as contraindications
 - F. Self-monitoring of drug therapy

- G. Proper storage and expiration dating of medications
 - H. Refill information
 - I. Actions to take in the event of a missed dose
 - J. Proper disposal of unused or expired medications
 - K. Other information, as applicable
3. Based on the assessment, the clinician should review written organization information available for patient and family/caregiver instruction.
4. Using the written information, the clinician will review the key points required, based on patient knowledge and skills as well as identified needs. This may include:
- A. Teaching the patient the purpose and side effects of medications, and the patient's role in identifying and preventing medication errors.
 - B. Assisting the patient in setting up medications for the first time.
 - C. Assessing the patient's ability to self-administer medications correctly, and document the patient's response and comprehension.
 - D. Answering questions/concerns expressed by the patient and family/caregiver regarding the patient's self-administration of medications.
 - E. Instructing the patient and family/caregiver regarding safe storage of medications:
 - 1. Medications should be stored separately from other toxic drugs and chemicals.
 - 2. Medication should be removed from storage during instruction and administration times.
 - 3. Medications should be kept out of the reach of children, pets, and confused or disoriented patients.
 - 4. The nurse will plan with the patient and family/caregiver for the safe, therapeutic storage of drugs during the assessment process.
 - 5. Drugs requiring refrigeration should be stored inside the refrigerator.
 - 6. Urine testing and other diagnostic materials should be stored away from all medications, heat, light, and moisture.

5. The clinician will include information, when appropriate, regarding poison control center numbers, allergies, pharmacy numbers, and emergency actions.
6. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Patient and family/caregiver understanding
 - C. Return demonstrations
 - D. Response to teaching
 - E. Updating medication profile

DRUG–FOOD INTERACTIONS**Policy No. 4-051.1****PURPOSE**

To provide guidelines for the instruction of patient and family/caregiver on potential drug–food interactions and counseling on nutrition intervention and/or modified diets.

POLICY

Patient and family/caregiver will receive information regarding potential drug–food interactions, and written and verbal instructions about nutrition and diets in accordance with applicable organization policies.

PROCEDURE

1. Upon admission, each patient and family/caregiver will receive information on food, drug, and alcohol interactions, describing the most common types of interactions known.
2. If the patient is receiving medications not listed in the information, the patient's pharmacist and/or physician will be contacted for any additional information.
3. The information will be reviewed with the patient in relation to the medications the patient is currently receiving.
4. During subsequent home visits, the information will be used as a resource whenever there is a change in the patient's medication regimen.
5. When a patient is on a special or modified diet or receiving nutritional interventions, written or verbal information will be discussed with the patient and family/caregiver including as appropriate, but not limited to:
 - A. Purpose of the diet or nutritional supplement
 - B. Relationship to disease process
 - C. Lifestyle modifications
 - D. Foods to avoid
 - E. Foods to be encouraged
 - F. Oral health

- G. Storage of the nutritional therapy solution to ensure stability, if applicable, including protecting solutions from contamination and spoilage and controlling lighting, ventilation, and humidity to prevent moisture or mold growth
6. Documentation of patient and family/caregiver instruction in the clinical record will include:
- A. Information taught
 - B. Patient and family/caregiver understanding
 - C. Return demonstrations
 - D. Response to teaching
 - E. Additional learning needs

PURPOSE

To provide guidelines for the instruction of patients and family/caregivers regarding pain and the management of pain as a part of treatment.

POLICY

As identified, patients and family/caregivers will receive information regarding pain and the management of pain as an integral part of care.

PROCEDURE

1. Clinicians will identify patients with pain or who are at risk for pain during initial and ongoing patient assessments.
2. The patient and family/caregiver will receive verbal or written instructions, as appropriate, regarding:
 - A. The pain process
 - B. The risk for pain
 - C. The pain assessment process
 - D. The importance of effective pain management
 - E. Methods for pain management, when identified as part of treatment
 - F. Potential limitations of pain management modalities
 - G. Side effects of pain treatment
3. Documentation of patient and family/caregiver instruction and understanding in the clinical record will include:
 - A. Information taught
 - B. Patient and family/caregiver understanding
 - C. Response to teaching
 - D. Additional learning needs

PURPOSE

To provide guidelines for the instruction of patients and family/caregivers in habilitation or rehabilitation techniques to facilitate adaptation and/or functional independence.

Definitions

1. **Habilitation:** Educational, medical, social, and other measures undertaken for individuals born with limited functional abilities.
2. **Rehabilitation:** The combined and coordinated use of educational, medical, social, and vocational measures for training or retraining individuals disabled by disease or injury. The goal is to enable individuals to achieve their highest possible level of functional ability.

POLICY

Patients will be assessed for habilitation or rehabilitation potential, and they will receive referrals and information as needed. (See "[Functional Assessment](#)" Policy No. 4-021.)

PROCEDURE

1. Upon admission, the patient will be assessed regarding functional limitations that need to be addressed to ensure adaptation to the home environment.
2. The functional assessment will include, but will not be limited to, activities of daily living:
 - A. Dressing
 - B. Feeding
 - C. Hygiene
 - D. Activity/exercise
 - E. Homemaking/housekeeping
 - F. Toileting/elimination

3. Based on the assessment, a physician's (or allowed practitioner's) order will be obtained, and a referral will be made to physical, speech, and/or occupational therapy, as needed.
4. The patient's physical status and functional abilities will be evaluated by a rehabilitation professional before instruction and treatment is initiated.
5. The therapists, when involved in the care of the patient, will be responsible for instructing the patient in habilitation or rehabilitation techniques in response to identified needs and patient treatment choices.
6. Verbal and/or written information regarding habilitation or rehabilitation techniques will be provided to the patient and family/caregiver, based on his/her ability to adapt to his/her environment.
7. Information regarding potential benefits and risks of habilitation or rehabilitation services will be provided to the patient in order to make an informed decision regarding services and treatment.
8. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Adaptations made to the environment
 - C. Patient and family/caregiver understanding
 - D. Return demonstrations in use of rehabilitation equipment, if appropriate
 - E. Response to teaching
 - F. Additional learning needs

APPROPRIATE USE OF RESTRAINTS AND SUPPLIES**Policy No. 4-054.1****PURPOSE**

To provide guidelines for the instruction of family/caregivers regarding the appropriate, safe, and effective use of restraints, alternatives to the use of restraints, and attention to the needs of the patient in restraints.

POLICY

Families/caregivers will receive information regarding the appropriate, safe, and effective use of restraints, alternatives to the use of restraints, and attention to the needs of the patient in restraints.

Definitions

A restraint is any method (chemical or physical) of restricting a patient's freedom of movement, physical activity, or normal access to the body.

1. **Physical Restraints**: Specially designed devices including: an arm board, side rails on a bed, a wrist or hand restraints, or lap belts. Household materials intended for other purposes can also be used as a restraint, such as a sheet tied around the waist and chair, or a sheet tucked over a patient and under the mattress to keep him/her in bed.
2. **Support Devices**: Special equipment that maintains and improves a patient's ability to function. These devices can include wedge cushions to provide support and positioning, special chairs, special splints, and specially designed eating utensils.
3. **Chemical Restraints**: Any drugs that are used to control behavioral symptoms as a substitute for good care practices. Behavioral symptoms are actions used when a patient is unable to communicate verbally due to a medical condition, which expresses distress. Some examples of behavioral symptoms include anger, agitation, screaming, continuous wandering, pacing, repetitive actions, or paranoia.
4. **Psychoactive Drugs**: These are the most common drugs that are used as chemical restraints. Psychoactive drugs include major tranquilizers or antipsychotics, sedatives/hypnotics, antidepressants, and minor tranquilizers or anxiolytics and other drugs used to treat a physical illness that has psychoactive effects.

PROCEDURE

1. Upon admission, each patient will be assessed regarding devices or medications being used within the home as restraints.

2. The clinician will, through interview and observations, determine the patient and family/caregiver level of understanding in the appropriate use of the restraint.
3. If the family/caregiver has identified knowledge and skill deficits, the clinician will provide additional instructions to the family/caregiver, including:
 - A. How the device or medication acts as a restraint
 - B. Correct and appropriate application of the device or the use of the medication
 - C. Attention to the needs of the patient while the device or medication is being used
 - D. Any alternatives to the use of the device or medication purely as a restraint
4. The organization will maintain generic written and pictorial teaching tools for common devices used as physical restraints. These tools will be used whenever the family/caregiver chooses to restrain the patient. The teaching tools will include:
 - A. Definition of the restraint
 - B. Why family/caregivers choose to use restraint
 - C. The dangers associated with the use of the restraint
 - D. Alternatives to using the restraint
5. Teaching materials will be provided for family/caregivers of patients, which address the potential for psychoactive medications to be used as chemical restraints. The material may be provided by the pharmacist directly to the family/caregivers or to the organization nursing personnel to discuss with them during home visits. The materials will include:
 - A. The definition of a chemical restraint
 - B. The actions and uses for a psychoactive drug
 - C. At what point this type of drug is considered a chemical restraint
6. Clinicians will assess on an ongoing basis the appropriate use of devices or medications used as restraints. Any suspected abuse/neglect will be reported, according to organization policy. (See "[Assessment of Possible Abuse/Neglect](#)" Policy No. 2-037.)
7. Documentation of instruction in the clinical record should include:
 - A. Information taught
 - B. Patient and family/caregiver understanding

- C. Return demonstrations
- D. Response to teaching

SAFE/EFFECTIVE USE OF EQUIPMENT AND SUPPLIES**Policy No. 4-055.1****PURPOSE**

To provide guidelines for the instruction of patients and family/caregivers regarding the safe, effective use of medical equipment and/or supplies.

POLICY

Patients will receive information regarding the safe and effective use of home medical equipment (HME) and supplies provided by Organization's Name, in accordance with applicable organization policies including, but not limited to, "[Safe and Appropriate Use of Medical Equipment and Supplies](#)" (see Policy No. 6-030) and "[Medical Equipment Malfunction](#)" (see Policy No. 6-032.) When medical equipment is supplied by another organization, organization personnel will be supportive in the patient teaching role.

PROCEDURE

1. Upon admission, the patient assessment will document the use of any medical equipment or supplies being used in the home. The HME company providing the equipment and any patient instruction materials to be left in the home will be identified.
2. The clinician will, through interview and observation, determine patient and family/caregiver level of understanding of the use of any equipment and supplies ordered for care, treatment, and service.
3. If the patient and family/caregiver have knowledge and skill deficits regarding the use of the HME or supplies and the clinician has a sound knowledge base regarding the equipment or supplies, the clinician may provide additional instruction to the patient and family/caregiver.
4. If the patient and family/caregiver have knowledge and skill deficits regarding the use of the HME and the clinician does not have a sound knowledge base regarding the equipment, the clinician should contact the HME company to further instruct the patient and family/caregiver.
5. The organization will maintain generic patient teaching tools from the HME company for common pieces of HME found in the home, including, but not limited to:
 - A. Oxygen
 - B. Humidifiers
 - C. Oxygen concentrators

- D. Oxygen cylinders
 - E. Suction machines
 - F. Mechanical lifts
 - G. Trapeze bars
 - H. Pressure pads/pumps
 - I. TENS units
 - J. Canes, walkers, wheelchairs
 - K. Hospital beds
 - L. Lift chairs
 - M. Bathroom aids
6. Specific operational information related to these items will be obtained from the HME company providing the equipment. This includes information such as:
- A. Basic purpose and description of the equipment
 - B. Basic operating instructions including electrical safety
 - C. Troubleshooting
 - D. Safety precautions and warnings, including manufacturer recalls
 - E. Cleaning and/or disinfecting
 - F. Infection control precautions as applicable
 - G. Backup equipment and accessories, if applicable
 - H. Emergency plans, when applicable
 - I. Correct use of the equipment
 - J. Checklists, when appropriate
 - K. Maintenance to be performed by patient and family/caregiver including calibration per manufacturer's guidelines
 - L. Storage and/or transport of equipment

7. If equipment operation has not been explained to the patient by the HME company, the clinician should provide or arrange education or contact the company to provide further information and/or instruction, as needed.
8. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Patient and family/caregiver understanding
 - C. Return demonstrations
 - D. Response to teaching

Provision of Care and Record Management
STORAGE, HANDLING, AND ACCESS
TO SUPPLIES AND GASES
Policy No. 4-056.1

PURPOSE

To provide guidelines for the instruction of patients and family/caregivers in storage, handling, and access to supplies and medical gases.

POLICY

Patients will receive information and instruction regarding storage, handling, and access to supplies and medical gases to promote safe usage.

Definitions

1. **Medical Equipment:** Any assistive device or piece of equipment used by home health personnel, patient, and/or family/caregiver to meet the patient's needs such as wheelchairs, walkers, canes, lifts, monitors.
2. **Home Health Supplies:** Those disposable items used by home health personnel, patients, and/or family/caregivers to meet the patient's home health needs, such as sterile dressings, syringes, catheters, tubing, and gloves.
3. **Gases, Oxygen and Related Equipment:** Any equipment used to deliver the gas to the patient, such as oxygen tank and tubing.

PROCEDURE

1. Upon admission, the patient assessment will include any medical gases and/or sterile supplies to be used in the home setting, as well as the name of the home medical equipment company providing such service/equipment.
2. The selection, delivery, set-up, initial instruction, and maintenance of home medical equipment are the responsibility of the equipment company.
3. The initial home assessment will include, but will not be limited to:
 - A. Use of oxygen and oxygen-related equipment, including medical gas cylinders
 - B. The areas where medical gases are stored
 - C. The environment, including temperature where gases are stored
 - D. Use of sterile supplies related to care being provided by the organization

4. Upon admission, the patient and family/caregiver using supplies (sterile and non-sterile) will designate an appropriate area in the home for storage.
5. The patient and family/caregiver will be instructed, as applicable, on:
 - A. Storage of medical gases in a stable, ventilated, protected area
 - B. Protection from heat extremes
 - C. Safe filling of portable oxygen units
 - D. Proper handling of sterile supplies
 - E. Response to emergency situations and/or accidents in the home
 - F. Delivery of and access to supplies
6. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Adaptations made to the environment
 - C. Patient and family/caregiver understanding
 - D. Return demonstrations in use of medical equipment and/or supplies, if appropriate
 - E. Response to teaching
 - F. Additional learning needs

**IDENTIFICATION, HANDLING, AND DISPOSAL
OF HAZARDOUS WASTE
Policy No. 4-057.1****PURPOSE**

To provide guidelines for the instruction of patients and family/caregivers in the identification, handling, and disposal of hazardous materials and wastes.

POLICY

Patients will receive information and instruction regarding the identification, handling, and disposal of hazardous materials and wastes, in accordance with and applicable law regulations. (See "[Hazardous Waste Handling](#)" Policy No. 6-011, "[Contaminated Materials Disposition](#)" Policy No. 6-009, "[Contaminated Waste Disposal](#)" Policy No. 6-010, and "[Communication of Hazards to Personnel](#)" Policy No. 6-017.)

PROCEDURE

1. Upon admission, the patient assessment will include educational needs related to the identification, handling, and disposal of hazardous materials and wastes.
2. The assessment will include, but will not be limited to:
 - A. The potential need for and use of puncture-resistant needle containers
 - B. The potential need for and use of bags for soiled dressing/linens
 - C. The potential need for and use of gloves and protective clothing
3. The assessment will include the appropriate actions for both the clinician as well as the patient and family/caregiver while receiving home health services.
4. Each patient who has the potential for handling and disposing of hazardous materials will receive information on OSHA's bloodborne pathogens standards, as well as the home safety information that addresses hazardous waste in the home setting.
5. The materials should be reviewed initially with the patient and family/caregiver to assess their understanding of the actions to be taken to protect themselves. Completion of this instruction and full understanding are necessary prior to the patient and family/caregiver assuming care and performing interventions that may put them at risk.
6. On subsequent visits, clinicians will observe patient and family/caregiver performing appropriate care activities using information learned from the clinicians. Failure to perform activities according to accepted standards will result in re-instruction.

7. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Adaptations made to the environment
 - C. Patient and family/caregiver understanding
 - D. Return demonstrations in use of equipment/procedures, if appropriate
 - E. Response to teaching
 - F. Additional learning needs

INFECTION CONTROL PRECAUTIONS**Policy No. 4-058.1****PURPOSE**

To provide guidelines for the instruction of patients and family/caregivers in infection control precautions.

POLICY

Patients will receive information and instruction regarding infection control precautions, as well as teaching about preventing infection. (See "[Standard Precautions](#)" Policy No. 6-004, and "[Hand Hygiene](#)" Policy No. 6-006.)

PROCEDURE

1. Upon admission, the assessment will include patient and family/caregiver knowledge regarding infection control.
2. Based on the assessment and patient needs, the patient and family/caregiver will receive information and instruction on standard precautions, including such information as:
 - A. Hand washing
 - B. Protecting skin membranes
 - C. Use of antiseptic cleaners
 - D. Disposing of sharps in puncture-resistant containers
 - E. Breaking of needles
 - F. Food and drink in the patient area
 - G. Transmission of infections
 - H. Personal protective equipment
 - I. Cleaning and decontamination schedules, if appropriate
 - J. Handling of soiled laundry and linen
 - K. Emergency responses

3. When appropriate, the patient and family/caregiver will receive verbal and written information on standard precautions.
4. The use of standard precautions must be demonstrated by the patient and family/caregiver prior to them assuming responsibility for care.
5. Ongoing assessments should continually address the use of standard precautions.
6. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Adaptations made to the environment
 - C. Patient and family/caregiver understanding
 - D. Return demonstrations in use of equipment, if appropriate
 - E. Response to teaching
 - F. Additional learning needs

NATURAL DISASTERS/EMERGENCIES**Policy No. 4-059.1****PURPOSE**

To provide guidelines for specific instruction and information for patients and families/caregivers in relation to emergency preparedness and actions to take in the event of a natural disaster and/or emergency.

POLICY

Patients and families/caregivers will receive information on an emergency management plan during the initial visit. (See "[Guidelines for Emergency Management](#)" Addendum 4-059.A.)

PROCEDURE

1. Upon admission, the patient and family/caregiver will be assessed regarding their emergency management plan for the home, and any special needs will be noted.
2. The patient's and family/caregiver's understanding will be assessed on an ongoing basis. Instruction will be in accordance with applicable organization policies, including, but not limited to, "[Emergency Management Plan](#)" (see Policy No. 6-037.)
3. The patient and family/caregiver will be assessed and instructed regarding the components of an emergency preparedness/natural disaster plan that will include, but not be limited to:
 - A. Emergency phone access, including ambulance, police, fire, gas, electric, water
 - B. Emergency supplies, including food, water, heat, light, day-to-day necessities, and needed medical supplies
 - C. Disaster follow-up, including battery-powered access to local radio stations
 - D. Procedures to follow if care is disrupted by a natural disaster
 - E. Actions to take in preparation and during natural disasters, such as flood, storms or earthquakes
 - F. Actions to take in case of fire
 - G. Evacuation plans for the home and how to access shelters if necessary
4. Written information will be presented and reviewed, based on patient and family/caregiver knowledge, skills, and identified needs.
5. The Case Manager will document all patient and family/caregiver education information in the clinical notes. Noncompliance and/or lack of understanding will be documented, and a plan for instruction will be developed as part of the plan of care.

ADDENDUM 4-059.A

GUIDELINES FOR EMERGENCY MANAGEMENT

SEVERE WEATHER/EARTHQUAKES

1. Have emergency equipment and medical supplies readily available.
2. Close all drapes.
3. Move away from windows.
4. CLOSE exit doors.
5. Go to inside room of building with no windows, if available.
6. Do not enter damaged portions of the building until instructed.
7. Monitor weather bulletins/radio announcements.
8. Do not exit building until instructed.
9. REMAIN CALM. DO NOT PANIC.

FLOODS

(Flood warnings, alerts, or an actual flood)

Precautions before the flood:

1. Make sure emergency supplies and equipment are readily available.
2. Do not touch any electrical equipment unless it is dry.

Precautions if evacuation of building is ordered:

1. Travel only routes designated.
2. Do not try to cross a stream or other water areas unless you are sure it is safe.
3. Monitor local radio broadcast.
4. Watch for fallen trees, live wires, etc.
5. Watch for washed-out roads, earth slides, broken water lines, etc.
6. Watch for areas where rivers, lakes, or streams may flood suddenly.

After the flood:

1. Do not enter the building until an all-clear has been given.
2. Do not use any open flame devices until the building has been inspected for possible gas leaks.
3. Do not turn on any electrical equipment that may have gotten wet.
4. Shovel out mud while it is still moist.

Flash floods:

1. Remember, flash floods can happen without warning.
2. When a flash flood warning is issued, take immediate action.
3. Follow all instructions issued without delay.

SNOW EMERGENCY*(Snow emergency or winter storms)*

1. Keep a one (1) to two (2) week supply of heating fuel, food, and water on hand in case of isolation at home.
2. Keep your car properly serviced, with snow tires and filled with gas.
3. Keep emergency supplies in the car:
 - A. Container of sand
 - B. Shovel
 - C. Windshield scraper
 - D. Tow chain or rope
 - E. Flares
 - F. Blanket
 - G. Flashlight
4. Dress appropriately—wear several layers of loose, lightweight, warm clothing, mittens, and winter headgear to cover head and face.
5. Carry a cellular phone (if available).
6. Drive with all possible caution. If caught in a blizzard, seek refuge immediately. Keep car radio on for weather information
7. If your car breaks down—turn flashers on or hang a cloth from the radio aerial; stay in your car. If your car is stuck in snow or traffic jam and car is running, crack windows to prevent carbon monoxide poisoning and keep exhaust pipe free of snow. If engine is not running, you do not need to crack windows.

Additional or alternate guidelines should be included by the organization based on its hazard vulnerability analysis and subsequent plan.

(See "[Emergency Management Plan](#)" Policy No. 6-037.)

PURPOSE

To provide guidelines for the instruction of basic home safety.

POLICY

Patients will receive information regarding basic home safety in accordance with applicable organization policies including, but not limited to, "Environmental Safety—Patient" (see Policy No. 6-023) including:

1. Fire response
2. Electrical safety
3. Environmental and mobility safety
4. Bathroom safety
5. Medication safety

PROCEDURE

1. Upon admission, the patient's home will be assessed using the home safety assessment and checklist.
2. Based on the results of that assessment, the patient and family/caregiver learning needs will be identified.
3. Using the assessment, written and verbal information appropriate to the patient's environment will be used as a basis for patient instruction.
4. The information that will be reviewed with every patient will include:
 - A. Fire safety – Smoking, smoke detectors, fire escape route, burns, electric blankets, heating pads, oxygen therapy precautions, space heaters, cooking safety, flammable liquids, and storage of papers and boxes.
 - B. Electrical safety – Extension cords, electrical cords, overloaded circuits, outlets, light bulbs, grounding, and electrical appliances.

- C. Environmental and mobility safety – Fall prevention techniques, wheelchair safety, walker safety, exits/passageways, use of handrails, loose carpets, stairway safety, adequate lighting, emergency medical plan, disaster plan. (See "[Fall Reduction Program](#)" Addendum 4-060.A.)
- D. Bathroom safety – Nonskid mats, slippery surfaces, grab bars, water temperature.
- 5. During subsequent home visits, clinicians will continually assess the patient and family/caregiver compliance to home safety and re-instruct when safety issues surface.
- 6. Documentation of patient and family/caregiver instruction in the clinical record will include:
 - A. Information taught
 - B. Adaptations made to the environment
 - C. Patient and family/caregiver understanding
 - D. Return demonstrations in use of equipment, if appropriate
 - E. Response to teaching
 - F. Additional learning needs

ADDENDUM 4-060.A

FALL REDUCTION PROGRAM

(Insert Organization's Fall Reduction Program here.

Note: Guidelines for developing your organization's program included.)

Reference

<https://ncchamp.org/home>

These are suggested Fall Reduction Program components that should be included in your agency-specific program. Your program should be developed by an interdisciplinary team. This list is a guideline and is not all-inclusive.

1. Determine what fall risk assessment tool (s) will be used.
 - A. Home health: To “get credit” for performing Fall Risk Assessment on OASIS, must:
 1. Be multifactorial; example: Missouri Alliance for Home Care tool at www.homecaremissouri.org/projects/falls/index.php AND
 2. Include a validated tool; example: Timed Up and Go (TUG) or Tinetti
2. Determine the definition of what a “fall” is and how it will be reported.
 - A. Incident forms should include areas for reporting witnessed and non-witnessed falls
 - B. Incident forms should have areas for time of day the fall occurred, where the fall occurred, what the patient was doing when the fall occurred and if the fall resulted in an injury, a trip to the ER or hospital admission
 - C. Fall incidents should be tabulated, trended and shared with appropriate committees (e.g. PI, Safety committees) and staff. The data can be used for PI projects.
3. Determine the patient education information you will utilize.
 - A. Materials need to be written at an appropriate education level and culturally appropriate as determined by your patient demographics
 - B. Handouts should include information on the risks and causes of falls, home safety and modifications for fall prevention (e.g. stair railings, proper lighting, proper footwear) and ways to reduce risk
4. Develop a staff education program that includes but is not limited to:
 - A. Major risk factors for falling, such as taking certain classes of medications, vision issues, and strength and balance deficits
 - B. How to perform a fall risk assessment
 - C. How to plan and implement the correct interventions based on the assessment results
 - D. How to complete appropriate OASIS items
 - E. When to appropriately refer to other disciplines
 - F. How to complete incident reports for patient falls reporting

PATIENT EDUCATION RELATED TO DISCHARGE PLANNING**Policy No. 4-061.1****PURPOSE**

To provide guidelines for organization personnel in the planning for discharge and patient and family/caregiver education related to that discharge.

POLICY

Patient will be active participant in the planning of discharge from home care services and will be verbally informed of the date and reason for discharge. Patients will receive instructions upon discharge to facilitate self-care and health care follow-up.

(See "[Discharge Planning](#)" Policy No. 4-009.)

PROCEDURE

1. Discharge planning and instruction will begin upon admission to the organization.
2. On admission, the clinician will inform the patient and his/her family/caregiver as to the expected length of service, emphasizing the need to resume, to the extent possible, a return to a functioning level of care and activity.
3. During subsequent visits, the patient and family/caregiver should be continually assessed as to his/her readiness to be discharged from home care, including instruction needed to assure a smooth transition.
4. As part of the discharge planning process, each discipline providing care will focus patient instruction on care and service requirements when home care is no longer needed.
5. Organization personnel will assist the patient regarding his/her discharge by:
 - A. Consulting with the patient and family/caregiver regarding the need for discharge
 - B. Serving as a referral source for patient and family/caregiver in obtaining follow-up support services
 - C. Consulting with the patient and family/caregiver regarding the need for discharge instruction
6. Discharge instruction will begin prior to the last visit. Instructions may include, but will not be limited to:
 - A. Medication instruction

- B. Activities of daily living instructions
 - C. Care instructions such as wound, diet, symptom control, etc.
 - D. Contaminated waste disposal
 - E. Standard precautions
7. Documentation of patient and family/caregiver instruction in the clinical record by each discipline will include:
- A. Information taught
 - B. Adaptations made to the environment
 - C. Patient and family/caregiver understanding
 - D. Return demonstration in use of equipment, if appropriate
 - E. Response to teaching
 - F. Additional learning needs
8. The clinician discharging the patient will provide written and verbal instructions. A copy of written instructions on a discharge instruction form will be given to the patient; the original will be maintained in the clinical record.

PURPOSE

To provide guidelines for identifying and providing educational resources required to achieve learning objectives for patients and family/caregivers.

POLICY

The organization will select and provide educational resources for the patient, based on patient and family/caregiver educational needs.

PROCEDURE

1. The organization will provide educational resources that include, but are not limited to:
 - A. Members of the home health team
 - B. Written instructions developed by home health pertinent to services, treatments, medications, etc.
 - C. Written information provided by the community, as well as by regional and national companies and associations
 - D. Written information provided by the patient's hospital
 - E. The patient's physician
 - F. Community resources
2. Educational methods will be based on the patient's and family/caregiver's ability to comprehend information and individual learning styles and may include, but will not be limited to, any of the following methods of instruction:
 - A. Verbal instruction
 - B. Written instruction
 - C. Demonstration and return demonstration
 - D. Verbal demonstration
 - E. Role playing
 - F. Videos/DVDs

G. Computer CD/websites

3. When appropriate, the organization will arrange for additional education to be provided by community resources.
4. Patient and family/caregiver education will be provided in a language and at a level the patient and family/caregiver can be expected to understand, including the use of special devices, interpreters, and other aids needed to meet the patient's specialized needs.

PURPOSE

To provide guidelines for the use of community resources for patient and family/caregiver education.

POLICY

Organization personnel, as well as the patient and family/caregiver, will have access to community resources in the provision of patient education appropriate to patient and family/caregiver needs.

PROCEDURE

1. The organization will maintain a list of community resources and appropriate contacts that have patient education materials available for use in the home health setting.
2. Resources available may include, but will not be limited to:
 - A. American Diabetic Association
 - B. American Heart Association
 - C. American Lung Association
 - D. Cancer Society
 - E. Drug and pharmaceutical companies
 - F. National groups supporting diseases, such as ALS, MS, etc.
 - G. Support groups, such as Ostomy, Y-Me, etc.
3. The organization will use this information in conjunction with other materials developed by the organization.
4. Use of community and outside resources should be documented in the clinical record.

PURPOSE

To define the process and timeframes for retention of clinical, administrative, financial and personnel records.

POLICY

The clinical record will be retained for seven (7) years from the month in which the organization's cost report to which the record(s) applies is filed with the intermediary, or seven (7) years after the end of the fiscal year to which the record(s) applies, unless state law stipulates longer. For Medicare managed care, patient records will be retained for ten (10) years. For minors, the seven (7) year retention requirement begins upon reaching the age of 18, unless state law stipulates longer. The organization shall retain the emergency management plan for at least four (4) years.

Administrative, financial and personnel records will be retained in a manner that allows each to be easily retrievable and in keeping with timeframes of applicable local, state and federal law. All billing records will be retained for a minimum of five (5) years past the month of filing the applicable cost report or until the cost report is settled unless state law stipulates a longer period of time. Governing Body meeting minutes will be retained for a minimum of five (5) years. HIPAA Security audits/logs and compliance hotline reports/investigations will be retained for a minimum of six (6) years.

PROCEDURE

Clinical Records

1. All active (open) paper records will be filed alphabetically and stored in a secure area with access by approved personnel only. Electronic medical records will be stored in a secure manner that is easily retrievable if records are requested.
2. When a patient is discharged/expires:
 - A. Paper records
 1. Original documents will be submitted to the appropriate department
 2. Documents will be reviewed for completeness
 3. The paper record will be closed and filed alphabetically with other discharged records within 30 days of discharge and be stored in a secure area with access by approved personnel only.
 4. Closed (discharged) records will be stored on premises for two (2) years. After two (2) years, the records will be properly labeled, boxed and stored off site in a secure storage facility.

B. Electronic records

1. Discharged records will be closed, reviewed and stored in accordance with the software workflow within 30 days of discharge.
3. The clinical record will remain the property of the organization.
4. The organization will keep all records of cases involved in litigation until the case is concluded, even if it goes beyond the retention time period prescribed by law.
5. In the event the organization should cease operation, all clinical records, administrative and financial files will be sent to and stored at another designated location and the appropriate legal authorities will be notified of the record disposition.

Administrative, Financial and Personnel Records

1. Applicable local, state and federal regulations regarding record retention will be reviewed on an ongoing basis.
2. A review of administrative, financial and personnel records produced by Organization's Name will be completed to determine which records meet criteria for inclusion in local, state and federal regulations. A directory of organization-specific reports and their respective retention times may be maintained by Organization's Name.
3. Records may be retained in paper form or as a part of the organization's computer back-up system so long as they are easily retrievable.
4. Paper records will be stored by type of record first and record date second in a secure area with access by authorized personnel only.
5. Those records identified to meet regulatory criteria will be maintained at the organization's primary site for three (3) years and after three (3) years the records will be properly labeled, boxed and stored off site in a secure storage facility.

If ownership changes:

1. Both the licensee and the applicant for the new license shall, prior to the change in ownership, provide the Department with written documentation that:
 - A. The new licensee will have custody of the patient's health records upon transfer of the agency and the health records are available to both the new and former licensee and other authorized persons; or
 - B. If the agency stores records in an off-site location, the following requirements shall apply
 1. Timely accessibility of stored records on a 24 hour basis, seven days a week
 2. Records are organized and systematically maintained

3. Protection of the clinical records from loss, destruction or unauthorized use
4. A current written agreement with storage facility
5. Policies and procedures which address the retention, retrieval and security for off-site centralized storage of inactive patient records

Destruction of Records (Intentional)

1. In the absence of an Investigation, Litigation, Cost Report, or Legal Hold, records may be destroyed upon the termination of the applicable mandatory retention period.
2. The appropriate method of destruction depends on the record's physical form or medium and subject matter or content.
3. Records that include Protected Health Information (PHI) or patient-specific identifiers will be destroyed or disposed of so that the personal data cannot practically be read or reconstructed.
4. Paper records will be redacted, burned, pulverized or shredded and electronic records will be destroyed or erased.
5. Absent any special instructions or unique circumstances, records generally will be destroyed at the end of their retention period; retaining any record past its mandatory retention period should be on an exceptions-only basis after weighing the potential usefulness of the record against cost or space limitations.

Destruction of Records (Unintentional)

1. Records destroyed unintentionally due to fire, natural disaster or human error, etc. will be recovered by means appropriate to the medium that they were stored.
2. Electronic records will be recovered by accessing the back-up system.
3. Paper records will be recreated to the extent possible by using available source and organization documents.

ASSEMBLY OF CLINICAL RECORD**Policy No. 4-065.1****PURPOSE**

To establish a consistent, organized structure for the clinical record.

POLICY

All clinical records will be assembled and maintained according to policy and applicable state and federal law. The structure will be defined and approved by the Management of Information Committee. Any changes must be approved by that committee.

(See "[Contents of Clinical Record](#)" Policy No. 4-047.)

PROCEDURE

1. The contents of the record will be assembled as follows:

(If utilizing electronic clinical records, the following information would need to be retrievable.)

Section I: Patient Data

- A. Referral and intake forms
- B. Insurance verification forms
- C. Family/caregiver resource data form
- D. Transfer forms from other organization
- E. Signed notice of patient rights
- F. Consent for treatment

Section II: Physician Orders

- A. Plan of care/treatment
- B. Verbal orders
- C. Pharmaceutical orders

Section III: Clinical Notes

- A. Case Conference Forms
- B. Communication notes
- C. 486 (if requested)

Section IV: Plan of Care

- A. Plan of care (485)
- B. Face-to-Face Encounter documentation
- C. Medication profile
- D. Home Health Aide assignment

Section V: Nurses Clinical Notes

- A. Initial, comprehensive and ongoing comprehensive assessments
- B. Clinical notes

Section VI: Home Health Aide Notes

- A. Home Health Aide clinical notes
- B. Supervisory visit notes

Section VII: Social Service Notes

- A. Initial assessment and ongoing assessments
- B. Social service notes

Section VIII: Therapy/Rehabilitation/Nutrition Notes

- A. Initial assessment and ongoing assessments
- B. PT, PTA, OT, COTA, SLP, and RD notes

Section IX: Lab and Special Reports

- A. Lab results

Section X: Advance Directives

- A. Living will
- B. Durable Power of Attorney for health care
- C. DNR/DNI

Section XI: Miscellaneous

- A. Communication with insurers, other clinicians, etc.
 - B. HME and supply documentation
2. Documentation will be assembled in chronological order, most recent first.
 3. Documentation will include that which is provided directly by home health and by contracted personnel.
 4. All personnel, including contracted personnel, will only use organization-approved forms.
 5. The clinical record for patient receiving only personal care and/or environmental support/chore services will be set up in the same manner as the skilled record, including the documentation required for these services.

INTAKE PROCESS**Policy No. 4-066.1****PURPOSE**

To establish the process for acceptance and entry of patient into the organization.

POLICY

Referrals will be accepted 24 hours a day, seven (7) days per week. Personnel will be available 24 hours a day to accept patients into home care service.

The organization accepts only those patients whose needs can be met by the services it provides.

PROCEDURE

1. Home health care referrals will be documented on a referral and intake form.
2. Patient referrals for home care services may be accepted by clinicians, including the director, supervisor, program coordinators, and nurses.
3. Referral information may be accepted by any of the following methods:
 - A. Telephone
 - B. Fax
 - C. Written order
 - D. Secure email
4. Referrals may be accepted from any of the following:
 - A. Doctors of Medicine, Osteopathy, Podiatry, or Dental Surgery, as well as Psychiatrists and Dentists
 - B. Discharge planners from inpatient and outpatient services
 - C. Social service agencies
 - D. Individual patients or their family/caregiver(s)
 - E. Case Managers and/or insurance company representative
 - F. Other home care organizations

5. During scheduled business hours (office hours are from 8:00 a.m. to 5:00 p.m., Monday through Friday), calls will first be received by the receptionist. Patient referral calls will be transferred to a staff member designated to accept referrals.
 - A. To accept referrals, information regarding a patient's demographics, diagnosis, services needed, medications, attending physician, and hospitalization, will be taken in order to make the initial determination of whether the patient's needs can be met and if he/she meets the eligibility criteria. (See "[Admission Criteria and Process](#)" Policy No. 2-003.) The information is reviewed for completeness.
 - B. When the payer source is private insurance, the insurance coverage will be verified and an insurance information form completed.
 - C. Intake information is given to the Clinical Supervisor to accept the referral information and complete a referral and intake form.
 - D. If the referral is not from a physician, the patient's physician (or allowed practitioner) will be contacted to confirm service needs and to obtain verbal orders.
 - E. The Clinical Supervisor will assign personnel and schedule an initial assessment visit. The initial assessment visit will be performed either within 48 hours of the referral, or within 48 hours of a patient's return home, or on the start of care date ordered by the physician (or allowed practitioner).
 - F. If service cannot be provided, intake personnel will give the caller the names of other agencies that can provide the required services. A log will be maintained on all patients that cannot be serviced.
6. After scheduled business hours (weekends and evenings) the organization can be accessed through the answering service.
 - A. The answering service will contact the nurse on-call via telephone or beeper.
 - B. The on-call nurse will complete the initial intake information from the referral source and relay the information as follows:
 1. If the referral is on Saturday or Sunday, the on-call nurse will determine if the patient needs to be seen, or if he/she can wait until Monday morning for follow-up by the Clinical Supervisor.
 2. If the patient must be seen, the on-call nurse schedule an initial visit.
 3. If the patient can wait until Monday, the on-call nurse will bring the intake information into the office on Monday morning for scheduling by the Clinical Supervisor.

CARE/SERVICE COORDINATION**Policy No. 4-067.1****PURPOSE**

To ensure the coordination of services for each patient and to minimize the potential for missed, conflicting, or duplicated services.

POLICY

Each patient will be assigned a Case Manager upon admission by the Clinical Director or Clinical Supervisor. It will be the responsibility of the Case Manager to facilitate communication about changes in the patient status among assigned personnel.

Timely and ongoing communication is the responsibility of each team member of the interdisciplinary group and will be appropriate to the needs and abilities of the patient, and relevant to the care/service provided. The clinician will be responsible for facilitating communications about changes in the patient's status among the assigned personnel.

PROCEDURE

1. The program supervisor will assign the patient to a Case Manager based on the patient's need and level of care required, geographic area, and qualifications of organization personnel needed.
 - A. A registered nurse will be assigned to a patient receiving skilled nursing.
 - B. A physical therapist or speech therapist will be assigned to a patient receiving physical or speech therapy only.
 - C. An occupational therapist may be assigned to a patient after the case has been opened by a registered nurse, physical therapist, or speech therapist.
2. The assigned clinician will be qualified through education, training, and/or experience and will:
 - A. Understand the principles of care/service provided
 - B. Know the required qualifications for organization personnel providing care/service and know which organization personnel possess these qualifications
 - C. Know the scope of care/service that can be provided by various organization personnel
 - D. Understand the nature of the patient population served
3. The name of the Case Manager will be clearly documented in the clinical record.

4. Care coordination will include, but not be limited to:
 - A. Clearly outlining each clinician's responsibilities to avoid duplication of services
 - B. Developing complementary actions and goals
 - C. Regularly occurring telephone or e-mail communication among team members
 - D. Planned face-to-face conferences
 - E. Timely documentation of coordination of care activities
 - F. Appropriate involvement of the patient and caregivers.
 - G. Limiting the number of personnel assigned to a case, when possible
 - H. Proper communication upon "hand-off" of patient, including the opportunity for questions between the giver and receiver of patient information
 - I. Integration of services, whether services are provided directly or under arrangement, to assure the identification of patient needs and factors that could affect patient safety and treatment effectiveness and the coordination of care provided by all disciplines.
 - J. Coordination of care delivery to meet the patient's needs, and involve the patient, representative, and caregivers, as appropriate, in the coordination of care activities.
 - K. Ensuring that each patient, and their caregiver(s), where applicable, receive ongoing education and training provided by the organization regarding the care and services identified in the plan of care. The organization shall provide training, as necessary, to ensure a timely discharge.
5. It will be the responsibility of the primary clinician to facilitate communication about changes in the patient's status among all assigned disciplines.
6. Organization personnel will communicate changes in a timely manner via telephone, one-on-one meetings, case conferences, and home visits. Documentation of all communications will be included in the clinical record on a communication note, case conference summary, or clinical note. Documentation will include: the date and time of the communication, individuals involved with the communication, information discussed, and the outcome of the communication.
7. When the patient requires more than one (1) service from the organization, the Case Manager will be responsible for cooperative care planning in order to assure that goals, actions, and the interrelationship of services is not duplicated and to minimize the potential for missed or conflicting services.
8. Written evidence of care coordination may be found in the plan of care/service, case conference summary forms, clinical notes in the patient's clinical record or interdisciplinary group meeting notes.

9. All organization personnel involved in patient care/service, including those providing contracted services, will have access to the plan of care/service and all other relevant patient information to ensure coordination and continuity. All personnel will be knowledgeable regarding patient needs, goals of care, and services.
10. Contract organization personnel will participate in preparation of the plan of care/service; submit weekly documentation of services provided including clinical notes, schedule of visits, and patient evaluations/assessments; and participate in multidisciplinary (interdisciplinary group) case conferences when a patient in their caseload is being discussed.

PURPOSE

To define acceptable and unacceptable standard abbreviations and symbols for use in clinical/service documentation.

POLICY

Organization's Name personnel will use standard approved abbreviations and symbols in documenting in the clinical/service record.

GUIDELINES

1. Home care personnel will use the approved home care and service abbreviations when documenting in the clinical/service record. (See "[Approved Home Care/Service Abbreviations](#)" Addendum 4-068.A.)
2. Home care personnel will not use unapproved home care and service abbreviations when documenting in the clinical/service record. (See "[Unacceptable Home Care/Service Abbreviations](#)" Addendum 4-068.B.)
3. Additional abbreviations and symbols may be added to the standard list if approved by the Executive Director/Administrator.

ADDENDUM 4-068.A

APPROVED HOME CARE/SERVICE ABBREVIATIONS

(Insert agency "Approved List" if developed)

APPROVED ABBREVIATIONS

Ä	before	AVSS	afebrile, vital signs stable
A	abortus	ax	axillary
	of each	Ba	barium
AAA	abdominal aortic aneurysm	B	bilateral
abd	abdomen	bil	bilateral
Abd	abduction	BDAE	Boston Diagnostic Aphasia Examination
ABG	arterial blood gas	BE.	barium enema
ac	before meals	bid	twice a day
ACL	anterior cruciate ligament	biw	twice a week
AD	right ear	B/K	below knee
ADA	American Diabetic Association	BKA	below knee amputation
Add	adduction	BLE	bilateral lower extremities
ADL	activities of daily living	BM	bowel movement
ad lib	as desired	BP	blood pressure
A/E	above elbow	BPH	benign prostatic hypertrophy
AE	as evidenced by	BR	bed rest
AF	atrial fibrillation	BRP	bathroom privileges
A-F Bypass	Aorta-Femoral Bypass	BS	blood sugar
AFB	acid fast bacillus	BSC	bedside commode
AFC	adult foster care	BSO	bilateral salpingo-oophorectomy
AFO	ankle foot orthosis	BT	brief therapy
AG	anti-gravity	BUE	both upper extremities
AIDS	acquired immune deficiency syndrome	BUN	blood urea nitrogen
A/K	above knee	bx	biopsy
AKA	above knee amputation		with
alb	albumin	C	centigrade
Alk phos	alkaline phosphatase	C-1, C-2	first cervical vertebra, etc.
ALPS	aphasia language performance scale	ca	calcium
AM	morning	CA	carcinoma
AMA	against medical advice	CABG	coronary artery bypass graft
amb	ambulatory	CAD	coronary artery disease
AMI	acute myocardial infarction	cal	calorie
amp	ampule	caps	capsule
amt	amount	CAT	computed axial tomography
Ant	anterior	cath	catheterize
A/O	alert and oriented	CBC	complete blood count
a&p	anterior/posterior	CBI	continuous bladder irrigation
A&P	auscultation and percussion	cc	cubic centimeter
AP	apical pulse	CC	chief complaint
APS	adult protective services	cert	certificate/certification
approx	approximate	CF	cystic fibrosis
appt	appointment	CGA	contact guard assist
Aq	water	CHD	coronary heart disease
ARDS	adult respiratory distress syndrome	CHF	congestive heart failure
ARE	active resisted exercises	CHHA	certified home health aide
arom	active range of motion	CHI	closed head injury
ASA	acetylsalicylic acid	CHO	carbohydrate
ASAP	as soon as possible	Chol	cholesterol
ASD	atrial septal defect	Chole	cholecystectomy
ASHD	arteriosclerotic heart disease	circ	circulation
asst	assistance	Circ	circumcision
AV node	atrioventricular node	CK	creatinine kinase
avg	average	Cl	chloride

APPROVED ABBREVIATIONS

Clt	client	E. coli	Escherichia coli
cm	centimeter	ECT	electroconvulsive therapy
CNS	central nervous system	EEG	electroencephalogram
CO	cardiac output	EENT	eyes, ears, nose and throat
CO ₂	carbon dioxide	e.g.	for example
coag	coagulation	EKG	electrocardiogram
c/o	complaint of	elix	elixir
conc	concentration	EMB	electromyography
cont	continuously	EMS	electrical muscle stimulation
COPD	chronic obstructive pulmonary disease	EOM	extraocular movement
cor	heart (Latin)	Eq	equivalent
CP	chest pain	ER	Emergency Room
cp	cerebral palsy	ESR	eosinophilic sedimentation rate
CPG	creatinine phosphatase	est	estimated
CPR	cardiopulmonary resuscitation	etiol	etiology
creat	creatinine	ETOH	ethanol
CRP	community resources planning	ext	extension
C&S	culture and sensitivity	ext rot	external rotation
C-section	cesarean section	exp	expired
CSF	cerebrospinal fluid	F	Fahrenheit
CTA	clear to auscultation	f	Flexion
CV	cardiovascular	FBS	fasting blood sugar
CVA	cerebrovascular accident	Fe	iron
CVD	cardiovascular disease	Fem/Pop	femoral-popliteal
CVP	central venous pressure	FH	family history
cx	cervix	fld	fluid
CXR	chest x-ray	flex	flexion
DC	discontinue	FOB	foot of bed
D&C	dilatation and curettage	freq	frequent
DDD	degenerative disc disease	FTT	failure to thrive
DIC	Disseminated Intravascular Coagulopathy	FU	follow-up
diff	differential	FUO	fever of unknown origin
dist	distention	FWB	full weightbearing
DJD	degenerative joint disease	fx	fracture
DM	diabetes mellitus	GB	gallbladder
DNR	do not resuscitate	GC	Gonococcus
DOA	dead on arrival	GE	gravity eliminated
DOB	date of birth	GFPT	good/fair/poor/trace
DOE	dyspnea on exertion	GI	gastrointestinal
dr	dram	GLC	glaucoma
drng	drainage	gluc	glucose
drsg	dressing	Gm	gram
DS	Dorsi Flexion	gr	gram
dschg	discharge	gt	gait
DSD	dry sterile dressing	gtt	drops
DSS	Department of Social Services	GTT	glucose tolerance test
DTR	deep tendon reflexes	GU	genitourinary
DVT	deep venous thrombosis	GYN	gynecology
D/W	dextrose and water	h	hour
D5W	dextrose and 5% water	HA	headache
dx	diagnosis	hams	hamstring
ea	each	Hcl	hydrochloric acid
ECF	extended care facility	HCO ₃	bicarbonate concentrate
ECG	electrocardiogram	Hct	hematocrit

APPROVED ABBREVIATIONS

HEP	home exercise program	lat	lateral
HEENT	head, eyes, ears, nose and throat	lb	pound
Hgb	hemoglobin	LBBB	left bundle branch block
HG	home glucose monitoring	LD	left deltoid
H&H	hemoglobin and hematocrit	LCL	lateral collateral ligament
HHA	home health aide	LE	lower extremity
HHN	hand held nebulizer	lg	large
HKAFO	hip, knee, ankle, foot, orthosis	LGM	left gluteus maximus
HIV	human immunovirus	lig	ligament
HMD	hyaline membrane disease	liq	liquid
HOB	head of bed	LLE	left lower extremity
H&P	history and physical	LLL	left lower lobe
HR	heart rate	LLO	long leg orthosis
hs	hours of sleep	LLQ	left lower quadrant
ht	height	LMP	last menstrual period
HTN	hypertension	LNMP	last normal menstrual period
HV	home visit	Lpm	liters per minute
hx	history	LP	lumbar puncture
H2O	water	LRPD	long range planning & decision-making
H2O2	hydrogen peroxide	LS	lumbosacral
HPI	history of present illness	lt	light
I&D	incision and drainage	LTM	long-term memory
IDDM	insulin dependent diabetes	LTR	lateral tongue response
i.e.	for example	LOB	loss of balance
IM	intramuscular	LOC	level of consciousness
inc	incomplete	loc	loss of consciousness
inf	inferior	LUE	left upper extremity
int	intermittent	LUQ	left upper quadrant
Int Rot	internal rotation	LVG	left ventro gluteal
I&O	intake and output	LVH	left ventricular hypertrophy
IOP	intraocular pressure	LVL	left vastus lateralis
IPPB	intermittent positive pressure breathing	lymphs	lymphocytes
IR	internal rotation	lyte	electrolytes
IR	inferior rectus	M	male
IS	incentive spirometry	MA	Medicaid
I/S	instruct and supervise	max	maximum
IV	intravenous	MBS	modified barium swallow
IVPB	intravenous piggyback	m/care	Medicare
JP	Jackson Pratt	mcaid	Medicaid
jt	joint	MCL	medial collateral ligament
jug	jugular	MCP	metacarpophalangeal joint
JVD	jugular venous distention	me	mistaken entry
k	potassium	meds	medications
KCL	potassium chloride	men	meniscectomy
Kg	kilogram	mEq	milliequivalent
KO	keep open	Mg	magnesium
KOR	keep open rate	mg	milligram
L	lumbar	mg%	milligram percent
L-1, L-2	first lumbar vertebra, etc.	MH	mental health
l	liter	M/H	moist heat
Lab	laboratory	MI	myocardial infarction
lac	laceration	mim	minimum
LAD	left anterior descending	min	minute
Lap	laparotomy	ml	milliliter

APPROVED ABBREVIATIONS

mm	millimeter	OU	both eyes
mod	moderate	oz	ounce
MOM	Milk of Magnesia	p	pulse
MOW	Meals on Wheels	P	plan
MP	metatarsophalangeal	P&A	percussion and auscultation
MR	mental retardation	PA	pulmonary artery
MRE	manual resisted exercises	PAC	premature atrial contraction
m.s.	morphine sulfate	path	pathology
M.S.	multiple sclerosis	pc	after meals
MSS	Medical Social Service	PCA	patient controlled analgesia
MTP	metatarsophalangeal joint	PCL	posterior cruciate ligament
MV	minute volume	PCTA	percutaneous transluminal angioplasty
MVA	motor vehicle accident	PCxR	portable chest x-ray
MVV	maximum voluntary ventilation	PE	pulmonary embolus
N	normal	Peds	pediatrics
Na	sodium	PCN	penicillin
N/A	not applicable	per	by
NaCl	sodium chloride	PERRLA	pupils equal, round, regular, and react to light and accommodation
NAS	no added salt	PEARL	pupils equal and reactive to light
NB	newborn	PF	Plantar Flexion
NC	nasal cannula	PH	past history
N/C	no complaint	pH	hydrogen ion concentrate
neg	negative	PI	present illness
NG	nasogastric	PID	pelvic inflammatory disease
NG tube	nasogastric feeding tube	PIP	proximal interphalangeal joint
NIDDM	non-insulin dependent diabetes	PKU	phenylketonuria
NKA	no known allergies	PM	evening
nsq	nursing	PMH	past medical history
NH	nursing home	PMI	point of maximum impulse
noc	night	PMR	pacemaker rhythm
NPC	non-productive cough	PND	paroxysmal nocturnal dyspnea
NPO	nothing by mouth	PNP	peak negative pressure
NS	normal saline	po	by mouth
NSR	normal sinus rhythm	PO	phone order
N&T	nose and throat	POC	plan of care
N&V	nausea and vomiting	post-op	following operation
NVD	neck vein distention	PP	postpartum
NWB	nonweightbearing	PPBS	post-prandial blood sugar
O	objective	PPDR	preproliferative diabetic retinopathy
O2	oxygen	PRE	passive/progressive resistive exercise
OA	osteoarthritis	PR	pupillary reflexes
OB	obstetrics	PR	public relations
OB-GYN	obstetrics and gynecology	preg	pregnancy
OBS	organic brain syndrome	pre-op	before operation
OD	right eye	prep	preparation
O/E	observe and evaluate	primip	primiparous
oint	ointment	PROM	Passive Range of Motion
OOB	out of bed	prn	as needed
oph	ophthalmology	pro-time	prothrombin time
O&P	ova and parasite	prog	prognosis
ORIF	open reduction and internal fixation	PSIS	posterior superior iliac spine
ortho	orthopedic(s)	psych	psychiatric
OS	left eye	PT	prothrombin time
OT	occupational therapy		

APPROVED ABBREVIATIONS

P.T.	physical therapy	S-A	sinoatrial node
pt	patient	SA	skilled assessment
PTT	partial thromboplastin time	SAS	Sklar Aphasic Scale
PUD	peptic ulcer disease	SAQ	short arc quads
PVB	premature ventricular beat	sat	saturation
PVC	premature ventricular contraction	SB	sinus bradycardia
PVD	peripheral vascular disease	SBA	standby assistance
PVT	private	SBE	subacute bacterial endocarditis
PWB	partial weightbearing	SBGM	self blood glucose monitoring
q	every	SC	subcutaneous
qam	every morning	SCM	sternocleidomastoid
qd	every day	scop	scopolamine
qh	every hour	sed rate	sedimentation rate
qhs	every night	seg	segmented
qid	four (4) times a day	SGOT	serum glutamic pyruvic transaminase
qns	quantity not sufficient	Sh	social history
qpm	every evening	SLP	speech language pathology/pathologist
qod	every other day	SLR	straight leg raising
qs	quantity sufficient	sn	skilled nursing
QS	quad sets	snf	skilled nursing facility
R	respirations	SO	significant other
RA	right atrial	SOB	shortness of breath
R.A.	rheumatoid arthritis	sol	solution
RBBB	right bundle branch block	s/p	surgical procedure
RBC	red blood cell	SP	speech
RD	right deltoid	SP Gr	specific gravity
re	regarding	SQ	subcutaneous
recert	recertification	SR	side rail
rehab	rehabilitation	SS	Social Security
REM	rapid eye movement	S&S	signs and symptoms
RGM	right gluteus maximus	SSD	Social Security Disability
RHR	resting heart rate	SSE	soapsuds enema
RLE	right lower extremity	SSI	supplemental security income
RLL	right lower lobe	Staph	Staphylococcus
RLQ	right lower quadrant	Stat	at once
RML	right middle lobe	STD	sexually transmitted disease
R/O	rule out	Strep	Streptococcus
ROM	range of motion	STm	short-term memory
ROS	review of systems	STT	short-term therapy
RS	review of systems	subclv	subclavian
R/T	related to	subq	subcutaneous
RTW	return to work	subj	subjective
RUE	right upper extremity	supp	suppository
RUL	right upper lobe	sup	supine
RUQ	right upper quadrant	sx	symptoms
RV	right ventricle	T	temperature
RVG	right ventro gluteal	T-1, T-2	first thoracic vertebra, etc.
RVL	right vastus lateralis	T&A	tonsillectomy and adenoidectomy
rx	take or treatment	tab	tablet
RX	prescription	TAH	total abdominal hysterectomy
	without	TARA	total articular replacement arthroplasty
S	sacral	TB	tuberculosis
S-1, S-2	first sacral vertebra, etc.	TBI	traumatic brain injury
S1, S2	first and second heart sounds	tblsp	tablespoon

APPROVED ABBREVIATIONS

TE fistula	trachea esophageal fistula	W/C	wheelchair
tech	technique	WFL	within functional limits
TENS	transcutaneous electric nerve stimulation	WMC	warm moist compress
TDWB	touch down/weightbearing	WNL	within normal limits
THR	total hip replacement	WP	whirlpool
TIA	transient ischemic attack	W/U	workup
tid	three (3) times a day	wt	weight
tint	tincture	x	exercise
tiw	three (3) times a week	XT	exotropia
TKR	total knee replacement	x's	times
TLC	total lung capacity	yo	year-old
TM	tympanic membrane	yoa	years of age
TO	telephone order	yr	year
TPR	temperature, pulse, respirations		
tr	trace		
trig	triglycerides		
tsp	teaspoon		
TTWB	toe touch/weightbearing		
TUR	transurethral resection		
TURB	transurethral resection of the bladder		
TURP	transurethral resection of the prostate		
TWE	tap water enema		
tx	traction		
Tx	therapy		
UA	urinalysis		
UE	upper extremity		
ung/oint	ointment		
unilat	unilateral		
UP	universal precautions		
URI	upper respiratory infection		
US	ultrasound		
US/EMS	ultrasound with electric muscle stimulation		
USA	ultrasound of the abdomen		
USN	ultrasonic nebulizer		
UTI	urinary tract infection		
UV	ultraviolet		
VA	Veterans Administration		
vag	vagina		
Vcc	visual acuity with correction		
VD	venereal disease		
VF	ventricular fibrillation		
vit	vitamin		
VO	verbal order		
vol	volume		
VQ	verbal que		
VS	vital signs		
VSD	Ventricular Septal Defect		
VSS	vital signs stable		
VTM	vertical tongue movement		
WAB	Western Aphasic Battery		
WB	weight bearing		
WBAT	weight bearing as tolerated		
WBC	white blood count		

SYMBOLS

(A)	axillary (temperature)		after
(R)	rectal (temperature)	"	minutes
M	murmur	↑	increased, elevated
R	right	↓	decreased, depressed
L	left	i	one (1)
24°	24 hours	ii	two (2)
=	equal	"✓"	check
Δ	change	♂	male
=	equivalent	♀	female
	with		parallel bars
	without	∴	therefore
	before		
		≅	roughly equal to, about, or approximately

ADDENDUM 4-068.B

UNACCEPTABLE HOME CARE/SERVICE ABBREVIATIONS

(Insert list of agency's official "Do Not Use" list.)

There is a website for the error prone abbreviations list. It is:

www.ismp.org/tools/errorproneabbreviations.pdf

UNACCEPTABLE HOME CARE ABBREVIATIONS

The following abbreviations have been identified as error prone abbreviations and should not be used:

U, u	unit	"x".0 mg	trailing zero in medication use
IU	international unit	. "X" mg	lack of leading zero in medication use
Q.D., QD, q.d., qd	once daily	MS, MSO ₄ , MgSO ₄	morphine sulfate
Q.O.D, QOD, qod, q.o.d.	every other day	MS, MSO ₄ , MgSO ₄	magnesium sulfate

Additional abbreviations, symbols, and acronyms identified to be considered as prohibited or unacceptable include the following:

ug	micrograms	D/C	discharge
H.S.	half strength or bed-time	c.c.	cubic centimeter
T.I.W.	three times a week	A.S., A.D., A.U.	left, right or both ears
S.C. or S.Q.	subcutaneous		

Additional Abbreviations, Acronyms and Symbols and Dose Designations (To be considered for inclusion in the agency's Official "Do Not Use" List):

Do Not Use	Potential Problem	Use Instead
> (greater than) < (less than)	Misinterpreted as the number "7" (seven) or the letter "L" confused for one another	Write "greater than" Write "less than"
Abbreviations for drug names	Misinterpreted due to similar abbreviations for multiple drugs	Write drug names in full
Apothecary units	Unfamiliar to many practitioners, confused with metric units	Use metric units
@	Mistaken for the number "2" (two)	Write "at"
cc	Mistaken for U (units) when poorly written	Write "ml" or "milliliters"
µg	Mistaken for mg (milligrams) resulting in one thousand-fold overdose	Write "mcg" or micrograms

The organization should also determine whether there are additional abbreviations, acronyms, symbols and dose designations that should not be used in the organization and added to this list.

Additional resources for dangerous abbreviations, acronyms, symbols and dose designations can be found here: www.ismp.org/Tools/errorproneabbreviations.pdf

MISSED VISITS
Policy No. 4-069.1**PURPOSE**

To ensure that missed visits are documented and does not affect the quality of patient care.

POLICY

Missed visits will be communicated to the clinical supervisor and the patient's physician. A missed visit will be rescheduled the same week if possible. Missed visits will be documented in the clinical record.

PROCEDURE

1. If a visit is missed for any reason, the clinician should attempt to reschedule it for the same week so that the physician ordered frequency is maintained and would not be considered a missed visit.
2. If a visit is missed and not rescheduled the clinician will:
 - A. Notify the physician and clinical supervisor of the missed visit and reason for missed visit
 - B. Document in the patient's clinical record the following information:
 1. Patient name
 2. Date and type of visit that was missed
 3. Reason for the missed visit
 4. Description of any unmet needs and how the patient's needs were met
 5. Physician notification including date, time, physician name, method of notification and the staff reporting the information
 6. Other person(s) that were notified of the missed visit
 7. Any other follow-up needed
 8. Signature of staff member reporting the information
3. If an aide visit is missed the aide will contact the case manager or clinical supervisor and the manager/supervisor will take the appropriate measures stated above.

4. The clinical supervisor will track the number and reasons for missed visits to assess patterns and to assure that the plans of care contain the correct number of planned visits.
5. If a patient is routinely refusing visits, the clinician and clinical supervisor should evaluate the appropriateness of the plan of care and make adjustments to the plan of care with input from the patient and physician.

SECTION FIVE

Quality Outcomes/Performance Improvement

Policy No.

Improving Organizational Performance

5-001

Addendum: Prioritization of Important Processes*

5-001.A

Addendum: Sample QAPI Plan

5-001.B

Patient Focused Performance Improvement

5-002

Measuring Performance of the Environmental Safety Program

5-003

Annual Organization Evaluation

5-004

Incident Reporting

5-005

Addendum: Examples of Specific Events or Occurrences That Must Be Reported

5-005.A

Serious Adverse Events

5-006

Root Cause Analysis/Action Plan

5-007

Addendum: Root Cause Analysis/Action Plan Form

5-007.A

Aggregation of Data/Information

5-008

External Databases

5-009

Patient and Family/Caregiver Experience of Care Survey

5-010

Addendum: Policy for Patient and Family/Caregiver Experience of Care Survey for Exempt and Non-Participating Home Health Agency

5-010.A

IMPROVING ORGANIZATIONAL PERFORMANCE**Policy No. 5-001.1****PURPOSE**

To establish patient outcomes as the primary focus of the organization's performance improvement activities.

To improve organizational performance by focusing on high risk, high volume, and problem-prone areas

POLICY

Senior management as designated by the Governing Body will have the responsibility of quality management: to guide the organization's efforts in improving organizational performance; to define expectations of the performance improvement activities; and to generate the plan and processes the organization will utilize to assess, improve and maintain quality of care and service including appropriateness and effectiveness of patient services.

Performance improvement results will be utilized to address problem issues, improve the quality of care and patient safety, and will be incorporated into program planning and process design and modifications.

All personnel will be active participants in the organization's performance improvement activities.

The Governing Body is responsible for ensuring that the performance improvement program is defined, implemented and maintained, and is evaluated annually.

PROCEDURE

1. Senior management with Governing Body oversight will:
 - A. Participate in educational activities to increase their level of understanding and ability to implement performance improvement activities. The educational activities may include: seminars, consultations, periodicals, and review of available information from other organizations (benchmarking).
 - B. Set expectations for performance improvement and manage processes to improve organization performance.
 - C. Focus on high risk, high volume, and problem-prone areas. Consider incidence, prevalence, and severity of problems in those areas and lead to an immediate correction of any identified problem that directly or potentially threaten the health and safety of patients

- D. Adopt a structured framework for performance improvement. The problem solving approach will stress the interrelationship of quality services provided, management activities, and sound business practices as applicable to the organization's:
1. Mission/philosophy
 2. Culture
 3. Strategic objectives
 4. Resources
 5. Operational components/responsibilities (financial, clinical/service, and personnel)
 6. Practice Standards
 7. Activities related to patient care and patient safety focusing on high risk, high volume and problem prone areas
 8. Clinical/service skills and competencies of personnel
 9. Quality indicators
 10. Data collection and analysis
 11. Frequency of activities
- E. Identify and set specific outcomes for measurable improvement and acceptable limits for findings. (See "[Prioritization of Important Processes](#)" Addendum 5-001.A.)
1. At least one important aspect related to care must be monitored through use of OASIS data
 2. At least one important administrative aspect of function or service care must be monitored
 3. Monitor indicators related to improved outcomes including the use of emergent care services, hospital admissions and re-admissions, and takes actions that address performance across the spectrum of care, including the prevention and reduction of medical errors
 4. Patient satisfaction surveys will be monitored (See "[Patient and Family/Caregiver Experience of Care Survey](#)" Policy No. 5-010.)
- F. Identify and participate in benchmarking activities that utilize:
1. Internal standards:
 - a. Measuring current performance against past performance

- b. Measuring against internally established goals
- 2. Processes and protocols
- 3. Practice or service guidelines
- 4. Industry research or best practices
- G. Ensure that the organization adheres to the CMS Conditions of Participation for Quality Assessment and Performance Improvement (QAPI) standards.
- H. Ensure that performance improvement projects are conducted that reflect the following standards:
 - 1. The number and scope of distinct improvement projects conducted annually must reflect the scope, complexity, and past performance of the organization's services and operations.
 - 2. The organization must document the quality improvement projects undertaken, the reasons for conducting these projects, and the measurable progress achieved on these projects
- I. Allocate resources for performance improvement activities by:
 - 1. Assigning organization personnel to participate in performance improvement activities
 - 2. Providing adequate time for organization personnel to participate in performance improvement teams and activities
 - 3. Creating and maintaining information systems and data management processes to support the collecting, managing and analyzing of data to improve performance
 - 4. Utilizing appropriate statistical techniques to analyze and display data
 - a. Statistical methodologies to consider include:
 - 1. Run charts that display summary comparison data
 - 2. Scatter diagrams
 - 3. Control charts that display variation and trends over time
 - 4. Histograms
 - 5. Pareto charts
 - 6. Cause and effect or fishbone diagrams
 - 7. Process flowcharts

- J. Assure that each performance improvement activity contains the following elements:
1. Description of the indicator(s)/activities to be conducted
 2. Frequency of activities
 3. Designation of responsible party
 4. Method(s) of data collection
 5. Acceptable limits for findings
 6. Who will receive the report
 7. Follow-up plans if findings fail to meet acceptable limits including plan(s) of correction
2. Provide organization personnel training in the approaches and methods of assessment and improvement.
3. All other organization personnel will:
- A. Be involved in performance improvement teams and activities.
 - B. Promote communication and coordination of performance improvement activities as well as contribute to those activities.
 - C. Forward relevant information regarding performance improvement activities to senior management and to the Performance Improvement Coordinator.
 - D. Take action on recommendations generated through performance improvement activities as outlined in the organization's written performance improvement plan.
4. Trends identified through performance improvement measurement and analysis will be reported to the Governing Body on a quarterly basis.
5. Results of performance improvement activities will be communicated with all staff via intranet, newsletters, email, etc.
6. The following policies are part of quality management (Title 22: 74742): 1-002 Governing Body; 1-011 Professional Advisory Committee; 1-012 Policy Decisions; 1-013 Development of Policies and Procedures; 4-048 Clinical Record Review; 5-002 Patient Focused Performance Improvement; 5-003 Measuring Performance of the Environmental Safety Program; 5-004 Annual Organization Evaluation; 5-005 Incident Reporting; 5-008 Aggregation of Data/Information; 5-010 Patient and Family/Caregiver Experience of Care Survey; 6-001 Management of Exposures in Personnel; 6-002 Record Keeping; 6-027 Infection Control Plan; 6-028 Tuberculosis Exposure Control Plan; 6-029 Bloodborne Pathogens and Hepatitis B Exposure Control Plan; 6-037 Emergency Management Plan.

ADDENDUM 5-001.A

PRIORITIZATION OF IMPORTANT PROCESSES

(To assist in outcomes to measure)

PRIORITIZATION OF IMPORTANT PROCESSES – EXAMPLE

Process	High Volume	High Risk	Problem-Pr one	Expensive to Correct	Impact on Quality	Impact Customer Satisfaction	Variations in Care
General Processes							
Patient Satisfaction							
Safety Program							
Incident Reports							
Documentation							
Infection Control							
Staff Education							
Patient Rights/Responsibilities							
Patient Grievances							
Use of Information							
Customer Service							
Patient Care Processes							
Intake Process							
Patient Assessment							
Pain Management							
Medication Administration							
Patient/Family Education							
Care Coordination							
Nursing Process							
Rehabilitation Therapy							
OASIS Accuracy							
Process Measures							
Outcome Measures							
Fall Risk Assessment							
Wound/Skin Care Management							
Foley/Urological Care							
Neurological Care							
Respiratory Care							
Cert/Recert Process							
Discharge Process							

Process	High Volume	High Risk	Problem-Pr one	Expensive to Correct	Impact on Quality	Impact Customer Satisfaction	Variations in Care
Personal Care							
Personal Care Plan							
CNA/HHA Communication							
Governance/Managerial							
Analysis of Effectiveness of Organizational Practices							
Development of Operational Plans							
Compliance with Regulatory Mandates							
Assigning/Acquiring Resources to Foster Quality							
Fostering Interdisciplinary Communication							
Assignment of Staff Responsibilities							
Management of Information							
Safety Plan Effectiveness							
Clerical							
Telephone Technique							
Coordination of Mail Between Offices							
Ordering of Supplies							
Ordering of Forms							
Tracking of Physician Orders							
Human Resources							
General Orientation/Competency Assessment							
Processing of Worker's Compensation Claims							
Handling of Employee Grievances							
Recruitment							
Personnel Policy Development							

Organization's Name

Quality Outcomes/Performance Improvement

Process	High Volume	High Risk	Problem-Pr one	Expensive to Correct	Impact on Quality	Impact Customer Satisfaction	Variations in Care
Financial Support							
Cost Analysis Reports							
Payroll							
Accounts Receivable Management							
Data Entry							
Charge Entry							
Generation of POC/Recert							
Discipline Visit Entry							
Insurance Verifications							
Other Organization-Specific Processes							
Totals							

Scoring Values

- 1 = Least Significant
- 2 = Mildly Significant
- 3 = Strongly Significant

ADDENDUM 5-001.B

SAMPLE QAPI PLAN

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Sample Quality Assessment and Performance Improvement (QAPI) Plan

Mission

All QAPI activities must support and be aligned with the mission of the organization.

The mission of the home health is to provide compassionate, high-quality healthcare for the best value, one person at a time, in their home setting.

To carry out this mission, the home health provides:

- A holistic program for the patient and family, caring for the physical, psychological and rehabilitative needs
- A dedicated team of health care professionals and staff who are qualified by experience and education
- Education to families, caregivers, and the community at large in the areas healthcare and chronic disease management in the home setting

Purpose

The purpose of the home health QAPI Plan is to provide a strategy for the systematic organization-wide implementation of quality assessment and performance improvement activities. This will ensure that the organization is providing appropriate, high-value, effective and efficient services in accordance with its mission and current standards of practice. Through QAPI activities, the organization provides a mechanism for identification and prioritization of opportunities for problem identification and improvement in care and operations.

Objectives

- To show measurable improvement in indicators that demonstrate an improvement in patient outcomes
- To measure, analyze and track quality indicators, including adverse events, to enable the assessment of processes of care, home health services and operations
- To collect data to monitor the effectiveness and safety of services and quality of care as well as to identify opportunities for improvement
- To conduct Performance Improvement Projects aimed at performance improvement and to track performance to ensure that it is sustained
- To document QAPI activities including reasons for Performance Improvement Projects and progress achieved on these projects
- To keep current with monitoring of required quality measures; submit reports in mandated format within required timeframes

Methodology

The QAPI Plan is operated through a committee structure that ensures that the QAPI program follows regulatory requirements. The QAPI Coordinator ensures that leadership priorities for improvement are evaluated. In addition, the QAPI Coordinator examines results from ongoing quality control activities to identify trends that will need follow-up action by the QAPI Committee.

The methodology selected to support and facilitate improvement activities is based on the Plan-Do-Study-Act (P-D-S-A) model. When an action for improvement is identified, an individual or project team may be designated to pilot the recommended action. A timeframe is established for implementing and evaluating the piloted action's outcome.

The effectiveness of any action will be assessed through a process that measures whether the performance expectations outlined in the action plan have been met. This may require additional data collection and analysis. Statistical analysis will be used in assessing performance outcomes.

Assignment of Responsibility

Resources will be made available to employees to assist them in gaining a basic understanding of QAPI principles. Inservices will be held periodically to reinforce the knowledge base. Each employee is responsible for the quality of care and services provided. The following summary of responsibilities provides a framework for the process of quality assessment and performance improvement.

The Governing Body is ultimately responsible for the QAPI Plan and for ensuring:

- That the QAPI Plan is ongoing, implemented and maintained
- That implemented QAPI activities address the appropriate priorities for the improved quality of care and patient safety
- Clear expectations for patient safety are met
- That all QAPI actions/changes are evaluated for effectiveness
- The QAPI Plan meets regulatory requirements

Responsibilities of QAPI Coordinator

The QAPI Coordinator chairs the QAPI Committee and is responsible for coordinating and facilitating all quality improvement activities. This includes, but is not limited to:

- Ensuring the collection of accurate and reliable data
- Participating in cross-organizational activities to assess and improve overall organizational quality and performance
- Assisting in the implementation of corrective actions as appropriate
- Evaluating the effectiveness of planned and implemented actions

- Encouraging staff participation in improvement activities by mentoring; advancing education by acting as a resource to all staff
- Aggregating, trending and analyzing data using appropriate statistical techniques
- Ensuring timely data submission of required quality measures
- Reporting significant findings to appropriate managers, staff and governing bodies

Responsibilities of QAPI Committee

The QAPI Committee is responsible for evaluating and prioritizing QAPI activities based on the aggregation of analysis of data collected. The QAPI Committee has the authority to issue recommendations for action or further study. Under the direction of the QAPI Coordinator, the committee issues a quarterly report summarizing QAPI activities and results of actions taken. The report is submitted to the Governing Body and appropriate managers and staff.

QAPI Committee members are appointed for staggered one-year terms that are renewable for a second term. The committee includes representatives from the following areas:

- Administration
- Clinical management
- Business office/billing/admitting
- Medical records
- Nursing
- Therapies (PT, OT, SLP)
- Dietitian or nutrition counselor
- Medical director or designee
- Support services
- Contracted services
- Medical social services
- Pharmacist, as needed

Department managers and supervisors are responsible for providing leadership to ensure the communication and coordination of QAPI activities. Managers are responsible for initiating immediate corrective action if problems identified threaten the safety of patients or staff. Additionally, they are responsible for participating in the QAPI Committee when assigned and identifying opportunities for improvement through their daily interactions.

Clinical and office personnel are responsible for participating in identifying opportunities for

improvement through their daily contact with patients, physicians and other employees. Staff may be requested to participate in the QAPI Committee or specific team activities such as data collection, analysis, action planning and implementation of new or improved processes as needed.

Evaluation of the QAPI Plan

A formal evaluation of all QAPI activities will occur annually. A written report will be completed by the QAPI Coordinator. After review by the directors of each department, the report will be presented to the Governing Body. The report will include a summary of all activities included as part of the QAPI Plan. The Governing Body will measure the effectiveness of the QAPI Plan based on the established objectives. The Governing Body will provide input into the identification and prioritization of future improvement activities.

Confidentiality

The QAPI Coordinator maintains all QAPI-related records in a secure storage area (physical area or HIPAA compliant electronic storage). Completed unusual occurrence/incident/ adverse event reports are maintained by the administration. Any requests for results of or data from the QAPI Plan will be forwarded to the designated administrator, who will respond only according to the organization's policy and procedure.

The QAPI Plan – Discussion

This QAPI Plan is effective because the QAPI efforts are grounded in the mission of the organization. The approach is a multi-disciplinary and collaborative one, and specific areas of responsibility are outlined.

This QAPI Plan is founded on basic QAPI principles. It demonstrates that everyone in the organization is involved through their participation in a variety of QAPI activities. In order to accomplish this, the leaders provide education, resources in QAPI principles, and time to participate in QAPI activities.

The improvement methodology is identified to ensure a systematic approach to improvement efforts. There is a provision in the QAPI Plan to evaluate the effectiveness of the QAPI activities.

This QAPI Plan demonstrates leadership involvement throughout the process. The Plan allows the flexibility needed if new problems or opportunities take priority.

PATIENT FOCUSED PERFORMANCE IMPROVEMENT
Policy No. 5-002.1**PURPOSE**

To delineate the process for defining and measuring quality of services in terms of patient outcomes.

POLICY

The organization's performance improvement processes will focus on the quality of patient and program outcomes. In addition, the assessment, planning, implementation of care and services, and evaluation of goal attainment will be individualized to the specific patient.

PROCEDURE

Patient Specific Outcomes (See "[Clinical Record Review](#)" Policy No. 4-048.)

1. At the time of admission and on an ongoing basis, an individualized plan of care/service including specific and measurable goals will be developed for each patient.
2. Time frames will be established for goal attainment.

Organization-wide Outcomes

1. As part of the organization-wide performance improvement process, opportunities for improvement related to patient outcomes will be identified through continuous measurement of patient satisfaction survey results, clinical record review, monitoring of incidents, complaints, grievances, and infection control reports and Potentially Avoidable Reports.
2. Program, or process related performance improvement activities will focus on opportunities to improve overall organizational performance.
3. When an opportunity to improve performance is identified, a focused study (indicator) will be developed to measure and improve associated processes.
4. Performance improvement documentation will be maintained by the Performance Improvement Coordinator.

**MEASURING PERFORMANCE OF THE
ENVIRONMENTAL SAFETY PROGRAM**
Policy No. 5-003.1

PURPOSE

To define a systematic process to measure environmental safety.

POLICY

Organization's Name will develop and maintain a systematic process to measure the effectiveness of the environmental safety program.

PROCEDURE

1. As part of the performance improvement program, the organization will assess, through defined measures, the effectiveness of the environmental safety program in:
 - A. Maintaining safe environments for patients as well as organization personnel
 - B. Teaching organization personnel and patients how to implement the program
 - C. Improving organization performance in environmental management
2. Measures will be developed which specifically address the components of the environmental safety program, and may include:
 - A. Incidents related to home environment of the patient, including:
 1. Equipment malfunctions
 2. Patient endangerment
 3. Falls
 4. Medication errors
 5. Fires
 6. Electrical issues
 - B. Incidents related to home environment but specific to home care personnel, including:
 1. Personnel endangerment

2. Equipment malfunction
3. Medication errors
- C. Incidents related to office environment, including:
 1. Equipment malfunction
 2. Fires
 3. Electrical issues
 4. Personnel falls/injuries
- D. Outcomes of office environment safety checks
3. Any areas demonstrating a pattern or trend will be analyzed by the Performance Improvement Committee for development of recommendations and actions.
4. A summary of the results of measures will be forwarded to the existing oversight committees and the Governing Body.

ANNUAL ORGANIZATION EVALUATION**Policy No. 5-004.1****PURPOSE**

To provide a process for the annual overall evaluation of the organization.

POLICY

The organization will appoint an authorized group or oversight committee (Professional Advisory Committee, Administrator, Director of Patient Care Services, at least one (1) physician, and other licensed professionals representative of the services provided) to conduct an annual evaluation to assess the organization's program. The complexity of the organization and the scope of services and products will define the parameters for data collection. The evaluation will include an analysis of the effectiveness of the organizational administrative practices, risk management, human resources, financial performance, and policies and procedures based on defined measures of program appropriateness and efficiency.

The Annual Evaluation Report is submitted to the appropriate oversight committees and the Governing Body for review. Portions of the annual evaluation may be used by the Marketing Department and to construct the annual report, if applicable

PROCEDURE

1. Within 45 days of the organization's fiscal year end, administrative personnel will prepare:
 - A. Organization statistics, fiscal, and billing information
 - B. Data collected from Patient Experience of Care Surveys (HHCAHPS)
 - C. Copies of organization policies and procedures for operations and patient care
 - D. Data collected from the an admissions or patient acceptance service policy
 - E. Summary information from the quarterly clinical/service record review
 - F. Summary information from quarterly clinical/service record review, as applicable
 - G. Environmental Safety Program review
 - H. A review of organizational structure and function
 - I. A review or organizational goal achievement
 - J. Benchmarking information
 - K. A review of the types of programs, services, and products provided
 - L. Human resource related information

- M. Adequacy of management and supervision, either on-site or by telecommunications, of support, paraprofessional, and professional personnel based at a minimum on the following considerations:
 - 1. The total patient census
 - 2. The numbers, qualifications, experience and current competence of the individuals providing each service
 - 3. The level of care/service required
 - 4. Service areas covered by the home health agency including personnel supervised out of branch offices
 - 5. The number and types of visits conducted
 - 6. The primary condition diagnosis of patients
 - 7. Services provided which require specialized training
 - 8. Dissatisfaction expressed by patients regarding the supervision of services
 - N. Summary information related to risk management
 - O. A review of organizational information systems
 - P. Quality Assessment Performance improvement findings
 - Q. Infection control program
 - R. Other pertinent information, as needed
2. The organization will be evaluated according to the following components, including, but not limited to:
- A. Organization and administration, including bylaws, policies, advisory committees, personnel policies, and contracts
 - B. Statistical/fiscal information, including Medicare cost reporting (as applicable), gross revenue by payer/service, revenue to expense analysis, contractual allowances per payer, charitable allowances, direct and indirect expenses per discipline, productivity, and bad debt
 - C. Human resource data, including number of personnel by functional level, turnover rates by type of termination, recruitment/retention effectiveness, personnel promotion/progression, and personnel development

- D. Clinical data including demographics of patients served by type of program; number of patients served by age, sex, diagnosis, program, and discipline; referral sources; number of admissions/re-admissions; number of non-accepted patients including reason; number of visits/hours/episodes by program and/or discipline; outcomes of care; quality indicators; length of stay, discharges from service and reasons; personnel productivity by discipline; and patient satisfaction
 - E. Risk management data including accidents/incidents/unusual occurrences, safety violations and/or hazards, equipment failures/misuse of equipment, occupational exposures, infection control activity findings, consumer complaints, employee grievances/conflict resolution, and other data required by local, state, or federal regulatory bodies and/or funding sources
 - F. Quality assessment performance improvement activities, including summary information from the four (4) most recent clinical record reviews, keep data results of ongoing monitoring/measures, and actions
3. The outcome of the evaluation will summarize findings from the previous year's services in scope and quality, and also may include:
- A. Whether the health and assistance needs of the service area/population are being met (based upon the request for service and the acceptance of patients)
 - B. The appropriateness and effectiveness of pain management
 - C. Whether there is sufficient qualified personnel available to provide the services offered
 - D. The extent of cooperation and communication with other agencies, community interest groups, and the effective use of community resources
 - E. The extent that organization policies and procedures are followed
 - F. The need for future planning, and methods of implementation for new or changing services as needs are identified
 - G. Whether the organization has achieved progress toward meeting annual goals
 - H. An evaluation of the implementation of the Environmental Safety Program
 - I. Whether there is a need for changes in:
 - 1. Services offered
 - 2. Service Limitations related to specialty services
 - 3. Service Durations or Service Frequency
 - 4. Training, Skills, and Competencies of Staff
 - 5. Staffing Levels

6. Operating hours
 7. Geographic service area
 8. Population groups service
 9. The system for continuity of care
 10. The financing of services
4. Variances from usual and expected patterns of performance will be analyzed. A summary of the findings will be prepared with recommendations made for each problem area, and will be discussed by the designated annual review group.
 5. The authorized group or oversight committee will review the summary and respond to areas requiring follow-up. The summary and responses will be forwarded to the Governing Body and senior management.
 6. The Governing Body will submit a copy of the annual evaluation with comments to its minutes.
 7. The Governing Body and senior management will act upon the recommendations, as appropriate.
 8. The Annual Evaluation Report will be retained as an administrative record and provide a basis for budgetary, strategic, operational planning, and marketing activities.

INCIDENT REPORTING
Policy No. 5-005.1**PURPOSE**

To define the reporting, follow-up, and feedback process for incidents involving patients and organization personnel.

POLICY

The organization will maintain a process for generating incident reports and follow-up corrective action, if applicable. The five (5) purposes of the incident report are:

1. To facilitate the early detection of problems or compensable events
2. To establish a foundation for early investigation of all potentially serious events
3. To develop a database for long-range problem detection analysis and correction
4. To enable cross-reference with other risk detection systems
5. To investigate and respond to serious adverse events, in accordance with accrediting bodies standards

All events or occurrences listed in "[Examples of Specific Events or Occurrences That Must Be Reported](#)" (see Addendum 5-005.A) must be reported, as well as any other occurrences presenting risks to patients.

The incident report is not a part of the patient's clinical/service record. No reference in the clinical/service record will be made indicating completion of an incident report.

The incident reporting system will be part of the organization's overall performance improvement and risk management plan.

Definitions

1. ***Incident:*** An unusual event involving organization personnel, a patient, and family/caregiver. The event is considered unusual if the result was unintended, undesirable, and/or unexpected. An incident is also any happening that is not consistent with the routine operation of the organization or the routine care of a patient. It may be actual or potential. (See "[Examples of Specific Events or Occurrences That Must Be Reported](#)" Addendum 5-005.A for further definition.)
2. ***Serious Adverse Event:*** An unexpected occurrence involving death or serious physical or psychological injury, or the risk thereof. Serious injury specifically includes loss of limb or function. The phrase "or risk thereof" includes any process variation for which recurrence would carry a significant chance of a serious outcome.

PROCEDURE

1. When an incident occurs, the individual discovering the incident will:
 - A. Notify your supervisor immediately (including after hours) with observations or identification of the incident.
 - B. Follow-up with the patient and family/caregiver, and/or patient's physician (or allowed practitioner), if indicated by your supervisor or designee within 24 hours or 4 hours if the incident is of an urgent nature.
 - C. Maintain the confidentiality of the information. The report is for internal use only and is not available to physician (or allowed practitioner) or other agents outside the organization.
 - D. Complete an incident report form within 24 hours of the incident.

Note: *For personal care providers, the immediate supervisor should implement the form.*

The form should include the following:

1. Pertinent patient information
2. Type of incident
3. Description of the incident or injury in narrative form; if no injury, state "no apparent injury"
4. Name of the family/caregiver, organization personnel, or another witness of the incident
5. Any drugs taken by the patient within eight (8) hours before the incident, including the dose, route, and time administered, especially for reporting falls
6. Name of person(s) who requested follow-up or needed notification; indicate who was notified, the time, and by whom
7. Whether organization personnel were injured, if applicable
8. Any action taken by the physician, if applicable
9. Nature of the injury; if other, please specify
10. Names of any witnesses, the relationship to the patient or organization personnel, and title, when applicable

2. The Program Director or designee will review and sign the incident report form, request any necessary follow-up from appropriate personnel, and initiate incident report follow-up form within 24 hours of investigation, as required.
3. The Program Director or designee will forward the incident and follow-up forms to the Executive Director/Administrator within 24 hours of Program Director investigation follow-up report. Copies of the incident reports will be forwarded to the Performance Improvement Coordinator for analysis, tracking, and trending.
4. The Performance Improvement Coordinator will review the incident reports and conduct follow-up as indicated. Corrective actions will be implemented and evaluated for effectiveness as indicated.
5. A summary of the analysis incident reports will go to the Executive Director/Administrator, who will review and forward report recommendations to the Governing Body on a quarterly basis.
6. Incidents requiring reporting to state and/or federal regulatory agencies:
 - A. All regulations and reporting forms will be available in the quality improvement department.
 - B. The Performance Improvement Coordinator or designee will review incidents to determine if the event meets reporting criteria.
 - C. As applicable, the Performance Improvement Coordinator or designee will complete and submit the necessary forms within the required time frame to the appropriate organization.
 - D. The Performance Improvement Coordinator will prepare and submit any subsequent or summary reports that may be required.
 - E. Reportable event files will be maintained according to applicable regulations.
7. Incidents that conform to Organization's Name's definition of serious adverse events will be immediately reported to the Executive Director/Administrator. Under the guidance of the Executive Director/Administrator, the Performance Improvement Coordinator will conduct a root cause analysis of the serious adverse event.

ADDENDUM 5-005.A

**EXAMPLES OF SPECIFIC EVENTS OR OCCURRENCES
THAT MUST BE REPORTED**

EXAMPLES OF SPECIFIC EVENTS OR OCCURRENCE THAT MUST BE REPORTED

1. Loss/Breakage

- Patients and family/caregivers report missing articles from home after home visit
- Organization personnel involved in damage/breakage to a patient's personal belongings or equipment
- Organization personnel's discovery of patient equipment or personal belongings missing from premises

2. Equipment/Medical Device

- Failure to provide/receive orders
- Non-delivery of an essential piece of equipment, whether the patient, vendor, or the referral source is responsible
- Malfunctioning equipment resulting in actual or potential injury to patient
- Organization personnel or the patient and family/caregiver observes or reports injury resulting from equipment use

3. Endangerment—Building Security

- Fire
- Unidentified persons in the home/premises

4. Patient Endangerment

- Suspected abuse by family/caregiver
- Patient confused and essentially helpless within adequate care by others
- Mentally competent adult chooses potentially unsafe home environment over institutionalization

Note: For all above, notification to the state Elder-at-Risk program and/or Adult or Child Protective Services must be made by mandated reporters, per program requirements.

5. Organization Personnel Endangerment

- Verbal and/or physical assault while on duty

6. Decubitus Ulcers

- Development of new decubitus ulcer that reaches worsening Stage III after start of care

7. Refusal of Treatment

- The patient and family/caregiver refuses treatment after start of care, against the professional advice of organization personnel

**EXAMPLES OF SPECIFIC EVENTS OR OCCURRENCE
THAT MUST BE REPORTED** *(continued)***8. Problem with Procedure, Protocol Error**

- The patient and family/caregiver state the procedure/treatment has traumatized the patient
- The patient and family/caregiver makes an error in the procedure after they were evaluated as independent or competent to perform a procedure
- Physician reports care that is inappropriate, unscheduled, provided without orders, or caused the family/caregiver to complain about the care provided
- Attorney initiates telephone call to complain about or inquire about alleged patient injury
- Staff member does not follow prescribed protocol, whether or not patient is injured
- Staff member discovers difficulty in carrying out assigned orders
- Staff member performs a procedure without patient authorization or appropriate physician (or allowed practitioner) orders
- Patient care performed outside the scope of practice
- Professional providing care not certified to perform specific procedure
- Failure to provide ordered care for patient once a case has been accepted
- Repeated complaints related to billing

9. Untoward Outcome

- Drug reaction or toxic effect, immediate or delayed
 - Signs/symptoms of adverse or toxic effects
 - Anaphylaxis
- Intravenous therapy complications
 - Extravasation
 - Infiltration (area of induration is in excess of 5 cm)
 - Hematoma
 - Phlebitis
 - Central venous catheter—elated sepsis
 - Questionable placement of central venous catheters
 - Vascular impairment of any extremity involving arteries/veins
 - Transfusion reaction (acute/non-acute systemic allergic, acute hemolytic or delayed)
- Treatment or procedure problems resulting in:

EXAMPLES OF SPECIFIC EVENTS OR OCCURRENCE THAT MUST BE REPORTED *(continued)*

- Aspiration of foreign matter into the respiratory tract, even if no problems present at time
- Loss of range of motion
- Laceration of skin
- Fracture, displaced, or undisplaced
- Progressive worsening of decubitus ulcer, even though physician (or allowed practitioner) orders are being followed
- Infection of any wound or organ, not present on admission to care
- Bruise, abrasion, contusion, or pressure sore, not present on admission to care—responsibility with reporting elder/child abuse
- Any hemorrhage
- Neurological impairment, central or peripheral, not present on admission to care; i.e., convulsions, coma, loss of taste, sight, hearing, numbness, weakness, or paralysis
- Retained foreign body, not intended
- Noncompliance by the patient and family/caregiver resulting in injury requiring emergency intervention or hospitalization
- Staff member neglects to report significant findings to the physician. In the case of non-licensed staff member, the reporting should be to the Clinical Supervisor who would then be responsible to the physician.

10. Consent Problems

- No consent obtained
- Wrong date on consent
- The patient withdraws consent by refusing treatment

11. Attended/Unattended Fall

- The patient sustains injury subsequent to falling while organization personnel present
- The patient and family/caregiver reports a fall sustained by the patient during the course of his/her time on service

12. Medication Error

- The patient and family/caregiver do not follow physician (or allowed practitioner) orders, nurse, or therapist instructions in administering medications; e.g., titrating of IV opiates outside of physician's (or allowed practitioner's) ordered titration parameters
- Medication errors that include errors of medication, time, dose, route of administration during intervention by the nurse

**EXAMPLES OF SPECIFIC EVENTS OR OCCURRENCE
THAT MUST BE REPORTED** *(continued)*

- Missing a scheduled administration of an IM/IV medication for any reason; e.g., staffing difficulties
13. Cardiac Arrest
- Witnessed cardiac or respiratory arrest—unless “no code—physician order” is written in the clinical record
14. Unexpected death including a successful suicide
15. Other
- Staff member accepts compensation or private employment from the patient and family/caregiver
 - Any reference to legal action by a patient, family/caregiver, physician, or attorney
 - Any occurrence or observation that the organization personnel believes involves actual or potential injury to the patient, the company, or its organization personnel
 - Suicide threats or attempts
 - Any act of violence
 - Psychological injury

SERIOUS ADVERSE EVENTS
Policy No. 5-006.1**PURPOSE**

To have a positive impact in improving patient care.

To focus Organization's Name's attention after a serious adverse event on understanding the causes that underlie the event and on making changes in systems and processes to reduce the probability of such an event in the future.

To increase general knowledge about serious adverse events, their causes, and strategies for prevention.

To ensure maximum risk-prevention and loss-reduction activities on the part of Organization's Name in response to a serious adverse event.

POLICY

1. Incidents that conform to Organization's Name's definition of serious adverse events will be immediately reported to the Executive Director/Administrator or designee. Any member of the staff may report the incident.
2. Under the guidance of the Executive Director/Administrator, the Performance Improvement Coordinator will conduct a root cause analysis of the serious adverse event. The planning and preparing phase of the root cause analysis and action plan will be completed within 45 days of the event or of becoming aware of the event.
3. Undesirable patterns or trends in performance and serious adverse events will be intensively analyzed.
4. High-risk, high volume, and problem-prone areas related to the care and services provided will be correlated with the listing of frequently occurring serious adverse events identified in the organization's risk management data, and information about problematic processes generated by other professional organizations.
5. At a minimum, performance measures will be selected related to the following processes as appropriate to care and services:
 - A. Medication use
 - B. Use of blood and blood components
 - C. Equipment malfunction
 - D. Oxygen use

E. Care or services provided to high-risk populations

Definitions

1. **Serious Adverse Event:** An unexpected occurrence involving death or serious physical or psychological injury, or the risk thereof. Serious injury specifically includes loss of limb or function. The phrase, "or the risk thereof" includes any process variation for which a recurrence would carry a significant chance of a serious adverse outcome. Such events are called "serious adverse events" because they signal the need for immediate investigation and response.
2. **Root Cause Analysis:** A process for identifying the basic or causal factors that underlie variation in performance, including the occurrence or possible occurrence of a serious adverse event. A root cause analysis focuses primarily on systems and processes, not individual performance. It progresses from special causes in clinical processes to common causes in organizational processes, and identifies potential improvements in processes or systems that would tend to decrease the likelihood of such events in the future, or determines, after analysis, that no such improvement opportunities exist. (See "[Root Cause Analysis/Action Plan](#)" Policy No. 5-007.)
3. **Action Plan:** The product of the root cause analysis is an action plan that identifies the strategies that the organization intends to implement to reduce the risk of similar events occurring in the future. The plan should address responsibility for implementation, oversight, pilot testing as appropriate, time lines, and strategies for measuring the effectiveness of the action.
4. **Proximate Cause:** The obvious, direct cause of an accident, such as human error or equipment malfunction.
5. **Special Causes:** A cause that contributed to the accident but is outside the system and the organization, as it could not be foreseen or prevented.
6. **Common Causes:** Systematic problems that could lead to other accidents or injuries.
7. **Risk Points:** Procedures or a point of time when personnel could have averted the accident if they took extra care.
8. **Major Permanent Loss of Function:** Sensory, motor, physiologic, or intellectual impairment not present on admission requiring continued treatment or lifestyle change. When "major permanent loss of function" cannot be immediately determined, applicability of the policy is not established until either the patient is discharged with continued major loss of function, or two (2) weeks have elapsed with persistent major loss of function, whichever occurs first.

PROCEDURE

Stage I: Immediately after the event, or hearing about the event (the first 48 hours)

1. Inform the Executive Director/Administrator or designee so he/she can provide information to any appropriate outside parties.
2. Make sure the organization is doing everything possible to provide follow-up care/services to ensure the best possible outcomes for injured parties/property and/or personnel.
3. Ensure that all parties involved in the event receive appropriate information to avoid miscommunication. Establish a mechanism for updates, as needed, using a designated spokesperson for consistency. The spokesperson should be someone familiar to and trusted by the patient and family. Personal rapport and concern are to be conveyed.
4. Follow any immediate regulator reporting requirement, such as the Occupational Safety and Health Administration in the case of an employee death, the Center for Missing and Exploited Children, adult/child protective services, etc.
5. Document appropriate information in occurrence reports and other risk management forms and submit to the Executive Director/Administrator or designee and the Performance Improvement Coordinator. Document objective clinical findings in the clinical record as necessary.
6. Document all confidential attorney patient communications and address to the corporate attorney.
7. Gather detailed information about the event.
8. Remind personnel of the confidentiality surrounding the event and the patient. Assure that personnel are aware of and communicate with the identified spokesperson/liaison, as appropriate.

Stage II: Within 45 days of the event or becoming aware of the event

1. The Performance Improvement Coordinator interviews all parties to the event, obtains written statements to gather an accurate description of the sequence of events.
2. Determine the root cause(s) of the event, including an analysis of all processes and systems related to its occurrence. Involve all appropriate personnel, as determined by the Executive Director/Administrator or designee in this analysis.
3. Determine potential improvements in processes or systems that would tend to decrease the likelihood of such events occurring in the future. Examples may include a change in communication, forms, training, equipment, policies, and procedures. If none exist, indicate in the conclusion of the analysis the determination that no such opportunities exist.

4. Establish a plan to address identified opportunities for improvement or formulation of a rationale for not undertaking such changes. Indicate time frame, person responsible, and criteria to evaluate effectiveness of the actions.
5. The Executive Director/Administrator or designee will notify all appropriate organization insurance carriers and plan for the proper handling and protection of the clinical record and other potential evidence (including log books, policies, procedures, or schedules) needed in anticipation of possible litigation.

Stage III: Post-event reporting

1. The Performance Improvement Coordinator will create a summary of all incidents managed through the serious adverse event intervention action plan.
2. The Executive Director/Administrator will review the summary and forward report recommendations to the Governing Body on a quarterly basis.

ROOT CAUSE ANALYSIS/ACTION PLAN
Policy No. 5-007.1**PURPOSE**

To discover underlying causes and understand why a serious adverse event occurred. To discover what went wrong with the organization's systems and processes that can be altered to reduce the likelihood of reoccurrence.

POLICY***Root Cause Analysis***

1. When a serious adverse event is discovered, a root cause analysis will be conducted to identify the basic or causal factors that underlie variation in performance.
2. The root cause analysis will focus primarily on systems and processes, not individual performance.
3. The analysis will progress from special causes in clinical/service processes, unusual circumstances or events that are difficult to anticipate, to common causes in organization processes. It will identify potential improvement in processes or systems that would tend to decrease the likelihood of such events in the future, or determines, after analysis, that no such improvement opportunities exist.
4. Ordinarily, common cause variation can only be improved by redesigning a process. The intention will be not to accept an event as either human error or equipment breakdown, but to delve into what preceded the event or allowed it to occur.

Action Plan

1. An action plan will be initiated after the root cause analysis is completed.
2. The action plan will be the product of the root cause analysis, which identifies the strategies the organization intends to implement to reduce the risk of similar events occurring in the future.
3. The plan will address responsibility for implementation, oversight, pilot testing as appropriate, time lines, and strategies for measuring the effectiveness of the actions.

PROCEDURE***Questions to be asked including:***

1. What happened?

2. Why did it happen?
3. What processes were involved?
4. What systems underlie these processes?
5. How did the systems or processes fail?

Process for conducting a root cause analysis:

1. It is critical that the team understands the purpose of the investigation; keeping in mind that issues obvious to one member may not be to another.
2. Brainstorming is a good method to find the most likely contributing causes.
3. Following brainstorming, the team should further assess potential causes.
4. At this point, the team should be able to begin to determine the cause or causes of the problem and whether it is a special cause or a common cause issue.
5. It is important that the team not wait to finish the analysis before designing and implementing changes that may be appropriate and necessary to prevent recurrence.
6. During the process, it is important to determine progress and whether any course adjustments need to be made.
7. As redesign occurs, there may need to be changes in training, policies, procedures, forms, equipment, etc.
8. Monitoring for expected results from the redesign should be ongoing but completed no later than six (6) months from interventions.

A root cause analysis will be acceptable if it has the following characteristics:

1. The analysis focuses primarily on systems and processes, not individual performance.
2. The analysis progresses from special causes in clinical/service processes to common causes in organization processes.
3. The analysis repeatedly digs deeper by asking "Why?"
4. The analysis identifies changes, which could be made in systems and processes—either through redesign or development of new systems or processes—that would reduce the risk of such events recurring.
5. The analysis is thorough and credible.

The root cause analysis is thorough if it includes:

1. A determination of the precipitating human and other factors most directly associated with the serious adverse event, and the processes and systems related to its occurrence
2. Analysis of the underlying systems and processes through a series of "Why?" questions to determine where redesign might reduce risk
3. Identification of risk points and their potential contributions to this type of event
4. A determination of potential improvements in processes or systems that would tend to decrease the likelihood of such events in the future, or a determination, after analysis, that no such improvement opportunities exist

For the root cause analysis to be credible, it must:

1. Include participation by the leadership of the organization and by the individuals most closely involved in the processes and systems under review
2. Be internally consistent
3. Provide an explanation for all findings of "not applicable" or "no problem"
4. Include examination of any relevant literature

An action plan will be considered acceptable if it:

1. Identifies a resulting action plan that describes the organization's risk reduction strategies or formulates a rationale for not undertaking such changes, and
2. Identifies who is responsible for implementing improvement actions, when the actions will be implemented (including any pilot testing), and how the effectiveness of the actions will be evaluated

The organization will:

1. Document its analysis within 45 days of the serious adverse event or knowledge of the event, and begin the action plan.
2. Implement the recommended improvements.
3. Monitor the changes for effectiveness.

ADDENDUM 5-007.A

ROOT CAUSE ANALYSIS/ACTION PLAN FORM

ROOT CAUSE ANALYSIS AND ACTION PLAN

This three (3)-page template will help to organize the steps in a root cause analysis. Not all possibilities and questions will apply in every case, and there may be others that will emerge in the course of the analysis. However, all possibilities and questions should be fully considered in your quest for "root causes" and risk reduction.

As an aid to avoiding "loose ends," the three (3) columns provided on the right are to be checked off for later reference:

"Root cause?" should be answered "yes" or "no" for each finding. A root cause is typically a finding related to a process or system that has a potential for redesign to reduce risk. If a particular finding that is relevant to the event is not a root cause, be sure that it is addressed later in the analysis with a "Why?" question. Each finding that is identified as a root cause should be considered for an action and addressed in the action plan.

"Ask why?" should be checked off whenever it is reasonable to ask why the particular finding occurred (or didn't occur when it should have)—in other words, to explore further. Each item checked in this column should be addressed later in the analysis with a "Why?" question. It is expected that any significant findings that are not identified as root causes will have check marks in this column. Also, items that are identified as root causes will often be checked in this column, since many root causes themselves have "roots."

"Take action?" Should be checked for any finding that can reasonably be considered a possible risk reduction strategy. Each item checked in this column should be addressed later in the action plan. It will be helpful to write the number of the associated action items on page 3 in the "Take Action?" column for each of the findings that require action.

Level of Analysis		Questions	Findings	Root Cause?	Ask "Why?"	Take Action?
What happened?	Serious adverse event	What are the details of the event? (Brief description)				
		When did the event occur? (Date, day of week, time)				
		What area/service was impacted?				
Why did it happen? What were the most proximate factors? (Typically "special cause" variations)	The process or activity in which the event occurred	What was the intended flow process? (A flow diagram may be helpful here)				
		Were there any steps in the process that did not occur as intended?				
	Human factors	What human factors were relevant to the outcome?				
	Equipment factors	How did the equipment performance affect the outcome?				
	Controllable environmental factors	What factors directly affected the outcome?				
	Uncontrollable environmental factors	What uncontrollable external factors influenced this outcome?				
	Other	Are there any other factors that directly influenced this outcome?				
		What are the other areas in the organization where this could happen?				

Level of Analysis		Questions	Findings	Root Cause?	Ask "Why?"	Take Action
Why did that happen? What systems and processes underlie those proximate factors? (Common cause variation here may lead to special cause variation in dependent processes.)	Human resource issues	Was the staff properly qualified and currently competent for their responsibilities at the time of the event?				
		How did actual staffing compare with ideal levels?				
		What is the plan for dealing with contingencies that would tend to reduce effective staffing levels?				
		To what degree is staff performance in the operant process(es) addressed?				
		How can orientation & inservice training be improved to reduce the risk of such events in the future?				
	Information management issues	To what degree was all necessary information available when needed? Accurate? Complete? Unambiguous?				
		To what degree was communication among participants adequate for this situation?				
	Environmental management issues	To what degree was the physical environment appropriate for the processes being carried out for this situation?				
		What systems are in place to identify environmental risks?				
		What emergency and failure-mode responses have been planned and tested?				
	Leadership issues: Corporate culture	How does the organization's culture support risk reduction?				
	Encouragement of communication	What are the barriers to communication of potential risk factors?				
	Clear communication of priorities	How is the prevention of adverse outcomes communicated as a high priority?				

	Uncontrollable factors	What can be done to protect against the effects of these uncontrollable factors?				
	Technology	Was available technology used as intended? How might technology be introduced or redesigned to reduce risk in the future?				

	Risk Reduction Strategies	MEASURES OF EFFECTIVENESS
For each of the findings identified in the analysis as needing an action, indicate the planned action, expected implementation date, and associated measure of effectiveness, OR . . . If, after consideration of such a finding, a decision is made not to implement an associated risk reduction strategy, indicate the rational for not taking action at this time. Check to be sure that the selected measure will provide data that will permit assessment of the effectiveness of the action. Consider whether pilot testing of a planned improvement should be conducted. Improvements to reduce risk should ultimately be implemented in all areas where applicable, not just where the event occurred. Identify where the improvements will be implemented.	Action item #1:	Measure:
	Action item #2	Measure:
	Action item #3	Measure:
	Action item #4	Measure:
	Action item #5	Measure:
	Action item #6	Measure:
	Action item #7	Measure:
	Action item #8	Measure:
Cite any books or journal articles that were referenced when developing this analysis and action plan:		

AGGREGATION OF DATA/INFORMATION
Policy No. 5-008.1**PURPOSE**

To define the data/information that will be accumulated and aggregated for leadership and management decisions.

POLICY

The organization will maintain data/information that will support its leadership and management decisions.

Statistical reports will identify organizational trends and will be used for planning processes.

PROCEDURE

1. Senior management will identify the information that is needed for management, operations, performance improvement, and patient care.
2. Senior management will identify and define the types of information to be collected and aggregated.
3. Data to be considered for collection to monitor performance of the organization include the following:
 - A. Pharmaceutical transactions including medication, use of blood, and use of blood components
 - B. Information about hazards and safety practices, including:
 1. Summaries of incidents (see "[Incident Reporting](#)" Policy No. 5-005)
 2. Summaries of recalls
 3. Summaries of user errors
 4. Summaries of fires
 - C. Annual inspection of all areas of the facility to identify environmental hazards and unsafe practices
 - D. Environmental hazards in the patient's environment
 - E. Appropriateness and effectiveness of pain management

- F. Care or services provided to high-risk populations
 - G. Information about incidents, including a break down by type
 - H. Reports and recommendations of any regulatory, accreditation or inspection organization, and actions taken
 - I. Measures or processes and outcomes for assessing performance as part of the performance improvement plan, including analysis of levels, patterns, or trends over time that trigger further evaluation
 - J. Summaries of actions taken as a result of performance improvement activities, including risk management, utilization review, infection control, safety management, outcome reports regarding processes or services, and performance measures from acceptable databases
 - K. Financial data/information, such as accounts receivable, accounts payable, budget management, fee setting, billing, and collection
 - L. Data recording the verification of licensure of physicians (or allowed practitioners) from whom orders are accepted
 - M. Personnel opinion and needs
 - N. Patient demographics and diagnoses
 - O. Educational systems and outcomes
 - P. Performance comparisons internally and externally over time
4. Based on data collected, organizational leaders will determine which performance areas will be important to monitor, what data will be collected to determine performance, the detail of the data collected and the frequency at which the data will be collected. Decisions will be based on the organization's mission, care and services provided, and population served. Information to be gathered and aggregated will be assigned to individuals within the organization. Summary reports will be generated and sent to senior management.

EXTERNAL DATABASES

Policy No. 5-009.1

PURPOSE

To ensure access to external information to assist organization personnel in performing their functions, and for comparative purposes.

POLICY

The organization and its personnel may access external databases and bodies of expert knowledge, when available, in the performance of their functions. These functions include the evaluation of organization performance and the identification of deviations from expected trends.

External databases, when available, will be used for comparative analysis for improving organizational performance.

The organization will contribute to external databases when required by law or regulation and accrediting bodies as appropriate to the organization's mission and scope of service. The organization will maintain confidentiality and security of information when contributing or using an external database for comparative purposes.

Benchmarking software products will provide quarterly reports of selected benchmarks.

**PATIENT AND FAMILY/CAREGIVER
EXPERIENCE OF CARE SURVEY
Policy No. 5-010.1**

PURPOSE

To delineate the process for responding to the needs and expectations of patients and their representatives utilizing the Home Health Care CAHPS (HHCAHPS) survey.

POLICY

The organization will maintain a patient experience of care program, designed to obtain fresh feedback from patients, family/caregivers, and patient representatives, to assist the organization in improving performance as well as planning for the design of care and services. The organization will utilize the Home Health Care CAHPS survey for this program.

PROCEDURE

1. The organization's leadership will choose and contract with an approved Home Health Care CAHPS survey vendor to conduct the survey process, transmit result data to CMS and provide the organization with reports on data submission and survey results for public reporting. (Approved vendor list available at <https://homehealthcahps.org>)
2. The organization's leadership will appoint a Home Health Care CAHPS Survey Administrator. The administrator's role and responsibilities are as follows:
 - A. Register as the Home Health Care CAHPS Survey Administrator.
 - B. Designate another individual within the organization as the backup Administrator.
 - C. Complete and/or approve each new non-administrator user on the online Registration Form.
 - D. Grant individual non-administrator users access to specific functions.
 - E. Update non-administrator user information.
 - F. Remove access and/or approve the removal of access for users who are no longer authorized to access the private side of the portal.
 - G. Serve as the main point of contact with the Home Health Care CAHPS Survey Data Center and the approved survey vendor.
3. Other organization roles and responsibilities are as follows:

- A. Authorize the contracted survey vendor to collect and submit Home Health Care CAHPS Survey data to the Home Health Care CAHPS Data Center on the organization's behalf.
- B. Work with the approved vendor to determine a date of each month by which the vendor will need sample frame information.
- C. By the agreed upon date, compile and deliver to the survey vendor a complete and accurate list of patients that will allow the vendor to administer the survey.
- D. Review data submission reports to ensure that the survey vendor has submitted data on time and without data problems.
- E. Review Home Health Care CAHPS Survey results prior to public reporting.
- F. Avoid influencing patients in any way about how to answer the Home Health Care CAHPS Survey. Unacceptable practices include:
 - 1. Giving a copy of the survey questionnaire to the patient
 - 2. Contacting patients to ask if they want to participate in the survey
 - 3. Sending letters or contacting patients to advise them on how to respond to the survey questions
 - 4. Telling patients that they are expected to respond favorably or in a certain way to survey questions
- 4. Utilizing vendor survey results report, the data will be summarized to identify any patterns, trends, and issues. Further analysis will be performed, using performance improvement tools, when areas in need of improvement are identified.
- 5. Results will be sent to the Performance Improvement Committee, and senior management will forward the information to the Governing Body.
- 6. Results will be reviewed with organization staff through department/team meetings, employee newsletter, performance improvement bulletin boards, intranet, etc.
- 7. The information will be utilized in the planning process to design care and services that meet the needs and expectations of patients and their representatives.
- 8. Any areas that do not meet the mission, goals, and objectives of the organization will be immediately addressed by senior management.

ADDENDUM 5-010.A

**POLICY: PATIENT AND FAMILY/CAREGIVER EXPERIENCE OF
CARE SURVEY FOR EXEMPT OR NON-PARTICIPATING HOME
HEALTH AGENCY**

**(FOR ORGANIZATIONS THAT SERVE FEWER THAN 60-
HH-CAHPS ELIGIBLE PATIENTS ANNUALLY)**

Must have filed a Participation Exemption Request Form at

<https://homehealthcahps.org/ForHHAs/ParticipationExemptionRequestForm.aspx>

**PATIENT AND FAMILY/CAREGIVER
EXPERIENCE OF CARE SURVEY
Policy No. 5-010.A**

PURPOSE

To delineate the process for monitoring, measuring, and responding to the needs and expectations of patients and their representatives.

POLICY

The organization will maintain a patient experience of care program, designed to obtain feedback from patient and family/caregiver and patient representatives, to assist the organization in improving performance as well as planning for the design of care and services.

PROCEDURE

1. The organization will obtain information from patients/family/caregivers regarding the care and service provided at least monthly.
2. During the first week of every month, a random list of patients to be surveyed will be generated.
3. Each patient and family/caregiver on the list will be numbered sequentially, beginning with the number one (1). This number, along with the number representing the current month, will be printed at the top of the Experience of Care Survey.
4. The Experience of Care Survey will be mailed to each customer on the list, along with a self-addressed, stamped envelope for the return.
5. The Experience of Care Survey will address important indications of satisfaction, such as:
 - A. Patient and family/caregiver satisfaction with care and services provided
 - B. Satisfaction with providers of care/service
 - C. Satisfaction with self participation of care/service
 - D. Understanding of and agreement with changes in the plan of care/service
 - E. Understanding of the Patient Bill of Rights information
 - F. Suggestions for improvement in care and service delivery

6. Each month, the responses will be summarized by the Performance Improvement Coordinator, including:
 - A. Response rate
 - B. Overall experience of care
 - C. Individual perception of care for each question
7. The data will be summarized to identify any patterns, trends, or issues. Further analysis will be performed, using performance improvement tools, when areas in need of improvement are identified.
8. Results will be sent to the Performance Improvement Committee, and senior management will forward the information on to the appropriate review committees and the Governing Body.
9. The information will be utilized for the purpose of program planning and developing strategies to improve care and service delivery.
10. Any issues that do not support the mission of the organization will be immediately addressed by senior management.
11. Patient experience of care data will be retained by the organization for a period of three (3) years.

SECTION SIX

Risk Management: Infection and Safety Control

Policy No.

Management of Exposures in Personnel

6-001

Record Keeping

6-002

Occupational Exposure Information and Training

6-003

Standard Precautions

6-004

Addendum: Standard Precautions Information for Personnel

6-004.A

Personal Protective Equipment

6-005

Addendum: Protective Device Checklist

6-005.A

Addendum: Required Personal Protective Equipment Form

6-005.B

Hand Hygiene

6-006

Clean vs. Aseptic Technique

6-007

Infection Control/Expanded Precautions

6-008

Addendum: Bed Bug Guidance

6-008.A

Contaminated Materials Disposition

6-009

Contaminated Waste Disposal

6-010

Hazardous Waste Handling

6-011

Addendum: Hazardous Waste Disposal State and Local Regulations*

6-011.A

Accidental Exposure to Blood

6-012

[Bag Technique](#)

6-013

[Evaluating and Maintaining Records of Infections Among Patients](#)

6-014

Addendum: [Infection Identification—Patient Report](#)

6-014.A

[Evaluating and Maintaining Records of Infections Among Personnel](#)

6-015

Addendum: [Infection Identification—Personnel Report](#)

6-015.A

[Reporting of Communicable Diseases](#)

6-016

[Communication of Hazards to Personnel](#)

6-017

[Environmental Safety Program](#)

6-018

[Environmental Safety—Office](#)

6-019

Addendum: [Office Environment Checklist](#)

6-019.A

[Fire Safety—Office](#)

6-020

SECTION SIX

Risk Management: Infection and Safety Control

Policy No.

Utilities Management—Office

6-021

Equipment Management—Office

6-022

Environmental Safety—Patient

6-023

Fire Safety—Patient

6-024

Utilities Management—Patient

6-025

Equipment Management—Patient

6-026

Infection Control Plan

6-027

Tuberculosis Exposure Control Plan

6-028

Addendum: Individual TB Risk Assessment

6-028.A

Bloodborne Pathogens and Hepatitis B Exposure Control Plan

6-029

Addendum: Hepatitis B Vaccination Documentation Form

6-029.A

Addendum: Hepatitis B Vaccination Declination Form

6-029.B

Addendum: Recognizing the Dangers

6-029.C

Addendum: Occupational Exposure Risk By Job Classification

6-029.D

Safe and Appropriate Use of Home Medical Equipment and Supplies

6-030

Storage of Medications and Nutritional Therapies

6-031

Medical Equipment Malfunction

6-032

[Safe Medical Device Act](#)

6-033

[Organization Personnel Safety—Personal Safety](#)

6-034

[Organization Personnel Safety—Unsafe Home Visits](#)

6-035

[Vehicle Accident Reporting](#)

6-036

[Emergency Management Plan](#)

6-037

Addendum: <u>Pyramid Phone Communication Plan</u> *	6-037.A
Addendum: <u>Command Structure</u> *	6-037.B
Addendum: <u>Flow Chart of Alternate Roles and Responsibilities</u> *	6-037.C
Addendum: <u>Hazard Vulnerability Analysis</u> *	6-037.D
Addendum: <u>Plans for Priority Events</u> *	6-037.E
Addendum: <u>Weather Report/Road Conditions</u> *	6-037.F
Addendum: <u>Emergency Management Plan Evaluation Form</u> *	6-037.G

SECTION SIX

Risk Management: Infection and Safety Control

Policy No.

Addendum: [Integrated Healthcare Systems](#)*

6-037.H

[Waived Testing](#)

6-038

Addendum: [Organization List and Criteria for Waived Test Performed](#)*

6-038.A

[Home Glucose Monitoring](#)

6-039

MANAGEMENT OF EXPOSURES IN PERSONNEL
Policy No. 6-001.1**PURPOSE**

To properly identify, manage, and report personnel exposures to infections.

POLICY

Organization's Name will identify and report personnel exposures to bloodborne pathogens and infections on a routine basis through consistent enforcement of its procedures.

General Considerations

In all patient-care activities, personnel can decrease the risk of acquiring or transmitting infection by careful hand washing and by providing care to patients with potentially transmissible diseases according to standard precautions. (See "[Standard Precautions](#)" Policy No. 6-004.)

The following exposures by personnel should be immediately reported on the "[Infection Identification—Personnel Report](#)" form (see Addendum 6-015.A), and "[Infection Identification—Patient Report](#)" form (see Addendum 6-014.A), and to the Program Director/Supervisor for appropriate follow-up.

1. Cuts, abrasions, needle sticks, wounds, etc., obtained during patient contact.*
2. A splash by blood or body fluids on any mucous membranes, cuts, open lesions, or skin rashes.

* Needle sticks are considered injuries and are to be reported in accordance with OSHA's record keeping requirements effective January of 2002. OSHA 300 Log and 301 Incident Report will be used to maintain records of all sharps injuries.

Personnel can receive advice about health counseling from the organization infection control nurse, the Program Director/Supervisor, or the public health department.

PROCEDURE***Post Bloodborne Pathogen Exposure Evaluation and Follow-up: General Procedures***

1. After a report of an exposure incident to blood/body fluids, Organization's Name will immediately make available to the exposed staff member a confidential medical evaluation and follow-up. This evaluation and follow-up will include:
 - A. Documentation of the route(s) of exposure, and the circumstances under which the exposure incident occurred.
 - B. Identification and documentation of the source individual, unless this identification is not feasible or prohibited by state or local law.

- C. The source individual's blood will be tested by an accredited lab at no cost to the employee, as soon as feasible and after consent is obtained. If consent is not obtained, then it will be established that legally required consent cannot be obtained. When the source individual's consent is not required by law, the source individual's blood, if available, will be tested and the results documented.
 - D. When the source individual is already known to be infected with HBV or HIV, testing for the source individual's HBV or HIV status need not be repeated.
 - E. Results of the source individual's testing will be made available to the exposed organization personnel, and the exposed personnel will be informed of applicable laws and regulations concerning disclosure of the identity and infectious status of the source individual.
 - F. Post-exposure, evaluation, follow-up, and prophylaxis will be provided at no cost to the exposed organization employee, and as recommended by the U.S. Public Health Service.
 - G. Post-exposure counseling will be provided when medically indicated.
2. Organization's Name will provide information for medical evaluation to the licensed physician who evaluates the exposed organization personnel after an exposure incident:
- A. A copy of the OSHA regulation §1910.1030 Bloodborne Pathogens, as necessary
 - B. A description of the exposed organization staff member's duties as they relate to the exposure incident
 - C. Documentation of the route(s) of exposure and circumstances under which exposure occurred
 - D. Results of the source individual's blood testing, if available
 - E. All clinical records relevant to the appropriate treatment of organization personnel, including vaccination status (HBV vaccination status). (Organization's Name's is responsible to maintain these records)
3. Organization's Name will obtain and provide the exposed staff member with a copy of the evaluation and the physician's written opinion within fifteen (15) days of the completion of the evaluation.
- A. The physician's written opinion for HBV exposure will be limited to whether HBV vaccination is indicated for the exposed staff member, and if the exposed organization staff member has received such vaccination.
 - B. The physician's written opinion for post-exposure evaluation and follow-up will be limited to the following information:
 - 1. That the exposed organization staff member has been informed of the results of the evaluation.

2. That the exposed organization staff member has been told about any medical conditions resulting from exposure to blood or other potentially infectious materials which require further evaluation or treatment.
3. All other findings or diagnoses will remain confidential and will not be included in the written report.

Personnel Restriction Due to Suspected or Known Infection

1. Personnel providing patient care who have been exposed to or who exhibit signs and symptoms of potentially transmissible conditions should report this information to their Program Supervisor. The Program Supervisor may exclude these individuals from direct patient contact. These conditions might include, but are not limited to the following:
 - A. Diarrhea: Personnel with diarrhea that is severe or accompanied by other symptoms (such as fever, abdominal cramps, or bloody stools), or lasts longer than twenty-four (24) hours should be excluded from direct patient contact pending evaluation by a physician.
 - B. Herpes Infections: Personnel with primary, recurrent or facial herpes simplex infections should be excluded from direct patient contact with high-risk patients, (e.g., newborns, patients with burns or immunocompromised patients) until the lesions are healed. Personnel with herpes simplex infections of the fingers or hands (herpetic whitlow) should be excluded from direct patient contact until lesions are healed. Personnel with herpes zoster should be excluded from direct patient contact until lesions are healed.
 - C. Respiratory Infections: Personnel with respiratory infections should be excluded from direct patient contact with high-risk patients, (e.g., neonates, patients with chronic obstructive lung disease, or immunocompromised patients).
 - D. Streptococcal Disease: If a Group A streptococcal disease is suspected, appropriate cultures should be taken, and personnel should be excluded from direct patient contact until they have received adequate therapy for twenty-four (24) hours, or until streptococcal infection has been ruled out.
 - E. Other Diseases: Personnel exhibiting symptoms of other infectious diseases may be excluded from direct patient care pending examination by a physician; these include open, draining wounds, conjunctivitis, etc.
2. Management of personnel who are potential carriers—Increases in bacterial infections that can be traced to personnel who are potential carriers should be reported. The identified personnel should be cultured (at no cost to the organization personnel). If cultures are positive, personnel should be excluded from direct patient contact until the pathogen is eradicated or the risk of disease transmission is eliminated.
3. Management of personnel with Tuberculosis (see "[Tuberculosis Exposure Control Plan](#)" Policy No. 6-028)—The management of TB in the workplace will consist of the following areas: medical surveillance (at no cost to the organization personnel), evaluation and management of systematic organization personnel, exposure management, training and information, respiratory protection, and accurate record keeping.

A. Medical surveillance:

1. Where required by state and/or local regulations, all patient care personnel will be given a two (2) step TB skin test and baseline screening at the time of employment if no previously documented TB skin test had been performed in the last twelve (12) months. This will be conducted at no cost to the organization personnel. The tuberculosis test shall be administered by a licensed health care professional who is specifically trained for the procedure.
2. In addition to the TB screening test, all patient care personnel will be given an individual TB risk assessment to establish a baseline. (See "[Individual TB Risk Assessment](#)" Addendum 6-028.A)
3. All patient care personnel will be retested (TST) in accordance with risk determination and state/local regulations. Personnel working with intermediate- to high-risk patients (e.g., HIV+, children, oncology, etc.) will be retested depending upon exposure risk (See "[Tuberculosis Exposure Control Plan](#)" Policy No. 6-028).
4. Employees who present evidence of a previous positive tuberculosis test or that he or she has previously been treated for tuberculosis infection or disease shall be excluded from the tuberculosis screening testing
5. An employee shall not be required to undergo the annual tuberculosis screening requirements if local health officer certifies in writing that less frequent testing may be conducted, and the rationale for less frequent testing is in accordance with applicable federal, state, and local requirements and established professional standards.
6. Personnel already known to have significant reactions should not have a chest X-ray unless they have pulmonary symptoms of TB.

B. Evaluation and management of personnel who are symptomatic, who have a positive TB skin test, or conversion with repeat testing:

1. Personnel with current pulmonary or laryngeal TB whose sputum smear shows tuberculosis bacilli should be excluded from work until adequate treatment has begun or until sputum cultures show no growth.
2. Personnel who have current TB at a site other than the lungs or larynx should be allowed to continue their usual activities.
3. Personnel who discontinue medications for current pulmonary or laryngeal disease before their recommended course of therapy has been completed should not be allowed to work.
 - a. All personnel with significant skin test reactions who do not have current tuberculosis and who have not had previous adequate therapy should be advised to have additional testing and physician follow-up. Return to work will be pending physician release with a written notice.

- b. Personnel who cannot take or do not accept or complete preventive treatment should have their work situations evaluated and may require reassignment. A change in assignment should be considered, if these persons work with high-risk patients.
 - c. Personnel with a history of TB are at risk for recurring infections. These personnel should be instructed to report to their physician for evaluation if TB symptoms develop.
 - 4. Personnel with a newly positive test result (with confirmation from those persons at low risk) should undergo a symptom evaluation and chest radiograph to assess for TB.
- C. Post-exposure prophylaxis:
- 1. After exposure to an active case of TB, all personnel, except those already known to have positive skin test reactions, should be skin tested eight (8) to ten (10) weeks after the exposure. Personnel whose skin tests convert should have a physical examination, appropriate laboratory and radiographic evaluations to determine whether they have infectious TB.
 - 2. Unless a skin test was given during the three (3) months prior to exposure, a baseline skin test should be done as soon as possible after the exposure to assist in interpreting the eight (8) to ten (10) weeks post-exposure skin test.
 - 3. Personnel already known to have significant reactions should not have a chest x-ray unless they have pulmonary symptoms of TB.
 - 4. Any incident involving exposure, including positive TB skin tests, will be recorded on the OSHA 300 form. The locations are:
 - Atlanta, GA (404) 562-2300,
 - Chicago, IL (312) 353-2220,
 - Denver, CO (303) 844-1600,
 - New York, NY (212) 337-2378,
 - San Francisco, CA (415) 975-4310,
 - Boston, MA (617) 565-9860,
 - Dallas, TX (214) 767-4731,
 - Kansas City, MO (816) 426-5861,
 - Philadelphia, PA (215) 596-1201,
 - Seattle, WA (206) 553-5930.
- D. Training and information:
- 1. All personnel will be informed of the signs and symptoms of TB, the hazards of TB transmission, the risk of TB in Organization's Name settings, current statistics about TB in health care workers, and the current CDC guidelines. Training will be conducted annually.
 - 2. Training will include the purpose of TB controls, proper use of protective equipment, medical surveillance procedures, and protocols specific to Organization's Name.

3. Personnel should be counseled about the risk of developing the disease, risks they may pose to their contacts, and instructed to seek evaluation of any signs and symptoms that may be due to TB.

E. Respiratory protection:

1. In accordance with CDC guidelines, all patient care personnel will use a high efficiency particulate air (HEPA) respirator in the following situations:
 - a. When appropriate ventilation is not available and the patient exhibits signs and symptoms that suggest a high potential for infectiousness, or
 - b. The potentially infectious patient is undergoing a procedure that is likely to produce bursts of aerosolized infectious particles (e.g., aerosolized pentamidine administration, sputum induction, and suctioning)
2. All personal protective equipment will be furnished to the appropriate personnel at no cost to the organization personnel. Training in proper usage, fit, and storage will be provided to appropriate personnel.
3. Repeated failure to comply with established policies may result in disciplinary action.

F. Accurate record keeping:

1. Organization's Name will maintain records on appropriate personnel. The files will contain, but will not be limited to, a record of TB skin testing, any personnel exposure incidents, medical evaluation, and treatment, as applicable.
2. Organization's Name will maintain a log of TB infections (positive TST) and any personnel exposure incidents. This information will be recorded on the OSHA Form 300.
3. Each clinical/service record of potentially infectious patients will be tagged according to organization policy to alert all families/caregivers to follow protocols outlined by Organization's Name.

4. Management of personnel exposed to varicella or Herpes Zoster

- A. After exposure to varicella (chicken pox) or Herpes Zoster (shingles), personnel not known to be immune to varicella (by history or serology) should be excluded from work beginning on the twelfth (12th) day after exposure and remain away from work for the maximum incubation period of varicella (twenty-one (21) days).
- B. Personnel who have an onset of varicella should be excluded from work until all lesions have dried and crusted.

5. Management of personnel with hepatitis infections

- A. Personnel who are suspected of being infected with Hepatitis A virus (HAV) should not take care of patients until seven (7) days after the onset of jaundice.
 - B. Personnel at substantial risk of direct contact with body fluids will be offered HBV vaccinations *free of charge*.
 - C. Personnel who are known carriers of HBV surface antigen (HBsAg) should be counseled about precautions to minimize their risk of infecting others and must strictly adhere to standard precautions at all times.
 - 1. Personnel who have no exudative lesions on the hands, who are acutely infected with HBV, are known to be carriers of HBsAg, or have hepatitis non A/non B (NANB) should not be restricted from patient care responsibilities, unless there is evidence of disease transmission.
 - 2. Personnel who have no exudative lesions on the hands, who are acutely infected with HBV, are known to be carriers of HBsAg, or have hepatitis NANB should wear gloves for procedures that involve trauma to tissues or direct contact with mucous membranes or non-intact skin.
 - 3. Personnel with exudative lesions on the hands who are HBsAg positive should either wear gloves for all direct patient contact and when handling equipment that will touch mucous membranes or non-intact skin, or abstain from all direct patient care.
 - D. Personnel who initially decline the HBV vaccination may, at a later date while they are still covered under this requirement, decide to accept the vaccination. Organization's Name will make the HBV vaccination available to them at that time.
 - E. Personnel who decline to accept the HBV vaccination must sign the Hepatitis B Vaccine Declination Statement.
 - F. Where a routine booster dose(s) of HBV vaccine is recommended by the U.S. Public Health Service at a future date, such booster dose(s) will be made available, in accordance with this policy.
6. Management of personnel with HIV
- A. Personnel with impaired immune system resulting from HIV infection should be counseled about the potential risk associated with taking care of patients with transmissible infections.
 - B. Personnel with impaired immune systems from HIV infections are at an increased risk of acquiring or experiencing serious complications of infectious diseases (measles, varicella, hepatitis), and therefore, they should continue to strictly adhere to standard precautions.
 - C. Personnel with impaired immune systems resulting from HIV infection should be informed of precautions to minimize their risk of infecting others and their risk of being infected while carrying out their job responsibilities.

- D. Personnel with exudative lesions or weeping dermatitis should refrain from all direct patient care and from handling patient care equipment until the condition resolves.
 - E. Whether personnel can adequately and safely perform patient care duties should be determined on an individual basis with the decision being made by the organization personnel's physician in conjunction with organization personnel health department and their Program Supervisor. Personnel may be excluded from direct patient contact for high-risk patients (e.g., newborns, patient with burns or immunocompromised patients).
7. Management of personnel following possible exposure to HIV and/or HBV
- A. Serological testing should be done if any personnel are concerned that they may have been infected with HIV. Personnel infected with HIV usually develop the antibody against the virus within six (6) to twelve (12) weeks after infection.
 - 1. If organization personnel has a parenteral (e.g., needlestick or cut) or mucous membrane (e.g., splash to the eye or mouth) exposure to blood or other body fluids or has a cutaneous exposure involving large amounts of blood or prolonged contact with blood—especially when the exposed skin is chapped, abraded, or afflicted with dermatitis—the source patient should be informed of the incident and tested for serologic evidence of HIV and HBV infection after consent is obtained.
 - 2. If the source patient has AIDS/HBV, is positive for HIV/HBV antibody, or refuses to be tested, personnel should be counseled regarding the risk of infection and evaluated clinically and serologically for evidence of HIV/HBV infection as soon as possible after the exposure.
 - 3. Personnel will be advised to report and seek medical evaluation for any acute febrile illness (particularly one characterized by fever, rash, or lymphadenopathy) that occurs within twelve (12) weeks after exposure.
 - 4. HIV seronegative personnel should be retested six (6) weeks post-exposure and on a periodic basis thereafter (e.g., twelve (12) weeks and six (6) months after exposure) to determine whether transmission has occurred. No further follow-up of persons exposed to infection described above is necessary if the source patient is seronegative, unless the source patient is at high-risk of HIV infection. In the latter case, a subsequent specimen (e.g., twelve (12) weeks following exposure) may be obtained from the exposed personnel for antibody testing.
 - 5. Follow-up procedures will be taken for personnel exposed, or potentially exposed, to HBV. The types of procedures depend on the immunization status of the worker and HBV serologic status of the source patient.

RECORD KEEPING
Policy No. 6-002.1**PURPOSE**

To identify contents of personnel health records pertaining to occupational exposure, injuries and illnesses and the OSHA reporting requirements.

POLICY

Organization's Name will establish and maintain an accurate record for personnel with occupational exposure. This record will include, but will not be limited to:

1. A copy of the Infection Identification—Personnel Report
2. A copy of the personnel's HBV vaccination status, including the dates of all the HBV vaccinations and any clinical records relative to personnel's ability to receive vaccinations
3. A copy of all results of examinations, medical testing and follow-up procedures
4. The employer's copy of the physician's written opinion
5. A copy of the information provided to the examining physician

PROCEDURE

1. The organization maintains a record of occupational injuries and illnesses and will make available in the event of an OSHA inspection.
2. If Organization's Name has fewer than ten (10) employees at all times during the last calendar year, the organization is not required to keep an OSHA injury and illness record unless OSHA or the Bureau of Labor Statistics has informed the Executive Director/Administrator or Owner in writing of the requirements.
3. If Organization's Name has more than ten (10) employees at any time during the last calendar year, the Director of Human Resources or designee shall keep an OSHA injury and illness record.
4. Regardless of Organization's Name's size, all work-related incidents resulting in fatality, in-patient hospitalization of one or more employees, an employee amputation or loss of an eye shall be reported to OSHA.
5. The Director of Human Resources or designee shall keep all records of fatalities, injuries and illnesses that is work-related and is a new case.

Note: An injury or illness is considered "work-related" if an event or exposure in the work environment either caused or contributed to the resulting condition or significantly aggravated a pre-existing injury or illness. Work-relatedness is presumed for injuries and illnesses resulting from events or exposures occurring in the work environment.

Note: An injury or illness is considered to be a “new-case” if the employee has not previously experienced a recorded injury or illness of the same type that affects the same part of the body or the employee previously experienced a recorded injury or illness of the same type that affected the same part of the body but had recovered completely (all signs and symptoms had disappeared) from the previous injury or illness and an event or exposure in the work environment caused the signs or symptoms to reappear.

6. The Director of Human Resources or designee shall determine whether an injury or illness meets the general recording criteria if it results in any of the following:
 - A. Death
 - B. Days away from work
 - C. Restricted work or transfer to another job
 - D. Medical treatment beyond first aid
 - E. Loss of consciousness.
7. The Director of Human Resources or designee shall also consider if it meets the general recording criteria if it involves a significant injury or illness diagnosed by a physician or licensed health care professional.
8. The following shall be recorded on the OSHA 300 Log:
 - A. All work-related needlestick injuries and cuts from sharp objects that are contaminated with another person's blood or potentially infectious materials.
 - B. If an employee is medically removed under medical surveillance.
 - C. If an employee experiences occupational hearing loss
 - D. If an employee has been occupationally exposed to anyone with a known case of active tuberculosis and that employee develops a tuberculosis infection.
 1. The employee shall have evidence of a positive skin test or diagnosis by a physician or other licensed health care professional.
 2. The “respiratory condition” column shall be checked.
9. The employee's name shall not be entered on the OSHA 300 Log for certain privacy concern cases. Privacy concern cases include:
 - A. An injury or illness to an intimate body part of the reproductive system;
 - B. An injury or illness resulting from sexual assault
 - C. Mental illnesses

- D. HIV infection, hepatitis, or tuberculosis;
 - E. Needlestick injuries and cuts from sharp objects that are contaminated with another person's blood or other potentially infectious materials
 - F. Other illnesses, if the employee voluntarily requests that their name not be entered on the log.
10. Each employee injury and illness will be reported on the "Log of Work-Related Injuries and Illnesses" (OSHA Form 300) or an equivalent form and will prepare a supplementary "Injury and Illness Incident Report" (OSHA Form 301) or equivalent that provides additional details about each case reported on the "Log".
 11. All recordable injury or illnesses shall be entered on the OSHA 300 Log or 301 Incident Report within seven (7) calendar days of receiving information that the injury or illness has occurred.
 12. Within eight (8) hours after the death of any employee as a result of a work-related incident, the Director of Human Resources shall report the fatality to OSHA.
 13. Within twenty-four (24) hours after the in-patient hospitalization of one or more employees or an employee's amputation or an employee's loss of an eye, as a result of a work-related incident, the Director of Human Resources shall make a report to OSHA.
 14. The report of a fatality, inpatient hospitalization, amputation or loss of an eye shall be completed by one of the following methods:
 - A. Telephone or in person to the OSHA Area Office that is nearest to the site of the incident.
 - B. By telephone to the OSHA toll-free central telephone number, 1-800-321-OSHA.
 - C. By electronic submission using the reporting application located on OSHA's public Web site at www.osha.gov
 15. The report of a fatality, inpatient hospitalization, amputation or loss of an eye shall include the following:
 - A. The organization name
 - B. The location of the work-related incident
 - C. The time of the work-related incident
 - D. The type of reportable event (i.e., fatality, in-patient hospitalization, amputation, or loss of an eye)
 - E. The number of employees who suffered a fatality, in-patient hospitalization, amputation or loss of an eye

- F. The names of employees who suffered a fatality, in-patient hospitalization, amputation or loss of an eye.
 - G. Your contact person and his phone number; and
 - H. A brief description of the work-related incident.
16. Annually, the organization is required to prepare and post a "Summary of Work-Related Injuries and Illnesses" (OSHA Form 300A) in a visible location in the workplace (main office and branches).
 17. The organization shall maintain the OSHA 300 Log, the privacy case list (if one exists), the annual summary and the OSHA 301 Incident Report Forms for five (5) years following the end of the calendar year that the records cover.
 18. Each employee or authorized employee representative, upon hire and annually, shall be oriented on how to report a work-related injury or illness, how to access injury and illness records.
 - A. If an employee, former employee, or personal representative requests a copy of the OSHA 301 Incident Report form, it shall be provided to the employee or former employee by the end of the next business day.
 - B. If an authorized employee representative asks for copies of the OSHA 301 Incident Reports for an establishment where the agent represents employees under a collective bargaining agreement, you must give copies of those forms to the authorized employee representative within seven (7) calendar days.
 19. The organization shall not discriminate against any employee for reporting a work-related injury or illness.
 20. If the organization has 250 or more employees, the organization is required to electronically submit information from the records of occupational injuries and illnesses to OSHA or OSHA's designee on an annual basis.
 21. Organization's Name will ensure that personnel health records are kept confidential and are not disclosed or reported without personnel's express written consent to any person, within or outside the workplace, except as required by law.
 22. All medical information and records as listed above will be maintained for the duration of personnel's employment plus thirty (30) years.

**OCCUPATIONAL EXPOSURE
INFORMATION AND TRAINING
Policy No. 6-003.1****PURPOSE**

To outline methods to inform and train personnel regarding occupational exposures.

POLICY

The organization will provide information and training for all personnel at risk for occupational exposure.

PROCEDURE

1. All personnel at risk for occupational exposure will participate in a training program. This training will be:
 - A. Provided at no cost to organization personnel and during working hours
 - B. Provided during orientation, prior to the time of initial assignment to tasks where occupational exposure may take place
 - C. Provided annually thereafter, within one (1) year of their previous training
 - D. Provided within ninety (90) days after the effective date of a bloodborne pathogen exposure
2. Organization's Name will provide additional training when changes, such as modification of tasks or procedures or institution of new tasks or procedures, affect the organization personnel's occupational exposure. The additional training may be limited to addressing the new occupational exposures.
3. The training program will consist of material appropriate in content and vocabulary to educational level, literacy, and language ability of the organization personnel being trained. It will contain, at a minimum, the following elements:
 - A. Distribution of a copy of the regulatory text on the bloodborne pathogen standard, and an explanation of its content to each applicable employee
 - B. A general explanation of the epidemiology and symptoms of bloodborne diseases, and the modes of transmission of bloodborne pathogens
 - C. An explanation of the organization's exposure control plan, and the means by which personnel can obtain a copy of the written plan

- D. An explanation of the appropriate methods for recognizing tasks and other activities that may involve exposure to blood and other potentially infectious materials
 - E. An explanation of the use and limitations of methods that will prevent or reduce exposure, including appropriate engineering controls, work practices, and personal protective equipment
 - F. Information on the types, basis for selection, proper use, location, removal, handling, decontamination and disposal of personal protective equipment
 - G. Information on the HBV vaccine, including information on its efficacy, safety, method of administration, the benefits of being vaccinated, and that the vaccine and vaccination will be offered free of charge
 - H. Information on appropriate actions to take and persons to contact in an emergency involving blood or other potentially infectious materials
 - I. An explanation of the procedure to follow if an exposure incident occurs, including the method of reporting the incident and the medical follow-up that will be made available
 - J. Information on the post-exposure evaluation and follow-up that Organization's Name is required to provide for personnel following an exposure incident
 - K. An explanation of the signs, labels, and/or color-coding required by law and regulation
 - L. An opportunity for interactive questions and answers with the person conducting the training session
 - M. The person conducting the training will be knowledgeable in the subject matter as it relates to the special workplace that the training will address
4. The organization will maintain training records for three (3) years from the date training occurred for all personnel, including, but not limited to:
- A. Date, contents, and summary of training sessions
 - B. Names and qualifications of persons conducting training
 - C. Names and job titles of all persons attending training sessions

STANDARD PRECAUTIONS**Policy No. 6-004.1****PURPOSE**

To reduce the risk of exposure to and transmission of infections when caring for patients.

POLICY

Organization personnel will adhere to the following precautions and will instruct patients and family/caregivers in infection control precautions, as appropriate to the patient's care needs.

Note: Patients may be given a copy of this procedure, if needed as a teaching tool.

Note: Organization's Name has the right to limit the practice of organization personnel, if patient safety is in question.

Definition

Under standard precautions, blood and certain body fluids of all patients are considered potentially infectious for bloodborne pathogens, such as human immunodeficiency virus (HIV), and hepatitis B virus (HBV). Standard precautions apply to blood and other body fluids potentially containing blood or bloodborne pathogens. These body fluids include emesis, sputum, feces, urine, semen, vaginal secretions, cerebrospinal fluid (CSF), synovial fluid, pleural fluid, pericardial fluid, and amniotic fluid. Standard precautions should be used with other fluids, such as nasal secretions, saliva, sweat, and tears when they contain visible blood or other potentially infectious materials and it is impossible to differentiate between body fluids.

PROCEDURE***General Precautions******Hand Hygiene***

1. Hand hygiene will be performed to prevent cross-contamination between the patient and personnel. (See also "[Hand Hygiene](#)" Policy No. 6-006.)
2. When hands are visibly dirty, contaminated with proteinaceous material, or are visibly soiled with blood or other body fluids, wash hands with either a non-antimicrobial or antimicrobial soap and water.
3. When hands are not visibly soiled, use an alcohol-based hand rub for routinely decontaminating hands.
4. An alternative to use of an alcohol based hand rub is to wash hands with an antimicrobial soap and water.

Personal Protective Equipment

(See "[Personal Protective Equipment](#)" Policy No. 6-005.)

1. Gloves:

A. The use of gloves (intact latex or vinyl of appropriate size and quality) is important when personnel has cuts, abraded skin, chapped hands, dermatitis, etc. Gloves are to be worn when:

1. There is actual or potential contact with blood or other potentially infectious materials
2. Contact with non-intact or abraded skin is anticipated
3. Touching contaminated items or surfaces
4. Performing invasive procedures
5. Handling any drainage appliance
6. Taking a rectal temperature
7. Shaving a patient with a safety razor
8. Obtaining laboratory specimens
9. Patients have active bleeding
10. Cleaning of body fluids and decontamination procedures
11. Performing wound care
12. Entering the room of, or providing care for, patients who are colonized or infected with vancomycin-resistant enterococci or multidrug-resistant *Staphylococcus aureus* (MRSA)
13. Handling soiled linen

B. Sterile gloves are to be worn for sterile procedures.

C. Gloves are to be changed:

1. Between tasks and procedures on the same patient
2. During changing or cleaning an incontinent patient
3. After removing an old dressing
4. When the integrity of the glove is in doubt

- D. Gloves should never be washed or disinfected for reuse.
 - E. General purpose utility gloves (e.g., rubber household gloves) will be used for housekeeping chores involving potential blood contact and for instrument cleaning and decontamination procedures. Utility gloves may be decontaminated and reused, but should be discarded if they are peeling, cracked, or discolored, or if they have punctures, tears, or other evidence of deterioration.
 - F. Gloves are not necessarily needed for general care or during casual contact, such as bathing of intact skin or assisting with ambulation.
 - G. Gloves are to be worn by the family/caregiver when direct contact with any body substance is anticipated (blood, urine, pus, feces, saliva, drainage of any kind.)
2. Gowns:
- A. The use of gowns is required when splashes to the skin and/or clothing are likely or when caring for patients with epidemiologically important microorganisms, such as multi-drug resistant organisms.
 - B. The gowns will be made of or lined with fluid-proof or fluid-resistant material and will protect all areas of exposed skin. The type and characteristics will depend on the task and degree of exposure anticipated.
3. Mask/Protective Eyewear:
- A. Masks, protective eye wear, or face shields are required when contamination of mucosal membranes, eyes, mouth, or nose is possible, such as splashes or aerosolization of material.
 - B. They are not required for routine care.
 - C. A NIOSH-certified N95 respiratory mask must be worn when caring for patients with suspected or confirmed *Mycobacterium Tuberculosis*.
4. Resuscitation Equipment:
- A. One (1)-way valve pocket masks, resuscitation bags, or other ventilation devices will be provided to personnel where the need for emergency mouth-to-mouth resuscitation would be required.

Sharps

- 1. After use, needles and other sharps will be placed directly into a puncture-proof container located in the immediate patient care area. Needles must not be recapped, bent, broken, or clipped.
- 2. Whenever possible, needleless protective devices will be utilized in the provision of patient care. Appropriate personnel will be involved in the selection of these products.

Laboratory Specimens

1. Laboratory specimens should be transported in a Ziplock bag or other leak-proof container.
2. The leak-proof container should be transported to the office or alternate lab site in a puncture-resistant container that is properly labeled.
3. Specimens transported to the office will be placed in a designated storage container located in the dirty supply area.

Labels

1. Biohazard labels will be used to prevent accidental injury or illness to personnel exposed to hazardous or potentially hazardous conditions that are out of the ordinary, unexpected, or not readily apparent.
2. Labels will state—BIOHAZARD—or the hazard symbol, readable at the minimum distance of five (5) feet.
3. Personnel will be informed as to the meaning of the labels.
4. Labels will be affixed as close as possible to respective hazards.
5. Labels will be used to identify equipment, containers, refrigerators, and rooms containing hazardous agents.
6. If labels are not used, other effective means will be used, such as RED bagging.

Housekeeping and Hygiene

1. Housekeeping procedures at Organization's Name's location will be implemented to ensure that the worksite is maintained in a clean and sanitary condition. The following guidelines will be implemented at Organization's Name's office. These same guidelines will be implemented and taught to patients and family/caregivers. Organization's Name recognizes that patients have a right to refuse to follow these guidelines.
 - A. Organization's Name will ensure that the worksite is maintained in a clean and sanitary condition. The organization will determine and implement an appropriate written schedule for cleaning and decontamination based upon the location within the facility; type of surface to be cleaned; and tasks or procedures to be performed in the area. All equipment, environmental and working surfaces shall be cleaned and decontaminated after contact with blood or other potentially infectious materials.
 - B. An appropriate disinfectant (e.g., household bleach 5.25% mixed 1:10 with water) should be used to clean floors, toilet bowl, tub, shower, sink, countertops, and soiled furniture.
 1. This solution will be discarded after each use, or at least every twenty-four (24) hours.

- C. Sponge and mops used to clean up body fluid spills should not be rinsed out in the kitchen sink or used where food is prepared.
- D. Dirty mop water should be poured down the toilet, rather than the sink.
- E. Rooms will be kept well aired to decrease the risk of colds, flu and other airborne communicable disease.
- F. Infectious organisms may be found in animal wastes, birdcages, cat litter boxes, and fish tanks. They should be maintained by someone other than a person with HIV disease or other causes of immunosuppression.
- G. Humidifiers and air conditioners can harbor infectious organisms, and should be cleaned and serviced regularly.
- H. All bins, pails, cans (e.g., waste cans) intended for reuse which have a reasonable likelihood for becoming contaminated with blood and other potentially infectious materials, will be inspected and decontaminated weekly. They will also be cleaned and decontaminated immediately, or as soon as feasible, upon visible contamination.

2. Blood/Body Fluid Spills

- A. Blood/body fluid spills should be mopped or wiped up using disposable towels or wipes with hot soapy water, then disinfected with bleach. If the cleanup is done by hand, disposable gloves must be worn.
- B. Disposable towels or wipes used in the cleanup should be bagged to prevent leaking and exposure to others. A heavy-duty plastic bag should be used for bagging this type of waste with double bagging. The bag should be disposed in accordance with local and state regulations.

3. Hygiene

- A. Personal items, such as toothbrushes, razors, and enema equipment, should never be shared.
- B. Maintaining a state of personal cleanliness is the key to reducing infection transmission from person to person. This includes bathing regularly, washing hands after use of bathroom facilities, after contact with one's own body fluids, and before preparing food.

Waste Disposal

(See also, "[Hazardous Waste Handling](#)" Policy No. 6-011 and "[Contaminated Waste Disposal](#)" Policy No. 6-010.)

1. General Waste:

- A. Materials not contaminated or visible soiled with blood or other infectious waste, such as diapers, incontinence pads, non-soiled PPE, dressing wrappers, or IV tubing not used for blood administration.

- B. General waste should be disposed of in a securely fastened plastic bag and can be placed into the patient's trash receptacle.
- 2. Regulated Medical Waste:
 - A. According to OSHA guidelines, these may include:
 - 1. Liquid or semi-liquid blood or other potentially infectious material
 - 2. Contaminated items that would release blood or other potentially infectious materials in a liquid or semi-liquid state if compressed
 - 3. Items that are caked with blood or other potentially infectious materials and are capable of releasing these materials during handling
 - 4. Pathological and microbiological wastes containing blood or other potentially infectious material
 - B. Place regulated medical waste into a leak-proof, heavy duty, securely fastened plastic bag. Items should be double bagged when the potential for contamination of the outside of the first bag is present. Items should also be double bagged when the first bag may be at risk for tearing. They should be disposed of according to local and/or state regulation.
- 3. Syringe/Sharps Disposal:
 - A. In the home setting, a sharps disposal container will be available for use by the clinician or patient and family/caregiver. The uncapped needle will be placed directly into the disposal container. When 3/4 full, the disposal container will be sealed and transported to the dirty supply area located in the office. A new container should be provided to the patient for any future use.
 - B. If the patient is self-administering medications and generating one (1) or more syringes per day, the patient should notify the garbage collection service, or the local Department of Public Health, to obtain information on local and/or state regulations for proper disposal.

Laundry

- 1. Handling and Changing of Linens:
 - A. Contaminated laundry should be handled as little as possible with minimal agitation.
 - B. Towels and washcloths should not be shared by different users.
 - C. Gloves and other appropriate personal protective equipment are to be worn when handling soiled linen.

- D. Soiled clothing and linens should be soaked as promptly as possible. Ideally, they should be machine washed in hot (160° F) soapy water. If appropriate, (e.g., colorfast material), a cup of bleach may be added to the water. If low temperature (less than 150° F) laundry cycles are used, chemicals suitable for low-temperature washing at proper use concentration should be used.
- E. When contaminated laundry is wet and likely to soak through or leak from the bag to the container, the laundry should be transported in containers or bags that prevent leakage to the exterior.
- F. Laundry and linens should be carried away from the body.

Equipment/Nondisposable Instruments

1. Bedpans/Urinals/Commodes:

- A. Bedpans and urinals should be used by only one (1) patient and should be cleaned on a regular basis with household detergent.
- B. Shared commodes do not require special precautions unless blood, contaminated body substance, or fluid is present. If soiled, the commode should be cleaned with a 1:10 dilution of bleach.

2. Thermometers:

- A. Thermometers are not supplied by Organization's Name, but may be owned by patients.
- B. Electronic thermometers with disposable sheaths need no special precautions unless they become visibly soiled. When thermometers are soiled, they should be wiped with a disinfectant solution.
- C. Glass thermometers used in the home should be rinsed with soap and water before and after use. If the thermometer will be used by more than one (1) family/caregiver member, it should be soaked in 70–90% ethyl alcohol for thirty (30) minutes followed by a rinse under a stream of water in between users.

3. Medical Equipment/Supplies:

- A. Any nondisposable equipment or supply utilized by the clinician in the provision of care will be disinfected by thoroughly wiping down with an organization approved disinfectant before use with another patient. Examples would be but not limited to:
 - 1. TENS unit
 - 2. Ultrasound equipment
 - 3. Pulse Oximeter finger clip
 - 4. Nylon BP cuffs

5. Stethoscope

- B. Any nondisposable equipment returned to organization stock will be transported in a designated dirty area in trunk and then will be placed in the dirty supply area and then thoroughly wiped down with an organization-approved disinfectant. After proper cleaning, the equipment may be returned to stock for patient use.
- C. In the event a nondisposable piece of equipment comes in contact with blood or body fluids, a 1:10 dilution of bleach or other organization-approved disinfectant is used to clean it. Soiled blood pressure cuffs will be washed in hot, soapy water.
- D. Dressing supplies contaminated with the patient's blood or body fluids should be double bagged in plastic bags, tied securely, and labeled "contaminated" then placed with household trash for garbage pickup (according to local and state regulations).

Kitchen/Food Preparation

1. Hand washing: Proper hand washing techniques should be observed prior to touching food.
2. General hygiene: "Tasting" of food during cooking should be done with a new, clean spoon each time. Wash the spoon with soap and water immediately after "tasting."
3. Cleaning of kitchen: Counters, sinks, and floors in the kitchen should be free from food particles and cleaned with a disinfectant regularly.
4. Refrigerator: The interior of the refrigerator should be cleaned with soap and warm water regularly to control molds.
5. Food freshness: Observe expiration dates and general freshness of food. Do not use cracked eggs due to the likelihood of Salmonella contamination.
6. Food storage: Store open packages of food (e.g., sugar) in covered containers to discourage infestation.
7. Food preparation: Pork, poultry, and eggs should be thoroughly cooked before eating. Porous (e.g., wood) cutting boards used for poultry should not also be used for fruits and vegetables.
8. Dishes/utensils: Wash dishes and utensils in hot soapy water. The water should be hot enough to require the use of lined gloves. Allow dishes to air dry. Known infected persons do not need separate dishes or utensils provided they are washed as described.
9. Sponges: Sponges used to clean in the kitchen should not be the same sponges used to clean bathrooms and body fluid spills. Sponges used to clean bathrooms and body fluid spills should be disinfected with bleach and changed periodically.

Special Considerations for a Person with HIV Disease

1. Unpasteurized milk, raw eggs, or products containing raw eggs or cracked or non-intact eggs should be avoided. They have been associated with Salmonella infections and may be problematic, especially for the person with HIV disease or other immune-suppressed diseases.
2. All fresh produce should be washed thoroughly.

Other Considerations

1. Eating, drinking, smoking, applying makeup or lip balm, or handling contact lenses should be avoided in work areas where there is a reasonable chance of exposure.
2. Sterile technique will be employed for sterile dressing changes, IV insertion, IV site care, phlebotomy, tracheal suctioning, insertion of a urinary catheter, and whenever appropriate to prevent infection.
3. Disinfectants:
 - A. HIV is inactivated rapidly after being exposed to chemical germicides.
 - B. HIV can be inactivated after exposure for ten (10) minutes to any of the following:
 1. Chlorine bleach (1:10 dilution)
 2. Alcohol (70–95%)
 3. Quaternary Ammonium (TRI-GUAT)
 4. Phenolic (Vesphene II)

ADDENDUM 6-004.A

STANDARD PRECAUTIONS INFORMATION FOR PERSONNEL

All personnel should be made aware of the following housekeeping requirements of the OSHA standard on bloodborne pathogens:

1. Decontamination of Surfaces

- [] Immediately after completion of procedures.
- [] Immediately after end of work shifts.
- [] Immediately after becoming overtly contaminated with blood or other potentially infectious materials.

2. Protective Covering of Equipment and Environmental Surfaces

- [] Protective covering (plastic wrap, aluminum foil, imperviously-backed absorbent paper).
- [] Remove and replace at end of work shift.
- [] Replace when overtly contaminated with blood or other potentially infectious materials.

3. Decontamination of Equipment

- [] Routinely check for contamination.
- [] Decontaminate when contaminated with blood or other potentially infectious materials.
- [] Decontaminate prior to servicing or shipping.

4. Decontamination of Receptacles

- [] Inspect, clean, and disinfect on a regularly scheduled basis any reusable bins, pails, cans and similar receptacles which have a potential of becoming contaminated.
- [] Clean and decontaminate immediately, or as soon as possible, when visibly contaminated.

5. Clean Up

- [] Do not use hands to pick up broken glassware, which may be contaminated.
- [] Use mechanical means (brush and dustpan, tongs, or forceps) to pick up potentially contaminated broken glassware.

6. Handling of Specimens

- [] Place in a closeable, leakproof container prior to storage or transport.
- [] Color-code or label specimens according to OSHA standard on bloodborne pathogens.
- [] If it is likely that the primary container will be contaminated, place a second leakproof container over first container.
- [] If it is likely that the primary container will be punctured, place primary container in a leakproof, puncture-resistant secondary container.
- [] Color-code or label second container in same manner as primary container.

7. Reusable Items

- [] Decontaminate prior to washing or reprocessing if contaminated with blood or other potentially infectious materials.

8. Handling of Infectious Waste

- [] Place in closeable, leakproof containers or bags prior to disposal.
- [] Color-code or label containers or bags according to the OSHA standard.
- [] Place a second closeable, leakproof container or bag over the outside of the first container or bag if it is likely outside contamination of the primary container or bag will occur.
- [] Close and color-code or label the secondary container or bag in same manner as primary container.
- [] Observe all federal, state, and local laws when disposing of infectious waste.
- [] Dispose of sharps immediately after use.
- [] Dispose of sharps in a closeable, puncture-resistant, disposable container that is leakproof on sides and bottom.
- [] Label sharps disposal containers according to the OSHA standard.
- [] Make sharps disposal containers easily accessible in immediate area of sharps use. Routinely replace sharps disposal containers.
- [] Do not allow sharps disposal container to overfill.

9. Handling of Laundry

- [] Treat laundry that is contaminated with blood or other potentially infectious materials as if contaminated.
- [] Handle such laundry as little as possible and minimize agitation of laundry.
- [] Bag contaminated laundry at area of use.
- [] Do not sort or rinse contaminated laundry in patient areas.
- [] Label or color-code bags in which contaminated laundry is placed and transported.
- [] Place and transport contaminated laundry in a leakproof bag if it is wet or presents a potential for soak-through or leakage from the bag.
- [] Ensure that laundry workers wear protective clothing and other personal protective equipment to prevent occupational exposure during handling and sorting of laundry.

PERSONAL PROTECTIVE EQUIPMENT**Policy No. 6-005.1****PURPOSE**

To define personal protective equipment requirements and indications for use in patient care.

POLICY

Organization's Name will supply and make accessible appropriate personal protective equipment consistent with the tasks being performed. Organization's Name will provide guidelines (see "[Protective Device Checklist](#)" Addendum 6-005.A) to assist organization personnel in selecting appropriate personal protective equipment.

Note: Personal protective equipment for eyes, face, head, and extremities, protective shields and barriers reduce the incidence of contamination but cannot prevent penetrating injuries due to needles and other sharp instruments.

PROCEDURE

1. On assignments where personnel have a risk of occupational exposure, the organization will furnish, repair, clean, and launder, at no cost to personnel, appropriate personal protective equipment so that barrier precautions can be observed and compliance with this policy can be maintained.
2. Personal protective equipment includes, but is not limited to, powderless gloves, gowns, laboratory coats, face shields or masks, eye protection, mouthpieces, resuscitation masks/devices, pocket masks, or other ventilation devices.
3. Personal protective equipment will be considered appropriate only if it does not permit blood or other potentially infectious materials to pass through to or reach the organization personnel's work clothes, street clothes, undergarments, skin, eyes, mouth, or other mucous membranes under normal conditions of use and for the duration of time which the protective equipment will be used.
4. All personnel must use appropriate personal protective equipment when exposed to blood or other potentially infectious materials.
5. This equipment will be readily accessible at the work site or will be issued to personnel prior to assignments where personal protective equipment is needed.
6. If, in rare and under extraordinary circumstances, organization personnel decline to use personal protective equipment for brief and temporary periods, they must do so only when in their professional judgment, in that specific instance, its use would have prevented the delivery of health care or public safety services or would have posed an increased hazard to the safety of themselves or a coworker.

7. When this occurs, Organization's Name will investigate the circumstances involved in order to determine whether changes can be instituted to prevent such occurrences.

8. Garments penetrated by blood or other potentially infectious materials, will be removed immediately, or as soon as feasible.
9. All personal protective equipment will be removed prior to leaving the work area.
 - A. When removed, it will be placed in an appropriately designated container or area for storage, washing, decontamination, or disposal.
10. Powderless gloves will be worn when it can be reasonably anticipated that personnel may have contact with blood or other potentially infectious materials; when performing vascular access procedures; and when handling or touching contaminated items or surfaces.
 - A. Any variety of powderless latex or vinyl glove is acceptable as long as they are impervious to liquids and strong enough to withstand the rigors of the tasks to be performed.
11. Personnel who are allergic to latex or vinyl gloves should notify their Program Supervisor so that hypoallergenic gloves, glove liners, or other similar alternatives may be provided to them.
12. Gloves must not be so large and loose fitting as to easily slip off during use.
13. Disposable gloves will be removed and discarded after contact with each person, fluid item, surface, if torn or punctured, or when their ability to function as a barrier is compromised.
14. Hands must be decontaminated immediately, or as soon as feasible, after removal of gloves or any other personal protective equipment.
15. A new set of gloves will be used for contact with each patient.
16. Gloves may not be washed or disinfected for reuse.
17. When utility gloves are used, they may be decontaminated for reuse as long as the integrity of the glove is not compromised.
 - A. However, they must be discarded if they are cracked, peeling, torn, punctured, or exhibit other signs of deterioration or when their ability to function as a barrier is compromised.
18. Masks, in combination with eye protection devices, such as goggles or glasses with solid side shields, or chin length face shields, and gowns or aprons must be worn for all tasks or procedures which are likely to generate droplets, sprays, spatters or splashes of blood, or other potentially infectious materials and where eye, nose, or mouth contamination can be reasonably anticipated.

19. Appropriate protective clothing, such as gowns, aprons, lab coats, clinic jackets, or similar outer garments, will be worn in occupational exposure situations.
 - A. The type and characteristics will depend upon the task and degree of exposure anticipated.
20. Appropriate protective clothing, such as gowns, aprons, lab coats, clinic jackets, or similar outer garments, will be worn when administering hazardous medications.
21. Personnel whose tasks include participation in CPR must use a one (1)-way valve mask when performing mouth-to-mouth resuscitation.
 - A. The masks will be provided by Organization's Name.
22. Linen, clothing, or other materials that are visibly contaminated with blood, body fluids, or other infectious materials must be placed in bags or containers that are impervious to moisture before transport for cleaning.
 - A. Gloves must be worn while bagging these materials.

ADDENDUM 6-005.A

PROTECTIVE DEVICE CHECKLIST

PROTECTIVE DEVICE CHECKLIST

Protective devices are usually required during the following patient care activities. The family/caregiver should assess the individual patient situation to determine if additional protective devices are required.

Patient Care Activities	Protective Devices				
	HAND Washing	Gloves	Gown/ Plastic Apron	Mask	Eye Protection
Bagged specimen handling	X				
Bed change, visibly soiled	X	X	S		
Blood gases	X	X			
Blood glucose monitoring	X	X			
Care of patients with vomiting/diarrhea	X	X	S		
Clean up of incontinent patient—feces/urine	X	X	X		
Collecting specimens—stool, urine, sputum, wound	X	X			
Coughing patient, forceful and/or productive—direct contact	X			X	
Denture handling	X	X			
Diaper change	X	X			
Direct contact with blood/body substance	X	X			
Ear oximetry	X				
Enema	X	X	S		
Equipment cleaning	X	X	S		
Fecal impaction removal	X	X	S		
Feeding	X				
Foley irrigation	X	X			
Gastric lavage	X	X	S		
Inserting rectal suppository	X	X			
Inserting vaginal suppository	X	X			
Keto urine checks	X	X			
Medication administration:					
Oral	X	X			
IV piggyback	X	X			
IV direct or into hub of catheter	X	X			
Nasotracheal or endotracheal suctioning	X	X	S	X	X
NG tube placement	X	X	S		
Oral exam with hand	X	X			
Oral exam with tongue blade	X	X			
Oral/nasal care	X	X		M	M

Legend: X = routinely S = If soiling likely M = If splattering likely

PROTECTIVE DEVICE CHECKLIST

Patient Care Activities	Protective Devices				
	Hand Washing	Gloves	Gown/ Plastic Apron	Mask	Eye Protection
Oral suctioning	X	X	M		
Ostomy care, irrigation, emptying bag	X	X	S		
Oxygen cannula or mask placement	X				
Physical assessment	X	S			
Post-mortem care	X	X	S		
Postural drainage	X	X	S		
Pressure to control bleeding	X	X	S	M	M
Rectal temperature	X	S			
Routine bath	X	S			
Routine breathing treatment	X	S		X	
Shaving	X				
Sitz bath	X				
Soiled equipment handling	X	X			
Sputum induction	X	X		X	X
Traction	X				
Tube feeding	X				
Vaginal irrigation	X	X	S		
Venipuncture opening specimens	X X	X X	S	X	X
Ventilatory tubing changes	X	X	S		
Vital signs—oral temperature, pulse, respiration, blood pressure	X				
Washing hair	X				
Wound care	X				
Dressing change—burn		X	S		
Dressing change—lg. amt. drainage	X				
Dressing—routine	X	X			
I & D of abscess	X	X	S	M	M
Suture/staple removal—clean/dry	X	X			
Suture/staple removal—drainage	X	X	S	M	M
Topical ointment to lesion	X	X			
Tracheostomy care	X	X	S	M	M
Wound irrigation	X	X	S		
Wound packing		X	S	M	M

Legend:

X = routinely

S = If soiling likely

M = If splattering likely

ADDENDUM 6-005.B

REQUIRED PERSONAL PROTECTIVE EQUIPMENT FORM

REQUIRED PERSONAL PROTECTIVE EQUIPMENT FORM

(Organization's Name)

Required Personal Protective Equipment

Name: _____ Date: _____

Job Title: _____

Task: _____

☐ Sterile Medical Gloves

☐ Utility Gloves

☐ Gown

☐ Mask

☐ Eye Protection

☐ Other (Specify) _____

☐ Non-sterile Medical Gloves

☐ Lab Coat or Apron

☐ Face Protection

Position Tasks: _____

Other Tasks Performed: _____

Copy to Organization Personnel: _____

HAND HYGIENE
Policy No. 6-006.1**PURPOSE**

To prevent cross-contamination and home care-acquired infections.

POLICY

Personnel providing care in the home setting will regularly wash their hands, per the most recently published CDC regulations and guidelines for hand hygiene in healthcare settings.

When hands are visibly dirty, contaminated with proteinaceous material, or are visibly soiled with blood or other body fluids, they should be washed with either a non-antimicrobial or antimicrobial soap and water.

When hands are not visibly soiled, they should be washed using an alcohol-based hand rub for routinely decontaminating hands. An alternative to use of an alcohol-based hand rub is to wash hands with an antimicrobial soap and water.

PROCEDURE***Hand decontamination with an alcohol-based hand rub***

Equipment: Organization-approved, alcohol-based hand rub which conforms to CDC Guideline for Hand Hygiene.

1. Apply alcohol-based hand rub product to palm of one (1) hand and rub hands together, covering all surfaces of hands and fingers (including under nails) until hands are dry.
2. The volume of product needed to reduce the number of bacteria on hands will vary by product, so product directions must be followed.
3. Hand decontamination using an alcohol-based hand rub should be performed:
 - A. Before having direct contact with patients
 - B. Before donning sterile gloves when performing sterile procedures; before inserting indwelling urinary catheters, peripheral vascular catheters, or other invasive devices
 - C. After contact with a patient's intact skin (when taking a pulse, blood pressure or lifting a patient)

- D. After contact with body fluids or excretions, mucous membranes, non-intact skin, and wound dressings, if hands are not visibly contaminated.
 - E. When moving from a contaminated body site to a clean body site during patient care.
 - F. After contact with inanimate objects (including medical equipment) in the immediate vicinity of the patient.
 - G. After removing gloves.
4. At any time, personnel may choose to wash their hands with soap and running water in addition to using the alcohol-based hand rub, especially if personnel feel there is a "build-up" on the hands after repeated use of the alcohol-based hand rub.

Hand Washing with Soap and Water

Equipment: Paper towels and liquid soap; if water and liquid soap are not available, waterless hand washing products will be used.

1. Wet hands and apply the soap, and rub hands together vigorously; avoid use of hot water because repeated exposure to hot water may increase risk of dermatitis.
2. Wash hands for at least fifteen (15) seconds covering all surfaces of the hands and fingers.
3. Rinse with warm water and dry the hands with a disposable towel from the fingers toward the forearm.
4. Use a dry disposable towel to turn off faucet.
5. Hand washing using soap and water should be performed:
 - A. Before eating
 - B. After using the restroom
 - C. When hands are visibly dirty or contaminated
 - D. If exposure to *Bacillus anthracis* is suspected or proven

CLEAN vs. ASEPTIC TECHNIQUE**Policy No. 6-007.1****PURPOSE**

To define the difference between an aseptic (near sterile) and clean environment for patient-care-related procedures.

POLICY

All personnel will use the appropriate technique for the procedure being performed.

PROCEDURE*Clean Technique*

Clean technique refers to the infection control strategies utilized to reduce the number of microorganisms or to reduce or prevent the transmission of microorganisms from one (1) person to another.

Clean technique includes the following strategies:

1. Keep the work area clean with an appropriate disinfecting or cleaning solution.
2. Wipe the area with alcohol or the disinfectant solution prior to performing the procedure.
3. Decontaminate the hands using an alcohol-based hand rub before and after the procedure.
4. Incorporate the use of barrier precautions
5. Clean the area after the procedure.
6. Keep traffic in the area to a minimum, if possible.
7. Avoid direct air currents to the area from open windows, doors, or heat or air conditioning vents.
8. Remember that anything around the clean work area is considered "dirty," including personal clothing.
9. If unsure if an item is clean, throw it out or clean it prior to use.

Aseptic or Sterile Technique

Aseptic, or near-sterile, technique refers to infection control strategies used to render and maintain objects and areas maximally free of microorganisms.

Aseptic technique includes the following strategies:

1. Wash hands thoroughly with antimicrobial soap and warm water.
2. Establish a sterile field to prevent transmission of microorganisms from the environment or from the clinician to the patient.
3. Use sterile gloves to perform the procedure.
4. Use additional PPE, such as sterile gowns and masks, as indicated for specific procedures.
5. Maintain cleanliness of the environment immediately adjacent to the sterile field.
6. When possible, keep the door to the patient's room closed during the aseptic or sterile procedure.
7. Ask family members to leave the room during the procedure.
8. Maintain all equipment used in a sterile condition, or use disposable, one (1)-time-use sterile equipment.

INFECTION CONTROL/EXPANDED PRECAUTIONS**Policy No. 6-008.1****PURPOSE**

To outline the necessary precautions to prevent transmission of infectious diseases. This policy also addresses bed bugs.

POLICY

In addition to standard precautions, organization personnel will follow strict specifications when caring for patients with infectious diseases and those with bed bug infestation.

GUIDELINES***Airborne Precautions as Recommended by the Centers for Disease Control***

1. Airborne precautions are designed to prevent transmission of infectious diseases primarily over short distances through airborne droplet nuclei.
2. Diseases which require airborne precautions are:
 - A. Measles
 - B. Varicella (including disseminated zoster)*
 - C. Tuberculosis (active pulmonary or laryngeal)
 - D. Smallpox
 - E. SARS
3. Specifications for Airborne Precautions:
 - A. Keep patient in one (1) room of the home as much as possible, unless other members of the household are immune (e.g., measles or chickenpox).
 - B. NIOSH-certified N95 respiratory mask should be worn by personnel for patients with suspected or known active pulmonary or laryngeal TB.
 - C. When a patient comes out of the room, he/she should wear a surgical mask.
 - D. Gloves are indicated for contact with respiratory secretions.
 - E. Hands must be washed after touching the patient or potentially contaminated articles.
 - F. Articles, including linen, should be thoroughly cleaned, disinfected, or discarded.

- G. Do not use fans in the patient's room.

Droplet Precautions

1. Droplet precautions are used for patients with known or suspected serious illnesses transmitted by large particle droplets.
2. Diseases which require droplet precautions are:
 - A. *Hemophilus Influenza* type B disease including meningitis, pneumonia, epiglottitis, and sepsis
 - B. *Neisseria Meningitides* disease, including meningitis, pneumonia, and sepsis
 - C. *Diphtheria* (pharyngeal)
 - D. *Mycoplasma pneumonia*
 - E. *Pertussis*
 - F. *Pneumonic* plague
 - G. *Staphylococcal* (group A) pharyngitis, pneumonia, or scarlet fever in infants and young children
 - H. *Serious* viral infections spread by droplet transmission
 1. Adenovirus*
 2. Influenza
 3. Mumps
 4. Parvovirus B19
 5. Rubella
3. Specifications for droplet precautions:
 - A. Place patient in a room away from susceptible individuals.
 - B. Surgical masks are to be worn when working within three (3) feet of the patient when he/she is coughing and does not reliably cover mouth, and until sputum smear is negative on culture.
 - C. Instruct patient to cover his/her mouth and nose when coughing or sneezing.

- D. Patients should not share personal items such as drinking cups.
- E. Do not use fans in the patient's room.
- F. Hands must be washed after touching the patient or potentially contaminated articles.
- G. Pregnant personnel susceptible to Rubella should not take care of patients with Rubella.

Contact Precautions

1. Contact precautions are used for patients with known or suspected serious illnesses easily transmitted by direct patient contact or by contact with items in the patient's environment.
2. Diseases which require contact precautions are:
 - A. Gastrointestinal, respiratory, skin, or wound infections or colonization with multidrug-resistant bacteria judged by the infection control program, based on current state, regional, or national recommendations, to be of special clinical and epidemiologic significance.
 - B. Enteric infections with a low infectious dose or prolonged environmental survival, including:
 1. *Clostridium difficile*
 2. For diapered or incontinent patients, enterohemorrhagic *Escherichia coli* O157:H7, *Shigella*, hepatitis A or rotavirus
 - C. Respiratory syncytial virus, parainfluenza virus, or enteroviral infections in infants and young children.
 - D. Skin infections that are highly contagious or that may occur on dry skin, including:
 1. Diphtheria (cutaneous)
 2. Herpes simplex virus (neonatal or mucocutaneous)
 3. Impetigo
 4. Major (noncontained) abscesses, cellulitis, or decubiti
 5. Pediculosis
 6. Scabies
 7. Staphylococcal furunculosis in infants and young children

- 8. Zoster (disseminated or in the immunocompromised host)*
- E. Viral hemorrhagic conjunctivitis
- F. Viral hemorrhagic infections (Ebola, Lassa, Marburg)
- 3. Specifications for Contact Precautions:
 - A. Minimize the number of staff members assigned to the patient.
 - B. Make efforts to schedule the patient as the last visit of the day or, minimally, avoid scheduling surgical or open-wound patients after the infected patient.
 - C. Leave the nursing bag in the car and take only a minimal number of required supplies into the home.
 - D. Leave stethoscope and blood pressure cuffs in the home until precautions are discontinued. Upon removal from the home, the equipment must be decontaminated using approved disinfectants.
 - E. Clean and disinfect contaminated patient care items, equipment, and surfaces on a daily basis.
 - F. Linen and laundry require no special treatment.
 - G. Gloves should be worn by personnel when entering the room of a patient infected or colonized with multidrug-resistant microorganisms. Otherwise, gloves should be used with patient contact.
 - H. Gloves must be removed prior to leaving the patient's room, and hands should be washed immediately using an antimicrobial soap and water.
 - I. Personnel should wear a gown when caring for patients requiring contact precautions when:
 - 1. There is substantial contact with the patient, environmental surfaces, or items in the patient's room.
 - 2. The patient is incontinent, has diarrhea, an ileostomy, a colostomy, or uncontained wound drainage.
 - 3. The gown should not be worn outside the patient's room and should not be removed until ready to leave the room, and should be discarded properly.
- 4. Precautions for bed bug infestation:
 - A. See Addendum 6-008.A "[Bed Bug Guidance](#)"

* Certain infections require more than one (1) type of precaution.

ADDENDUM 6-008.A

BED BUG GUIDANCE

(Insert specific organization policy)

Reference: Bed Bug Action Plan for Home Health Care and Social Workers

https://vtechworks.lib.vt.edu/bitstream/handle/10919/55941/VCE2014a_bed_bug_action_plan_home.pdf?sequence=1&isAllowed=y

Bed bugs are spreading rapidly within the United States. Home care workers that routinely visit patients' homes are at risk of contracting bed bugs. Homecare staff needs to be bed bug conscious if they are to avoid transporting bed bugs in their cars, taking bed bugs to the office, to their homes and possibly taking them to another patient's home.

Preparing for Home Visits

- Always wear simple clothing when visiting a patient's home. Avoid shirts with buttons and pockets (professional looking, long-sleeved, light colored T-shirts are advisable). Avoid cargo pants or pants with cuffs. Simple shoes that can be thrown in a hot dryer and that have minimal tread are also recommended. Do not accessorize with anything, particularly scarves, jewelry or handbags.

The following information is strictly to be used as guidelines when entering a home that currently has or is suspected to have bed bugs.

1. Prepare for the visit by acquiring the following items:
 - A. Folding chair
 - B. Rubber boots or shoe covers
 - C. Gloves
 - D. Fanny pack
 - E. Large garbage bags/lawn bags
 - F. White garbage bags
 - G. Container of wet wipes
 - H. Extra set of clothes/Tyvek suit (for severe infestation)
2. Wear protective booties at all times.
3. Wear light colored clothing so you can observe any bed bugs on your person.
4. Do not sit on upholstered furniture or the bed. Look in the cracks of hard chairs before sitting down or use the folding chair.
5. Carry only those items with you that are essential to the home visit. Leave everything else in the car. A fanny pack can be used to hold your wallet, personal items, spare gloves and booties.
6. A plastic clipboard can be used to hold your paperwork.
7. Avoid putting anything on upholstered furniture, bedding, or carpeted floors.
8. If assisting the patient on a bed or couch, put down a white garbage bag between yourself and the furniture.

9. Examine yourself and clothing when leaving the home and before entering your vehicle. Remove your booties immediately and seal them in a plastic bag. Dispose of the bag before you get in the car.
10. Remove any affected clothing, or Tyvek, seal them in a plastic garbage bag, and launder personal clothing on a high heat setting. If Tyvek was used, dispose of it at the home.
11. If you find a bed bug on yourself, use a wet wipe to capture the bug. Use another wet wipe to wipe down the surrounding area, paying attention to seams, buttons and other bed bug hiding places.

If you discover bed bugs in the home during your visit, remain calm.

1. Record the infestation in your notes so you will be prepared for the next visit.
2. Perform a self-inspection of your clothing such as the back of your pants, tread of your shoes, shoelaces, socks, cuffs and collar.
3. If you find a bed bug on yourself, use a wet wipe to capture the bug. Use another wet wipe to wipe down the surrounding area, paying attention to seams, buttons and other bed bug hiding places.

If you are repeatedly visiting infested homes:

1. Protect yourself and other patients by always wearing booties and protective coveralls and using a bed bug containment kit. The kit should contain:
 - A. A portable, hard surface chair or stool
 - B. A fanny pack for holding items like identification, cell phone, additional booties and gloves
 - C. A change of clothes and shoes (kept in your vehicle)
 - D. A plastic storage bin with a sealed lid that is large enough to contain items below or items that you might suspect to be infested.
 1. Protective booties and Tyvek type coveralls
 2. Disposable gloves
 3. A roll of light colored duct tape
 4. Small plastic garbage bags
 5. A roll of clear plastic drum liners
 6. Flashlight
 7. Narrow banded spatula (for crushing bed bugs)

8. A fresh container of wet wipes
9. Plastic box-type clipboard containing paper and pens

CONTAMINATED MATERIALS DISPOSITION**Policy No. 6-009.1****PURPOSE**

To provide guidelines for the handling of contaminated materials.

POLICY

Contaminated materials will be handled in accordance with standard precautions and all applicable laws and regulations. (See "[Standard Precautions](#)" Policy No. 6-004.)

Definition

Contaminated materials: Materials that have been exposed to or contaminated by blood or body fluids. These materials may be transported to destinations outside the patient's home (i.e., blood specimens to laboratories).

PROCEDURE**1. Laboratory Specimen Collection and Delivery**

- A. Once the specimen is collected, it should be labeled (patient's name, date, time specimen obtained, doctor's name), placed inside a sturdy and sealable plastic bag, and secured to prevent leakage during transport. Each type of specimen (i.e., blood, urine, feces, sputum, etc.) should be placed in a separate and labeled plastic bag.
- B. Care should be taken when collecting the specimen to avoid contaminating the outside of the container and the laboratory requisition accompanying the specimen.
- C. An impermeable container with a biohazard label will be used to transport the specimen to the laboratory to prevent leakage, should the collection container spill or break.

2. Linen

- A. Patients at risk for soiling linen with infected materials should designate a laundry bag/pillow case in which to place soiled linen.
- B. Soiled linen will be kept separate from clean linen and should be washed separately from other linens.
- C. Soiled linen is not to come in contact with personnel's clothing. Soiled linen should be carried away from the body. Personnel's clothing may be protected by a disposable apron, as appropriate.

- D. Personnel will not sit on the floor during patient's care unless a barrier is covering the floor and the patient cannot be cared for from another position.

3. Equipment

- A. Cleaning reusable equipment that may come in contact with mucous membranes or body fluids: (This refers to equipment that personnel transports from patient to patient in the performance of their duties, i.e., BP cuffs, stethoscope, thermometers, scales, ultrasound.)
 - 1. Wipe exposed portions of equipment with alcohol or other appropriate cleaning solution.
 - 2. Return equipment to carrying case (see "[Bag Technique](#)" Policy No. 6-013).
- B. Cleaning reusable equipment in the office: This refers to equipment that has been assigned to one (1) patient for use during the course of his/her care (i.e., BP cuffs, stethoscopes, hardback charts).
 - 1. Upon equipment malfunction or after patient discharge from care/service, dirty equipment should be placed in a plastic bag and secured for transport to the office.
 - 2. Dirty equipment should be separated from clean equipment/supplies in the personnel's automobile for transport to the office.
 - 3. Dirty equipment will be returned to the office's designated dirty area for cleaning. It will be cleaned with an appropriate solution before being returned to the clean storage area.
 - a. BP cuffs—spray with disinfectant spray and allow to air dry.
 - b. Stethoscopes—disinfect with alcohol for three (3) minutes, using friction, and allow to air dry on clean surface.
 - c. Hard back charts—spray with disinfectant spray and allow to air dry.

CONTAMINATED WASTE DISPOSAL**Policy No. 6-010.1****PURPOSE**

To ensure the protection of organization personnel, patients and family/caregivers, and the community through proper handling of contaminated waste.

POLICY

Organization's Name will dispose of contaminated waste according to state and local regulations. Organization's Name will educate personnel and, as appropriate, patients and family/caregivers on procedures for handling and disposing of contaminated waste.

Definitions

1. **Contaminated Waste:** Disposable materials that have been exposed to or contaminated by blood or body fluids.
2. **Infectious Wastes:**
 - A. **Sharps:** Any waste capable of producing injury, including, but not limited to, contaminated needles, syringes, scalpels, and disposable instruments.
 - B. **Blood, Blood Products, and Body Fluids:** All waste blood, blood products, and body fluids greater than 20ml. (2/3 oz.) in volume that exist in a free, liquid state and cannot be safely poured down a drain.
 - C. **Microbiological Waste:** Cultures and stocks of infectious agents and associated biologicals, including culture dishes and devices used to transfer, inoculate, and mix cultures.
 - D. **Contaminated Lab Waste:** All lab specimens consisting of blood or body fluids that cannot be disposed of by safely pouring down a drain.

PROCEDURE***Contaminated Waste***

1. Personnel will receive inservice education to identify all possible contaminated waste as appropriate to the care and services provided.
2. Personnel will teach patients and family/caregivers proper contaminated waste disposal.
3. Contaminated paper wastes (disposable gloves, gowns, masks, paper towels, tubings, dressings, etc.), should be placed in a plastic puncture resistant bag and secured. It should be double bagged and, if possible, placed in a plastic trash container with tight lid and labeled, as appropriate. (See also "[Standard Precautions](#)" Policy No. 6-004.)

Sharps

1. Needles, syringes, and razor blades should be placed in a puncture-proof container and labeled as contaminated waste when 3/4 full. Puncture-proof containers should be bagged, secured, and returned to the office for disposal.
2. NEVER recap needles. Do not attempt to break or bend needles; always dispose of the unit as a whole. The nurse may recap sterile needles (e.g., after prefilling insulin syringes) using the one (1)-handed scoop method or a safety device. The nurse may teach the patient to recap his/her own needles using a one (1)-handed method.

Blood and Body Fluids

1. Fluids (i.e., urine, feces, solutions, etc.) should be poured down the toilet and immediately flushed. Care should be taken to avoid splashing.

Contaminated Lab Waste

1. Lab specimens, waste that cannot be disposed of by safely pouring down a drain, may be delivered to the lab after being bagged, securely closed, and labeled.
 - A. Place specimen in a leak-proof, impermeable, biohazard-labeled transport container.
 - B. Carry the bagged specimen in the container to the lab and hand directly to lab personnel.

HAZARDOUS WASTE HANDLING**Policy No. 6-011.1****PURPOSE**

To ensure the protection of organization personnel, patients, families/caregivers, and the community through proper handling of contaminated waste.

POLICY

Organization's Name does not supply hazardous chemicals (i.e., chemotherapeutic agents) to patients. They may, however, be involved in the administration of such agents in the home care setting.

Home care personnel are responsible to instruct the patient and family/caregiver regarding the handling and disposal of hazardous materials/wastes in a safe and sanitary manner. All hazardous wastes should be discarded according to state and local regulations. (See ["Hazardous Waste Disposal State and Local Regulations"](#) Addendum 6-011.A.)

*Chemical Hazard Communication "OSHA 3084", [Department of Labor](#). (See ["Contaminated Materials Disposition"](#) Policy No. 6-009, and ["Contaminated Waste Disposal"](#) Policy No. 6-010).

Definition

Hazardous Waste: Those chemicals/materials that may potentially cause or contribute to many serious health effects or present a safety hazard and have the potential to cause fire, explosion, or serious accidents.

PROCEDURE

1. Gloves should be worn when handling tubing and administering chemotherapeutic agents.
2. Sharp instruments and disposables: Needles will not be recapped, bent or broken by hand, or removed from disposable syringes and manipulated by hand.
3. A spill kit will be provided to all patients receiving chemotherapeutic agents.
4. Following administration, hazardous waste should be placed in a puncture resistance container (taped and secured) prior to disposal. It should be double bagged and placed in a plastic lined trash container and tagged, as appropriate, with biohazard symbols.

ADDENDUM 6-011.A

**HAZARDOUS WASTE DISPOSAL
STATE AND LOCAL REGULATIONS**

(Insert here)

Reference: Environmental Protection Agency
www.epa.gov/osw/nonhaz/industrial/medical/index.htm

(Has links to state programs)

ACCIDENTAL EXPOSURE TO BLOOD**Policy No. 6-012.1****PURPOSE**

To define a process for the handling of a blood spill.

POLICY

All organization personnel, patients and family/caregivers will be instructed in the proper handling of an accidental blood spill in accordance with defined containment principles for bloodborne pathogens.

PROCEDURE

1. A spill cleanup kit will be available for use by the clinician/technician and the patient and family/caregiver with instructions for use in the event of a spill. The disposal spill kit will include:
 - A. Gloves—double gloves advised
 - B. Gowns or aprons
 - C. Mask, goggles—if splatter or splashes are anticipated
 - D. Absorbent beads (if applicable and available)
 - E. Paper towels or disposal cloths
 - F. Plastic bags—double bags are advised
 - G. Disinfectant solution
2. If a spill occurs, it will be cleaned up immediately by trained individuals. Organization policies and procedures and spill kit directions will be followed.
3. The patient and/or family/caregiver will be instructed to call the nurse immediately to report the occurrence of a spill or exposure.
4. All spills and exposures will be reported and documented through the organization incident reporting mechanism.
5. All available information will be given to the family/caregiver concerning possible exposure concerns to enable them to make a decision regarding the best place of treatment for the patient.

6. The clinician/technician will teach the patient and family/caregiver what to do if a spill or accidental exposure occurs.

Spill

1. Put on two (2) pairs of gloves.
2. Put on a gown or apron and mask or goggles as indicated by the type of spill.
3. Cover the spill with the absorbent beads (if applicable) to bind the material.
4. Wipe up the material with towels or cloths.
5. Place the cloths in the first plastic bag.
6. Clean the area with the disinfectant solution. Place the paper towels or cloths in the first plastic bag.
7. Remove the outer pair of gloves and place in the first bag.
8. Securely tie the first bag.
9. Place the first bag in the second bag.
10. Place all protective clothing and equipment in the second bag, removing the inner pair of gloves last and placing them in the second bag.
11. Securely tie the second bag and appropriately label the outer bag.
12. Wash hands.
13. Transport the bag for proper disposal.
14. Use extreme care to prevent contamination to self. ALWAYS WASH HANDS BEFORE AND AFTER CONTACT. Report the incident to the Clinical Supervisor.

Accidental Exposure

1. Remove contaminated gloves or gowns immediately and discard properly.
2. Wash skin contaminated with soap (not a germicidal agent) and water.
3. Flood an eye that is accidentally exposed with water or an isotonic eyewash for at least five (5) minutes.
4. Obtain a medical evaluation as soon as possible and document the incident according to established organization policies.

BAG TECHNIQUE
Policy No. 6-013.1**PURPOSE**

To describe the procedure for maintaining a clean nursing bag/computer bag and preventing cross-contamination.

POLICY

As part of the infection/exposure control plan, Organization's Name personnel will consistently implement principles to maximize efficient use of the patient's care supply bag when used in caring for patients.

PROCEDURE

1. The bag may have the following contents:
 - A. Hand washing equipment—alcohol based hand rub and skin cleanser, soap, and paper towels
 - B. Assessment equipment (as appropriate to the level of care being provided)—thermometers, stethoscopes, a hem gauge to measure wounds, sphygmomanometer, and urine testing equipment
 - C. Disposable supplies (as appropriate to the level of care being provided)—plastic thermometer covers (if applicable), sterile and non-sterile gloves, plastic aprons, dressings, adhesive tape, alcohol swabs, tongue blades, applicators, lubricant jelly, scissors, bandages, syringes and needles, vacutainer equipment for venipuncture, skin cleanser, paper towels, and a CPR mask
 - D. Paper supplies (if applicable)—printed forms and materials necessary to teach patients and family/caregivers and document patient care
 - E. Laptop computer or other documentation device
2. Personnel must regularly check the expiration date of any disposable supplies kept in the nursing bag. Expired supplies should be returned for disposal.
3. The bag will be cleaned as soon as feasible when it is grossly contaminated or dirty. Soap and water, alcohol, or another approved cleaning agent will be used.

Bag Technique

1. The bag will be placed on a clean surface (i.e., a surface that can be easily disinfected) in the car and in the home.

2. Prior to administering care, alcohol-based hand rub or soap and paper towels will be removed, and hands will be washed. These supplies will be left at the sink for hand washing at the end of the visit. Hand washing will always be completed before opening the bag.
3. After hand washing, the supplies and/or equipment needed for the visit will be removed from the bag.
4. The bag will contain a designated clean and dirty area. The clean area contains unused or cleaned supplies/equipment, and the dirty area is designated for contaminated materials (i.e., used equipment, etc.).
5. When the visit is completed, reusable equipment will be cleaned using alcohol, soap and water, or other appropriate solution, hands will be washed, and equipment and supplies will be returned to the bag.
6. Hands will be decontaminated prior to returning clean equipment to bag.
7. If paper towels/newspapers have been used as protective barrier for bag placement in the patient's home, they will be discarded.

EVALUATING AND MAINTAINING RECORDS OF INFECTIONS AMONG PATIENTS

Policy No. 6-014.1

PURPOSE

To provide for a reliable, consistent method of surveillance of infections occurring in the home health care population. There are three (3) types of surveillance: total surveillance, priority targeted surveillance, and problem-oriented surveillance.

POLICY

All patients with a new, actual, or suspected infection will have a patient infection report completed within 24 hours of discovery. If trends are discovered, target surveillance or problem-oriented surveillance may be initiated for specific high-risk patient populations if high volume, problem-prone areas are identified.

Definitions

1. **New Infection:** Any infection that occurs that was not documented as present at the time the patient was admitted to care/services; or, an infection that occurs 48 hours after admission.

Note: If an infection develops within 48 hours of home care admission it may be a nosocomial infection.

2. **Suspected Infection:** A situation in which clinical observations strongly suggest the presence of an infection, but empirical data to support the suspicion is not possible or available at the time of the report.
3. **Reportable Communicable Disease:** Some suspected or positively identified communicable diseases must be reported to Public Health agencies. (See "[Reporting of Communicable Diseases](#)" Policy No. 6-016.)
4. **Total Surveillance:** Surveillance of all infections to include sources of infections such as nosocomial or home-acquired infections.
5. **Targeted Surveillance:** Focuses on specific populations or procedures, such as patients with enteral feedings who suffer higher-than-expected incidence of diarrhea.
6. **Problem-oriented Surveillance:** Measures the occurrence of a specific infection in multiple patients. Also called outbreak-response surveillance.

PROCEDURE

1. The Infection Identification—Patient Report form is completed when any of the following occur:

- A. A new, actual, or suspected infection is clinically observed by personnel
 - B. A culture is ordered and performed, and the results are positive
 - C. A new antibiotic is ordered
 - D. A patient is admitted to a hospital due to an actual or suspected infection
 - E. An unanticipated death due to an actual or suspected infection
 - F. A major permanent loss of function associated with an actual or suspected infection
 - G. A reportable, communicable infection is identified
 - H. Patient has temperatures greater than 101 degrees F
 - I. Patient has conjunctivitis
2. The Case Manager or the nurse caring for the patient in his/her absence should complete the Infection Identification Patient Report within 24 hours of discovering the infection.
 3. Each section of the report should be completed in its entirety.

Note: If patient was admitted to home care from a hospital and a new infection was identified by the clinician within the first 48 hours of service, the source of the suspected infection may be nosocomial.

4. After completion, the original form will be sent to the Performance Improvement Coordinator, and a copy will be submitted to the appropriate personnel for follow-up.
5. The Performance Improvement Coordinator summarizes, trends, and analyzes the reports. The break down and summary may include, but will not be limited to:
 - A. Number of infection types
 - B. Sources of infection
 - C. Diagnoses
 - D. Age/sex of patients
 - E. Family/caregiver
 - F. Pathogens, if identified
 - G. Follow-up taken

6. The Performance Improvement Coordinator, in conjunction with the Program Supervisor, will investigate possible causal factors and recommend appropriate action to contain the transmission of the infection. Actions may include implementing target or problem surveillance to aid in identifying causal factors and appropriate recommendations.
7. A report on infections will be given at each Performance Improvement Committee meeting. A summary will be forwarded to the appropriate oversight committee of the Governing Body.
8. Information from the summary, analysis, and discussion will be used as part of the organization's risk analysis and annual evaluation of the organization's infection prevention and control activities.

Reference for Healthcare-Associated Infections: <http://www.cdc.gov/HAI/InfectionTypes.html>

ADDENDUM 6-014.A

INFECTION IDENTIFICATION—PATIENT REPORT

INFECTION IDENTIFICATION—PATIENT REPORT

Patient Name:

Report Date:

Patient Number:

SOC Date: _

Dx:

Primary Nurse:

Reported by:

Signs and Symptoms Observed Related to Presence of an Infection:

(Check and/or describe)

Date first observed:

☐ Cloudy Urine☐ Foul Urine☐ Dysuria/Flank Pain☐ Purulent Drainage☐ Erythema☐ Fever/Chills☐ Change in sputum color☐ Cough☐ Dyspnea☐ Wound Pain☐ Increased Amount of Wound Drainage☐ Other:

Explanation/Description:

Site of Suspected/Actual Infection:☐ Urinary☐ Wound☐ Surgical Incision☐

Respiratory

☐ IV Site☐ Oral☐ Gastrointestinal☐

Blood

☐ Other:

Source of Suspected Infection (if known):

Factors Associated with the Development of the Infection:

Has the Physician Been Notified of Above Observations?

___ Yes ___ No

If yes, name of MD who was notified: _____ Phone #:

Date notified: _____ Time: _____

Orders Received:

Lab Orders:

Medications Ordered:

Date/Time Started: _____ / _____

Other:

Results/Findings: Pathogens Identified: _____

* New Diagnosis: _____ Yes _____ No; If Yes, State: _____

Follow-Up Required/Additional Comments: _____

* Is the Infection Communicable and Reportable? ___ Yes ___ No; If yes, report to supervisor within 24 hrs.

Organization's Name

Risk Management: Infection and Safety Control

Reported by: (Name) _____

Date: _____

Time: _____

Signature of Person Reporting: _____

Date: _____

Manager Receiving Report: _____

Date Received: _____

ORIGINAL—QA/I

YELLOW—Director

**EVALUATING AND MAINTAINING RECORDS
OF INFECTIONS AMONG PERSONNEL
Policy No. 6-015.1**

PURPOSE

To provide for a reliable, consistent method of surveillance of infections occurring among personnel.

POLICY

All personnel with a new, actual, or suspected infection will have a personnel infection report completed within 24 hours of discovery.

PROCEDURE

1. The staff member will notify his/her supervisor of a suspected illness or exposure.
2. The staff member's physician will identify the type of infection experienced, if any, the treatment needed, and when the staff member may safely return to work.
3. A written statement from the physician will be necessary for the staff member to return to work.
4. The Program Supervisor or Infection Control Nurse will identify any other personnel and/or patients who may have been exposed. Appropriate notification and recommendations for treatment will be provided, when indicated.
5. An infection identification personnel report is completed and forwarded to the designated individual responsible for infection reporting, with a copy to the Performance Improvement Coordinator.
6. The Performance Improvement Coordinator will summarize, trend, and analyze the reports. The breakdown and summary may include, but will not be limited to:
 - A. Number of types of infections
 - B. Diagnoses
 - C. Age/sex of employees
 - D. Patients cared for
 - E. Pathogens
 - F. Orders/medications

7. The Performance Improvement Coordinator, in conjunction with the Program Supervisor or Infection Control Nurse, will investigate possible causal factors and recommend appropriate action to contain the transmission of the infection.
8. Specific problem-oriented surveillance will be initiated, if appropriate.
9. A report on infections will be given at each Performance Improvement Committee meeting. A summary will be forwarded to the appropriate committee of the Governing Body.
10. Information from the summary, analysis, and discussion will be used to improve patient care and reduce the risk of personnel-to-patient infection transmission.

ADDENDUM 6-015.A

INFECTION IDENTIFICATION—PERSONNEL REPORT

INFECTION IDENTIFICATION—PERSONNEL REPORT

Report Date:

Employee:

Diagnosis:

Signs and Symptoms of Suspected Infection:

Source of Suspected Infection:

Physician Contact: Yes _____ No _____ Date _____

Pathogen Identified: Yes _____ No _____

How: Lab Report:

Physician Diagnosis:

Other:

Work Restriction: (if applicable)

(See Summary of Inpatient Recommendations and Work Restrictions)

Date of Physician Release: (if applicable)

Date(s) of Work Restriction: From: _____ To: _____

Additional Information Follow-up: _____

Form Completed By:

REPORTING OF COMMUNICABLE DISEASES

Policy No. 6-016.1

PURPOSE

To promote compliance with local, state, and federal communicable disease reporting requirements.

POLICY

All known or suspected cases of reportable diseases will be promptly reported to the local health officer or other public health official, as mandated by state law.

All cases of "unusual" diseases not listed below, such as Glanders, Herpangina, Histoplasmosis, Toxoplasmosis, Echinococcosis, Listeriosis, Cat Scratch Fever, and Rickettsialpox, will also be reported.

PROCEDURE

1. A verbal report will be made by the clinician/technician to the Program Supervisor as soon as the medical diagnosis is made. A subsequent written report will be made, upon request of the public health officer.
2. The following diseases are to be reported to the local health department in which the patient resides.

(See "[Patient Requests for Accounting of PHI Disclosures](#)" Policy No. 2-025 and "[Uses and Disclosures of PHI](#)" Policy No. 2-017.)

CLASS A (Individual Case Reports Required Within 24 Hours)

(1) Diseases of major public health concern because of endemicity and/or potential for epidemic spread

Campylobacter	Meningococcal Disease
Chlamydial Infections	Meningitis, Aseptic, including
(nonspecific Urethritis, Cervicitis,	Lymphocyte Choriomeningitis, and
Salpingitis, Neonatal Conjunctivitis,	Viral Meningoencephalitis
Pneumonia, and Lymphogranuloma	Meningitis, other bacterial
Venereum)	Mumps
Encephalitis	Mycobacterial Disease
Arthropod-borne	Tuberculosis
Other viral	Other
Post-Infection	Pelvic Inflammatory Disease,
Giardiasis	Gonococcal
Gonococcal Infections	Pertussis
Hepatitis:	Reyes Syndrome
A	Rocky Mountain Spotted Fever
B	Rubella (including Congenital
Unspecified	Rubella Syndrome)
Legionnaire's Disease	Salmonellosis
Measles	Shigellosis
Invasive Haemophilus Influenzae	Syphilis

(2) Low Frequency diseases of a major public health concern

Acquired Immunodeficiency	Lyme Disease
Syndrome	Plague
(AIDS)	Poliomyelitis (including vaccine-associated)
AIDS-Related Complex (ARC)	Psittacosis (Ornithosis)
Amebiasis	Rabies
Anthrax	Rheumatic Fever
Botulism	Smallpox
Brucellosis	Streptococcal B in newborn
Chancroid	Sudden Infant Death Syndrome (SIDS)
Cholera	Tetanus
Cytomegalovirus (congenital only)	Toxic Shock Syndrome (TSS)
Dengue	Toxoplasmosis (congenital)
Diphtheria	Trichinosis
Granuloma Inguinale	Tularemia
Herpes (congenital only)	Typhoid Fever
Leprosy	Typhus Fever
Leptospirosis	Vibriosis
Listeriosis	Yellow Fever
Malaria	Yersiniosis
Mucocutaneous Lymph Node	
Syndrome (Kawasaki Disease)	

CLASS B (Report by Number of Cases Only)

Chickenpox
Herpes-genital

Influenza
Streptococcal Infections

CLASS C (Report Situation When Epidemic is suspected)

Blastomycosis
Conjunctivitis, acute
Diarrhea of newborn
Foodborne disease
Histoplasmosis
Infectious mononucleosis
Nosocomial infections of any type

Pediculosis
Scabies
Sporotrichosis
Staphylococcal skin infections
Toxoplasmosis
Waterborne disease

COMMUNICATION OF HAZARDS TO PERSONNEL
Policy No. 6-017.1**PURPOSE**

To communicate the risk of occupational exposure to hazardous materials to personnel.

POLICY

Hazards will be communicated to organization personnel by the proper use of labels and signs, according to applicable laws and regulations.

PROCEDURE

1. Biohazard warning labels will be affixed to containers of regulated waste; refrigerators, and freezers containing blood or other potentially infectious material; and other containers used to store, transport blood or other potentially infectious materials.
2. Labels required by this section will include the biohazard legend.
3. Labels will be fluorescent orange or orange-red, with lettering or symbols in a contrasting color.
4. Labels must be affixed as close as feasible to the container by string, wire, adhesive, or other method that prevents their loss or unintentional removal.
5. Red bags or red containers may be substituted for labels.
6. Individual containers of blood or other potentially infectious materials that are placed in a labeled container during storage, transport, shipment, or disposal will be exempted from the labeling requirement.
7. Labels required for contaminated equipment will be in accordance with this policy and will also state which portions of the equipment remain contaminated.
8. Regulated waste that has been decontaminated need not be labeled or color-coded.
9. In accordance with OSHA's Hazard Communication Standard, Organization's Name will maintain Safety Data Sheets (SDS) for all hazardous chemicals and medications maintained or used by the organization's personnel. SDS information will be readily available to personnel at all times.

ENVIRONMENTAL SAFETY PROGRAM**Policy No. 6-018.1****PURPOSE**

To establish the process by which the organization will address safety and risk management for both patients and organization personnel.

Definition

1. ***Environments:*** Building(s), delivery vehicle(s), equipment, and people.

POLICY

The organization will maintain an environmental safety program that addresses the office environment as well as the patient care environment, including, but not limited to:

1. Environmental safety, addressing hazards, injuries, storing and handling of environmental cleaning supplies, and unsafe practices
2. Security, addressing unsafe areas, on-call, isolation, and security concerns
3. Hazardous materials/wastes, addressing OSHA, EPA regulations, hazardous spills, health hazards, and reporting spills/exposures
4. Emergency management, addressing continuing care, communication, and prioritizing patients
5. Fire safety, addressing fire response, fire hazards, fire escape, and communication
6. Equipment management, addressing maintenance, recalls, cleaning, and set-up including the proper use, handling and care of desktop computers and laptop/clinical documentation devices
7. Utilities, addressing electrical outlets, grounding, and batteries

The senior management will have the responsibility for the following activities:

1. Designing the environmental safety program
2. Teaching organization personnel and patients how to implement the environmental safety program
3. Implementation of the environment of care processes
4. Measuring and assessing the effectiveness of the design

5. Improving the performance of the environment of care function

The organization will maintain a systematic process to measure the effectiveness of the environmental safety program.

The policies included in this section comprise the environmental safety program and are considered standard operating procedures.

PROCEDURE

1. The Executive Director/Administrator, in conjunction with the senior management, will educate all personnel about the environmental safety policies and procedures and their responsibilities in the implementation.
2. Organization personnel will receive an orientation to the environmental safety components including safety data sheet format and label elements.
3. Knowledge and competence will be demonstrated during the orientation and probationary period as well as throughout the year.
4. The environmental safety program inservices will be scheduled annually. Attendance will be mandatory and will be documented in the personnel file. Verification of attendance for the mandatory environmental safety program inservices from other institutions will be accepted, provided they are attended within the same calendar year.
5. Program supervisors and clinical/technical personnel will educate patients and family/caregivers in safety measures in the home to minimize hazards related to care provided.
6. The environmental safety program will be evaluated as part of the annual program evaluation.
7. As part of the performance improvement program, the organization will assess, through defined measures, the effectiveness of the environmental safety program in:
 - A. Maintaining safe environments for patients and organization personnel
 - B. Educating organization personnel and patients how to implement the program
 - C. Improving the organization's performance in environmental management
8. Measures will be developed which specifically address the components of the environmental safety program, and may include:

- A. Incidents related to home environment of the patient, including:
 - 1. Equipment malfunctions
 - 2. Patient endangerment
 - 3. Falls
 - 4. Medication errors
 - 5. Fires
 - 6. Electrical issues
 - B. Incidents related to home environment but specific to organization personnel, including:
 - 1. Organization personnel endangerment
 - 2. Equipment malfunction
 - 3. Medication errors
 - C. Incidents related to office environment, including:
 - 1. Equipment malfunction
 - 2. Fires
 - 3. Electrical issues
 - 4. Organization personnel falls/injuries
 - D. Outcomes of office environment safety checks, including fire drills
- 9. Any areas demonstrating a pattern or trend will be analyzed by the Performance Improvement Committee for development of recommendations and actions.
 - 10. A summary of the results of measures will be forwarded to the existing oversight committees and the Governing Body.

ENVIRONMENTAL SAFETY—OFFICE**Policy No. 6-019.1****PURPOSE**

To outline general office safety practices of the organization.

POLICY

Organization's Name is committed to provide a safe environment for all personnel, and will instruct all personnel to take responsibility in maintaining a safe office environment.

Physical facilities will be in continuous compliance with OSHA (federal and state), CDC, ADA, and applicable state or local regulations for maintaining a safe employee environment.

PROCEDURE

1. All personnel will receive education on basic safety rules.
2. All personnel will be responsible for recognizing and avoiding unsafe conditions, as well as controlling or eliminating any hazards or exposures to injury.
3. Basic safety guidelines will be incorporated into the daily work habits of all personnel, both at the office and in the patient care environment:

Body Mechanics While Lifting Objects

- A. To lift an object, squat or bend knees, take hold of the item, and straighten up.
- B. Divide the weight of the object between both hands.
- C. Leg or thigh muscles must be used for lifting objects.
- D. Keep back straight when lifting.
- E. Keep object close, avoid reaching, do not jerk.
- F. Secure firm footing before lifting.
- G. Ask for assistance with heavy objects.
- H. Use weight lift belt for lifting heavy objects (over 25 pounds).

Prevention of Slips and Falls

- A. Wear supportive, closed-toe shoes.
- B. Clean up noted spills and trash.
- C. Identify wet floors.
- D. Observe WET FLOOR signs.
- E. Stay off wet floors until dry.
- F. Walk, do not run or slide across floor.
- G. Keep wastebaskets, stools, stands, and other mobile equipment out of aisles and other areas intended for walkways.

Prevention of Bruises, Lacerations, Skin Tears

- A. Keep all desk and file drawers closed when not in use.
- B. Open only one (1) file drawer at a time.
- C. Knock before entering a room.
- D. Take time to look before leaving a room.
- E. Check furniture regularly for rough edges, splinters, sharp edges, or loose casters.
- F. File drawers should never be "bumped" closed with body.
- G. Use sharp or pointed tools correctly and store in a safe manner.

Prevention of Burns and Skin Reactions

- A. Avoid skin contact with all chemicals and/or contaminants.
- B. Handle chemicals cautiously.
- C. Never spray chemicals toward face of another person.
- D. Use proper mixing ratios with all chemicals/cleaning solutions.
- E. If cleaning compounds produce fumes, use only in well-ventilated areas.

- F. Mix only those chemicals together for which specific instructions have been written, to avoid making a dangerous combination.
- G. Wash promptly if any chemical comes in contact with skin.
- 4. All areas of the organization, including passageways, storerooms, and service rooms, will be kept clean and orderly and in a sanitary condition.
- 5. Personnel will not consume food or beverage in toilet rooms or in any other area exposed to toxic material.
- 6. The organization will ensure that medical personnel or consultants are readily available for advice on matters of office health. First aid supplies will also be readily available.
- 7. All storage will be stacked, blocked, interlocked, and limited in height so that it is secure against sliding or collapse.
 - A. Storage areas will be kept free from any accumulation of material that constitutes a hazard or pest harborage.
 - B. Where equipment is used, sufficient safe clearance will be allowed for aisles, at loading docks, through doorways, etc.
- 8. Medications and nutrition therapy solutions will be prepared under proper conditions of sanitation, temperature, light, moisture, ventilation, segregation, safety, and security.
 - A. Medication and nutrition therapy preparation will only be done by personnel with documented competencies regarding medication and nutrition therapy preparation.
 - B. The environment where medications or nutritional therapies are stored will be appropriate to the therapy preparations, in the office and patient environment. As appropriate to the setting, areas to consider include:
 - 1. Functionally separate areas for sterile product preparation to minimize the possibility of particulate and microbial contamination.
 - 2. A suitable environment for preparing sterile products.
 - 3. Adequate safety equipment to protect personnel preparing cytotoxic or hazardous medications.
 - 4. Clutter-free, clean work surface for medication preparation or nutritional therapy solutions.
- 9. The organization will have adequate toilet facilities that are separate for each sex.

- A. Where toilet rooms will be occupied by no more than one (1) person at a time, they should be able to be locked from the inside, and must contain at least one (1) water closet; separate toilet rooms for each sex is not needed.
 - B. Adequate washing facilities will be provided in every toilet room or be adjacent thereto.
 - C. Covered receptacles will be kept in all toilet rooms used by women.
 - D. A suitable cleansing agent, individual hand towels or other approved apparatus for drying the hands, and receptacles for disposing of hand towels, will be provided at washing facilities.
10. The organization will annually conduct a formal office environment inspection, using the Office Environment Checklist. (See "[Office Environment Checklist](#)" Addendum 6-019.A.)
- A. A calendar of inspection will be determined by the Performance Improvement Committee.
 - B. The office environment inspection will include at a minimum:
 - 1. Posting of certificates of occupancy as required by local law and regulation
 - 2. Fire and emergency exits
 - a. Clearly detail areas of entrance and egress
 - b. Lighted exit signs when required are functional
 - 3. Areas containing hazardous materials are clearly marked and access is controlled
 - 4. Hazardous chemicals and solutions are clearly labeled and stored in locked storage areas.
 - 5. Facilities are barrier free and/or special arrangements are made to provide access as required by the ADA.
 - C. Annual fire drill will be conducted
 - D. Fire drills (as applicable) and health inspection reports will indicate compliance and be available for review
11. The following safety precautions have been established for all personnel to follow when emergency conditions warrant such action. These safety precautions are not all-inconclusive. Others may be added or become necessary during the actual emergency.

Severe Weather/Earthquakes

- A. Have emergency equipment and medical supplies readily available.

- B. Close all drapes.
- C. Move away from windows.
- D. Stay away from windows.
- E. CLOSE exit doors.
- F. Go to inside room of building with no windows, if available.
- G. Do not enter damaged portions of the building until instructed.
- H. Monitor weather bulletins/radio announcements.
- I. Do not exit building until instructed.
- J. REMAIN CALM. DO NOT PANIC.

Floods

(Flood warnings, alerts, or an actual flood.)

- A. Precautions before the flood:
 - 1. Make sure emergency supplies and equipment are readily available.
 - 2. When a flood alert or warning is issued, store drinking water.
 - 3. TURN OFF all unnecessary electrical equipment.
 - 4. Do not touch any electrical equipment unless it is dry.
- B. Precautions if evacuation of building is ordered:
 - 1. Travel only routes designated.
 - 2. Do not try to cross a stream or other water areas unless you are sure it is safe.
 - 3. Monitor local radio broadcast.
 - 4. Watch for fallen trees, live wires, etc.
 - 5. Watch for washed-out roads, earth slides, broken water lines, etc.
 - 6. Watch for areas where rivers, lakes, or streams may flood suddenly.

C. After the flood:

1. Do not enter the building until an all-clear has been given.
2. Do not use any open flame devices until the building has been inspected for possible gas leaks.
3. Do not turn on any electrical equipment that may have gotten wet.
4. Shovel out mud while it is still moist.

D. Flash Floods:

1. Remember, flash floods can happen without warning.
2. When a flash flood warning is issued, take immediate action.
3. Follow all instructions issued without delay.

Snow Emergency

(Snow emergencies or winter storms)

- A. Keep a one (1) to two (2) week supply of heating fuel, food, and water on hand in case of isolation in office.
- B. Keep emergency supplies on hand, e.g., blankets and flashlights.
- C. Carry a cellular phone (if available).
- D. Dress appropriately—wear several layers of loose, lightweight warm clothing, mittens, and winter headgear to cover head and face.

ADDENDUM 6-019.A

OFFICE ENVIRONMENT CHECKLIST

OFFICE ENVIRONMENT CHECKLIST

Reviewer: _____ Survey Date: _____

When an unsafe condition or operation is observed, place a check (x) in the appropriate column. Submit any suggestions or recommendations to eliminate the hazard/concern on this form. Return the checklist to the Performance Improvement Coordinator.

OFFICE ENVIRONMENT CHECKLIST	Unsafe Condition or Operation
1. Area is tidy and well kept	
2. Work area is clean, orderly, and safely arranged	
3. Floors are well maintained—afford secure footing and are free of obstructions	
4. Non-slip surfaces are provided	
5. Aisles are sufficiently wide to provide easy movement	
6. Adequate storage areas are provided	
7. Stairs are free of chips, cracks	
8. Handrails are provided and secured	
9. Workspace and stairwells are properly illuminated	
10. Temperature is comfortable	
11. Area is free of odors	
12. Noise level is acceptable	
13. Ventilation is adequate	
14. Personnel use proper lifting and handling techniques	
15. Personnel exercise safe work habits	
16. Often used items are within easy access	
17. Heavy items are stored at waist height	
18. Step ladders or stools are used to access items stored on high shelves	
19. Non-slip feet are on all ladders, stools, etc.	
20. Furniture and fixtures are free of splinters/sharp edges	
21. Desk and file drawers open easily	
22. File cabinets are anchored to prevent tipping	
23. Heavy machines are properly and securely mounted	
24. Repetitive motions are minimized	
25. Electric machinery equipment are secured with ground wire	
26. Electric cords, plugs, and switches are in good repair	
27. Moving parts and "pinch points" are guarded	
28. Operators of machines are trained	
29. Scissors, knives, pins, razor blades, other sharp items are safely stored	

OFFICE ENVIRONMENT CHECKLIST, *continued*

OFFICE ENVIRONMENT CHECKLIST	Unsafe Condition Or Operation
30. Smoking is prohibited in specified areas	
31. Flammable liquids are stored in metal cabinets	
32. Used cleaning waste or rags are kept in closed metal containers	
33. Storage areas are kept clean and orderly with maintenance of proper temperature if applicable to stored solutions	
34. Adequate ventilation, lighting, and humidity control in storage area to prevent moisture, condensation, and mold growth	
35. Electric machines or heat producing elements are turned off when not in use	
36. Heating elements—coffee makers, griddles, portable electric heaters are properly wired, safely placed, and on a regular maintenance schedule	
37. Proper type and number of fire extinguishers are available	
38. Fire extinguishers are checked per regulation	
39. Evacuation route maps are prominently displayed	
40. Exit lights are functioning properly	
41. Absence of "trip" hazards	
42. Patient care items are stored off the floor	
43. Items are not stored within 18" of ceiling sprinkler	
44. Fire exit corridors are free of obstructions	
45. Personnel are trained to use extinguishers and are aware of use	
46. Personnel are instructed in fire reporting and emergency duties	
47. Fire drills are conducted in facilities	
48. Safety Data Sheets are posted for chemicals utilized	

FIRE SAFETY—OFFICE
Policy No. 6-020.1**PURPOSE**

To delineate the fire response process for the organization's office.

POLICY

Organization's Name is committed to promoting personnel safety in the event of a fire. At a minimum, all personnel will be knowledgeable of:

1. The principles of fire safety
2. The plan for evacuation of all occupants
3. The location of office exit doors, and the fire escape route to exit the building
4. The phone number(s) of the fire department kept in the office, in addition to 911

PROCEDURE

1. All personnel have the responsibility to practice fire prevention by:
 - A. Avoiding accumulations of excessive flammable material and trash
 - B. Only smoking in designated areas outside of the facility and following the organization policy on non-smoking
 - C. Reporting conditions which could result in fire
 - D. Knowing the locations of fire alarm devices, fire and smoke barrier doors, fire extinguisher, and fire exits
 - E. Knowing what to do when hearing a fire alarm sound
 - F. Knowing what to do when a fire, the smell of smoke, or odors of any burning substances are discovered
 - G. When storing items on shelves, be sure the items are at least 18" from ceiling, automatic detector or sprinkler head
2. The organization will maintain evacuation and relocation plan. Maps will be posted throughout the office.

3. If any personnel discover a fire, the following actions will be taken:
 - A. Evacuate anyone in immediate danger.
 - B. Close the door to the room of the fire origin, if possible.
 - C. Pull the fire alarm, if available.
 - D. Evacuate the building.
 - E. Notify the fire department (if phone accessible).
 - F. Stay calm—don't panic.
4. All personnel will:
 - A. Keep all doors closed that are posted "KEEP CLOSED"
 - B. Keep all corridors and walkways free from obstruction
 - C. Report any potential fire source to administration
 - D. Report any damage to the fire warning system
5. The office building will be in compliance with fire regulations, but at a minimum:
 - A. The office will have exits sufficient to permit the prompt escape of occupants, in case of emergency.
 - B. Where organization personnel may be endangered by the blocking of any single means of departure due to fire or smoke, there will be at least two (2) alternate means of exit remote from each other.
 - C. Exits and paths to exits will be unobstructed and accessible at all times.
 - D. All exits will discharge directly to the street or other open space that gives safe access to a public way.
 - E. Exits will be marked by readily visible, suitably illuminated exit signs.
6. The fire warning systems and fire extinguishers will be inspected and/or treated minimally twice a year, or more often as dictated by the local fire department.

PURPOSE

To outline the responsibilities of the office in utilities management.

POLICY

All personnel will be knowledgeable of the principles of electrical safety and utilities management.

PROCEDURE

1. All personnel will take personal responsibility to keep the electrical equipment used as part of the workday in proper working order.
2. All personnel will be responsible for recognizing and avoiding unsafe conditions, as well as for controlling or eliminating potential safety issues with regard to electrical equipment and utilities management.
3. The following basic safety guidelines will be incorporated into the daily work habits of all personnel, both at the office as well as in the patient care setting
 - A. Power outlets, covers, plugs, and cords are to be visually checked by organization personnel on a routine basis.
 - B. Report any cracked wall cover plates, frayed cords, or broken cords to the office manager.
 - C. Never attempt to plug or unplug electrical cords with wet hands or while on a wet floor.
 - D. Use of extension cords as a permanent use item is prohibited.
 - E. Appliance or equipment cords may not extend across walkways or corridors.
 - F. Immediately take out of service and tag any electrical equipment that gives a shock, makes a peculiar noise, or smells of burning.
 - G. Report equipment problems to the manufacturer immediately.
4. The telephone system will be tested on a regularly scheduled basis via periodic calls to the office at various hours of the day, week, and time. Any problems noted will be communicated to the telephone company.

5. Any problems or potential problems with fire, water, electric, and gas utilities will be reported to the appropriate company.
6. A file will be kept of all correspondence with utility companies.
7. The organization will conduct formal office environmental inspection annually, using the Office Environment Checklist (see "[Office Environment Checklist](#)" Addendum 6-019.A) and incorporating utilities management.
 - A. A calendar of inspections will be determined by the Performance Improvement Committee.
 - B. An informal inspection (not documented) should be done daily.

EQUIPMENT MANAGEMENT—OFFICE**Policy No. 6-022.1****PURPOSE**

To outline the responsibilities of the organization and organization personnel in office equipment management.

POLICY

All personnel will be knowledgeable regarding equipment management and safety.

PROCEDURE

1. It is the responsibility of all organization personnel to follow the manufacturer's guidelines in using any piece of equipment.
2. All personnel will be responsible for recognizing and avoiding unsafe conditions with regard to use of equipment.
3. All organization personnel will take personal responsibility to keep the equipment in the office in safe working order.
4. All new office equipment (i.e., computers, printers, facsimiles) will be checked for proper functioning prior to general office use.
 - A. The manufacturer's guidelines will be reviewed prior to using the equipment.
 - B. Equipment will be checked, when appropriate, by a Biomedical Engineer, for pre-acceptance testing, inspection, and approval before use.
5. For equipment that is leased or rented (i.e., copying machines, etc.), the rental company will be contacted for any problems in functioning.
6. The organization will conduct formal office environment inspections annually, using the Office Environment Checklist (see "[Office Environment Checklist](#)" Addendum 6-019.A) and incorporating equipment management:
 - A. A calendar of inspections will be determined by the Performance Improvement Committee.
 - B. An informal inspection (not documented) should be done daily.

ENVIRONMENTAL SAFETY—PATIENT
Policy No. 6-023.1

PURPOSE

To outline the mechanism to identify any environmental, mobility, and bathroom safety risks related to patient care.

POLICY

The organization is committed to promoting a safe environment for care for the patient and family/caregiver.

PROCEDURE

1. During the initial visit, the clinician/technician admitting the patient for care/service, will conduct a home safety check to identify environmental safety issues related to the type of care/service to be provided.
2. The home safety assessment will be documented in the patient/service record and will include, but will not be limited to:
 - A. Infection control, including hand hygiene practices, respiratory hygiene practices and contact precautions as needed
 - B. Medication safety (as applicable)
 - C. Fall prevention
 - D. Lighting
 - E. Communication
 - F. Bathroom safety
 - G. Fire safety
 - H. Electrical safety
 - I. Child safety (as applicable)
3. Based on the assessment, the clinician/technician will provide prevention tips and suggestions on reducing any environmental safety risks.
4. Each patient will receive written home safety instructions.

5. All personnel will be alert to safety factors in the home environment. The patient and family/caregiver will be encouraged, as appropriate to:
 - A. Have grab bars installed in the bathroom
 - B. Use non-skid mats or emery strips in the tub
 - C. Use a shower stool or transfer bath bench
 - D. Remove throw rugs or other environmental hazards such as loose extension cords, small mats, and slippery, waxed floors
 - E. Use assistive equipment such as toilet handrails, or walking belt, as indicated by the patient's condition
 - F. Always lock any wheeled equipment
 - G. Utilize a medication sheet to ensure proper administration of prescribed medications
 - H. Apply distinct and complete labeling of medications, including large letters indicating if it is for internal or external use, and good illumination of the medication cabinet in order to avoid errors in self-administered medications.
6. Patient education regarding environmental hazards and risks will be documented in the patient/service record, including:
 - A. What was taught (reference written safety instruction, as necessary)
 - B. Level of understanding
 - C. Patient and family/caregiver response to teaching
7. Additional resources to assist in patient education can be requested by organization personnel.
8. These safety guidelines have been established for all patients and personnel to follow, but they are not all-inclusive. Others may be added or become necessary when natural, biological, or environmental emergencies occur.

Severe Weather/Earthquakes

- A. Have emergency equipment and medical supplies readily available.
- B. Move away from windows.

- C. Stay away from windows.
- D. Store all glass and other breakable objects.
- E. If electricity is out:
 - 1. Use flashlights. Do not use candles until certain there is no gas leak.
 - 2. For patients on oxygen concentrators, switch to cylinders.
 - 3. Where possible, switch equipment to battery backup.
 - 4. For life support equipment, arrange for additional battery backup or move to an alternate location with power.
- F. Close all drapes.
- G. CLOSE exit doors.
- H. Monitor weather bulletins/radio announcements.
- I. Do not exit building until instructed.
- J. REMAIN CALM. DO NOT PANIC.

Floods

(Flood warnings, alerts, or an actual flood.)

- A. Precautions before the flood:
 - 1. When a flood alert or warning is issued, store drinking water.
 - 2. TURN OFF all unnecessary electrical appliances.
 - 3. Do not touch any electrical appliance unless it is dry.
 - 4. Open basement windows to equalize water pressure on the walls and foundation.
 - 5. Move patient to upper floor or to other designated areas.
 - 6. Move all essential equipment and supplies to safe areas.
- B. After the flood:
 - 1. Do not use any open flame devices until the building has been inspected for possible gas leaks.

2. Watch for live electrical wires.
3. Do not turn on any electrical appliance until it has been inspected.
4. Do not use any food supplies that have come in contact with floodwaters.
5. Provide emergency medical treatment required.

C. Flash floods:

1. Remember, flash floods can happen without warning.
2. When a flash flood warning is issued, take immediate action.
3. Protect patient and yourself to the best of your ability.

Snow Emergency

(Snow emergencies or winter storms)

- A. Keep a one (1) to two (2) week supply of heating fuel, food, and water on hand in case of isolation at home.
- B. Keep your car properly serviced, with snow tires and filled with gas.
- C. Keep emergency supplies in the car:
 1. Container of sand
 2. Shovel
 3. Windshield scraper
 4. Tow chain or rope
 5. Flares
 6. Blanket
 7. Flashlight
- D. Notify organization of destination, schedule, and approximate time of arrival.
- E. Dress appropriately—wear several layers of loose, lightweight warm clothing, mittens, and winter head gear to cover head and face.
- F. Carry a cellular phone (if available).

- G. Drive with all possible caution. If caught in a blizzard, seek refuge immediately. Keep car radio on for weather information.
- H. If your car breaks down—turn flashers on or hang a cloth from the radio aerial; stay in your car. If car is stuck in snow or traffic jam and car is running, crack windows to prevent carbon monoxide poisoning and keep exhaust pipe free of snow. If engine is not running, you do not need to crack windows.

FIRE SAFETY—PATIENT
Policy No. 6-024.1**PURPOSE**

To define the requirements of a fire safety assessment and fire response for patients and organization personnel while in the home.

POLICY

Organization's Name is committed to promoting patient safety in the event of a fire. At a minimum, all personnel will:

1. Be knowledgeable of the principles of fire safety.
2. Assess each patient home for fire safety on the first visit, and subsequently thereafter, as needed.

PROCEDURE

1. During the initial home visit, the admitting clinician/technician will conduct a home safety check, which includes an assessment of fire safety.
2. The assessment will include, but will not be limited to:
 - A. Presence of smoke alarms
 - B. Presence of an emergency exit plan
 - C. Smoking in the home
 - D. Oxygen use
 - E. Fire extinguisher availability
 - F. Fire hazards related to medical equipment or supplies
 - G. The patient's ability to summon emergency assistance
3. In addition, the admitting clinician/technician should identify the emergency numbers to call in the event of a fire (i.e., police, fire, 911). This information should be kept near the patient's main telephone.
4. Based on the assessment, prevention tips and suggestions should be made to reduce the risk of fires and potential hazards in the home. Prevention tips may include, but are not limited to:

- A. Development of a written emergency plan, including an escape route and designated meeting place
 - B. Installation of smoke detectors and routine maintenance
 - C. Precautions to take when smoking
 - D. Causes of fire and fire prevention activities
 - E. Relocation of heaters away from passageways and flammable materials
 - F. Storage of flammable and combustible items away from the range and oven
 - G. Safe use of oxygen to reduce home fire risks
5. Patient education will be documented in the clinical/service record, including:
- A. What was taught (reference written home safety instructions, when applicable)
 - B. Recommendations made to reduce environmental hazards
 - C. Level of understanding
 - D. Patient and/or family/caregiver response to teaching
6. If organization personnel discover a fire while in the home, the following steps should be taken:
- A. Remove anyone in IMMEDIATE danger.
 - B. Close the door to the room of fire origin, if possible.
 - C. Notify the fire department.
 - D. Stay calm; don't panic.
 - E. Remain with the patient and family/caregiver.
 - F. Notify the office.
7. The organization will maintain a list of all disabled, homebound patients and will notify the fire department, upon their admission to service, and upon discharge from services.

UTILITIES MANAGEMENT—PATIENT
Policy No. 6-025.1**PURPOSE**

To outline the responsibilities of the organization and organization personnel regarding utilities management in the patient's home.

POLICY

All personnel will be knowledgeable of the principles of electrical safety and utilities management, and will assess patients' homes for utility safety on the first visit, and subsequently thereafter, as warranted, to identify potential or actual safety risks.

PROCEDURE

1. During the initial home visit, the admitting clinician/technician will conduct a home safety check, which includes an assessment of utility safety.
2. The assessment will include, but will not be limited to:
 - A. Placement of electrical cords
 - B. Condition of electrical cords/outlets
 - C. Use of extension cords
 - D. Proper grounding of appliances/equipment
 - E. Appropriate size and type of light bulbs
3. Based on the assessment, prevention tips and suggestions will be made to reduce the risk of problems and potential hazards in the home. Prevention tips may include, but are not limited to:
 - A. Remove cords from under furniture or carpeting.
 - B. Replace damaged or frayed cords.
 - C. Follow UL rating label guidelines when using extension cords.
 - D. Never use nails or staples to secure wires.
 - E. Replace bulbs with the correct type and wattage.

- F. Never defeat the grounding feature; move cords and appliances away from sink areas and hot surfaces.
- 4. Patient education will be documented in the clinical/service record, including:
 - A. What was taught (reference written home safety instructions, as necessary)
 - B. Suggestions made to reduce environmental hazards
 - C. Level of understanding
 - D. Patient and/or family/caregiver response to teaching

EQUIPMENT MANAGEMENT—PATIENT
Policy No. 6-026.1**PURPOSE**

To outline the responsibilities of the organization and organization personnel regarding equipment management in the patient's home.

POLICY

All personnel will be knowledgeable regarding equipment management and safety as it applies to the home setting.

PROCEDURE

1. During the initial home visit, the admitting clinician will assess the patient and/or family/caregiver level of knowledge in using any equipment found in the home.
2. The clinician will, through interview and observation, determine the patient's and/or family/caregiver's level of understanding in the use of medical equipment.
3. The clinician will follow the guidelines set forth in "Safe and Appropriate Use of Medical Equipment and Medical Supplies." (See "[Safe and Appropriate Use of Home Medical Equipment and Supplies](#)" Policy No. 6-030 for further actions.)
4. If a clinician, identifies any safety hazards or potential safety hazards, the clinician/technician will notify the home medical equipment company and document the findings.
5. Any potential and real safety hazards will be to be documented on an incident report form.
6. Patient education will be documented in the clinical/service record, including:
 - A. What was taught (reference teaching materials)
 - B. Suggestions made to reduce environmental hazards
 - C. Level of understanding
 - D. Patient and family/caregiver response to teaching

INFECTION CONTROL PLAN**Policy No. 6-027.1****PURPOSE**

To delineate an infection control plan to meet the following goals:

1. Establish the mechanism by which the organization will address surveillance, prevention, identification, control and reporting of infections, utilizing current scientific methods and epidemiologic principles
2. Guide organization personnel in the care and services they provide in relation to infection control practices
3. Educate organization personnel, patients and family/caregivers, and others in the prevention and control of infections
4. Provide for surveillance systems to track the occurrence and transmission of infections
5. Comply with all applicable local, state, federal regulations, and accepted standards of practice including, but not limited to:
 - A. State and federal OSHA mandates
 - B. CDC recommendations and guidelines
 - C. State Practice Acts
 - D. Agency for Healthcare Research and Quality (AHRQ)
 - E. Association for Professionals in Infection Control and Epidemiology (APIC)
 - F. Local regulations including reporting of outbreaks or undue prevalence of infections, parasitic disease or infestation

POLICY

Organization's Name is committed to reducing the risk of acquisition and transmission of health care associated infections (HAIs). Recognized prevention and control mechanisms will be implemented for planning, surveillance, identification, prevention/controls, and reporting procedures. To determine the effectiveness of the infection control plan, Organization's Name will measure, assess, improve, and redesign (as appropriate) the surveillance, identification, prevention, and control function annually through its performance improvement program. [Refer to Occupational Safety and Health Administration (OSHA) website for most recent standards on occupational exposure (www.osha.gov)]

Definitions

The following definitions describe terms used by Organization's Name throughout this section.

1. **Aseptic:** Near-sterile technique referring to methods used to prevent the spread of microorganisms.
2. **Blood:** Human blood, human blood components, and products made from human blood.
3. **Bloodborne Pathogens:** Pathogenic microorganisms that are present in human blood and can cause disease in humans. These pathogens include, but are not limited to, hepatitis B virus (HBV) and human immunodeficiency virus (HIV).
4. **Body Fluids:** Emesis, sputum, feces, urine, semen, vaginal secretions, cerebrospinal fluid (CSF), synovial fluid, pleural fluid, pericardial fluid, amniotic fluid, and human breast milk; along with other fluids such as nasal secretions, saliva, sweat, and tears.
5. **Clinical Laboratory:** Workplace where diagnostic or other screening procedures are performed on blood or other potentially infectious materials.
6. **Contaminated:** The presence, or the reasonably anticipated presence, of blood or other potentially infectious materials on an item or surface.
7. **Contaminated Materials:** Reusable materials that have been exposed to or contaminated by blood or body fluids. These materials may be transported to destinations outside the patient's home (i.e., blood specimens to laboratories).
8. **Contaminated Laundry:** Laundry that has been soiled with blood or other potentially infectious materials, or may contain sharps.
9. **Contaminated Sharps:** Any object capable of cutting or penetrating the skin that has been in contact with blood or body fluids, including, but not limited to, needles, scalpels, broken glass, broken capillary tubes, and exposed ends of dental wires.
10. **Contaminated Wastes:** Disposable materials that have been exposed to or contaminated by blood or body fluids.
11. **Decontamination:** The use of physical or chemical means to remove, inactivate, or destroy bloodborne pathogens on a surface or item to the point where they are no longer capable of transmitting infectious particles and the surface or item is rendered safe for handling, use, or disposal.
12. **Engineering Controls:** Controls that isolate or remove the bloodborne pathogens hazard from the workplace (e.g., sharps disposal containers, self-sheathing needles, needleless IV systems).
13. **Exposure Incident:** A specific eye, mouth, or other mucous membrane, non-intact skin, or parenteral contact with blood or other potentially infectious material(s) that results from the performance of an organization personnel's duties.
14. **Hand Antisepsis:** Refers to either antiseptic hand wash or antiseptic hand rub.

15. Hand Hygiene: A general term that applies to either hand washing, antiseptic hand wash, antiseptic hand rub, or surgical hand antisepsis.
16. Hand Washing: Washing hands with plain (i.e., non-antimicrobial) soap and water.
17. Infectious Wastes are defined as:
 - A. Sharps: Any waste capable of producing injury including, but not limited to, contaminated needles, syringes, scalpels, and disposable instruments.
 - B. Blood, Blood Products, and Body Fluids: All waste blood, blood products, and body fluids greater than 20ml. (2/3 oz.) in volume that exist in a free liquid state and cannot be carefully poured down a drain.
 - C. Microbiological Waste: Cultures and stocks of infectious agents and associated biologicals including culture dishes and devices used to transfer, inoculate, and mix cultures.
 - D. Contaminated Lab Waste: All lab specimens consisting of blood or body fluids that cannot be disposed of by careful pouring down a drain.
18. Licensed Healthcare Professional: A person whose legally permitted scope of practice allows him/her to independently perform the activities required by the hepatitis B vaccination and post-exposure evaluation and follow-up.
19. HBV: Hepatitis B virus.
20. HIV: Human immunodeficiency virus.
21. Occupational Exposure: Reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious material(s) that may result from the performance of an organization personnel's duties.
22. Other Potentially Infectious Materials: Any body fluid that potentially contains blood, e.g., feces, nasal secretions, sputum, sweat, tears, urine, emesis, human breast milk, saliva, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids;
 - A. Any unfixed tissue or organ, other than intact skin, from a human, living or dead
 - B. HIV-containing cell or tissue cultures, organ cultures; and HIV or HBV containing culture medium or other solutions; and blood, organs, or other tissues from experimental animals infected with HIV or HBV
23. Penetrating Injury: Piercing mucous membranes or the skin barrier through such events as needle sticks, human bites, cuts, and abrasions.

24. Personal Protective Equipment (PPE): Specialized clothing or equipment worn by personnel for protection against a hazard. PPE will be considered "appropriate" only if it does not permit blood or other potentially infectious materials to pass through or reach organization personnel's work clothes, street clothes, undergarments, skin, eyes, mouth, or other mucous membranes under normal conditions of use and for the duration of time which the protective equipment will be used. General work clothes, (e.g., uniforms, pants, shirts, or blouses) not intended to function as protection against a hazard, are not considered to be personal protective equipment.
25. PPD: Purified protein derivative for intradermal tuberculin skin tests.
26. Problem-oriented or outbreak response surveillance: Surveillance that is conducted to measure the occurrence of specific infections in multiple patients at the same time.
27. Regulated Waste: Liquid or semi-liquid blood or other potentially infectious materials in a liquid or semi-liquid state if compressed; items that are caked with dried blood or other potentially infectious materials and are capable of releasing these materials during handling; contaminated sharps; and pathological and microbiological wastes containing blood or other potentially infectious materials.
28. Source Individual: Any individual, living or dead, whose blood or other potentially infectious materials may be a source of occupational exposure to personnel.
29. Sterilize: The use of a physical or chemical procedure to destroy all microbial life including highly resistant bacterial endospores.
30. Standard Precautions: An approach to infection control where all human blood and certain human body fluids are treated as if known to be infectious for HIV, HBV, and other bloodborne pathogens.
31. Targeted or Priority-directed Surveillance: Surveillance activities that focus on specific patient populations or specific procedures.
32. TST: Tuberculin skin test
33. Waterless Antiseptic Agent: An antiseptic agent that does not require use of exogenous water. After applying such an agent, the hands are rubbed together until the agent has dried.
34. Work Practice Controls: Controls that reduce the likelihood of exposure by altering the manner in which a task is performed (e.g., prohibiting recapping of needles by a two (2)-handed technique).
35. Nosocomial Infections: This type of infection is known as a hospital-acquired infection. These infections are a result of treatment in a hospital. Infections are considered nosocomial if they first appear 48 hours or more after hospital admission or within 30 days after discharge.

PROCEDURE

1. Organization's Name will educate all personnel on infection control policies, procedures, and their responsibilities for implementation as contained throughout this section. New personnel will receive a copy of the standard precautions (see "[Standard Precautions](#)" Policy No. 6-004) in their orientation packets.
2. Personnel will be provided training on the basics of transmission of pathogens to patients and personnel, bloodborne diseases, the use of standard precautions, infectious waste management, and other infection control procedures when their work activities, as indicated below, may result in an exposure to blood, other potentially infectious materials, or under circumstances in which differentiation between body fluid types is difficult or impossible.
3. Infection control inservices will be scheduled no less than annually.
 - A. Attendance will be mandatory and will be documented.
 - B. Records of inservice attendance will be maintained in the personnel file.
4. The organization will utilize its safety and performance improvement process to identify risks for the acquisition and transmission of infectious agents on an ongoing basis.
5. The infection control plan will be monitored and evaluated in the annual program evaluation and in conjunction with the review of the organization's safety and performance improvement activities.
 - A. Success or failure of interventions for preventing and controlling infection will be addressed.
 - B. Evolution of relevant infection control and prevention guidelines based on evidence and/or expert consensus will be considered.
6. The Performance Improvement Coordinator will be responsible for managing and coordinating infection control activities and reporting of infection control activities to the Performance Improvement Committee and other appropriate authorities. Infection control activities and outcomes will be part of the organization's QAPI program. The Performance Improvement Coordinator will maintain qualifications for infection control responsibility through ongoing education and training.

TUBERCULOSIS EXPOSURE CONTROL PLAN**Policy No. 6-028.1****PURPOSE**

To provide guidelines to reduce the risk of occupational exposure to mycobacterium tuberculosis (TB).

POLICY

Organization's Name will minimize the occupational exposure to TB through the development of a TB exposure plan, organization personnel education, and implementation of this plan. An annual organization TB risk assessment will be completed to determine the need, type and frequency of testing/assessment for direct care personnel. Organization's Name will reassess the risk of transmission of TB to evaluate the effectiveness of the TB exposure plan. The risk assessment will include consideration of local and state regulations regarding Tuberculin Skin Tests (TSTs) for employees.

An occupational exposure to TB will be defined as face-to-face contact for more than ten (10) minutes or being in the same room with a patient having active pulmonary or laryngeal TB for more than thirty (30) minutes without the use of a mask. Any employee who may encounter these conditions during the performance of his/her assigned duties will be considered to be at risk.

PROCEDURE

1. Patients will be assessed on admission for high-risk categories or conditions, and signs and symptoms of TB.
2. Any patient who is considered high-risk and has exhibited a cough and at least one (1) other symptom will be identified as a potential TB patient.
3. The attending physician (or allowed practitioner) will be contacted for an order to perform a TST (with patient permission).
4. For patients with signs/symptoms suggestive of TB (persistent cough longer than two (2) weeks' duration, bloody sputum, night sweats, weight loss, anorexia, fever), respiratory precautions will be applied as part of the initial assessment and care/service planning process.
5. If a patient is already diagnosed with TB, the patient will be accepted onto care/service after:
 - A. A confirmed follow-up appointment with a licensed physician has been arranged
 - B. Sufficient medication has been given to the patient until the outpatient appointment
 - C. A registered nurse or respiratory therapist is assigned to coordinate the care/service

6. Patients should not be sent home if there are persons in the household who are at high-risk of active TB transmission (i.e., HIV, immunocompromised patients, or children younger than five (5) years old).
7. Personnel will:
 - A. Attend an inservice on the use of a NIOSH-certified N95 mask and will undergo fit testing based on organization's risk assessment or when a patient has been identified with known or suspected TB.
 - B. Wear a NIOSH-certified N95 mask when entering the home of a patient with known or suspected TB.
 - C. Place the patient on respiratory isolation only when there are children or immunocompromised individuals living in the home.
 - D. Collect sputum for acid-fast bacilli (AFB) in a well-ventilated area of the home, away from other household members. Opening a window to improve ventilation or specimen collection outdoors should be considered, when feasible.
 - E. Discontinue respiratory precautions when the patient has been on effective therapy, is improving clinically, and the sputum smear is negative for AFB on three (3) consecutive days.
 - F. Monitor patients with active TB for relapse with sputum smears every two (2) weeks until discharged from care/services.
8. All personnel will receive training/education upon hire, and annually thereafter. The education will include:
 - A. Basic concepts of TB transmission, pathogenesis, diagnosis, the difference between the signs and symptoms of latent and active TB
 - B. Potential for occupational exposure
 - C. Principles of infection control to reduce risk of transmission
 - D. Purpose of Tuberculin Skin Test
 - E. Principles of preventative therapy for latent infection
 - F. Principles of drug therapy
 - G. Individual responsibility
 - H. Responsibility of organization

9. All personnel will be tested for TB according to the risk assessment identification for which personnel have potential exposure to TB and in accordance with federal, state, and/or local regulations. OSHA has determined that home health care is not provided in one (1) of the workplaces where CDC has identified workers as having a higher incidence of TB infection than the general population.

A. Risk Definitions

1. Low-Risk—Personnel for whom the TST conversion rate is not greater than that for personnel for whom occupational exposure to TB is unlikely (i.e., office personnel) or than previous conversion rates for the same category of personnel. This includes personnel for whom there have been no clusters of TST conversions, no detection of TB transmission, and fewer than three (3) TB patients are cared for each year.
 2. Intermediate Risk—Personnel for whom the TST conversion rate is not greater than that for personnel for whom occupational exposure to TB is unlikely (i.e., office personnel) or than previous conversion rates for the same personnel. This includes personnel for whom no clusters of TST test conversions have occurred, no detection of TB transmission, and three (3) or more TB patients are cared for each year.
 3. High-Risk—Personnel for whom the TST conversion rate is significantly higher than personnel for whom exposure to TB is unlikely (i.e., office personnel) or than previous conversion rates for the same personnel and epidemiologic evaluation suggests transmission of TB. This includes personnel for whom a cluster of TST conversions have occurred, and possible person-to-person transmission of TB has been detected.
- B. In the absence of known exposure or evidence of ongoing TB transmission, personnel without LTBI should not undergo routine serial TB screening or testing at any interval after baseline. The organization may consider using serial TB screening of certain groups who might be at increased risk for TB exposure or in certain settings if transmission has occurred in the past.
- C. Upon hire, personnel must provide the organization evidence of a baseline TB skin or blood test, if there is no evidence of a baseline TB skin or blood test conducted at any point in the past. If the personnel is unable to provide evidence of a baseline TB skin or blood test, then the organization will conduct TB testing.
- D. Where required by state and/or local regulations, a two (2) step TST skin test will be given to all personnel upon hire, if no TST has been performed within twelve (12) months of hire date, and thereafter, according to risk assessment.
- E. In addition to the TB screening test, all patient care personnel will be given an individual TB risk assessment to establish a baseline (See "[Individual TB Risk Assessment](#)" Addendum 6-028.A).

1. If high-risk exposure has occurred since the administration of the baseline TB test, the organization will conduct an individual TB risk assessment and symptom evaluation prior to patient contact (See "[Individual TB Risk Assessment](#)" Addendum 6-028.A).
- F. The testing procedure, reading, documentation and record keeping of TST will be kept in compliance with applicable law and regulation. The person(s) conducting the testing procedures and reading the results of the TST will be competency tested in the appropriate procedures.
- G. The organization will conduct an annual TB risk assessment to determine the need, type and frequency of testing and assessment for all direct care personnel. Annual TB testing of personnel is not recommended, unless there is a known exposure or ongoing transmission.
 1. Those employees known to have positive TST results will be given an annual questionnaire regarding the existence of active TB signs and symptoms.
 2. Personnel with newly identified positive TB test results will be given a symptom evaluation and a chest radiograph to assess for TB disease.
10. Follow-up with a physician will be mandatory for any employee identified through the use of the questionnaire to have signs or symptoms of active TB.
11. When TB exposure is known or thought to have occurred, a TST will be administered to the individual as soon as possible. This test will serve as a baseline reading. Personnel known to be TST positive should not be retested.
 - A. If negative, a second TST will be administered eight (8) to ten (10) weeks post-exposure to determine if infection has occurred. If this TST is negative, no further testing is necessary.
12. TST converters will be sent to an occupational health provider, the employee's personal physician, or to the county health department for further interpretation and follow-up. Antimicrobial prophylaxis may be indicated.
13. All TST converters must have a chest x-ray to determine possible presence of active pulmonary TB. To detect the existence of extrapulmonary TB, additional physical assessment and testing may be necessary.
14. Work restriction will not be initiated if the employee is asymptomatic.
15. When active disease is present, the individual will be restricted from work assignments until antimicrobial prophylaxis has been initiated and there are three (3) negative AFB smears obtained on different days.
16. Organization's Name will comply with reporting and record keeping requirements according to state and federal law.

Note: For further information regarding Tuberculosis please refer to the:

1. Morbidity and Mortality Weekly Report (MMWR)
“Guidelines for Preventing the Transmission of *Mycobacterium tuberculosis* in Health-Care Settings, 2005”
December 30, 2005/Vol. 54/No. RR-17
<https://www.cdc.gov/mmwr/preview/mmwrhtml/rr5417a1.htm>
2. Visit <https://www.cdc.gov/tb/education/professionaltools.htm> for a list of resources by topic.
For more information, contact:
CDC
TB Surveillance Department
Atlanta, GA
(404) 639-1819
www.cdc.gov

ADDENDUM 6-028.A

INDIVIDUAL TB RISK ASSESSMENT

INDIVIDUAL TB RISK ASSESSMENT

Health care personnel should be considered to be at increased risk for TB if they answer "yes" to any of the following statements.

1. Temporary or permanent resident (for ≥ 1 month) in a country with a high TB rate (i.e., any country other than Australia, Canada, New Zealand, the United States and those in western or northern Europe)

OR

2. Current or planned immunosuppression, including human immunodeficiency virus infection, receipt of an organ transplant, treatment with a TNF-alpha antagonist (e.g., infliximab, etanercept, or other), chronic steroids (equivalent of prednisone ≥ 15 mg/day for ≥ 1 month), or other immunosuppressive medication

OR

3. Close contact with someone who has had infectious TB disease since the last TB test.

Have you had any of the following symptoms?	Descriptions	Yes	No
1. Unexplained productive cough	Cough greater than three weeks in duration		
2. Unexplained fever	Persistent temp elevations longer than one (1) month		
3. Night sweats	Persistent sweating that leaves sheets and bedclothes wet		
4. Shortness of breath/chest pain	Presently having shortness of breath or chest pain		
5. Unexplained weight loss/appetite	Loss of appetite with unexplained weight loss		
6. Unexplained fatigue	Very tired for no reason		
7. Have you been exposed to anyone with TB?	Personally or professionally within the past twelve (12) months		
8. Have you traveled outside the U.S.?	In the past twelve (12) months		

**BLOODBORNE PATHOGENS AND HEPATITIS B
EXPOSURE CONTROL PLAN
Policy No. 6-029.1****PURPOSE**

To provide guidelines to reduce the risk of occupational exposure to bloodborne pathogens, including Hepatitis B (HBV).

POLICY

Organization's Name will minimize the occupational exposure to bloodborne pathogens, including Hepatitis B, through the development of an exposure plan, organization personnel education, and implementation of this plan.

PROCEDURE

1. The organization shall ensure that the exposure control plan is made accessible to employees.
2. The organization's exposure control plan shall be updated and revised at least annually and whenever necessary to reflect new or modified tasks and procedures which affect occupational exposure and to reflect new or revised employee positions with occupational exposure.
3. Information will be provided to new direct care employees upon hire regarding the risks of exposure to bloodborne pathogens, HBV vaccine, including information on its efficacy, safety, method of administration, and the benefits of being vaccinated. (See "[Recognizing the Dangers](#)" Addendum 6-029.C and "[Occupational Exposure Risk By Job Classification](#)" Addendum 6-029.D.)
4. Determination of the new direct care employee's current HBV vaccination will be completed in the pre-employment health assessment
5. The vaccine and vaccination will be offered free of charge within ten (10) days of employment.
6. An employee who reports having received the HBV vaccination previously will be required to provide Organization's Name with a record of the vaccination and any antibody testing that may have been performed.
7. An employee who has not been previously vaccinated and who declines administration of the HBV vaccine is required to sign a declination statement. This statement will be kept as part of the individual's health record.
8. An employee who wishes to receive the HBV vaccine will be administered three (3) doses over a six (6) month period at the recommended intervals. Documentation of each dose will be maintained in the individual's health record.

9. The use of standard precautions in the work place is considered effective preventive methodology in the care of patients with suspected or diagnosed bloodborne pathogen infections. Standard precautions should be implemented as outlined in "[Standard Precautions](#)" (see Policy No. 6-004).

ADDENDUM 6-029.A

HEPATITIS B VACCINATION DOCUMENTATION FORM

HEPATITIS B VACCINATION DOCUMENTATION FORM

Organization Personnel:

Position:

HBV VACCINATION				
Series	DATE	Administered By	Lot #	Organization Signature

ADDENDUM 6-029.B

HEPATITIS B VACCINATION DECLINATION FORM

(MANDATORY)

HEPATITIS B VACCINATION DECLINATION FORM (MANDATORY)

I understand that due to my occupational exposure to blood or other potentially infectious materials I may be at risk of acquiring hepatitis B virus (HBV) infection. I have been given the opportunity to be vaccinated with hepatitis B vaccine, at no charge to myself. However, I decline hepatitis B vaccination at this time. I understand that by declining this vaccine, I continue to be at risk of acquiring hepatitis B, a serious disease. If in the future I continue to have occupational exposure to blood or other potentially infectious materials and I want to be vaccinated with hepatitis B vaccine, I can receive the vaccination series at no charge to me.

Date: _____

Organization Personnel Signature

I decline to receive a Hepatitis B Vaccination because I have been previously vaccinated. I agree to provide Organization's Name with a record of the vaccination and any antibody testing that may have been performed.

Date: _____

Organization Employee Signature

ADDENDUM 6-029.C

RECOGNIZING THE DANGERS

RECOGNIZING THE DANGERS

Name:

Job Title:

Organization Personnel Identification Number:

Employer/Facility:

Department:

Date of Training:

Organization Personnel's Signature:

I have received training covering the following. (Please check the appropriate boxes below):

- ☐ Spread of HIV and HBV infection in the general population
- ☐ Number of health care workers who have gotten HIV and HBV infections on the job
- ☐ Symptoms and effects of HIV and HBV infection
- ☐ Ways of preventing, detecting, and treating HIV and HBV infections

In addition, I have been informed that:

- ☐ Sexual contacts of people who have HIV and HBV infections and IV drug abusers are at special risk of getting the AIDS or HBV, and
- ☐ The risk to health care workers is increasing as HIV and HBV spread in the general population.

ADDENDUM 6-029.D

**OCCUPATIONAL EXPOSURE RISK
BY JOB CLASSIFICATION**

BLOODBORNE PATHOGEN EXPOSURE DETERMINATION**Employees with Occupational Exposure**

Job Classification	All	Some	None	Comments
Administrator/CEO	X			When performing clinical tasks
Clinical Director	X			When performing clinical tasks
Program Supervisor (Clinical Managers)	X			When performing clinical tasks
PI Coordinator	X			When performing clinical tasks
R.N.	X			
LPN/LVN	X			
Home Health Aide	X			
Physical Therapist/PTA		X		
Occupational Therapist/COTA		X		
Speech Therapist		X		
Medical Social Worker		X		
Dietitian		X		
Medical Director		X		
Office Personnel			X	
Volunteer		X		

SAFE AND APPROPRIATE USE OF MEDICAL EQUIPMENT AND SUPPLIES

Policy No. 6-030.1

PURPOSE

All home medical equipment and medical supplies will be managed in a safe and appropriate manner. Medical equipment used in the provision of care is maintained, tested, and inspected.

Definitions

1. *Home Medical Equipment*: Any assistive device or piece of equipment used by home care personnel, patient, or family/caregiver to meet the patient's home care needs (i.e., walker, commode, Hoyer lift, apnea monitor, etc.).
2. *Home Care Supplies*: Those disposable items used by home care personnel, patient, and/or family/caregiver to meet the patient's home care needs (i.e., dressings, syringes, catheters, tubing, gloves, etc.).
3. *Oxygen and Related Equipment*: Oxygen gas and any equipment used to deliver the gas to the patient (i.e., oxygen tank and tubing, pulmo-aid, concentrator, etc.).

PROCEDURE

1. The selection, delivery, setup, and maintenance of home medical equipment and oxygen is the responsibility of the home medical equipment company. Any equipment used in the provision of patient care will have routine and preventive maintenance, as defined by the manufacturer's guidelines or at least annually.
2. The office may provide a storage area for home care supplies. During orientation and on an ongoing basis (as needed), personnel will be instructed regarding access, handling, and delivery of these supplies. The Clinical Supervisor will establish a tracking mechanism to locate equipment used in the provision of care/service.
3. The Program Supervisor or designee will assume the responsibility for instructing personnel on safe and appropriate use of home medical equipment, supplies, and oxygen. This process will be achieved through orientation, annual safety and risk management inservices, and ongoing programs as needs are identified.
4. It is the responsibility of the clinician/technician, as appropriate, to instruct patients and their family/caregivers regarding the safe and appropriate use of home medical equipment, home care supplies, and oxygen. These needs will be identified during the initial assessment via the home safety checklist, and ongoing assessment with follow-up, as appropriate, in the plan of care. Evidence of this instruction will be documented in the clinical record.

5. The following equipment used by clinical personnel will have routine and preventative maintenance as defined in the manufacturer's guidelines:
 - A. Home glucose monitors
 - B. Rehabilitative equipment

**STORAGE OF MEDICATIONS
AND NUTRITIONAL THERAPIES**
Policy No. 6-031.1

PURPOSE

To ensure that medications and nutritional therapy solutions are properly handled and stored.

POLICY

Medications and nutritional therapy solutions will be properly stored in the organization and in patient homes.

PROCEDURE

1. Storage of medication in the organization will be consistent with applicable law and regulation.
2. Investigational medications and cytotoxic medications will be stored in a secure location and appropriately identified.
3. Medications used for external use and disinfectants will be stored separately from internal and injectable medications.
4. All medications, chemicals, and biologicals will be labeled for contents, with expiration dates clearly identified.
5. Medications and nutritional therapy solutions will be stored under conditions that enhance stability. Elements to be considered include:
 - A. Appropriate storage temperatures, utilizing appropriate thermometers and temperature logs
 - B. Protecting solutions from contamination and spoilage
 - C. Controlling lighting, ventilation, and humidity
 - D. Prevention of moisture, condensation, and mold growth
 - E. Thorough cleaning and sanitizing of all surfaces, supplies, and equipment after each use
6. The environment where medications or nutritional therapies are prepared will be appropriate to the therapy preparations, in the office and patient home. Areas to consider include:

- A. Functionally separate areas for sterile product preparation
 - B. An environment suitable to preparation of sterile products
 - C. Safety equipment to protect personnel preparing cytotoxic or hazardous medications
 - D. Clutter free, clean work surface for medication preparation or nutritional therapy solutions
7. Medication and nutrition therapy preparation will only be done by personnel with documented competencies regarding medication and nutrition therapy preparation. Multi-dose vials will be discarded 28 days after first use.
8. Clinicians with the appropriate competencies will provide instruction and education to the patient and family/caregiver and document evidence of instruction in the clinical/service record.

MEDICAL EQUIPMENT MALFUNCTION

Policy No. 6-032.1

PURPOSE

To define the process to follow in the event of medical equipment malfunction.

POLICY

It is organization policy to report and document any medical equipment malfunction and serious injury, illness, or death associated with any medical equipment (whether the equipment has malfunctioned or not).

PROCEDURE

1. Personnel should report any medical equipment malfunction to their Program Supervisor.
2. The Program Supervisor and/or clinician/technician will be responsible for reporting the malfunction to the equipment company.
3. An incident report will be completed. (See "[Incident Reporting](#)" Policy No. 5-005.)
4. If the medical equipment malfunction or misuse results in serious injury, illness, or death Risk Management should be notified immediately. Risk Management must report the illness, injury, or death in association with any medical device to the FDA within ten (10) working days of the event, in compliance with the Safe Medical Devices Act of 1990 and the 1995 Final Rule requiring home health care compliance, effective 7/31/96.

SAFE MEDICAL DEVICE ACT**Policy No. 6-033.1****PURPOSE**

To ensure that Organization's Name is in compliance with the Safe Medical Device Reporting (SMDR) regulations.

POLICY

When it has been determined that a medical device has, or may have, caused or contributed to the serious injury of a patient, the organization will make a report to the manufacturer of the device, if known, and to the FDA when the manufacturer is not known.

When it has been determined that a medical device has, or may have, caused the death of a patient, the organization will make a report to the manufacturer of the device and to the FDA.

The Executive Director/Administrator will be responsible for determining when a reportable event has occurred and will complete all required FDA reports.

PROCEDURE

1. A report using FDA Form 3500A will be completed and submitted to the manufacturer of a device when it is suspected or determined that a device has caused serious injury to a patient. The report will be completed within ten (10) working days of discovery of the serious injury to the patient.
2. A report using FDA Form 3500A will be completed and submitted to the manufacturer of a device and the FDA when it is suspected or determined that a device has caused a patient death. The report will be completed within ten (10) working days of discovery of the patient's death.
3. Information to be reported on Form 3500A will include:
 - A. Patient information
 - B. Type of adverse event
 - C. A description of the event
 - D. Relevant laboratory/test data and patient history
 - E. Manufacturer and identification of the suspect device and certain other information about the device

- F. Initial reporter of the event
 - G. User facility name, address and contact
 - H. Where and when the report was sent
4. In addition to individual device reports, an annual report to the FDA will be made, using FDA form 3419, summarizing all reports sent to manufacturers and the FDA in the previous year. The annual report is due by January 1st of each year where a reportable occurrence has occurred.
 5. Information to be reported annually on Form 3419A will include:
 - A. CMS provider number or FDA assigned reporting number
 - B. Reporting year, reporting period, and report date
 - C. Complete name and address of the user facility
 - D. Name, title, and address of the contact person
 - E. Lowest and highest report numbers of the reports submitted to the FDA and/or manufacturer during the reporting period
 - F. Basic information about each reported event or a copy of the FDA Form 3500A that was submitted for each event
 6. A file will be established and maintained for each reportable event and will include:
 - A. Information related to the event—including all documentation of the reporting decisions and the decision-making process
 - B. Copies of all completed Medical Device Reporting forms and other information submitted to the FDA, distributors, and manufacturers
 7. All records will be maintained for a period of two (2) years after the reportable event.
 8. Safe Medical Device Reporting inservice education will be provided to personnel on an annual basis. Documentation of inservices will include:
 - A. Dates and times of sessions
 - B. Written curriculum outlines describe training content
 - C. Records of attendance

ORGANIZATION PERSONNEL SAFETY—PERSONAL SAFETY
Policy No. 6-034.1**PURPOSE**

To provide guidelines for personnel to ensure their own personal safety and security while providing care/service in the community setting.

POLICY

Self-protection to enhance personal safety is the responsibility of all personnel. Safety review and training will be provided during orientation, and as part of ongoing inservice programs.

PROCEDURE

1. The Executive Director/Administrator is responsible for the development, implementation, and monitoring of security and safety training activities.
2. Access to or egress from security-sensitive areas will be identified by senior management and managed by monitoring areas directly and locking access when appropriate personnel are not present.
3. For patient care staff: There will be established daily plans to keep a designated contact person informed about staff whereabouts throughout the day. A contact person will follow up if an employee does not report as expected.
4. All employees will wear Organization's Name approved identification badges at all times when working.

General Safety Precautions

1. Know exactly where you are going before you leave the office.
2. Know the community where visits are being made.
3. Do not take personal safety for granted.
4. Keep alert to avoid becoming a victim of an attack or robbery. Personnel should look for the unexpected and avoid taking unnecessary chances.
5. Review concerns with the Program Supervisor prior to the visit, if the environment does not feel safe.
6. Leave the home as quickly as possible, and contact the office when safe, if an unsafe situation should arise during a visit.
7. Carry identification, including the phone number of Organization's Name, police and fire departments of the municipalities in your territory. (Use 911 when appropriate.)

8. Be sure that your vehicle is in good working order and that you have sufficient fuel. Always keep all doors locked.
9. Consider having a spare set of keys in the home care bag or briefcase or keep keys in a magnetic holder hidden on the outside of the car. Locking keys in the car can happen unexpectedly.
10. Do not carry excessive amounts of cash. Do carry enough money for emergency transportation and phone calls.
11. If possible, avoid carrying a purse. If you do carry a purse and are driving, lock it in your trunk before leaving the office and leave it there while visiting patients. Keep money and identification in an inside pocket.
12. Never leave laptop/device in plain sight in vehicle. Place laptop/device in trunk of car if unable to take into home due to filth, infection control or other reasons.
13. Dress appropriately. If you do not wear uniforms, wear conservative street clothes. Do not wear suggestive clothing. Wear shoes that fit comfortably and well so that you can move quickly and safely, if necessary. Wear a nametag and carry some form of identification so patients can be assured you are a valid representative of Organization's Name.
14. Never knock on unmarked doors or on the doors of homes other than those of patients and family/caregivers whom you are visiting. Never enter a vacant home.
15. Do not enter if there are any doubts about the safety of entering a home or an apartment building. Call the Program Supervisor, or return to the office.
16. If a night visit is being made in a questionably safe area, plan ahead by contacting the local police to assist, if there are no other personnel to function as escort.
17. If anyone in the house appears to be drunk or under the effects of drugs, do what is essential for the patient and leave or refuse to provide services in a clearly hazardous situation.
18. If any weapons are present, leave and report this to the Program Supervisor.
19. If a pet is hostile, ask that it be contained, or leave.
20. Report to the Program Supervisor at the completion of the visit if any situation occurs.

Precautions To Take While Walking

1. Avoid groups of people lingering on corners or in doorways. Cross the street to avoid them.
2. Stay near people who are moving about. Walking in lonely, isolated areas may invite attack. Do not take short cuts down alleys, through buildings, or across private property. Avoid narrow or confined spaces.

3. Carry keys in your hand. This will enable you to get into your car immediately and you can use them as a method of self-defense. Hold the key ring in the palm of your hand and put one (1) key between each of your four (4) fingers with the sharp ends sticking out. You may want to attach a whistle on your key ring, which can be used to summon help.
4. Think about your appearance. The nametag and lab coat will indicate your purpose to the public.
5. Walk confidently. Know where you are going. If you don't, go to a store and ask or call for directions.

Self-Defense Measures

1. If you think you are being followed by someone on foot—cross the street, vary your pace, change directions. If the person persists, go to a lighted store and call the police.
2. If you think you are being followed by someone in a car—turn around and proceed in the opposite direction. If the person persists, jot down the car's license number and proceed to the nearest police station.
3. If you are being robbed:
 - A. Size up the situation—think quickly about the wisest response.
 - B. Stay alert—take note of the assailant's characteristics so you can give a full description to the police.
 - C. Stall for time by talking or fumbling for money—someone may come to your rescue.
 - D. Try to get away, if there are people around or open stores nearby—you may be able to run toward them.
 - E. Don't be heroic by taking foolish chances—your assailant may become violent. It's better to lose your money, medications, or equipment than your life.
4. If you are attacked:
 - A. Use your natural defenses
 - B. Make a scene—take your assailant by surprise
 - C. Jab with your elbow
 - D. Twist to break free
 - E. Bite hard
 - F. Scratch with your fingernails

- G. Yell to alert passersby or scare off assailant
- H. Swing briefcase/home care bag at the head of the assailant
- I. Bend assailant's fingers back
- J. Turn over any medications the assailant wants—don't be a hero!

ORGANIZATION PERSONNEL SAFETY—UNSAFE HOME VISITS
Policy No. 6-035.1

PURPOSE

To provide guidelines for addressing unsafe home visits.

POLICY

When scheduled or unscheduled home visits are thought to be “unsafe,” personnel and Program Supervisor(s) will decide upon the course of action to be taken.

PROCEDURE

1. The clinician/technician will discuss the visit circumstances with his/her immediate Program Supervisor.
2. If at all possible, the visit will be made during daylight.
3. The local police or security service will be contacted to arrange for an officer to meet the clinician/technician at the patient's home.
4. The clinician/technician will consult with the Program Supervisor regarding the advisability of canceling the visit.
5. Should the decision be that no visit will be made, the Program Supervisor will:
 - A. Contact the patient's physician, as indicated
 - B. Discuss options/alternatives for home care with the patient and physician, as indicated

VEHICLE ACCIDENT REPORTING**Policy No. 6-036.1****PURPOSE**

To define a process for documenting and reporting an on-duty vehicle accident.

POLICY

All vehicular accidents that occur during on-duty time will be reported.

PROCEDURE

1. In case of a vehicular accident involving organization personnel, or patient and organization personnel, during on-duty time, organization personnel will stop immediately and render aid to injured persons, as necessary, and within the individual's scope of practice.
2. A police officer will be present to file an accident report, unless on private property. Emergency services will be notified, as appropriate.
3. Organization personnel will obtain information including name, license number, and car insurance information from persons involved in the accident and/or witnesses.
4. Organization personnel, or the patient as appropriate, will provide proper vehicle insurance information to other persons involved in the accident.
 - A. If a patient's vehicle is involved in the accident, the patient's vehicle insurance and organization personnel's driver's license will be provided. The patient will be encouraged to notify his/her insurance agent.
 - B. If an organization personnel's private vehicle is involved in the accident, the personnel's auto insurance and driver's license will be provided. The individual will be advised to notify their insurance agent.
 - C. If an organization owned/leased vehicle is involved in the accident, organization vehicle insurance information will be provided.
5. Organization personnel will notify the office or on-call scheduler of the accident as soon as reasonably possible. The scheduler will notify the supervisor on-call immediately.
6. Organization personnel will fill out a written incident report as soon as possible, but no later than the next working day.
7. The Executive Director/Administrator or designee will notify legal counsel of the accident and forward the written incident report.

8. Organization personnel will be instructed to discuss the accident with the police, appropriate department personnel, and legal counsel only.
9. As required by state (DOT) regulations, a drug screen may be performed on the employee involved in the accident.

EMERGENCY MANAGEMENT PLAN**Policy No. 6-037.1****PURPOSE**

To establish a plan which will allow for continuation of services in the event of a disaster, natural or man-made, affecting the organization or the community and to ensure the safe management of staff while securing patient and employee records.

Definitions

1. ***Community Partners:*** Community partners are considered any emergency management officials (fire, police, emergency medical services, etc.) for full-scale and community-based exercises, however can also mean community partners that assist in an emergency, such as surrounding providers and suppliers.
2. ***Emergency:*** A natural or man-made event that significantly disrupts the environment of care, such as damage to the organization's buildings or grounds due to a severe storm or earthquake; that significantly disrupts care and services, such as loss of utilities due to floods, civil disturbances, accidents or emergencies within the organization or community; or that results in sudden, significantly changed or increased demands for the organization's services, such as emerging infectious diseases, bioterrorist attack, building collapse, or a plane crash in the organization's community.
3. ***Full-Scale:*** A full scale exercise is an operations-based exercise that typically involves multiple agencies, jurisdictions, and disciplines performing functional (for example, joint field office, emergency operation centers, etc.) and integration of operational elements involved in the response to a disaster event, i.e., "boots on the ground" response activities (for example, hospital staff treat mock patients). Though there is no specific number of entities required to participate in a full-scale community-based exercise, it is recommended that it be a collaborative exercise which involves at a minimum local or state emergency officials and is robust to develop community-based responses to potential threats.
4. ***Functional Exercise (FE):*** The Department of Homeland Security's (DHS's) Homeland Security Exercise and Evaluation Program (HSEEP) explains that FEs are an operations-based exercise that is designed to validate and evaluate capabilities, multiple functions and/or sub-functions, or interdependent groups of functions. FEs are typically focused on exercising plans, policies, procedures, and staff members involved in management, direction, command, and control functions.
5. ***Hazard Vulnerability Analysis (HVA):*** The identification of potential emergencies and the direct and indirect effects these emergencies may have on the home health organization's operations and the demand for its services.
6. ***Mitigation Activities:*** Those activities an organization undertakes in trying to lessen the severity and impact of a potential emergency.

7. **Mock Disaster Drill:** A mock disaster drill is coordinated, supervised activity usually employed to validate a specific function or capability in a single agency or organization. Mock disaster drills are commonly used to provide training on new equipment, validate procedures, or practice and maintain current skills. For example, mock disaster drills may be appropriate for establishing a community-designated disaster receiving center or shelter. Mock disaster drills can also be used to determine if plans can be executed as designed, to assess whether more training is required, or to reinforce best practices. A mock disaster drill is useful as a stand-alone tool, but a series of drills can be used to prepare several organizations to collaborate in an FSE.
8. **Preparedness Activities:** Those activities an organization undertakes to build capacity and identify resources that may be utilized during an emergency.
9. **Response Activities:** The actions taken and procedures implemented by the organization when an emergency occurs.
10. **Recovery Activities:** The final phase of emergency management, related to strategies, actions and individual responsibilities necessary to restore the organization's services after an emergency.
11. **Workshop:** A workshop, for the purposes of this guidance, is a planning meeting, seminar or practice session, which establishes the strategy and structure for an exercise program.

POLICY

The organization will comply with all applicable, Federal, State, and local emergency preparedness requirements and will establish and maintain an emergency preparedness program that meets those regulations. The organization will establish an all-hazards approach to emergency preparedness that focuses on identifying hazards and developing emergency preparedness capabilities. The organization shall review and update the emergency preparedness plan every two (2) years.

The organization shall maintain written documentation of the emergency management plan for at least four (4) years.

Note: An all-hazards approach includes emergency preparedness for natural, man-made or facility emergencies including, but not limited to, care-related emergencies, weather related emergencies (e.g., extreme heat or cold), equipment/power failures, communication interruptions (e.g. cyber attacks), loss of facility or facility supplies (i.e., food, water and electricity) and emerging infectious disease threats (i.e., hazardous waste, bioterrorism, influenza, highly communicable diseases (such as Ebola, zika, SARS, COVID-19, etc.)) or pandemics.

PROCEDURE

Planning Process - Before an Emergency

Organization leadership will conduct a hazard vulnerability analysis (HVA) that is both facility-based and community-based to identify potential emergencies (risks) that could impact the need for services or the ability to provide care to patients.

1. The HVA is documented and evaluated at least every two (2) years to reflect changes in organization, risk conditions, patient information and changes in staff.
2. The HVA should evaluate the organizations risk and potential for hazards and should include all risks that could disrupt the organization's operations and necessitate emergency response planning to address the risk mitigation requirements and ensure continuity of care.
3. Strategies should be developed based on events identified in during the development of the HVA such as staffing strategy for staff shortages and a surge capacity strategy in the event the organization accepts additional patients during an emergency.
4. Staff will work with regional or county emergency management planning agencies, where available, in:
 - A. Establishing priorities among the potential emergencies identified in the hazard vulnerability analysis
 - B. Defining organization's role in relation to the community-wide emergency management program, use of volunteers, other emergency staff, including state or federally-designated health care professionals to address the organization's surge needs.
 - C. Developing an "all-hazards" command structure within the organization that links with the community command structure.
5. Specific procedures that describe mitigation, preparedness, response and recovery strategies, actions, and responsibilities will be developed for each priority emergency.
6. The organization shall develop policies and procedures for contingency operations, such as how the organization will meet the requirements for individualized care plans, staffing shortages or surge needs.
7. Based on the hazard vulnerability analysis and community planning activities, the organization's general emergency plan may be enhanced or revised according to identified potential emergencies and planning activities.

Note: If the organization has identified emerging infectious diseases on the HVA, the organization will need to consider the following: facility modifications (i.e., as isolation, social distancing, or capacity/surge limitations), screening patients and visitors (e.g., testing

procedures for staff, visitors, and patients) and personal protective equipment (PPE) measures.

Policy No. 6-037.4

8. The Executive Director/Administrator or designee has been designated as key leadership who is responsible for all emergency activities and makes the decision to implement the emergency plan upon becoming aware of an emergency situation. The Executive Director/Administrator and key leadership will determine the leadership command structure to ensure continuity of operations (See Addendum 6-037.B "[Command Structure](#)" and Addendum 6-037.C "[Flow Chart of Alternate Roles and Responsibilities](#)").
9. The decision to discontinue or terminate the emergency management plan (operations) will be made by the Executive Director/Administrator or designee. The Executive Director/Administrator or designee will assign recovery responsibilities by functional areas (e.g. the IT department will access off-site back-ups of the electronic medical records).
10. An alternate site will be designated in the event the office must be evacuated or is not accessible due to the emergency. Any clinical and financial records or blank documentation forms necessary for care during the emergency will be maintained off-site in the event they cannot be retrieved from the office.
11. Electronic health care records and other vital assets will be protected against loss from unexpected natural or man-made disasters (See Policy No. 2-053 "[Contingency Plan](#)").
 - A. All vital information will be backed up regularly
 - B. A backup must be in place and tested regularly for the ability to restore properly.
 - C. The backup must be stored at a non-local offsite facility or in a cloud-based environment so that it cannot be affected by the same disaster that befalls the main computer system.
12. Individualized plans for patients during a natural or man-made disaster will be included as part of the comprehensive patient assessment. The comprehensive assessment will be updated as frequently as the patient's condition warrants due to a major decline or improvement in the patient's health status and a revised risk categorization may be assigned.
13. The organization educates patients and their families on emergency preparedness at the time of admission including where they can go to shelter and what is required by the shelter.
14. During the comprehensive assessment, the patient will be placed in one of three categories for triaging patient care in the event the emergency management plan (operations) is implemented. The categories included:
 - A. Category I: Patients who cannot safely forego care and require home health intervention regardless of other conditions. Patients in this category may include: highly unstable patients with a high probability of inpatient admission if home health is not provided; IV therapy patients; highly skilled wound care patients with no family/caregiver or other outside support; patients in need of critical supplies or medications.

- B. Category II: Patients with recent exacerbation of disease process; patients requiring moderate level of skilled care that should be provided that day; patients with essential untrained family/caregivers not prepared to provide needed care.
 - C. Category III: Patients who can safely forego care or a scheduled visit without a high probability of harm or deleterious effects; this category may include homemaker patients, routine supervisory visits, evaluation visits, patients with frequencies of one (1) or two (2) times a week, if health status permits, or if a competent family member/caregiver is present
- 15. The agency maintains a hard-copy of a current patient list including contact information and categories identified for each patient in the event there is a power failure.
 - 16. Sufficient medical supplies and non-medical supplies will be maintained in a patient's home in the event of an emergency. The agency will contact suppliers prior to an emergency and develop a plan to receive additional supplies as needed.
 - 17. Sufficient medical supplies and non-medical supplies (including food, bedding, and other provisions) will be maintained in the organization's office in the event of an emergency and the need to shelter in-place. The agency will contact suppliers prior to an emergency and develop a plan to receive additional supplies as needed.
 - 18. Sufficient medical supplies such as personal protective equipment and critical care equipment will be maintained in the organization's office.
 - 19. The organization shall include in their emergency management plan the available transportation needs or options available in preparation for surge events.
 - 20. All staff will be identified by use of company badges. If volunteers are utilized, other forms of identification are utilized (i.e. wrist bands, badges, vests).

Communication Plan

The organization will maintain an emergency preparedness communication plan that complies with Federal, State, and local laws and is reviewed and updated at least every two (2) years. The plan includes the following:

- 1. Names and contact information for the following:
 - A. Staff
 - B. Entities providing services under arrangement
 - C. Patients' physician (or allowed practitioner)
 - D. Volunteers, if applicable
- 2. Contact information for the following:
 - A. Federal, state, tribal, regional and/or local emergency preparedness staff

- B. Other sources of assistance
- 3. Primary and alternate means for communicating with the staff, federal, State, tribal, regional and local emergency management agencies
- 4. A timeframe requirement for on-duty and off-duty staff to check in with the organization's designated individual (e.g. every two (2) to four (4) hours for on-duty staff and every eight (8) hours for off-duty staff).
- 5. A method for sharing information and medical documentation for patients under the organization's care as necessary.
- 6. A means of providing information about the general condition and location of patients under the organization's care as permitted
- 7. A means of informing state and local emergency preparedness officials before, during, and after an emergency on the following:
 - A. Patients that the organization has been unable to contact to determine service needs
 - B. Patients in need of evacuation due to their medical or behavioral health, or conditions of the home environment
 - C. On-duty staff that the organization has been unable to contact
- 8. A means of providing information about the organization's needs, and its ability to provide assistance, to the authority having jurisdiction, the Incident Command Center, or designee.

Training and Testing

The training and testing program is reviewed and updated at least every two (2) years. If the plan is significantly updated, the organization will complete additional training and testing.

Training:

- 1. The training includes initial training in emergency preparedness policies and procedures and their assigned emergency response roles to all new and existing staff, individuals providing service under arrangement, and volunteers if applicable,
- 2. The training is provided at least every two (2) years and demonstrates staff knowledge of emergency procedures.
- 3. Organization leadership will provide for orientation and annually thereafter of all personnel regarding participation in the emergency management plan. Education to include:
 - A. Specifics of organization's emergency management plan
 - B. Specific assigned emergency response role
 - C. Information on developing their family's emergency response plan

D. Information regarding accessing housing and transportation for staff if necessary

1. All training will be documented in the personnel record of staff, individuals under arrangement and volunteers, if applicable.

Testing:

1. Organization's Name will test the emergency management plan, at each site included in the plan, at least annually, either in response to an actual emergency or in a planned exercise.
2. The agency participates in a full-scale exercise that is community-based or when a community-based exercise is not accessible, conduct an annual individual, facility-based functional exercise every two (2) years.
 - A. In the event an actual natural or man-made emergency occurred, the agency will be exempt from engaging in its next required full-scale community-based exercise or individual facility-based functional exercise following the onset of the emergency event.
3. The agency conducts an additional exercise every two (2) years, opposite the year the full-scale or functional exercise is conducted, that may include, but is not limited to:
 - A. A second full-scale exercise that is community-based or individual facility-based functional exercise, or
 - B. A mock disaster drill, or
 - C. A table top exercise or workshop that is led by a facilitator and includes group discussion, using a narrated, clinically relevant emergency scenario and a set of problem statements, directed messages or prepared questions designed to challenge an emergency plan.
 1. Planned exercises are realistic scenarios and related to the priority emergencies defined by leadership and will focus on the organization's response to an emergency that is likely to affect continuation of care, treatment or services. Exercise scenarios will allow the organization to evaluate its handling of communication, resources and assets, staff and patients.
4. All actual responses and exercises will be documented and critiqued through a multidisciplinary process that includes administrative, clinical and support staff. Documentation will be used to identify deficiencies and opportunities for improvement based on all monitoring activities and observations during the exercise or actual emergency. The documentation will identify the effectiveness of the preparedness, mitigation, response and recovery activities of the emergency operations plan. (See "[Emergency Management Evaluation Form](#)" Addendum 6-037.G.)
5. Organization's Name will modify the emergency management plan in response to the critiques of the actual emergency event response and will prioritize processes identified for improvement, develop and institute plans for improvement. (See "[Emergency Management Evaluation Form](#)" Addendum 6-037.G.)

6. Organization's Name will modify the emergency management plan in response to the critiques of the exercise. The next scheduled planned exercise will evaluate the effectiveness of the improvements that were made in response to the critiques of the previous exercise. If improvements require substantive recourses that can't be accomplished by next planned exercise, interim improvements must be put in place until final resolution.
7. The evaluation results and proposed improvements of any exercise or actual emergency will be presented to all staff, individuals under agreement, volunteers as applicable and the Governing Body. The evaluation may be incorporated into the organization's QAPI program.

When an Emergency is Declared

1. Once the decision has been made to implement the emergency management plan, the Executive Director/Administrator or designee will initiate the Pyramid Phone Communication Plan (PPCP) to notify personnel and community authorities, as appropriate (See "[Pyramid Phone Communication Plan](#)" Addendum 6-037.A.). Personnel must listen to the organization-identified Emergency Broadcasting System for organization instructions and updates, if the telephone system is not functioning, and leadership staff is unable to initiate the PPCP. Additionally, as able, personnel are to report to the office or alternate site if the office building is not accessible and normal communication systems are not working.
2. The Executive Director/Administrator and supervisory personnel will be responsible for identifying and assigning staff to cover all essential functions.
3. Following the initiation of the PPCP, all available and qualified personnel will be mobilized to perform identified services.
4. Alternate communication systems will be implemented (e.g., call forwarding, cell phones, email, walkie-talkie, etc.) as needed. (See Addendum 6-037.A "[Pyramid Phone Communication Plan](#)")
5. The Clinical Supervisors or designee(s) will review the assigned category classifications for all current patients.
6. The agency will notify the patient/caregiver to obtain needed medications and supplies immediately upon implementation of the plan.
7. The agency will communicate when possible any information received about changes in patients' location to durable medical equipment (DME)/infusion suppliers in the event additional supplies need to be delivered.
8. If feasible, the Clinical Supervisors or designee(s) will assign all available, qualified personnel to care for Category I patients first and Category II patients second. Category III patients and any other patients who do not receive scheduled care will be contacted as soon as possible. If transportation is needed to safely get personnel to patient homes, it will be arranged via county support services, such as the police, fire, or sheriff's office.

9. If care cannot be provided to Category I patients, emergency transport to alternate care delivery sites will be arranged with community support agencies.
10. New patients will not be accepted for care until the emergency situation is controlled or staffing levels permit. Patients accepted, but not yet admitted, will be triaged as noted above.
11. If the community experiences an epidemic or infection, Organization Name will not continue to accept or treat infected patients unless a plan for managing an ongoing influx of potentially infectious patients over an extended time period has been implemented.
12. In the event of a prolonged emergency situation, the Executive Director/Administrator or designee will:
 - A. Determine staffing availability and limitations, including assistance available from external staffing agencies
 - B. Identify those patients who could be discharged from home health earlier than anticipated
 - C. Determine course of action based on above information
 - D. Identify patients with continuing care needs
 - E. Contact other area home health organizations to determine the degree to which they may be able to accept new patients, if the decision is made to transfer
 - F. Notify attending physicians regarding ability to continue caring for patients
 - G. Make transfer or discharge arrangements as necessary, notifying patients and family/caregivers as appropriate
 - H. In prolonged emergency situations, the organization will retain only those patients for whom it can safely and adequately provide care
13. Safety of patients and organization personnel will take priority in all emergency situations.
 - A. Weather and road conditions will be monitored via local weather reports and state patrol reports.
 - B. Natural or community disasters will be monitored via the Emergency Broadcasting System, reports from local authorities, reports from other local health care facilities in the event there is no telephone communication.
 - C. Strike conditions will be monitored via liaison with local representatives.
 - D. In the event the office building is determined to be unsafe, the Executive Director/Administrator or designee will communicate the location to which all personnel are to report for work.

14. In all emergency situations, the Executive Director/Administrator or designee will maintain communications and act as the spokesperson among other facilities, media, and community and safety authorities.

After the Emergency

1. Staff will contact the agency administrator (or designee) to report in and receive instructions
2. The agency administrator (or designee) and staff will make a survey of patients to find out their location and condition, enquire about possible injuries or deterioration of health status and will initiate corrective action
3. The actual response will be documented and critiqued through a multidisciplinary process that includes administrative, clinical and support staff. Documentation will be used to identify deficiencies and opportunities for improvement based on all monitoring activities and observations during the emergency. The documentation will identify the effectiveness of the preparedness, mitigation, response and recovery activities of the emergency operations plan. (See "[Emergency Management Evaluation Form](#)" Addendum 6-037.G.)
4. Organization's Name will modify the emergency management plan in response to the critiques of the emergency event response and will prioritize processes identified for improvement, develop and institute plans for improvement. (See "[Emergency Management Evaluation Form](#)" Addendum 6-037.G.)

ADDENDUM 6-037.A

PYRAMID PHONE COMMUNICATION PLAN

(Add staff, entities under arrangements, patients' physicians as noted on current patient information document)

(insert Organization's List and Communication Methods)

Primary Communication Method: _____

Alternate Communication Method: _____

ADDENDUM 6-037.B

COMMAND STRUCTURE

(Insert Organization's Structure)

ADDENDUM 6-037.C

FLOW CHART OF ALTERNATE ROLES AND RESPONSIBILITIES

(Insert Organization's Flow Charts)

ADDENDUM 6-037.D

HAZARD VULNERABILITY ANALYSIS

Resources for Emergency Management tools can be found at the following websites:

Threat and Hazard Identification and Risk Assessment Guide
Homeland Security

<https://www.hsd.org/?abstract&did=813555>

All-Hazards Preparedness Guide
Centers for Disease Control and Prevention (CDC)

https://www.cdc.gov/phpr/documents/ahpg_final_march_2013.pdf

ADDENDUM 6-037.E

PLANS FOR PRIORITY EVENTS

(Insert Individualized Plans as Appropriate to the Environment and Geographic Service Area)

1. Natural disasters
 - A. Severe weather
 1. Floods
 2. Snow/Ice
 3. Hurricane
 4. Tornado
 - B. Earthquakes
2. Environmental
3. Biological/Emerging Infectious Disease
4. Cyberattacks

ADDENDUM 6-037.F

WEATHER REPORT/ROAD CONDITIONS

(Insert Phone Numbers and Websites)

Weather Report

National Weather Forecast Office Phone Number and Website:

Weather Forecast Phone Number and Website:

Road Conditions

Highway Patrol Phone Number and Website:

State Highway Patrol Phone Number and Website:

ADDENDUM 6-037.G

EMERGENCY MANAGEMENT PLAN EVALUATION FORM

(Complete this form following an emergency drill or actual emergency)

EMERGENCY MANAGEMENT PLAN EVALUATION FORM

Evaluation completed for ____ actual event ____ drill? Date of event: _____

Summary of event: _____

	YES	NO	NA	COMMENTS
Plan was initiated in a timely manner				
Plan was initiated by the appropriate person of authority				
Efforts to contact staff were effective				
Efforts to contact patients were effective				
The patient classification system was current				
The patient classification system was accurate				
Patients identified as requiring ongoing care received visits in a timely manner				
Patients identified to not have immediate needs were contacted and given appropriate instructions				
Transfer of current home care patients to a more acute level of care was timely and effective				
Triage of referrals for admission was effective in identifying those patients requiring admission within the first 24 hours of referral				
New admissions were processed within 48 hours of referral				
The predefined surge capacity for admissions was an accurate estimate				
Staffing was adequate to meet patient needs				
If patient needs exceeded staffing availability, coordination with other home care agencies was effective to meet patient needs				
Critical supplies (food, water, etc.) were available as required				
In the event of telephone failure, backup communication systems were effective				
Information regarding road closures was available to staff coordinating and conducting ongoing patient visits				

EMERGENCY MANAGEMENT PLAN EVALUATION FORM*Continued*

	YES	NO	NA	COMMENTS
Evacuation of the organization's facility was accomplished in an orderly and timely manner				
If this was an actual event, the probability of occurrence was accurately predicted in the Hazard Vulnerability Analysis (HVA)				
Response efforts were compatible with those of the community				
If this was an actual event, staff support services (stress debriefing, spiritual/emotional support, etc.) were implemented and effective				
Mitigation activities were effective (List and evaluate mitigation activities relevant to this event) 1. 2. 3. 4. 5.				
Preparedness activities were effective (List and evaluate preparedness activities relevant to this event) 1. 2. 3. 4. 5.				
Response activities were effective (List and evaluate response activities relevant to this event) 1. 2. 3. 4. 5.				
Recovery activities were effective (List and evaluate recovery activities relevant to this event) 1. 2. 3. 4. 5.				

List and Prioritize Processes Identified for Improvement:

- 1.
- 2.
- 3.
- 4.
- 5.

EMERGENCY MANAGEMENT PLAN EVALUATION FORM

Continued

- 6.
- 7.
- 8.
- 9.
- 10.

Plans for Improvement:

ADDENDUM 6-037.H

INTEGRATED HEALTHCARE SYSTEMS

If an organization is part of a healthcare system consisting of multiple separately certified healthcare facilities that elects to have a unified integrated emergency preparedness program, the organization may choose to participate in the system's coordinated emergency preparedness program. If elected, the unified and integrated emergency preparedness program must do all of the following:

1. Demonstrate that each separately certified facility within the system actively participated in the development of the unified and integrated emergency preparedness program.
2. The plan be developed and maintained in a manner that takes into account each separately certified facility's unique circumstances, patient populations, and services offered.
3. The plan demonstrates that each separately certified facility is capable of actively using the unified and integrated emergency preparedness program and is in compliance with the program.
4. The plan includes a unified and integrated emergency plan that meets the requirements of the regulation. The unified and integrated emergency plan must also be based on and include all of the following:
 - A. A documented community-based risk assessment, utilizing an all-hazards approach.
 - B. A documented individual facility-based risk assessment for each separately certified facility within the health system, utilizing an all-hazards approach.
5. The plan includes integrated policies and procedures that meet the requirements set forth in the regulations, a coordinated communication plan and training and testing programs that meet the requirements of the regulations.

WAIVED TESTING
Policy No. 6-038.1**PURPOSE**

To define the organization's compliance with waived testing criteria and the need for a certificate of laboratory services.

POLICY

The Clinical Laboratory Improvement Act (CLIA) of 1988 requires all clinical laboratories to possess a CLIA certificate in order to perform testing of human specimens. Certain tests are listed on a waived list. Agencies that collect human specimens but do not perform tests are excluded from this act.

Written policies and procedures will be maintained by the organization defining the type of waived tests performed.

PROCEDURE

1. Organization's Name will perform permitted laboratory tests in the home.
2. Organization's Name will apply for and maintain a CLIA waiver on file.
3. Patients who require laboratory tests performed that are not within the scope of tests offered will have their specimens transported to a CLIA-certified laboratory for testing. A copy of the current CLIA certificate for the outside laboratory will be on file.
4. Policies and procedures for each type of waived test performed will include, at a minimum:
 - A. Indications and purpose of the test
 - B. Personnel responsible for performing and supervising waived testing
 - C. Training of staff in performing waived tests
 - D. Parameters for requiring follow-up or referral
 - E. Mechanism used to ensure accuracy of testing instruments used
 - F. (See "[Organization List and Criteria for Waived Tests Performed](#)" Addendum 6-038.A)
5. The nurse shall not substitute the organization's equipment when assisting with patient self-administered tests, except for a short duration when the patient has not yet obtained their own testing equipment

ADDENDUM 6-038.A

**ORGANIZATION LIST AND CRITERIA
FOR WAIVED TESTS PERFORMED**

(Insert List)

For more information:

<https://www.fda.gov/medical-devices/ivd-regulatory-assistance/clinical-laboratory-improvement-amendments-clia>

HOME GLUCOSE MONITORING**Policy No. 6-039.1****PURPOSE**

To provide guidelines for the safe use of home glucose monitoring devices.

POLICY

Nurses will utilize home glucose monitoring (HGM) devices in accordance with manufacturer's guidelines in order to ensure the quality and accuracy of blood glucose values. Home glucose monitoring performed in the patient's home for purposes of care and diagnosis will be considered definitive. Blood glucose monitoring utilized as a screening tool will be followed with confirmation testing as ordered by the physician (or allowed practitioner), prior to care or treatment decisions.

All nurses utilizing HGM devices will have evidence of:

1. Specific training on the HGM device
2. Orientation to the organization's process for monitoring
3. Current competence

Note: The nurse shall not substitute the organization's equipment when assisting with patient self-administered tests, except for a short duration when the patient has not yet obtained their own testing equipment.

PROCEDURE

1. Assignment of HGM Equipment
 - A. There will be one (1) acceptable brand of HGM device that will be issued and used for home glucose monitoring.
 - B. All nursing personnel who receive a HGM device for use in the service of the organization will sign an HGM agreement that outlines responsibility for care and return of the item.
 - C. The nurse will be responsible for maintaining the HGM device in optimal operating condition according to the manufacturer's instructions including cleaning, use of appropriate accessory equipment, proper operating procedure, and protection from damage.
 - D. When employment is terminated, or the nurse's function no longer requires the use of a HGM device, the machine will be returned to the appropriate Clinical Supervisor or designee with associated stock items and the quality control log.

2. Training and Supervision of Personnel

- A. All nurses will receive training in the HGM devices issued to them. Training will include instruction and return demonstration of the following procedures according to the manufacturer's directions. All of the procedures listed below are to be conducted in accordance with standard precautions. (See "[Standard Precautions](#)" Policy No. 6-004.)
 - 2. Calibration performance and recording
 - 3. Control solution checks and recording
 - 4. Obtaining an adequate blood specimen
 - 5. Blood testing procedure
 - 6. Cleaning the machine
 - 7. Storage of the machine
 - 8. How to change a battery
 - 9. Actions to take if a machine malfunction is suspected
- B. Return demonstration of the procedures (listed in 2A above) by full-time, per diem, and contract nursing personnel using HGM devices will be performed at least annually in the presence of the Clinical Supervisor or designee. This process will be documented and retained in the staff members personnel file.
- C. The Clinical Director will be responsible for supervision of personnel performing home glucose monitoring.

3. Calibration and Control Tests (see manufacturer's guidelines for specific procedures)

- A. Calibration of the HGM assigned to the nurse will be checked prior to every use with a patient. The calibration procedure of the HGM device will be performed when a new bottle of strips is opened for use and at other times as indicated by the manufacturer's guidelines.
- B. Control testing using control solution will be performed by a nurse each day that a HGM machine will be used, according to the manufacturer's directions, prior to use with a patient.

- C. Calibration and control test results will be recorded on a log form entitled, "quality control for home glucose monitoring devices." The nurse will maintain this log with the device. Completed logs will be turned into and reviewed by the Clinical Supervisor or designee and will be retained by the organization.

4. Blood Testing Procedure

A. Supplies Needed

1. HGM device
2. Test strips
3. Lancet
4. Optional lancet device
5. Tissue or cotton ball
6. Alcohol wipes
7. Non-sterile gloves
8. Biohazardous sharps container
9. Paper towel to cover surface where testing with the machine will take place
10. One 6-volt photographic alkaline battery

B. Before the Test:

1. Verify that proper equipment is available.
 - a. If the nurse is using a HGM device owned by the patient to provide care according to the plan of care, the nurse must ensure that the machine is in proper operating condition and that the nurse is trained on the machine. Calibration performance-control solution checks are required when a nurse uses patient-owned HGM devices. Recording of calibration and control tests will be recorded in the nurse's notes. Calibration and control tests will be performed per manufacturer's instructions for the patient-owned meter.
 - b. If the machine is not in acceptable operating condition, or if the nurse has not received the appropriate training, the nurse must use the HGM device issued by the organization for testing.

- c. A nurse who identifies his/her HGM training deficit must report this to the Clinical Supervisor or designee for follow-up and possible staffing changes.
2. Verify orders as needed.
3. Inform patient of the purpose and procedure.
4. Verify if a new vial of strips will be used.
 - a. Wash hands.
 - b. Assemble equipment on a clean towel, on a safe, flat surface with biohazardous sharps disposal unit available.
 - c. Don non-sterile gloves.

C. Testing

1. This entire procedure will be performed in accordance with standard precaution guidelines. (See "Standard Precautions" Policy No. 6-004.)
2. Blood glucose testing will be conducted in accordance with the procedure outlined by the manufacturer.
3. Blood glucose results will be recorded on the skilled nursing note.
4. All contaminated lancets will be discarded in a biohazardous sharps container with the patient receiving appropriate instruction on its use and purpose.

5. HGM Device Malfunctions

- A. If a device fails a control test or otherwise malfunctions, the nurse will consider performing the following procedures:
 1. Check the battery (replace if necessary and available)
 2. Recheck using fresh control solution and/or a fresh bottle or strips (if available)
 3. Check the cleanliness of the machine and clean according to manufacturer's instructions. If necessary, allow drying time
 4. Ensure that the size and location of the blood specimen was adequate
 5. Ensure that the testing procedure was followed properly

6. Ensure that the machine has been calibrated properly
 7. Inspect the machine for any damaged parts that can be detected visually.
 8. Refer to the troubleshooting guide in the HGM Manual
 9. Call the manufacturer's toll-free number for further instructions.
- B. If the machine continues to malfunction despite attempts to isolate the problem, the following procedure is to be followed:
1. The machine will be placed out of use.
 2. The patient's physician will be contacted to ascertain if a venipuncture for a blood sugar value is required.
 3. The HGM device will be returned to the office for return to the manufacturer.
 4. The nurse will obtain an HGM replacement.

ATTACHMENTS

Attachment I:

[ACHC Crosswalk](#)

Attachment II:

[Glossary of Terms](#)

ATTACHMENT I
ACHC CROSSWALK

Crosswalk

Accreditation Commission for Health Care

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ATTACHMENT II

GLOSSARY OF TERMS

GLOSSARY OF TERMS

Abuse: Any act that constitutes a violation of the prostitution or criminal sexual conduct statutes, the intentional and nontherapeutic infliction of pain or injury, or any persistent course of conduct intended to produce mental or emotional distress.

Adult: A person 18 years or older, or a person legally capable of consenting to his/her own medical treatment.

Advance Directive: A document in which a person states choices for medical treatment.

Adverse Drug Reaction: Any response to a drug that is noxious, unintended, and unexpected, which occurs at doses normally used in man for prophylaxis, diagnosis, or therapy of disease or for the modification of physiologic functions.

Allowed Practitioner: A physician assistant, nurse practitioner, or clinical nurse specialist.

Aseptic: Means clean. It does not mean sterile or sterilized, which means completely free of germs.

Assessment: The comprehensive approach to defining status, using a screening and interdisciplinary evaluation process.

Attending Physician: The physician who is primarily responsible for the medical care of a patient who is receiving hospice care services.

Blood: Human blood, human blood components, and products made from human blood.

Bloodborne Pathogens: Pathogenic microorganisms that are present in human blood and can cause disease in humans. These pathogens include, but are not limited to, Hepatitis B virus (HBV) and human immunodeficiency virus (HIV).

Body Fluids: Emesis, sputum, feces, urine, semen, vaginal secretions, cerebrospinal fluid (CSF), synovial fluid, pleural fluid, pericardial fluid, amniotic fluid, and human breast milk; along with other fluids such as nasal secretions, saliva, sweat, and tears.

Branch Office: An approved location or site from which a home health agency provides services within a portion of the total geographic area served by the parent agency. The parent home health agency must provide supervision and administrative control of any branch office. It is unnecessary for the branch office to independently meet the conditions of participation as a home health agency.

Care Plan: An individualized, written plan of care for a patient, including nursing diagnoses related to standards of care, goals, and interventions appropriate to each discipline. Documents include Plan of Treatment (485), Plans of Care (all disciplines).

Caretaker: An individual or facility responsible for the care of a patient as a result of a family/caregiver relationship, such as a relative or spouse, or responsible for all or some of the care voluntarily or by contract or agreements, such as organization personnel.

Clinical Laboratory: A workplace where diagnostic or other screening procedures are performed on blood or other potentially infectious materials.

Clinical Note: A notation of a contact with a patient that is written, timed and dated and which describes signs and symptoms, treatment, drugs administered and the patient's reaction or response, and any changes in physical or emotional condition during a given period of time.

Clinical Nurse Specialist: An individual as defined at §410.76(a) and §410.76(b) and who is working in collaboration with the physician as defined at §410.76(c)(3).

Clinician: Any nurse, PT, OT, ST, or MSW involved in the care of a patient, either directly or indirectly, including administrative, management and supervisory personnel.

Communicatively Impaired: A communicatively impaired individual has expressive or receptive language deficits which may be present after an illness or injury. This may include individuals with voice disorders, laryngectomy, glossectomy, or cognitive disorders.

Conflict Of Interest: Using any knowledge or information acquired through one's professional relations with one's patients, or in the conduct of organization business, to one's own advantage or profit.

Contaminated: The presence, or the reasonably anticipated presence, of blood or other potentially infectious materials on an item or surface.

Contaminated Laundry: Laundry that has been soiled with blood or other potentially infectious materials or may contain sharps.

Contaminated Materials: Materials which are reusable and which have been exposed to or contaminated by blood or body fluids. These materials may be transported to destinations outside the patient's home (i.e., blood specimens to laboratories).

Contaminated Sharps: Any contaminated object that can penetrate the skin, including, but not limited to, needles, scalpels, broken glass, broken capillary tubes, and exposed ends of dental wires.

Contaminated Wastes: Disposable materials that have been exposed to or contaminated by blood or body fluids.

Coordination: The communication of patient status and recommended care planning among all disciplines involved in the care of the patient.

Decontamination: The use of physical or chemical means to remove, inactivate, or destroy bloodborne pathogens on a surface or item to the point where they are no longer capable of transmitting infectious particles and the surface or item is rendered safe for handling, use, or disposal.

Drug Regimen: All prescription and over-the-counter (OTC) drugs that are currently ordered for the patient, including all drugs used on a prn basis.

Education: The role of information and instruction in care.

Engineering Controls: Controls (e.g., sharps disposal containers, self-sheathing needles) that isolate or remove the bloodborne pathogens hazard from the workplace.

Exposure Incident: A specific eye, mouth, other mucous membrane, non-intact skin, or parenteral contact with blood or other potentially infectious materials that results from the performance of organization personnel's duties.

Hand Washing Facilities: A facility providing an adequate supply of running potable water, soap and single-use towels or hot air drying machine.

Hazardous Waste: Chemicals or materials that may potentially cause or contribute to any serious health effects, may present a safety hazard, or have the potential to cause fire, explosion, or serious accidents.

HBV: The Hepatitis B virus.

Hearing Impaired: Difficulty hearing and/or discriminating verbal conversation either in a face-to-face situation or over the telephone. An individual with this impairment may require a hearing aid, telephone amplifier, TDD, or sign language interpreter.

HIV: The human immunodeficiency virus.

Home Care Supplies: Disposable items used by home care personnel, the patient, and/or family/caregiver to meet the patient's home care needs (i.e., dressings, syringes, catheters, tubing, gloves, etc.).

Home Medical Equipment (HME): Any assistive device or piece of equipment used by home care personnel, patient, and/or family/caregiver to meet the patient's home care needs (i.e., walker, commode, Hoyer lift, apnea monitor, etc.).

In Advance: HHA staff must complete the task prior to performing any hands-on care or any patient education.

Incident: An unusual event involving organization personnel, patient and/or family/caregiver. The event is considered unusual if the result was unintended, undesirable and/or unexpected. An incident is also any happening that is not consistent with the routine operation of the organization or the routine care of a patient. It may be actual or potential. (See "[Examples of Specific Events or Occurrences That Must Be Reported](#)" in the operations manual for further definition).

Individual Mandated to Report: A professional, or the professional's delegate, who is engaged in the care of patients or in education, social services, law enforcement, or any related occupations, who has knowledge of the abuse or neglect of a patient, has reasonable cause to believe that a patient is being or has been abused or neglected, or has knowledge that a patient has sustained a physical injury that is not reasonably explained by the history of injuries provided by the caretaker or caretakers of the patient. (Note that specific reporting requirements vary from state to state.)

Irregularity: Any departure from what is usual, proper, accepted, or right.

Limited English Proficiency (LEP): A person with Limited English Proficiency whose command of the English language is not sufficient to promote meaningful interaction for service.

Medical Equipment: Any assistive device or piece of equipment used by home care personnel, patient, and/or family/caregiver to meet the patient's home care needs, such as wheelchairs, walkers, canes, lifts, monitors.

Medication Error: Any error which includes, but is not limited to: a) Patient and family/caregiver not following physician's (or allowed practitioner) orders or nurse's instructions in administering medications, e.g., titration of IV opiates outside of the physician's (or allowed practitioner) ordered titration parameters; b) Wrong medication, wrong time, wrong dose, wrong route of administration during intervention by the nurse, extra dose or omission of ordered drug; or c) Missing a scheduled administration of a SQ/IM/IV medication for any reason, e.g., staffing difficulties, equipment, supplies.

Neglect: Failure of a caretaker to supply the patient with necessary food, clothing, shelter, health care, or supervision; or the absence, or likelihood of absence, of necessary food, clothing, shelter, health care, or supervision for a patient.

New Infection: Any infection that occurs that was not documented as present at the time the patient was admitted to home care.

Noncoverage: Patient is not eligible for services because of his/her insurance coverage criteria.

Nurse Practitioner: An individual as defined at §410.75(a) and §410.75(b) and who is working in collaboration with the physician as defined at §410.75(c)(3).

Nutrition Care: The interdisciplinary nature of nutrition care, including physicians, nurses, registered dietitians, pharmacists and others as appropriate; interventions and counseling on appropriate nutrition intake by integrating information from nutrition assessments.

Occupational Exposure: Reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious materials that may result from the performance of organization personnel's duties.

Other Potentially Infectious Materials: (1) The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, synovial fluid, pleural fluid, pericardial fluid, peritoneal fluid, amniotic fluid, saliva in dental procedures, any body fluid that is visibly contaminated with blood, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids; (2) any unfixed tissue or organ (other than intact skin) from a human (living or dead); and (3) HIV- or HBV-containing cell or tissue cultures, organ cultures, and culture medium or other solutions; and blood, organs or other issues from experimental animals infected with HIV or HBV.

Oxygen and Related Equipment: Oxygen gas and any equipment used to deliver the gas to the patient (e.g., oxygen tank and tubing, pulmo-aid, concentrator, etc.). These items are considered durable medical equipment.

Parent Home Health Agency: The agency that provides direct support and administrative control of a branch.

Patient Representative: A person appointed to make decisions for someone else. He/she may be formally appointed (as in a Durable Power of Attorney for Health Care) or, in the absence of a formal appointment, may be recognized by virtue of a relationship with the patient (such as the patient's next of kin or close family/caregiver).

Patient Self-Determination Act: A federal statute enacted as part of the 1990 Omnibus Budget Reconciliation Act (OBRA) (PL 101-508) which requires, among other things, that health care

facilities provide information regarding the right to formulate Advance Directives concerning health care decisions.

Personal Protective Equipment: Specialized clothing or equipment worn by organization personnel for protection against a hazard. General work clothes (e.g., uniforms, pants, shirts or blouses) not intended to function as protection against a hazard are not considered personal protective equipment.

Physician: A Doctor of Medicine, osteopathy, or podiatric medicine and who is not precluded from performing this function. (A doctor of podiatric medicine may perform only plan of treatment functions that are consistent with the functions he or she is authorized to perform under State law.)

Physician Assistant: An individual as defined at §410.74(a) and §410.74(c).

Plan of Care: The clinical plan of care, including pertinent diagnoses, mental status, types of services/equipment, frequency of visits, goals and interventions appropriate to each discipline, prognosis, rehabilitation potential, functional limitations, precautions, activities, nutritional requirements, medications, treatments, safety measures and instructions.

Primary Home Health Agency: The HHA which accepts the initial referral of a patient, and which provides services directly to the patient or via another health care provider under arrangements (as applicable).

Production Facility: A facility engaged in industrial-scale, large-volume production, or high concentration production of HIV or HBV.

Pseudo-patient: A person trained to participate in a role-play situation, or a computer-based mannequin device. A pseudo-patient must be capable of responding to and interacting with the home health aide trainee, and must demonstrate the general characteristics of the primary patient population served by the HHA in key areas such as age, frailty, functional status, and cognitive status.

Quality Indicator: A specific, valid, and reliable measure of access, care outcomes, or satisfaction, or a measure of a process of care.

Reduction of Services: A change in the patient's service plan in which one (1) or more existing services is discontinued.

Regulated Waste: Liquid or semi-liquid blood or other potentially infectious materials; contaminated items that would release blood or other liquid or semi-liquid infectious materials in a liquid or semi-liquid state if compressed; items that are caked with dried blood or other potentially infectious materials and are capable of releasing these materials during handling; contaminated sharps; and pathological and microbiological wastes containing blood or other potentially infectious materials.

Report: Any report received by the local welfare agency, police department, county sheriff, or licensing/regulation agency. Any report used for recording and communication of information within the organization.

Reportable Communicable Disease: Some suspected or positively identified communicable diseases must be reported to state public health agencies. (See "[Reporting of Communicable Diseases](#)" Policy No. 6-016.)

Representative: The patient's legal representative, such as a guardian, who makes health-care decisions on the patient's behalf, or a patient-selected representative who participates in making decisions related to the patient's care or well-being, including but not limited to, a family member or an advocate for the patient. The patient determines the role of the representative, to the extent possible.

Research Laboratory: A laboratory producing research-laboratory-scale amounts of HIV or HBV. Research laboratories may produce high concentrations of HIV or HBV, but not in the volume found in production facilities.

Simulation: A training and assessment technique that mimics the reality of the home care environment, including environmental distractions and constraints that evoke or replicate substantial aspects of the real world in a fully interactive fashion in order to teach and assess proficiency in performing skills, and to promote decision making and critical thinking.

Source Individual: Any individual, living or dead, whose blood or other potentially infectious materials may be a source of occupational exposure to organization personnel. Examples include, but are not limited to, hospital and clinic patients; patients in institutions for the developmentally disabled; trauma victims; patients of drug and alcohol facilities; residents of hospices and nursing homes; human remains; and individuals who donate or sell blood or blood components.

Standard Precautions: An approach to infection control. According to the concept of standard precautions, all human blood and certain human body fluids are treated as if known to be infectious for HIV, HBV, and other bloodborne pathogens.

Sterilize: The use of a physical or chemical procedure to destroy all microbial life, including highly resistant bacterial endospores.

Summary Report: The compilation of the pertinent factors of a patient's clinical notes that is submitted to the patient's physician, physician assistant, nurse practitioner, or clinical nurse specialist.

Supervised Practical Training: Training in a practicum laboratory or other setting in which the trainee demonstrates knowledge while providing covered services to an individual under the direct supervision of either a registered nurse or a licensed practical nurse who is under the supervision of a registered nurse.

Suspected Infection: A situation in which clinical observations strongly suggest the presence of an infection, but empirical data to support the suspicion is not possible or available at the time of the report.

Telecommunication Device for the Deaf (TDD): A small, typewriter-style instrument that allows a person to make or receive a telephone call directly without using another person to interpret.

Terminal Condition: An incurable condition caused by an injury, disease, or illness that, regardless of the application of life-sustaining procedures, would, within reasonable medical judgment, produce death, and where the application of life-sustaining procedures would only postpone the moment of death of the patient.

Termination/Discharge: Discontinuance of all organization services by the organization.

Transfer/Referral: Patients whose needs change significantly and/or who require care that cannot be provided by the organization.

Verbal Order: A physician, physician assistant, nurse practitioner, or clinical nurse specialist order that is spoken to appropriate personnel and later put in writing for the purposes of documenting as well as establishing or revising the patient's plan of care.

Work Practice Controls: Controls that reduce the likelihood of exposure by altering the manner in which a task is performed (e.g., prohibiting recapping of needles by a two (2)-handed technique).

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